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St. Vincent's Healthcare Group annual review 2004

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ANNUAL REVIEW 2004

ST. VINCENT'S HEALTHCARE GROUP

St. Vincent's Healthcare GROUP



ANNUAL REVIEW 2004



Mission Statement

We strive for excellence in meeting the holistic needs of our patients in a caring and healing environment in which the essential contribution of each member of staff is valued.

The values of human dignity, compassion, justice, quality and advocacy, rooted in the mission and philosophy of the Religious Sisters of Charity, guide us in our work.

We will, within the foregoing context, make every effort to maintain excellence in clinical care, teaching and research.

Comh mheas, comh bha, comh phártaíocht agus comh oibre bunsraith ár gcuid saothar uile.

(Mutual respect, empathy, partnership and cooperation are the foundation of all our work.)

Annual Review

St. Vincent's Healthcare Group Ltd.

incorporating

St. Vincent's University Hospital

St. Vincent's Private Hospital

and

St. Michael's Hospital



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Board of Directors



Professor Noel Whelan



Mr. Edmond J. Bergin



Sr. Eugene Butler



Ms. Louise English



Mr. Stewart Harrington

In Attendance



Mr. Nicholas C. Jermyn



Mr. Eamonn Fitzgerald



Ms. Pauline Doyle (to April 04)



Dr. Risteárd Ó Laoide



Professor M. X. FitzGerald



Mr. Joe Leyden



Mr. W. R. Quinlan



Dr. Brian Maurer



Ms. Gemma McCrohan



Mr. Patrick Meade



Mr. Michael Meagher



Sr. Anne MacEneaney



Mr. Conor Sexton



Dr. Michael Somers



Ms. Raphael McMullin (from May 04)



Mr. Cormac Maloney



Sr. Therese Culhane



Mr. Michael Redmond

Overview by

The Chairman of the Board of Directors

I have pleasure in presenting the annual review of St Vincent's Healthcare Group Ltd., for the year 2004. This is the second Annual Review of the group which comprises St Vincent's University Hospital, St Vincent's Private Hospital, and St Michael's Hospital (Dún Laoghaire, Co Dublin). The group forms part of the Caritas Consultative and Advisory Forum.

Details of the many challenging and exciting issues and problems which came to bear on the group's development during year 2004 are contained elsewhere in this review so I shall concentrate here on the core matters which have arisen during the year from the perspective of the group's Board of Directors, and which will shape the group's development in the years immediately ahead.

Ongoing Management and Group Integration

Year 2004 was an eventful year for St Vincent's Healthcare Group. The ongoing management of the constituent hospitals in the group took place in the context of further group integration, important changes in the group's environment, and stakeholder emphasis. Overall, the management of group operations took place in the context of serious resource constraint and increasing demand for the health services of the group. This mis-match between resources and demand for services was reconciled by increased productivity within the group's individual hospitals and by economies of scale as between the hospitals. This situation cannot continue indefinitely; and unless there is, next year, realistic resource provision in the context of what the public and the state require from the group, service levels will of necessity be cut.

A major, and at this stage on-going, problem is the need to increase bed numbers within the group. The group needs 200 extra beds to function optimally. This requires an additional investment of €40million approximately. In order to make every effort to obtain this investment the group's Board of Directors is examining how best it can harness both public and private sector investment for the provision of the extra beds required. The board is also developing action to attract philanthropic funding for the group's further development. It has, for instance, approved the creation of the "St Vincent's Foundation" (a fund raising body for the group), and the recruitment of an Executive Director for the Foundation. It is at present seeking a public spirited and visionary person to chair St Vincent's Foundation and give vision and leadership to its successful development.

Important Strategic Issues

During year 2004, many major issues came to bear on the group's planning and development. These included:

Research: One of the group's constituent hospitals, St Vincent's University Hospital, is one of the main teaching hospitals attached to the Medical and Health Sciences Faculty of University College Dublin. Effective teaching needs to be underpinned by appropriate research. So, medical research is constantly on the group's agenda. In



addition, during year 2004, major and exciting research challenges and opportunities in advanced medical research presented themselves. The Board of Directors undertook a strategic review of its research capacity with the help of prominent experts. It is in the process of establishing a strategic research committee which will involve prominent medical and other scientists of international acclaim.

Group Governance: The Board of Directors is committed to the creation and development of the highest standards of governance within the group. It has drawn up and approved a Charter for Good Governance in accordance with current international norms. These norms include procedures and practices to appraise and ensure board effectiveness.

Group Planning: Strategic plans are being drawn up for each of the three constituent hospitals in the group in the context of the group's integration strategy. Economies of scale and inter-hospital synergies are being pursued. The group Medical Board and group management are contributing actively to the process.

Networking, Alliances and Partnerships: The group board pursues a proactive and positive policy towards networking and creating alliances with other hospitals and health/medical entities. It does so to explore and realise as many possibilities as possible for inter-hospital synergy and benefits. Such activity includes the group's involvement with the many hospitals and health facilities in its region, and with the country-wide health facilities which (in conjunction with St Vincent's Healthcare Group) form the Caritas Consultative and Advisory Forum. Discussions have also commenced in relation to the creation of close rapport and partnership between St Vincent's Healthcare Group and both the National Maternity Hospital (Holles Street, Dublin) and the City of Dublin Skin and Cancer Hospital (Hume Street, Dublin) with the possibility of both of these important facilities being located on the campus of St Vincent's Healthcare Group.

Major Infrastructural Investment: The major current investment, in excess of €220 millions, in the future development of St Vincent's University Hospital (one of the major constituent hospitals of the group) in Elm Park, Dublin 4, continued during year 2004 - on time and on budget. We look forward to its completion and formal opening towards the end of year 2005. This investment will create a technologica lly and scientifically advanced hospital complex with the patient, and patient care, as its central tenet.

Health Sector Reform: The issues mentioned above are taking place in the context of much public health sector reform. During the years 2003 and 2004 a number of public health sector reform reports were issued and adopted by the government. The far-reaching structural reforms emanating from these reports present both challenges and opportunities, and St Vincent's Healthcare Group intends to respond to them in an innovative and positive way.

Thanks

I have focused my remarks in this overview on the core matters which affect and shape the development of St Vincent's Healthcare Group Ltd. There is no more fundamental matter in this regard than the commitment and help which we receive from so many persons. I have much pleasure, therefore, in extending the warm and appreciative thanks of the group board to the many friends and benefactors of, and participants in the exciting and rewarding development of the St Vincent's Healthcare Group. As in previous years, the list and number of persons we have to thank is indeed long. We thank:

- The shareholders of the group, the Religious Sisters of Charity, under the caring guidance of Sr. Una O'Neill, Superior General, and her colleague sisters;
- The staff of the group, and of the group's three constituent hospitals;
- The Minister for Health and Children, and the departmental officials of the Department of Health and Children;
- The Board members and officials of the Health Services Executive;
- The governing boards and staff of our many partner hospitals, and of University College Dublin;
- Our many supportive friends in Ireland, and around the world.

I extend my personal thanks to my colleague directors on the group board, and to the management of both the group and of the individual constituent group hospitals.

As we head into year 2005: the group board looks forward to delivering excellence in meeting the holistic needs of its hospitals' patients in a caring and healing environment. It shall continue to pursue excellence in clinical care, in teaching and in medical research; and it shall continue to be guided by the preservation of human dignity, by compassion, by justice, and, most importantly, by charity.

Professor Noel Whelan
Chairman
St Vincent's Healthcare Group Ltd.

Review by

The Group Chief Executive

Executive Review

Introduction

I am pleased to report for the second year on the annual review of the activity and expenditure of St. Vincent's Healthcare Group Ltd. (incorporating St. Vincent's University Hospital, St.Vincent's Private Hospital and St. Michael's Hospital) for the year ended 31st December 2004.

Health Reforms

Last year, a number of reports were released that contained various recommendations in relation to the way future health services should be delivered. The reports recommended that major structural and cultural reforms were needed within the health services. These changes are now in the process of being implemented and obviously there are significant challenges to be faced as with any major fundamental reform.

Hospital management, with the support of the Board of Directors, face the real challenges of delivering the day to day services (24 hours x 365 days), while at the same time dealing with the major reorganisation of the services. The public want, expect and have been promised that these reforms will bring about instant improvements on the ground. It is important that we note structural reforms in themselves do not automatically guarantee processes will change. The reforms should and will enable changes to take place but these will take time. While the level of investment in health has increased dramatically over the last few years, Ireland has one of the lowest spends on health compared with its counterparts in Europe. Notwithstanding this, as a country we provide doctors, nurses and other healthcare professionals who are equal, if not better, than our European partners. If one was to realistically look at the levels of capital and revenue invested over the last 10/15 years, compared with our European counterparts, our health services compare very favourably.

I reported last year that St. Vincent's Healthcare Group needed to take stock, consolidate and stabilise the operations within the various hospitals in the Group while the Board of Directors and management considered what impact the various health reform initiatives would have on the Group and its mission i.e.

"We strive for excellence in meeting the needs of our patients in a caring and healing environment in which the essential contribution of each member of staff is valued.The values of human dignity, compassion, justice, quality and advocacy, rooted in the mission and philosophy of the Religious Sisters of Charity, guide us in our work. We will make every effort to maintain excellence in clinical care, teaching and research.

Comh mheas, comh bha, comh phártaíaocht agus comh oibre bunsraith ár gcuid saothar uile.(Mutual respect, empathy, partnership and co-operation are the foundation of all our work)".



Management has concentrated on the following :

a) The Strategic Plan for the Group
b) Performance and Quality Indicators to measure the Group hospitals against its peers
c) Charter/Code of Good Governance for the Group
d) Prepare the Group for Accreditation
e) A strategy for fund-raising within the Group
f) Education & Research Strategy
g) Teaching agreement between UCD and St.Vincent's Healthcare Group
h) An agreement between UCD (DMMC) and the Group for the development of a Genome Research Centre

I am pleased to report that the above initiatives, (a) to (h) have been dealt with in 2004. I will refer to some of these initiatives later in my report.

A key strategic issue for the Group was to continue its involvement in national and regional programmes and to network and develop partnerships with other healthcare facilities at primary and secondary levels within the delivery process chain of patient services.

Networks/Partnerships

As you will see from individual department reports, St. Vincent's Healthcare Group has continued to play a major role in various national, supra-regional and regional programmes in various specialties i.e. Cardiovascular, Cancer, Transplantation, Emergency Medicine and Renal, to mention just a few. The Group has and continues to develop major links with the primary care system. The ongoing development of EastDoc and DLDoc are good examples of the healthcare Group recognising the importance of the acute system linking with primary care systems within the healthcare delivery chain.

St. Vincent's/St. Luke's Association agreement is another example of the Group's strength in developing real partnerships. St. Vincent's Healthcare Group treats more cancer patients than any other general hospital in the nation so therefore it is logical that St. Vincent's Healthcare Group is linked with St. Luke's Hospital, the national radiotherapy centre. St. Vincent's Hospitals are known locally, regionally, nationally and internationally for their expertise in the surgical, medical and radiation oncology treatment of breast, lung and liver, bowel and prostate cancers. The hospitals within the Group have a long tradition of innovation and leadership in the development and management of cancer care. Our scientists and clinicians are leaders who co-operate in national and international clinical trials. The Group hospitals are committed to evidence based research, care and education in an environment where the needs of patients and families are put first. As Group Chief Executive, I believe we should highlight and not under-estimate the importance and benefits of the linkages between St. Vincent's & St. Luke's from a patient care and staff point of view. I think it is worth noting that in 1996, St. Vincent's and St. Luke's entered into an agreement which involved myself (as CEO of St. Vincent's University Hospital) taking on the additional role as CEO of St. Luke's Hospital. I held this dual role until 2000 when Mr. Lorcan Birthistle was appointed.

I am pleased to report that the Association Agreement is growing from strength to strength with additional services being integrated and developed to ensure that between the two agencies we deliver a modern and energetic service which will meet the needs of our current and future patients.

"It is the joint wish and aim of the Boards of St. Vincent's University Hospital and St. Luke's Hospital that, while maintaining their respective autonomies they should continue their association with a view to creating and enhancing added value in the operations (medical, administrative and managerial) of their respective hospitals in the interests of patients, their families and staff. This Association Agreement follows on from the agreement of understanding between the two hospital Boards dated June 1996".

I believe the proposals before the Minister from St. Vincent's Healthcare Group and St. Luke's Hospital will deliver the badly needed expansion of radiotherapy services quickly. The proposals involve the private and public sectors coming together in partnership.

The Board of Directors, with management, continue to put a high emphasis on the need to continue this ongoing development of cancer services here within the Group with St. Luke's Hospital. The Group look forward to the outcome of the Minister's deliberations in relation to the development of Radiation Oncology services here in Ireland.

I should make reference to developments and ongoing discussions between the National Maternity Hospital (Holles Street) and St. Vincent's Healthcare Group, where agreement has been reached in principle that clinically it would be of benefit to patients to transfer services from the National Maternity Hospital to the campus of St. Vincent's University Hospital. The proposal would be to build new facilities that would be integrated with the services already being provided on the site. Therefore, maximising economies of scale and avoiding duplication of services. No doubt this matter will receive ongoing attention in 2005. Obviously there are issues and challenges with such a major proposal, however we look forward to ongoing discussions with our colleagues in the National Maternity Hospital re same.

The other major strategic initiative that has been discussed is the transfer of services from the City of Dublin Skin and Cancer Hospital (Hume Street) to St. Vincent's University Hospital's campus which has been ongoing for a number of years. I am pleased to report that discussions are nearing completion regarding the issues and challenges of such a transfer. Both hospital Boards have agreed to the transfer in principle and I would envisage concluding these discussions in early 2005.

I believe that partnerships and integration of services such as these are essential if we are to ensure that patients and their families receive quality care effectively and efficiently to which they are entitled.

Financial Overview

Once again, the hospitals within the Group faced a very difficult year from a financial and activity point of view. The financial and activity reports for the Group and individual hospital finances hide the very difficult year we had in the context of managing the finances and services within the Group. The Board of Directors and the Finance Committee spent a considerable amount of time dealing with the financial challenges faced by the Group. Throughout the various debates, the focus was about trying to maintain standards and patient services and the need to develop facilities was constantly at the forefront of everybody's mind.

I have to say in the context of the overall year when one considers the challenges, the financial results for the Group were extremely good. I must give due credit to everybody concerned, clinicians, management, staff etc. Activity for all three healthcare facilities was in line and in some areas increased on the previous year. Activity levels were tightly controlled giving priority to emergency admissions. Staff recruitment was controlled and restrictions had to be put in place. Various value for money initiatives and cost saving exercises were in place throughout the year and, as I said last year, I believe these ongoing initiatives will not be sustainable in 2005. The Group baseline funding will have to be increased in 2005 if we are to maintain standards and continue to provide services at the existing levels. Throughout 2004, the Board, management and clinicians were very aware of the need to manage the financial risks and, at the same time, the clinical and non-financial risks had to be managed as well.

The Group Finance Committee completed its review of the business and operations of St. Vincent's Private Hospital from a financial and patient activity perspective. Considerable progress was made with our main funder, VHI, in relation to baseline funding for services and while there is still some work to be done to improve reimbursement rates, these discussions will be ongoing throughout 2005.

Please note the results for the individual hospitals and Group which, when one considers the level of expenditure, overall expenditure was in line with budget for St. Vincent's University Hospital and St. Michael's Hospital respectively, i.e. St. Vincent's University Hospital €160.9m and St. Michael's Hospital €27.8m. St. Vincent's Private Hospital's total expenditure was €42.86m and had a surplus of €812k compared with last year's figure of €110k. The Group's overall expenditure for the year was €245.9m and the Group surplus for the year was €1.3m before transfer of reserves. Total net assets at year-end were valued at €241.6m.

Quality

Last year I indicated that the Group had agreed to apply for Accreditation through the Irish Health Service Accreditation Board (IHSAB) on a Group basis. This process of measuring an organisation's performance against pre-determined quality standards through self-assessment and peer review took place in May and I am pleased to report that St. Vincent's Healthcare Group received pre-accreditation advanced status which is an award in line with our peers and I would like to congratulate and compliment everybody involved.

One of the major benefits of the process of Accreditation has been to create the environment for staff at all levels to assess, review and consider whether or not the patient and patient support process of care within the Group are effective and efficient. As I reported last year, the Group identified over eighty quality initiatives, many of which are in the process of being implemented in 2005 and some had commenced implementation in 2004.

The Board of Directors for the Group had identified a number of major issues for management to pay particular attention to - Health & Safety Risk Management was one of these.

- The introduction of the Enterprise Liability Scheme for NCHDs and Consultant medical staff has had its challenges. As you know the fundamental principle underlying the Enterprise Liability Scheme was that the enterprise would assume liability for all its employees alleged clinical negligence or medical malpractice arising out of performance of their duties under the contract of employment.

- The hospital has decided and is in the process of implementing integrated clinical audit throughout the hospital at organisational level and it is anticipated that this process will be completed in 2005. The medical staff have spent a considerable amount of time looking at various options in relation to organisational clinical audit. I am happy to report that these processes are well established and are underway in the context of developing overall organisational structures to encapsulate clinical audit at all levels within the Group hospitals.

Governance

I should report that the Board of Directors has spent considerable time reviewing the governance structures. The Group has identified key competencies required at Board and hospital level in the context of ensuring that it is managed in an effective and efficient manner, having regard to its objectives laid out in the Memorandum and Articles of Association of the company. The Board has developed a Charter of Good Governance for St. Vincent's Healthcare Group.

I am pleased to report that in line with the development of risk management and quality improvement, internal audit structures at organisational level have been developed in line with the Group company policy to ensure that there are appropriate financial and other controls internally and externally in relation to the healthcare services we are delivering.

Challenges 2004

Bed Capacity

I believe it is important to report on the major challenges that the Group has had to face in 2004, which really have carried forward from 2003. Bed capacity is the single biggest issue for the Group. Unfortunately, I believe this is going to be an ongoing issue for the next few years. The number of patients who having completed their acute care stay in hospital is, to say the least, disturbing and has impacted dramatically on the hospital Group's ability to deliver elective and emergency services. Elective surgical and medical practices have been reduced to such an extent that one might argue that everybody who is brought into hospital now could be considered to require immediate attention and that the current "elective admission" is a thing of the past. This has had a detrimental effect on the development of services and the ability to train staff. Notwithstanding these challenges, I believe staff have worked well together and the hospitals as a Group is looking at various ways of managing the situation. One of the benefits and outcomes of all of this has been the rationalisation of our Emergency Medicine and Cardiac services within the Group, with the introduction of many innovative initiatives to deal with emergency admissions and alternative routes of patient care that have been developed to avoid people being admitted through the Emergency Department. Notwithstanding all of this work, it is important that we remember there are groups of people in our population who regardless of what we do will require admission. I am sorry to say they have tended to be our elderly patients who unfortunately, due to the lack of development of infrastructure at a secondary care level, end up being admitted through our Emergency Department. This is something which the Board of Directors and management have spent some considerable time discussing and working with other relevant healthcare facilities to try to deal with the situation. The fact remains that there are not sufficient long-term care facilities available to appropriately look after these patients and therefore they end up staying in acute hospital beds.

Environment

Infrastructure

The other major initiative and challenge which the hospital is left with is the management of risk and trying to maintain quality of care in environments where the infrastructure needs to be renovated and developed. The hospital's bed base was designed over fifty years ago and while renovation has taken place in some areas, the hospital needs to replace its current bed stock with new modern facilities and services to support same. This matter will continue to be looked at by the Board of Directors, with hospital management, as the single most important matter for the hospital Group to try and deal with over the next few years. It is estimated that the Group requires an additional bed base of in excess of 200 beds to deal with the needs of its patients and catchment area. It should be noted that these beds do not all need to be acute, some of them would be respite/intermediate and long-term care beds which would free up some of the acute beds currently occupied by patients who should be in alternative care. The balance of the current beds within the Group could be replaced and the freed up space would be utilised to renovate the current wards and develop services that should be part of any modern ward i.e. appropriate day and treatment rooms, bathroom facilities, shower and toilets etc.

Infection

Another challenge is the management of infection, which now has become an issue throughout Ireland in the context of MRSA and SRSV (more commonly known as the Winter Vomiting Bug). I think it is important to note that the clinicians, Board and management are at one in that they all agree one of the major challenges in 2005 is to ensure that the environment in which we provide care to our patients is improved. The new building project is being commissioned for operational use. The potential decanting of departments into the new building will leave space to renovate current wards and improve our bathroom and ward facilities. The hospital needs more single rooms to isolate patients with infection when required. This is not always possible.

However, it should be noted that the level of cross infection within St. Vincent's Healthcare Group is by no means above the norm and many of the issues we are addressing come down to education and good hygiene. Many of the infections, MRSA and SRSV, are already in the community. It should be noted that many of St. Vincent's Healthcare Group peers

have been modernised with single rooms and appropriate bathroom facilities. These facilities too have to deal with MRSA and SRSV. There is agreement that hand washing and more education around hygiene and infection control practices is required amongst healthcare workers and the public at large. MRSA and SRSV are now community-based infections and often when an outbreak of SRSV occurs in the hospital we find that the source has been a patient or visitor. This is not to say that this is not a problem for the hospitals within the Group. The Infection Control Committee for the Group is active and resources have been allocated to continue to deal with the issue of infection and the control of same.

Re-development Projects

I think it would be useful to highlight for staff within the Group some of the key issues identified by each of the Group hospitals as priorities that require attention.

- The environment and development of infrastructure. The key projects that management is looking at and hopes to process in 2004/05/06 are: -

St. Michael's Hospital

i. An appropriate connecting corridor between the annexe (old St. Michael's Private Hospital) and the current St. Michael's Public Hospital.
ii Development of three theatres side-by-side with the rationalisation of the current theatres which are single theatres on either site.
iii The replacement of the diagnostic equipment in Radiology with digital imaging, PACS (picture archival system) to connect with St. Vincent's University Hospital systems.
iv New day-care and endoscopy facilities to be developed.
v. Day hospital for the elderly.

St. Vincent's Private Hospital

i. The development of its current theatres to expand by 1 / 2 extra theatres.
ii The development of the Radiotherapy Department and the replacement of equipment which is already taking place in 2004 and will be completed by the end of 2005.
iii Development of day and ambulatory day care facilities.
iv The development of new consulting rooms to deal with the expanded consultant base at the hospital.

St. Vincent's University Hospital

The current redevelopment project is progressing well, ahead of schedule and within budget, with an expected practical completion date of February 2005. For further details please see the detailed St. Vincent's University Hospital Re-development report.

I should point out that while there have been many service developments in the three hospitals during the year, I do believe the use of the provincial house on St. Anthony's campus as a Cancer Support Centre was one of our major developments which has been awaited for some time. This will also accommodate the halfway house, which provides accommodation for patients and family members who are waiting for liver transplantation or who have just had a liver transplant and need support from the hospital for a number of weeks before or after surgery.

I believe the individual hospital reports contain a considerable amount of information about what is going on at departmental level within the three hospitals in the Group, so I have not included details of same in this executive summary.

Personal Thanks

I would like to take this opportunity to welcome all new staff. I would like to offer my best wishes, and that of all staff, to those staff who have retired and thank them for their service to their respective hospitals.

On behalf of hospital management I would like to thank the other healthcare facilities that support us in our work and many other voluntary groups who have fund-raised and supported the Group hospitals on a voluntary basis.

I believe it is important to thank the many people who serve on the various hospital committees and I would like to particularly thank Dr. Risteárd Ó Laoide, Chairman of the Group Medical Board and his medical colleagues for all their support and commitment throughout 2004.

Our thanks should also go to the Group's legal and financial advisors, Arthur Cox and Oliver Freaney & Co. respectively and the Design Team members, project management consultants, contractors, suppliers and all other outside contractors who support and work for the Group hospitals.

Particular thanks should go to Sister Anne MacEneaney (Sister Superior) and the community of Sisters for their support over 2004. I would like to express my sincere appreciation and thanks to Sister Una O'Neill (Superior General of the Religious Sisters of Charity), her team and to Sister Eileen Mary Durack (Head of Provincial Team).

On behalf of management, staff and patients, a sincere and particular thanks has to go to the Board of Directors and the Chairman, Professor Noel Whelan, who

have met on numerous occasions throughout 2004 and have spent a considerable amount of time with the Group management in discussions on many matters relating to the Group's current and future development. A number of strategic initiatives have been developed which I have referred to earlier and this has involved workshops and special board meetings over a number of hours which board members have given freely and voluntarily of their time. I believe people need to know that the pressure placed on Board of Directors nowadays under the Companies legislation and public accountability means that they put in a lot of time and effort so as to ensure that our Group hospitals are run effectively and efficiently and so they can reassure the owners, shareholders and the Religious Sisters of Charity, that their wishes and hopes for the future of patient care is in line with their philosophy and ethos.

To the staff in all three Group hospitals, I know 2004 has had its challenges and staff once again never cease to amaze me on how they deal with these challenges in the interest of delivering the best care possible having regard to the resources available to them.

Healthcare delivery today involves the huge dedication of staff and I can only say the expectations of the public and demands put upon staff is extraordinary.

I want to reassure staff that their work is appreciated, not withstanding the adverse media coverage about the health services and the terrible state it is in. I believe that the quality of care the health services provide, having regard to resources available is second to none and we do and can hold our heads high as providers of the services, notwithstanding the limited resources we have. One has to remember we are providing complex services 24 hours a day, 365 days a year, where mistakes can be fatal, how often do we hear about all the hundreds of thousands of patients who are looked after so well?

Let us remind ourselves that St.Vincent's Healthcare Group hospitals are at the cutting edge of medicine and that, in itself, is challenging notwithstanding the volume of services we are expected to provide.

I believe St.Vincent's Healthcare Group and its individual hospitals are stepping up to the mark and will flourish over the coming years. The shareholders have entrusted and put structures and a Board of Directors in place who are fully committed and willing to support the hospitals within the Group.

Once again, I would like to reassure the community whom we serve and the patients we look after that you do come first and that the philosophy of the hospitals within our Group is to serve our patients and their families.

Once again, on behalf of the management, the Board of Directors of St. Vincent's Healthcare Group, our Shareholders and ultimately the public, I want to take this opportunity of sincerely thanking staff for their work throughout 2004. Once again, as I said last year, we can only do our best and if each one of us does that, nothing more can be expected of you.

Thank You

Nicholas C. Jermyn
Group Chief Executive



"Nicholas C. Jermyn, Sr. Una O'Neill and Professor Noel Whelan at the opening of Lois Aoibhinn, Cancer Support Centre."

Hospital Re-development Review

Executive Review

St. Vincent's University Hospital's Redevelopment Project

I am pleased to report that there has been considerable work achieved this year on Phase 1 of the redevelopment of St. Vincent's University Hospital. As you know from previous reports, Phase 1 of the redevelopment got underway in September 2001 with practical completion of the new five storey building expected to be achieved in February 2005, approximately five weeks ahead of the contract date.

The total cost of the development is in the region of €230m with funding being provided from the National Development Programme and central government (Department of Health & Children). As I reported last year, the main elements of the redevelopment are:

- Intensive Care / High Dependency Unit
- Radiology Department
- Pathology Department
- Ambulatory Day Care including Outpatient Department, Day Care Units, Specialist Clinics, Endoscopy, Dialysis etc.
- Emergency Department
- Operating Theatres

The following are other new facilities included in the redevelopment:

- Multi-storey car-park completed October 2002
- 54 bedded Psychiatric Unit completed March 2003
- Pharmacy completed January 2003
- Mortuary extension completed March 2003
- New road and surface car-parking completed October 2003
- Waste marshalling yard completed June 2004

In addition to the above developments, a number of other developments are underway or in the process of being completed.

Fire up-grade works within the existing St. Vincent's University Hospital main buildings and the refurbishment of the lifts within the existing buildings are nearly completed. It should be noted that the new development also included the funding of the Breast Screening Unit for the National Breast Screening Programme. This building was completed in January 2004, eight weeks ahead of programme, and opened for business on 16th February 2004. All of these projects were within budget and have been completed on target.

I should now indicate the challenge ahead. The single most significant challenge which lies ahead is the operation and commissioning of the new five-storey building. A budget of €43m is available for the purchase of equipment for the new building.

The process of procurement of the equipment is already underway with tenders received for Radiology, PACS, Pathology, Dialysis and furniture. In addition to these packages there are a further forty tender packages that have to be dealt with. Before any of these can be put out to tender, specifications have to be developed in conjunction with the end users and approved by the Joint Equipment Group and Project Team.

Once procured, delivery, receipt, checking, testing, asset registering, installation and commissioning of the equipment requires to be managed. In addition to this, plans will be developed to facilitate the transfer of existing equipment to dovetail with the opening dates for each individual department in the new building. One of the most challenging transfers to be undertaken will be to the new Emergency Department. This has to be achieved while still providing emergency services.

User groups are currently developing operational policies for their new departments and looking at the logistics of moving from current locations to the new building. As current plans stand, it is important to note we would expect to see the first of many transfers to commence in the third quarter of 2005.

It is anticipated that various departments will have moved into their new facilities towards the end of 2005 and it may be an appropriate time to target an official opening ceremony.

I cannot stress the effort that it has taken to mobilise, build and develop such a new facility on campus while still remaining operational and providing services locally and regionally.

This achievement must be credited to the Design Team, the contractors and staff involved. I would like to take this opportunity of formally thanking everybody involved in the development.

Group Executive


Nicholas C. Jermyn


Mr Eamonn Fitzgerald


Mr. Ken Bale


Ms. Marianne Byrne


Mr. Noel Cassidy
(to July 04)


Ms. Gretta Colbert


Mr. Ian Maguire


Ms. Pauline Doyle
(to April 04)


Mr. Cormac Maloney


Dr. Brian Maurer


Dr. Risteárd Ó Laoide


Ms. Sorcha O'Quigley
(from August 04)


Mr. James Crowe


Mr. Seamus Murtagh


Ms. Raphael McMullin
(from May 04)


Mr. Michael Redmond


Mr. Peter Sheehan


Ms. June Stanley


Mr. Neil Twomey

Review by

Chairman of the Medical Board

Executive Review

As Chairman of the Medical Board I have pleasure in presenting the annual review for the year 2004.

Effective Bed Capacity

The multi-disciplinary bed management group continued to meet on a regular basis throughout the year.

The challenge facing this group continues to be extremely difficult due to the inadequate bed base, aging demographics served by the Hospital Group, the persistently high level of emergency department activity, and the lack of adequate step down facilities.

This has led to difficult conditions in the Emergency Department both for patients and staff. Outside the Emergency Department the effective bed shortage has had a significant impact on the capacity for elective surgery. Significant efforts have been made both at clinical and management levels to optimise the current situation. However, the persistent, significant, underlying, fundamental problems mean that the impact of such initiatives is limited.

At the time of writing, the Minister for Health and Children has proposed a 10-point initiative to address the problems in Emergency Departments nationally. It is hoped that effective implementation of these initiatives will lead to significant improvements in this difficult situation.

Research

In collaboration with the Dublin Molecular Medicine Centre, St. Vincent's University Hospital (SVUH) was awarded a grant to develop a Genome Research Unit (GRU) on the hospital site. The GRU will provide a state of the art ambulatory clinical research unit at the hospital. This will allow synergy between the research efforts of clinicians at SVUH, and bench based researchers at the Conway Institute at UCD. The GRU facility will be sited in the former medical residence within the hospital. The preparatory work involving the transfer of the medical residence to the convent area is near completion at the end of 2004. The GRU facility itself is due for completion in November 2005.

During 2003, Professor Pat Fottrell and Mr. Leo Kearns were asked by the Board of Directors to undertake a strategic review of research. The report and its recommendations were subsequently adopted by the St. Vincent's Healthcare Group Board in 2004. Two of the key recommendations include the setting up of a high level strategic Research Advisory Board and the appointment of a Director of Research. It is hoped that both will be in place in 2005.



Education

St Vincent's University Hospital/University College Dublin agreement was signed in December 2003. The schedules to this agreement were finalised and agreed in August 2004.

A recent medical council report on medical education, and the current review of undergraduate medical education chaired by Professor Fottrell makes this an opportune time to evaluate and develop the educational infrastructure on the St. Vincent's campus. The Board of Directors has therefore asked Professor Pat Fottrell and Mr. Leo Kearns to undertake a strategic review of education for the group. It is hoped that this review will be finalised in 2005.

Radiotherapy Proposal

A request for a radiotherapy proposal for the Eastern Region, South was received from the Department of Health in mid 2004. The development of this proposal was led by Professor John Armstrong and required a significant amount of time and effort from many people throughout the hospital. The final impressive proposal was testament to the effort and the co-operation of all involved. The deliberations of the Radiotherapy Review Group have not yet been published.

Project Development

The St Vincent's University Hospital project development progressed extremely well during 2004. The project continued to be on time and within budget, with a view to effective completion by early 2005. Subsequently, a significant amount of work involving commission and equipping the building will take place with a view to occupation in late 2005.

Accreditation

The St. Vincent's Healthcare Group took part in a Hospital Accreditation survey organised by the Irish Health Services Accreditation Board. This involved the co-operation of the three hospitals within the group at all levels. The group achieved a pre-accreditation advanced status, which is comparable to that achieved by the other major academic teaching hospitals who have undertaken this accreditation process.

St Vincent's Private Hospital

Dr. Brian Maurer continues to ably chair the St. Vincent's Private Hospital (SVPH) Medical Forum. During 2004, the Board of Directors sanctioned the further development of Radiotherapy facilities at SVPH. At the Group Medical Executive, strategies for synergies between SVPH and SVUH continue to be explored and developed.

St Michael's Hospital

Mr Donal Maguire succeeded Mr David Mulvin as chairman of the St. Michael's Hospital (SMH) Medical Forum. The Medical Executive thanks Mr Mulvin for his excellent contributions and wishes Mr Maguire well in his new role. Work continues on streamlining the delivery of clinical care in the Southeast region utilising the strengths of each individual institution. The Medical Board is pleased to facilitate the co-ordination of a medical strategy, which will maximise the facilities available for optimal patient care.

Consultant Changes

A number of new consultants commenced their practice at St Vincent's University Hospital in 2004. These included Mr. Sean Carroll, (Plastic Surgeon), Dr. Ingrid Browne (Consultant Anaesthetist), Dr. Niall Tubridy (Consultant Neurologist) and Dr. Susan Knowles (Consultant Microbiologist). The Medical Board welcomes our new colleagues and wishes them a happy and fulfilled professional career.

During 2004, four consultants retired from the group. Mr. J. Murphy, Professor N. Duignan, Dr. E. Gallagher and Dr. M. Slazenger. On behalf of the Medical Board, I would like to thank them for their dedication to the highest standards of clinical care over many years and to wish them a long, healthy and happy retirement.

Finally, I would like to thank the large number of consultant colleagues who have given generously of their time and expertise and who have been most supportive over the past year. In particular I would like to thank Dr. Alan Watson who completed his second term as Honorary Secretary of the Medical Board during 2004. I wish his successor Dr. Ken McDonald well in his new role.

I would also like to acknowledge the significant help and contribution of all the other members of the Medical Executive.

I would like to thank Ms. Ann Shannon and her team in the Medical Board office, without whose considerable support the Medical Executive would not function.

I would also like to thank the members of the hospital management, in particular Mr. Nicholas Jermyn, Group C.E.O., and Mr. Eamonn Fitzgerald, Deputy Group C.E.O., for their continued help and support.

Dr. Risteárd Ó Laoide
Chairman of the Medical Board
St. Vincent's Healthcare Group

Finance Report 2004

Executive Review

St. Vincent's Healthcare Group Ltd

St. Vincent's Healthcare Group Ltd includes the business of St. Vincent's University Hospital, St. Michael's Hospital and St. Vincent's Private Hospital.

The company results attached include the income, expenditure and assets of the three institutions. Branch accounts are prepared separately for each division.

The following notes have treated each branch separately.

St. Vincent's University Hospital Branch

Funding

The initial allocation in 2004 was €145.2m. Additional funding was secured from the ERHA during the year in relation to Emergency Services, Cancer Services, minor capital purchases and other service pressures. This funding, together with careful management of resources available enabled the Hospital to achieve a breakeven position at the end of the year. The final allocation for 2004 was €160.9m.

Income & Expenditure Account

Total expenditure (excl depreciation) in 2004 was €177.3m, an increase of €19.1m (12%) from 2003. The primary drivers of this increase were salaries & wages, drug expenditure and medical and surgical supplies.

Pay

Benchmarking and Sustaining Progress pay increases accounted for most of the increase in pay.

The cost of agency nursing also increased primarily due to the full year effect of VAT charged on all agency costs.

There were significant lump sum superannuation payments with the resignation of some long serving employees.

The staff ceiling imposed by the ERHA continues to be a difficulty and needs to be adjusted to reflect the posts which were funded in recent years.

Non Pay

Medical inflation continues to run much higher than the national rate of inflation. Drug usage and costs continue to escalate. The main increases are in areas such as Oncology, Rheumatology and Infection Control.

Medical and surgical appliance cost increases are due to increased usage of high cost treatments such as Spinal Cord Stimulators (Pain Management), ICD Implants (Cardiology), CAPD Programme (Dialysis) and Drug Eluting Stents.

X-Ray costs have also increased due to the significant increase in the utilisation of high cost PET Scanning chosen for their greater diagnostic ability.

Demand for the Pathology service remains high including the demand for high cost/complex histology tests relating to surgical oncology.

Maintenance costs continue to rise due to the ageing profile of our assets. A number of major maintenance contracts were undertaken during the year in relation to lift refurbishment and plantroom pipe works. Conversion of the old convent into clinical offices was also carried out in 2004.

Heat, power and light running expenses have also increased in line with rising global prices.

In 2004 there was a recruitment drive for non-national nurses.

From the 1st February 2004, the Government introduced a Clinical Indemnity Scheme, which covered all medical indemnity issues. This reduced the indemnity charge in the accounts in 2004.

Income

There were approved price increases for Semi-Private, Health Act Inpatient and A&E charges in 2004.

Activity

- Admissions for the year totalled 30,894.
- Operations increased by 2.7% to a total of 16,083.
- 45 Liver Transplants were performed in 2004 (32 were performed in 2003).
- The inpatient waiting lists (inc. day cases) at the year end totalled 4,207 (year 2003 was 4,117).
- Occupancy levels were 102.4% a decrease of 0.3% on last year.

Balance Sheet

Land and Buildings

Land and buildings have been re-valued at the 31st December 2004. This has added €28.5M to the asset values.

Capital Expenditure

Substantial progress was made in the main hospital development programme (€203 million), which commenced in September 2001. Expenditure on the main project development during 2004 amounted to €51.7m. €47m was spent on construction costs and €4.7m on professional fees, equipment and other costs.

Minor capital equipment grants totalling €3.3m were received in 2004.

Casemix

The hospital received a positive adjustment of €179k for its 2005 allocation.

Considerable effort was expended to turn around the previous year's negative out-turn.

St. Michael's Hospital Branch

Income & Expenditure Account

Total Income in 2004 was €3.2m, an increase of €0.4m on year 2003 (€2.8m).

Expenditure rose to €32.3m from €30.2m in 2003.

Pay

Salaries and wages were €21.0m (€21.2m in 2003), a decrease arising from the reduction in consultant's MDU subscription and once off adjustment to accruals in 2003.

Non Pay

Main increases in Non-Pay expenditure occurred under surgery and dispensary and establishment costs.

Total allocation received in the year was €27.8m (€26.5m in 2003).

Activity

Admissions for 2004 were 6,988 compared with 6,202 in the year 2003.

Balance Sheet

Land and Buildings

Land and buildings have been re-valued at the 31st December 2004. This has added €2.5M to the asset values.

St. Vincent's Private Hospital

Income & Expenditure Account

	Eur '000
Income	43,678
Pay costs:	20,736
Non Pay costs:	22,130
Net Profit	812

Pay

Pay costs in 2004 were €20.7m. The main components of these costs were Nursing/Portering €11.4m, Allied Health €3.7m and Support Services €2.4m.

Benchmarking and Sustaining Progress pay increases accounted for a significant amount of the increase in pay year on year. There was also an increase in the use of agency nursing and agency cost increases due to the full year effect of VAT being charged on agency fees.

Non Pay

Non-pay costs increased by €0.9m(4%) over 2003 to €22.1m. The main components of these costs were Pathology €1.6m, Pharmacy €5.2m, Medical Supplies €1.5m and Theatre €2.8m

The costs of both Pharmacy and Theatre rose due to activity increases. Service price increases were also incurred in the areas of Cardiac and ICU. Increased usage of high cost ICD Implants (Cardiology) also impacted. Improved cost control procedures and the achievement of better prices through Group purchasing economies resulted in a decrease in the cost of medical supplies.

Income

Direct income rose by €4.4m(11%) to €43.3m. This would be primarily due to increases granted by VHI and other insurers. There was also an increase in the number of high-cost radiology procedures carried out during the year. The main components of direct income are Accomodation €20m, Radiology €6.5m, Oncology €5.9m and Major Theatre €3.3m.

Activity

Admissions (inpatient)	9,127	(2.9% Inc)
Daycases	5,317	(0.3% Inc)
Oncology (daycases)	5,127	(16% Inc)
Major Theatre	3,808	(1.4% Dec)
Minor Theatre	2,967	(6% Dec)
Endoscopy	5,203	(10.6% Inc)
Diagnostic Imaging	29,324	(3.6% Inc)

Balance Sheet

Land and Buildings

Land and buildings have been re-valued at the 31st December 2004. This has added €9.6M to the asset values.

Capital Expenditure

Capital expenditure totalled €2.9m in 2004. €1.7m was spent in the Radiotherapy area, €0.1m on lift upgrades and €0.1m on computer equipment.

Future Developments

System developments in progress include the introduction of document scanning and electronic workflow management, which will reduce the amount of paperwork in the Finance Dept.

In addition, web based online requisitioning is being introduced which will make requisitioning easier for all users and make production of purchase orders more efficient.

It is hoped to continue with the integration of policies and procedures within the group and to continue with co-operation in relation to purchasing.

Cormac Maloney
Financial Controller

St. Vincent's Healthcare Group Limited

Company Accounts

For the year ended 31st December 2004

Income & Expenditure a/c	2004 Euro €	2003 Euro €
Income		
Patients Income	50,779,199	45,411,678
Other Income	7,761,499	6,406,770
	58,540,698	51,818,448
Grants Amortised		
Building & Equipment	7,547,036	6,602,668
Expenditure		
Salaries & Wages	159,985,084	144,943,071
Surgery & Dispensary	53,551,888	47,849,170
Provisions	1,981,361	2,157,448
Domestic	9,426,231	9,118,032
Clinical Support & Administration Services	6,547,496	5,967,320
Maintenance of Buildings etc	7,544,915	4,297,415
Miscellaneous	5,443,097	5,980,228
Bad Debts	174,871	201,928
Bank Interest & Finance Charges	246,980	209,258
Tax	0	4,083
Depreciation - Buildings & Equipment &Motor Vehicles	8,605,824	7,413,369
Total Expenditure	253,507,747	228,141,322
Excess of Expenditure over Income	-187,420,013	-169,720,206
Refundable from the ERHA	188,766,648	175,684,009
Income & Expenditure (Deficit) / Surplus	1,346,635	5,963,803
Accumulated (Deficit) / Surplus as at 31st December		
Surplus / (Deficit) for the Year	1,346,635	5,963,803
Transfer to other reserves	-577,406	-87,022
	769,229	5,876,781
Surplus carried forward from previous years	177,109	-5,699,672
Accumulated Surplus /(Deficit)	946,338	177,109



Balance Sheet as at Year End

	2004 Euro €	2003 Euro €
Income		
Assets		
Fixed Assets	392,393,291	300,754,169
Current Assets	51,419,715	45,082,717
	443,813,006	345,836,886
Liabilities / Reserves / Capital		
Creditors	53,816,189	47,019,357
Capitalisation Accounts	148,396,587	96,930,467
Capital & Other Reserves	240,653,892	201,709,953
Accumulated (Deficit) / Surplus	946,338	177,109
	443,813,006	345,836,886

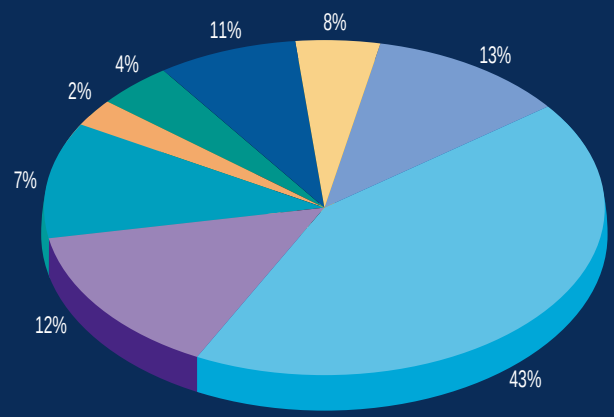
NOTE

Figures shown for 2004 & 2003 include SVUH, St. Michael's Hospital and St. Vincent's Private Hospital.

Financial Analysis

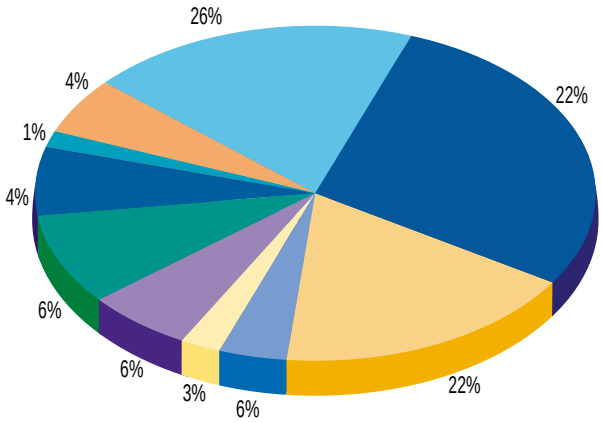
Executive Review

Pay Expenditure 2004



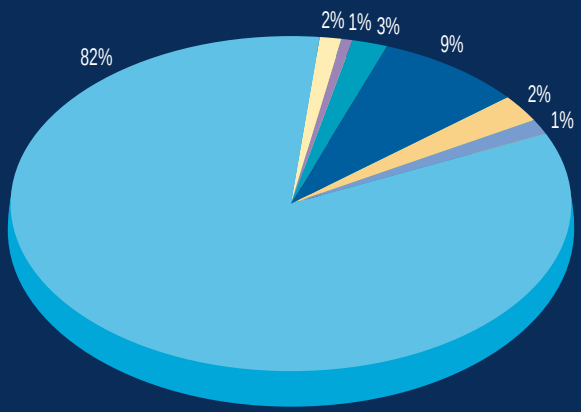
- Administration 11%
- Paramedical 12%
- Common Contacts 8%
- Catering / Housekeeping 7%
- Medical Salaries 13%
- Maintenance 2%
- Nurse & Allied 43%
- Superannuation 4%

Non Pay Expenditure 2004



- Medical & Surgical 22%
- Maintenance Charges 6%
- Medicines 22%
- Blood 4%
- Pathology Expenses 6%
- Courses & Seminars 1%
- Medical Equipment 3%
- X-Ray Expenses 4%
- Cleaning / Laundry 6%
- Sundry Expenses 26%

Income 2004

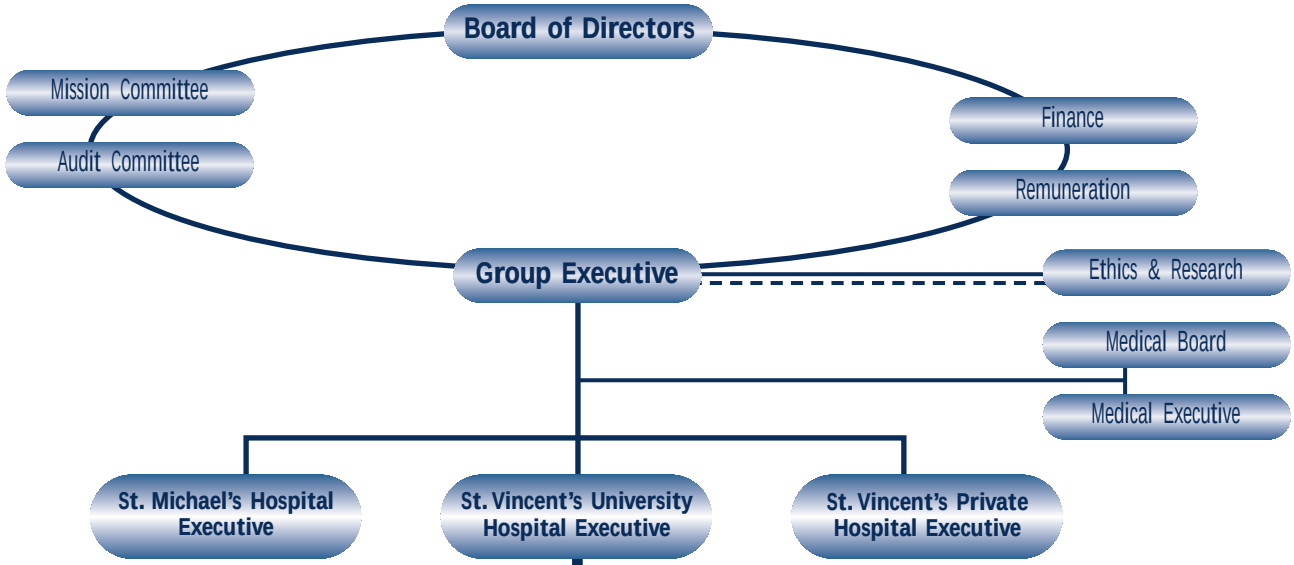


- Superannuation 9%
- Health Act Inpatient 2%
- Canteen 2%
- A&E Charge 1%
- RTA etc 1%
- Other Income 3%
- Semi-Private 82%

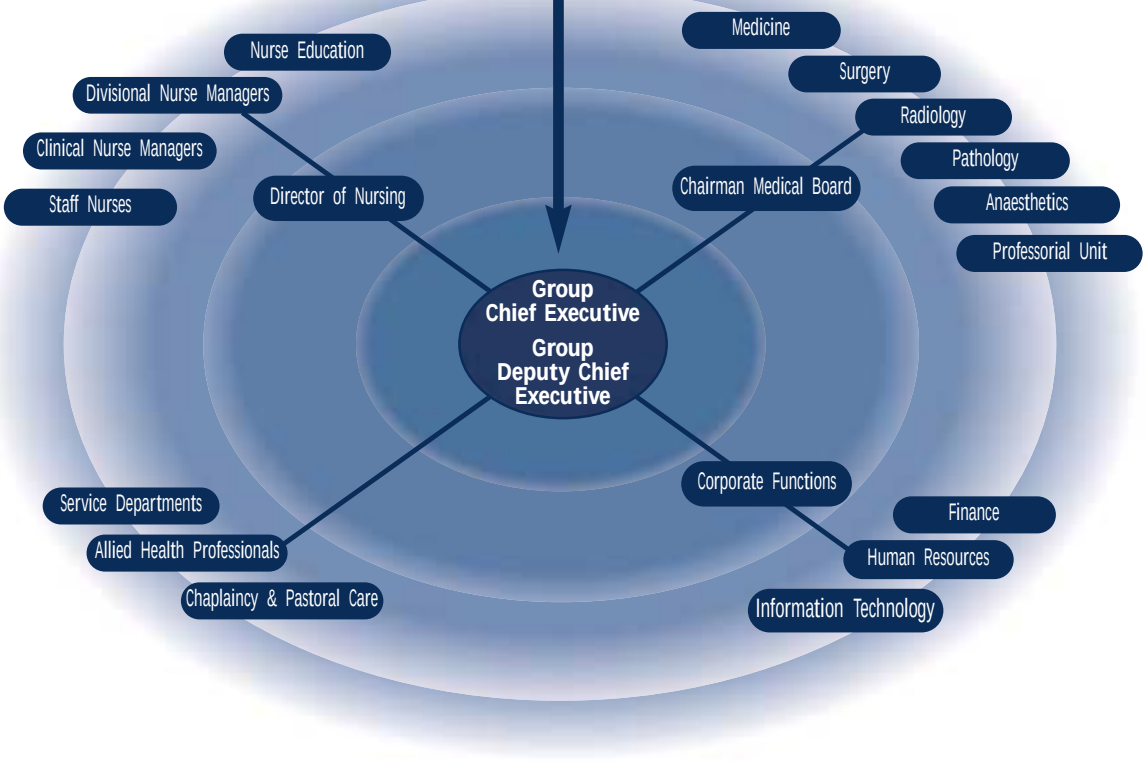
Figures shown include SVUH, SMH and SVPH

Governance & Management Structure

Healthcare Group Board Structure



Operational Structures



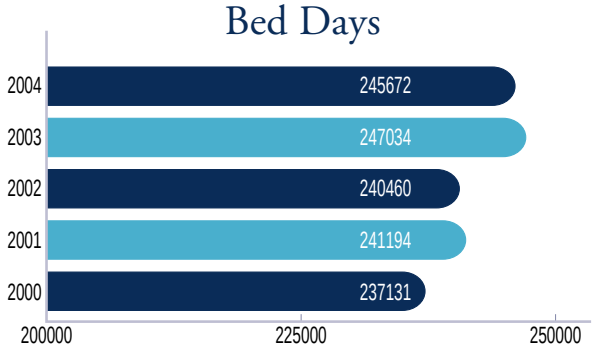
Clinical Network within Eastern Region



Statistics

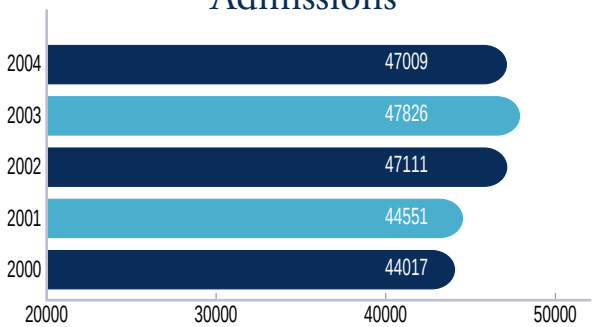
Executive Review

Bed Days



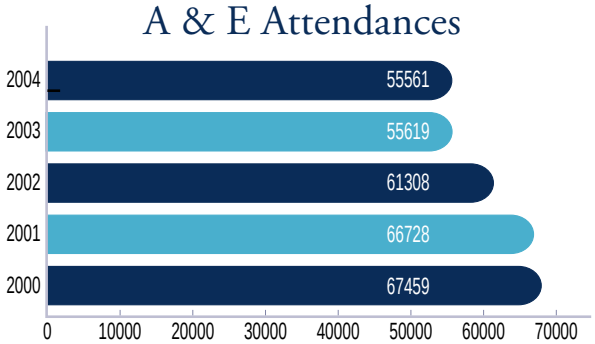
Figures shown above include SVUH, St. Michael's & SVPH.

Admissions



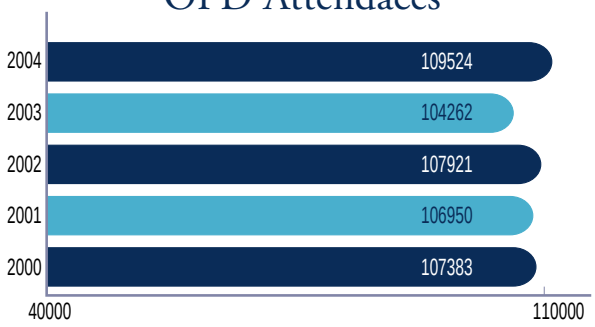
Figures shown above include SVUH, St. Michael's & SVPH.

A & E Attendances



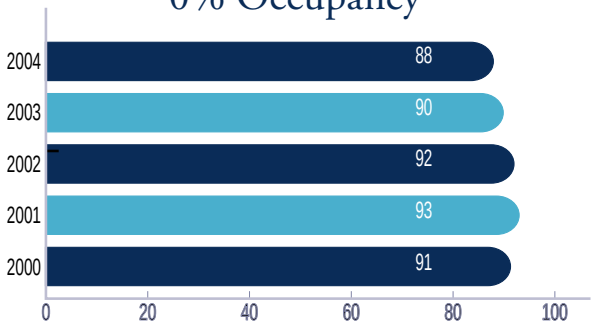
Figures shown above include SVUH, St. Michael's only.

OPD Attendaces



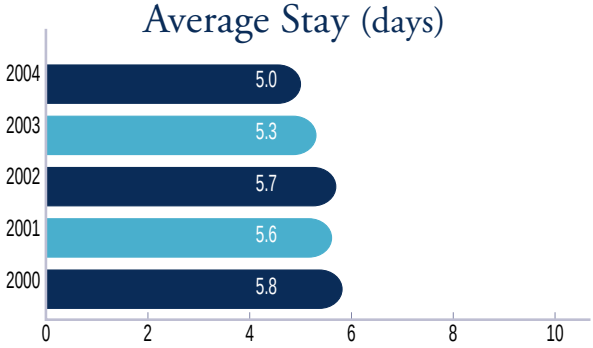
Figures shown above include SVUH, St. Michael's only.

0% Occupancy



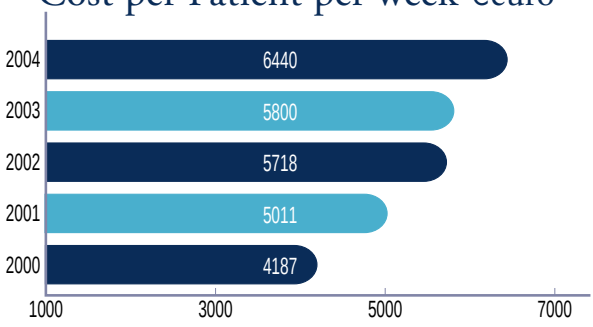
Figures shown above include SVUH, St. Michael's & SVPH.

Average Stay (days)



Figures shown above include SVUH, St. Michael's & SVPH.

Cost per Patient per week €euro



Figures shown above relate to SVUH only.

Departmental Statistics

DEPARTMENT			2002	2003	2004
CARDIOLOGY					
	Inpatients(incl. Day-care)	Admissions	1,239	1,410	1,572
		Discharges	1,416	1,614	1,710
	Outpatients	New patients	772	885	981
		Total attendances	4,144	4,622	5,564
NEPHROLOGY					
	Inpatients	Admissions	631	563	558
		Discharges	532	469	511
	Outpatients	New patients	134	165	154
		Total attendances	1,157	1,370	1,237
ONCOLOGY					
	Inpatients	Admissions	477	422	352
		Discharges	735	611	580
	Outpatients	New patients	253	223	259
		Total attendances	1,849	1,891	2,011
RADIOTHERAPY					
	Outpatients	New patients	264	264	254
		Total attendances	357	359	339
HAEMATOLOGY					
	Inpatients(incl. Day-care)	Admissions	264	251	193
		Discharges	310	282	220
	Outpatients	New patients	192	182	210
		Total attendances	1,286	1,164	1,280
ST ANNE'S DAY CENTRE (Oncology/Haematology)		Admissions	4,678	5,124	4,783
		Discharges	4,678	5,124	4,783
PLUS ward patients treated in centre			28	23	164

DEPARTMENT			2002	2003	2004
GENERAL MEDICAL					
	Inpatients	Admissions	160	214	232
		Discharges	139	168	191
	Outpatients	New patients	40	18	17
		Total attendances	140	111	85
PSYCHIATRY					
	Inpatients	Admissions	141	180	172
		Discharges	162	187	178
	Outpatients	New patients	250	227	229
		Total attendances	2,209	1,776	1,950
	Day Centre	Attendances	1,365	893	1,231
NEUROLOGY					
	Inpatients(incl. Day-care)	Admissions	208	361	344
		Discharges	243	415	394
	Outpatients	New patients	808	662	903
		Total attendances	3,286	2,764	3,387
DERMATOLOGY					
	Inpatients(incl. Day-care)	Admissions	3,002	3,176	3,023
		Discharges	3,011	3,182	3,025
	Outpatients	New patients	770	755	751
		Total attendances	2,402	2,542	2,333
RHEUMATOLOGY					
	Inpatients(incl. Day-care)	Admissions	866	850	643
		Discharges	817	783	540
	Outpatients	New patients	1,029	992	974
		Total attendances	5,770	5,515	5,609
RESPIRATORY & GENERAL MEDICINE					
	Inpatients(incl. Day-care)	Admissions	2,250	2,057	2,000
		Discharges	2,051	1,905	1,910
	Outpatients	New patients	572	599	631
		Total attendances	6,116	5,659	5,752

DEPARTMENT			2002	2003	2004
ENDOCRINOLOGY					
	Inpatients(incl. Day-care)	Admissions	519	492	513
		Discharges	428	416	459
	Outpatients	New patients	320	310	330
		Total attendances	2,334	2,312	2,359
DIABETES CLINIC					
	Outpatients	New patients	257	259	297
		Total attendances	3,152	3,245	3,479
DIABETES CENTRE					
		Attendances	3,391	4,260	5,020
GERIATRIC MEDICINE					
	Inpatients	Admissions	425	397	405
		Discharges	443	403	429
	Outpatients	New patients	280	277	252
		Total attendances	778	820	798
GASTROENTEROLOGY & GENERAL MED.					
	Inpatients(incl. Day-care)	Admissions	2,473	2,429	2,674
		Discharges	2,339	2,334	2,528
	Outpatients	New patients	924	949	1,064
		Total attendances	3,333	3,444	3,821
GENERAL SURGERY					
	Inpatients(incl. Day-care)	Admissions	4,309	4,235	4,107
		Discharges	4,272	4,211	4,081
	Outpatients	New patients	3,854	3,605	3,861
		Total attendances	14,395	13,209	13,304
LIVER CLINIC					
	Outpatients	New patients	137	166	185
		Total attendances	1,952	1,997	2,160
HEPATITIS C CLINIC					
	Outpatients	New patients	67	59	29
		Total attendances	950	913	848
COLORECTAL CLINIC					
	Outpatients	New patients	72	65	53
		Total attendances	677	705	696

DEPARTMENT			2002	2003	2004
PAIN RELIEF					
	Inpatients(incl. Day-care)	Admissions	460	426	463
		Discharges	463	429	468
	Outpatients	New patients	234	240	276
		Total attendances	2,499	2,708	3,238
VASCULAR SURGERY					
	Inpatients(incl. Day-care)	Admissions	566	513	546
		Discharges	582	527	554
	Outpatients	New patients	569	560	625
		Total attendances	2,775	2,678	2,960
ORTHOPAEDIC SURGERY					
	Inpatients(incl. Day-care)	Admissions	1,891	1,811	1,746
		Discharges	1,901	1,798	1,759
	Outpatients	New patients	3,430	3,412	3,671
		Total attendances	12,395	12,431	13,116
UROLOGY					
	Inpatients(incl. Day-care)	Admissions	2,458	2,244	2,229
		Discharges	2,452	2,247	2,228
	Outpatients	New patients	1,256	1,102	940
		Total attendances	5,155	4,399	3,965
PLASTIC SURGERY					
	Inpatients(incl. Day-care)	Admissions	1,040	1,289	1,028
		Discharges	1,040	1,295	1,041
	Outpatients	New patients	1,061	1,053	1,127
		Total attendances	3,739	3,650	3,795
CARDIO THORACIC					
	Inpatients(incl. Day-care)	Admissions	148	174	160
		Discharges	158	204	170
	Outpatients	New patients	198	156	179
		Total attendances	600	628	570
GYNAECOLOGY					
	Inpatients(incl. Day-care)	Admissions	260	474	396
		Discharges	269	491	400
	Outpatients	New patients	686	645	645
		Total attendances	1,911	1,652	1,608

DEPARTMENT			2002	2003	2004
OPHTHALMOLOGY					
	Inpatients	Admissions	458	458	442
		Discharges	456	451	434
	Day-care	Admissions	1,846	1,994	1,729
		Discharges	1,846	1,994	1,729
	Outpatients	New patients	773	781	729
		Total attendances	2,398	2,475	2,245
ORTHOPTICS					
		Total attendances	502	927	777
A & E Patients treated in Ophthalmology Dept.		Total attendances	1,261	1,016	1,064
E.N.T. SURGERY					
	Inpatients(incl. Day-care)	Admissions	718	673	578
		Discharges	715	673	578
	Outpatients	New patients	955	1,033	1,012
		Total attendances	2,098	2,196	2,370
AUDIOMETRICS					
		Tests	463	516	533
DENTAL SURGERY					
	Inpatients	Admissions	22	10	6
		Discharges	19	10	6
	Outpatients	New patients	2	0	0
		Total attendances	3	0	0
PODIATRY					
	Outpatients	Total attendances	349	271	345
SUMMARY					
	Total Inpatients	Admissions	31,509	32,227	30,894
		Discharges	31,477	32,223	30,906
	Total Outpatients	New patients	20,159	19,644	20,638
		Total attendances	89,860	87,235	90,879

Diagnostic and Treatment

Executive Review

DEPARTMENTS	2003	2004	%VARIANCE	Variance
Pathology Test	3,753,283	3,838,016	2.3%	84,733
X-Rays	104,909	112,938	7.7%	8,029
Dialysis Treatments	4,994	6,370	27.6%	1,376
Pulmonary Laboratory Attendances	3,608	3,138	-13.0%	-470
Physiotherapy Attendances	72,445	69,492	-4.1%	-2,953
Social Work Cases	10,539	10,671	1.3%	132
Dietetics Attendances	20,367	20,885	2.5%	518
Diabetes Centre	4,260	5,020	17.8%	760
Vascular Lab. Tests	1,322	1,429	8.1%	107
E.C.G. Recordings	17,301	16,833	-2.7%	-468
Theatres				
Main Theatre	9,966	9,859	-1.1%	-107
Cardiovascular Theatre	2,194	2,356	7.4%	162
Endoscopy Unit	3,494	3,868	10.7%	374
E.E.G./E.M.G. Tracings	995	1,548	55.6%	553
Occupational Therapy Treatment Units	51,115	41,530	-18.8%	-9,585
Nuclear Medicine Scans	3,596	3,716	3.3%	120

Health and Safety Review

Executive Review

St. Vincent's Healthcare Group recognises its responsibility to provide the safest possible environment for patients, relatives, visitors and staff members. We aim to meet and exceed our legislative requirements through awareness of, and adherence to, best practice.

The Group Executive has established a Health & Safety Committee as the structure through which the Group manages and co-ordinates matters pertaining to health, safety and welfare at work. The committee comprises of the co-ordinators of departmental safety groups and representatives from other key functions such as Occupational Health, Infection Control, Back Care & Ergonomics and General Management.

Local representation comes in the form of departmental or functional safety groups, which provide a forum for staff to raise and address safety issues within their own areas. At present, our local safety groups comprise of members from Administration, Allied Health Professions, Catering, Nursing, Pathology, Pharmacy, Portering, Radiology, Technical Services and Theatre, together with representation from the safety groups in St. Michael's Hospital and St. Vincent's Private Hospital. The work of these groups is a key element of our safety programme and we are grateful for their ongoing commitment.

In keeping with our commitment to provide a clear and up to date written policy on the management of safety, health and welfare at work, we continued with the review and update of our Safety Statement during 2004.

Ongoing participation in the Accreditation process and preparation for the Peer Review Survey brought a continuing focus on the management of safety and risk within the Group. This process has strengthened our awareness of the management of safety throughout the Group, and helps identify opportunities for improvement in these areas.

Finally, the provision of ongoing, relevant training is seen as an essential element of our health & safety programme and we continue to focus resources on key areas including Fire Safety, Manual Handling and Ergonomics, non-Violent Crisis Intervention and Sharps Awareness.

Larry Clancy
General Services Manager



Mission Committee

Executive Review

Mission Effectiveness Programmes Units 1 & 2 were held throughout the year and were well attended by staff from all departments. Feedback from groups was very positive and was presented to the Executive Council, to Heads of Departments meetings and discussed at our committee meetings. Staff from St Michael's Hospital who attended the programmes during the year found them interesting and informative and appreciated the opportunity to improve and expand their links with SVUH.

Staff Art and Photographic Exhibition

The exhibition held in May was a most enjoyable event. There were over 80 exhibits and both staff and visitors enjoyed the viewing. Exhibits were chosen for inclusion in the 2005 Hospital Calendar.

Celebrations

On Foundation Day, January 23rd Mass was celebrated for all staff. Stands were placed in the foyer displaying text and images of interest from the past and historical video clips, which were shown throughout the day in the conference room, complemented this.

To commemorate the feast of St Vincent de Paul on September 27th a remembrance service for deceased and bereaved staff members was held.

Hospital Choir

Our hospital choir rendered great service during the year enhancing the Liturgy and adding to the enjoyment of our Carol Service in December.

Committee Membership

Ms Marie Culliton, Senior Medical Scientist resigned from the committee during the year to take up a post as Laboratory Manager in Holles Street. Marie served on the committee for 3 years and we are grateful for her support and commitment. We will also miss her leadership and expertise in leading the choir.

Ms Anne Grumley, Divisional Nurse Manager who acted as Mission Liaison Person for the last five years retired this year. We acknowledge with gratitude Anne's valuable contribution in promoting Mission.

Sr. Eugene Butler
Chairperson, Mission Committee

Ethics and Medical Research Committee

Executive Review

The St. Vincent's Healthcare Group (SVGH) Ethics and Medical Research Committee (EMRC) met on 11 occasions during 2004 and the committee reviewed 24 clinical trial applications and 108 research studies. Following appropriate review and incorporation of recommended revisions, all studies were approved.

Dr. Richard Assaf, Consultant Anaesthetist and Clinical Risk Facilitator, has continued the enormous task of reviewing all serious unexpected adverse reaction reports arising from clinical trials and reporting these to the committee. We are very grateful to Richard for this service, which has at all times been performed to the highest standard.

On May 1st 2004, the EU implemented a new directive entitled: "European Communities Clinical Trials on Medicinal Products for Human Use"(Regulations 2004). The purpose of this Directive was to harmonise and standardise approval for Clinical Trials by regulatory authorities and ethics committees across all EU member states.

Dr. Tom Crotty, stood down as chairman after six years of tireless leadership and I would like to take this opportunity to thank Tom for his excellent stewardship over this time. In addition, the following also resigned having completed a full and dedicated period as members of the committee, Dr. S. Donnelly, Mr. S. Dudeney, Mr. M. Tolan, Mr. Fergus Clancy, Ms Mary Nevin and Ms Breda Kearns. I am delighted that Ms Joan McDonnell, who has a wealth of experience, continues to provide the highest level of support to the committee and all the SVHG staff and I thank Joan for her constant hard work and efficiency.

In compliance with the directive, the SVHG EMRC changed its conformation to 11 expert members and 7 lay members. The newly formed SVHG EMRC applied for and was duly designated a recognised committee acting for the whole state by the Minister of Health and Children, acting in his capacity under the Regulations, as the Ethics Committees Supervisory Body, in September 2004. Under this designation, the Committee may act to give an opinion on applications for clinical trials at sites other than those within the St. Vincent's Healthcare Group.

Since the new EMRC was formed we have revised our application form in an attempt to ensure maximum compliance with the EU directive and to avoid unnecessary confusion. Two forms are now used (i) for clinical trials of medicinal products for Human Use, and (ii) for investigator-driven and non-invasive research studies. In line with these changes our standard Operating Procedures (version 5 dated 5th August, 2004) has also been updated. We are currently developing standard Patient Information Leaflets and Consent Form templates, which should further streamline the application process.

Copies of all documents are available electronically and in hard copy from Ms Joan McDonnell, Ethics Office, ext: 4117 (joan.mcdonnell@ucd.ie).

Dr. D. Veale, Chairman.



St. Michael's Hospital

St. Michael's Hospital

Executive Review

Overview

Since St. Michael's Hospital joined the St. Vincent's Healthcare Group in 2001, the policy of developing and integrating the services and facilities of St. Michael's Hospital within the Healthcare Group has continued. In 2004, this trend was accelerated through Accreditation, where staff from the three hospitals within the group worked together on the Self Assessment process and Quality Improvement Plans.

In 2004, the hospital prepared and submitted a Service Plan in conjunction with St. Vincent's University Hospital to the Eastern Regional Health Authority. As in previous years the Service Plan detailed the hospital's expenditure, activity and service requirements for 2004. It also outlined the requirement for a capital-funding injection to upgrade the infrastructure in St. Michael's Hospital. Throughout 2004, a series of meetings where held with the ERHA both separately and as part of the St. Vincent's Healthcare Group, during which the issues detailed in the Service Plan where considered and discussed.

The Financial outturn for the year 2004 showed a small surplus. The factors, similar to that of 2003, which contributed to us maintaining expenditure within allocation, included once off adjustments in respect of debtors and stocks. When these adjustments are taken into consideration, the outturn reflects the difficult year that the hospital experienced in coping with the growing demands for services.

During periods of the year the SRSV outbreak was a contributing influencing factor on both activity and finance, leading to loss of revenue and increased costs, particularly in providing additional cleaning to the patient care areas.

Capital Expenditure received in 2004 was spent on essential maintenance, repairs, painting and commencing work on the link corridor between the Public Hospital and the former St. Michael's Private Hospital building. This corridor will enable the hospital to re-locate services and facilitate appropriate access and ease of patient transfer between the two buildings. It will also enable the hospital to consider future developments in relation to utilisation of space.

Demand for A&E services during the revised hours continued to increase, as did the demand for the GP Co-operative Service (DL Doc) and the GP Physiotherapy referral service. The GP Physiotherapy Service was provided in conjunction with the East Coast Area Health Board.

As mentioned previously, St. Michael's as part of the St. Vincent's Healthcare Group, participated in the Accreditation process of the Irish Health Service Accreditation Board. The survey took place in May 2004 and we received our award in January 2005. St. Michael's was strongly represented across all the Clinical and Clinical Support teams and on the Steering and Co-ordination groups. I would like to thank Ann Brennan, who undertook tremendous work at local level in St. Michael's Hospital in respect of the Accreditation process and all the other St. Michael's Hospital staff who participated at all levels.

Human Resources

2004 was another stable year for Human Resources as St. Michael's experienced relatively low staff turnover. We had a successful year in recruitment with high quality response levels to all advertised vacancies in addition to a regular flow of unsolicited applications. It was also a busy time for HR participating in the preparation to meet the standards of the Irish Health Services Accreditation Board and to co-ordinate the site visit at St. Michael's on behalf of SVHG. Further progress was made in integrating our HR policies and practices with our sister hospitals, SVUH and SVPH, as part of the SVHG. We also welcomed Nicola Vahey as the new Assistant to the HR Manager.

Other significant developments during the year included the introduction of the new Dignity at Work, Equal Opportunities and Diversity policies for the health service. We also commenced the introduction of Personal Development Planning (PDP) and all departmental and line managers are being offered training to carry out PDP with their staff. In addition, over 140 staff participated in professional and personal skills training courses and study during the year. The HR Forum for Department Heads and Line Managers met regularly throughout the year to share information and discuss issues of common interest relating to the management of human resources at St. Michael's.

2004 marked the retirement of Mr. John Doyle, as Portering Services Officer, after a long and distinguished career in St. Michael's Hospital. I take this opportunity to thank John for his contribution to St. Michael's Hospital, and to wish him a long happy and healthy retirement.






We welcome the appointment of Mr. Declan Mc Callig as the Portering Services Officer, and wish him well in his new position.

Ann Brennan our Quality Manager, who represents Ireland on the European Task Group, (on the harmonisation of Health Information) attended a meeting in Hoorn in the Netherlands during the year. At this meeting there was much discussion on the transition from the paper to the electronic record, and the desire to produce a common road map for Information Management in the years to come.

Nursing Administration

The Nursing Department in St. Michael's Hospital, including a number of specialist services, continues to provide high quality patient focused care in line with the mission of the Religious Sisters of Charity.

2004 saw some changes in the department, not least the departure of Ms. Angela Delahunt, who retired after many years of dedicated service to the hospital. Her service included a considerable commitment to many fundraising projects. In addition, on the completion of the Diploma in Nursing course, Ms. Anne Murphy, Tutor, also retired. We wish both of them and all nurses who resigned or retired during 2004 our best wishes.

In June, we welcomed Ms. Robyn Henderson as Assistant Director Of Nursing, who commenced work at the hospital and brought a wealth of expertise from her many years of nursing and organisational change management in New Zealand.

The Nursing Department also welcomed Claire Grant into the Practice Development Unit. The Unit continues to support and develop nursing services by evaluating practice throughout the hospital and encouraging nurses to improve their knowledge and skills in patient care. The unit worked closely with the Royal College of Surgeons to ensure that all students undertaking the Diploma in Nursing graduated at the completion of the course. Staff in the department continue to participate in the Preceptorship and Competency programme for BSc. Students in partnership with UCD.

Nursing Personnel

A significant achievement during the year was the establishment of a database of all nurses, the development of a generic template for nursing contracts and the regularising of many temporary nursing contracts. The nursing complement continues at 164 WTE and while the turnover rate of 9% for nurses is low, selection and recruitment of staff continued throughout the year. During the year 70 pre registration clinical placements and assessments for overseas nurses were offered and co-ordinated by Josephine Barrett.

New Developments

- Nurses throughout the hospital contributed to the Accreditation Process and welcomed the opportunity to self assess services and identify quality initiatives that could enhance the service that they provide.

- CNS, Mary Frances O'Driscoll, in conjunction with Dr. Tim McDonnell, established a nurse led respiratory service for established respiratory patients.

Hospital Representation

CNS, Anne Vaughan delivered two papers at national Cardiology Conferences: -

- *"Risk Factor Assessment and Compliance in Elderly Subjects Undergoing Cardiac Rehabilitation"* at the eighth World Congress of Cardiac Rehabilitation and Secondary Prevention in Dublin.

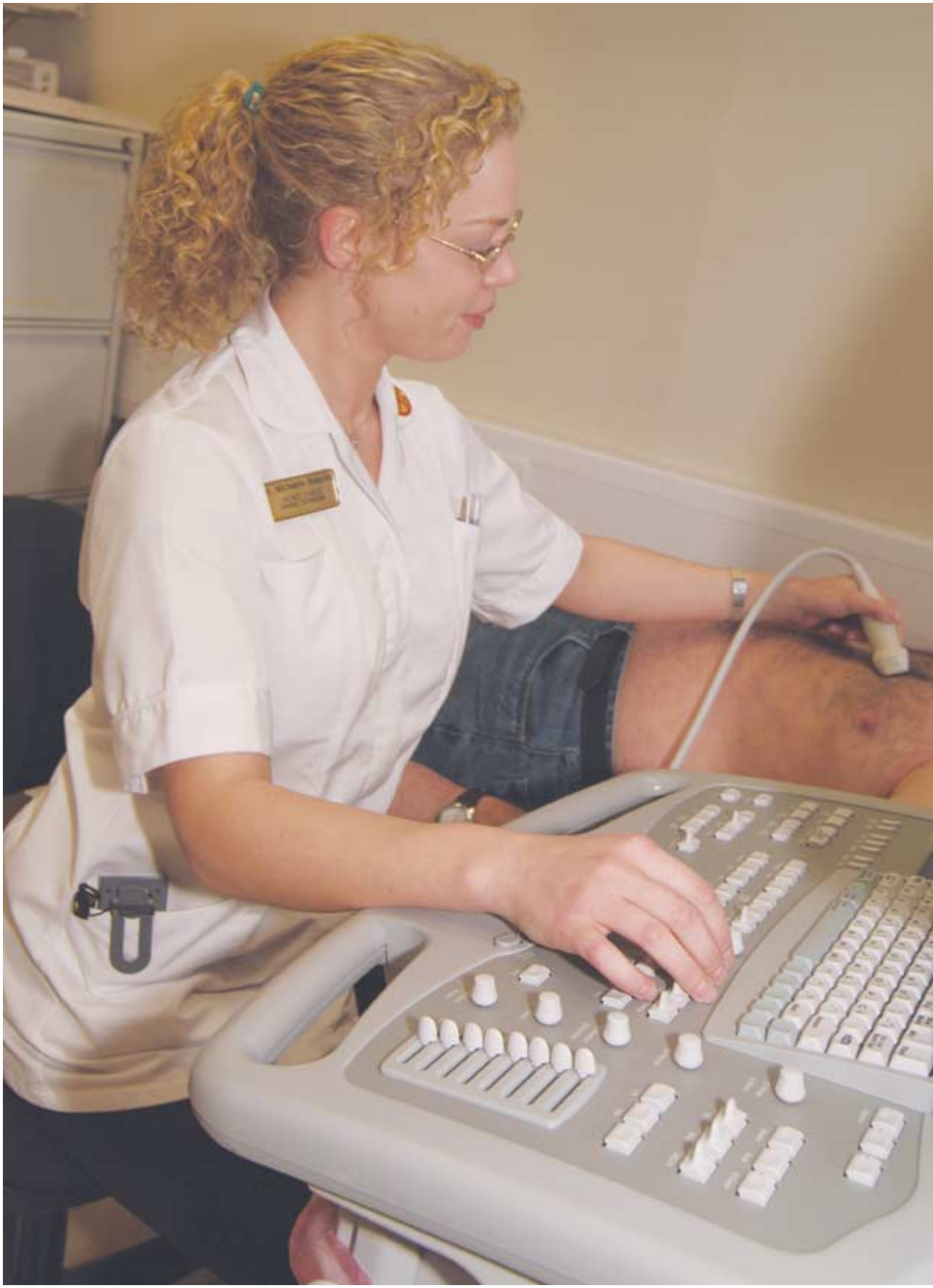
- *"Cardiac Rehabilitation for Elderly Subjects"* at the Irish Cardiac Society Annual Conference, Derry, Northern Ireland.

CNM, Margaret Dobbin presented a paper on the "History of Endoscopy" at the Irish Society of Endoscopy Nurses conference in Dublin Castle.

Student Nurse, Paula Bolger became the first nurse to represent the College of Surgeons at the Basketball Championships in Dubai.

Education

The department is committed to the ongoing education and development of nursing staff. In-service education continued and all Clinical Nurse Specialists delivered educational sessions and workshops throughout the year. A helpful contributor to the continuing education for nurses was the appointment of Ms. Breda Bennett, as librarian to the nurse's library. Ms. Bennett has been extremely helpful in cataloguing books, periodicals, and



documents in the library, as well as helping both students and registered nurses to source a wide variety of literature and publications.

Many newly graduated nurses continued with postgraduate education and started the one-year degree programme. Others commenced Higher Diploma Programmes'. Two senior nurses completed the two-year management programme in conjunction with St. Vincent's University Hospital and the National College of Ireland.

In an ever-increasing pace of change, development and increasing levels of specialisation, nurses in St. Michael's Hospital continue to be advocates for patients and strive to deliver efficient, effective, patient focused care at every opportunity.

Pharmacy Department

The Pharmacy Department has extended many of the initiatives established in 2003 and has further developed its service. It successfully participated in the Accreditation process throughout 2004. As a platform service, it documented pharmacy services and contributed information to many teams. More standard operating procedures (SOPs), guidelines and policies were developed in 2004. These included pharmacy SOPs, policies for administration of patients own drugs and self-administration of medication, and guidelines for supply of medication in A&E and other areas. Guidelines were also drawn up for the safe administration of high-risk medications in the hospital. An audit of antibiotic use was successfully completed and presented in 2004. Another audit is planned for 2005.

New storage facilities and procedures were introduced for hazardous materials. Approval was obtained for a system of continuous temperature monitoring of drug fridges (with remote alarm system) in conjunction with the Laboratory. It is hoped that this will be put in place in 2005. The Chief Pharmacist is involved with the Hospital Procurement Services Group in developing purchase agreements for dietary feeds and wound dressings. In addition, there have been initiatives re joint tenders between SVUH and SMH pharmacies. Considerable savings have already been achieved. The Chief Pharmacist in SMH now attends SVUH Drugs and Therapeutics (D+T) meetings, as well as SMH D&T meetings. The emergency drug trays in the hospital have been completely reviewed and updated by the Hospital Resuscitation Committee (with pharmacist input).

Plans for 2005 include greater documentation of service developments (e.g. poster presentations), computer training for staff and initiatives to address the issue of medication errors.

I.T. Department

The Integra Finance System was installed during the year. This resulted in a certain amount of reorganisation in a number of departments. Nevertheless, the system was implemented successfully with great co-operation all round.

Plans are in place to up-grade the link to St. Vincent's University Hospital. This will allow for greater communication between the two hospitals.

I.T. staff were kept busy expanding the network throughout the hospital with subsequent extra training needed in various areas. Cathal Prendergast left for a sabbatical in Australia and is expected back in early 2005. In July, we welcomed David Hogan to the I.T. Department.

Fundraising

In 2004, the Fundraising Committee continued to organise functions and raise funds on behalf of St. Michael's Hospital. I would like to thank the members of the Fundraising Committee and the volunteers who help run the St. Michael's Charity Shop. I would also like all those who attended fundraising functions or donated resources to the Hospital. A special thank you to the Chairperson of the Fundraising Committee, Ms. Ann Riordan.

Thanks

On behalf of the shareholders, the Board of Directors, management and patients, I would like to thank all staff in St. Michael's Hospital, who have shown a huge amount of loyalty, dedication and professionalism during 2004. I value deeply the contribution each staff member makes to meet the demands of both management and patients.

A special note of thanks to the various group and hospital committee members, medical and non-medical, for all the support in 2004.

I wish to thank the Group Chief Executive and the Executive of the ERHA who continued to give valued support to St. Michael's Hospital during the year.

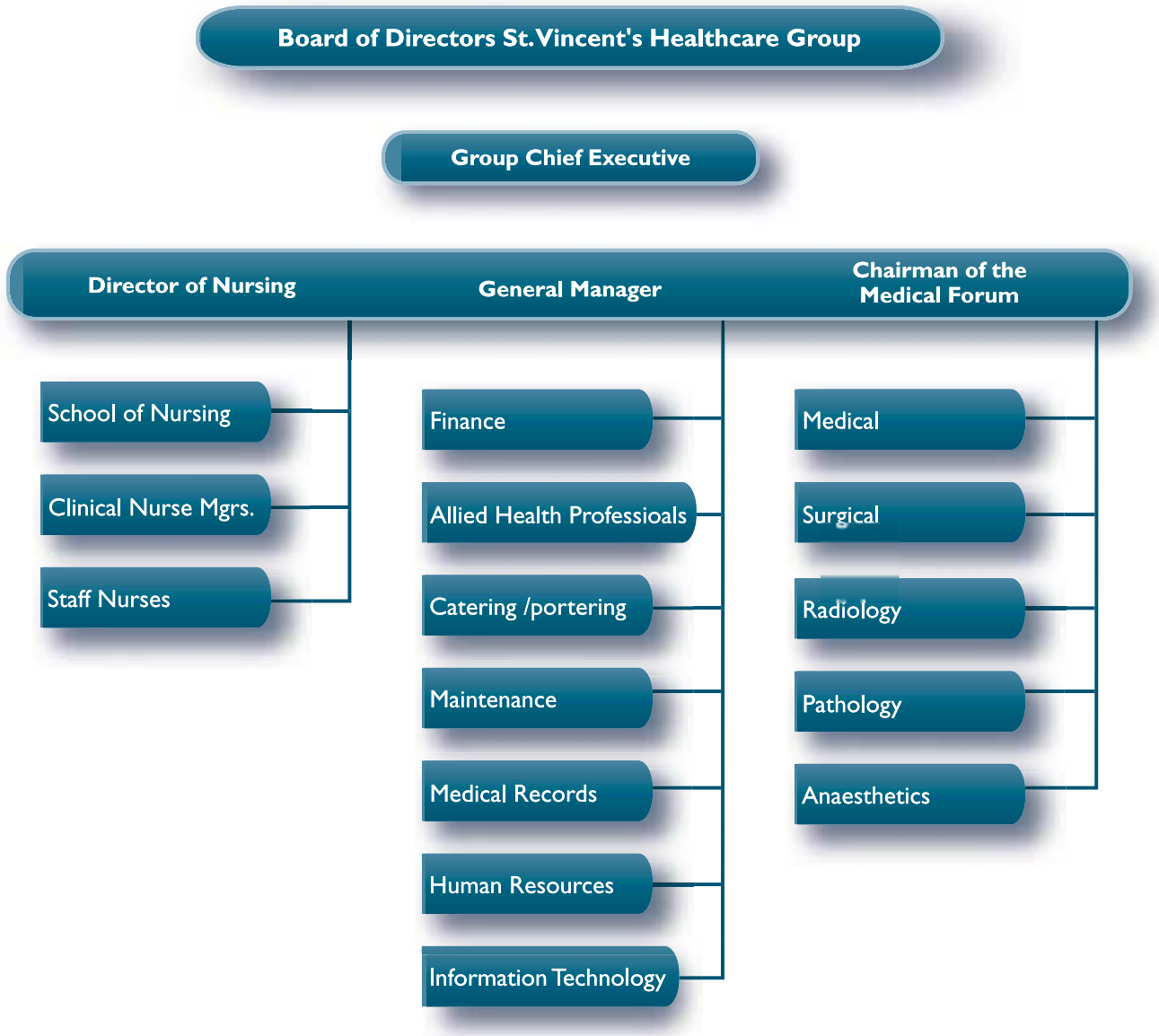
I would also like to acknowledge and thank the members of the Executive Council and the Medical Forum for their help and support during 2004.

Finally, all hospital staff performed exceptionally during 2004, and I wish to extend my personal thanks to each and every staff member in St. Michael's Hospital for their commitment to the hospital and to the delivery of care to the patients and their relatives.

Seamus Murtagh
Hospital Manager
St. Michael's Hospital

Organisation Structure

St. Michael's Hospital





St. Vincent's Private Hospital

St. Vincent's Private Hospital

Executive Review

Overview

St. Vincent's Private Hospital made progress during the year in negotiating improvements in the reimbursement arrangements for its services. Hospital turnover increased from €38.7m in 2003 to €43.4m in 2004 and a surplus of €812,000 was generated during the year compared to €110,000 in 2003. This was achieved through changes to the business model, which focused on a review of complex procedure costs, reimbursement and value for money initiatives. Our thanks are due to consultants and staff who contributed to the various improvements made during the year which are outlined in this report. The hospital currently has 164 inpatient beds, 36 day care spaces (including oncology), operating theatres for major and minor surgery, endoscopy, diagnostic imaging which includes general radiography, CT, ultrasound and MRI and comprehensive oncology and radiotherapy services.

Patient Activity

The total in-patient admissions were down marginally at 9,127 in 2004 compared with 9,397 in 2003, a reduction of 2.9% and average length of stay reduced from 5.31 to 5.15 days over the same period. Day cases, on the other hand, increased from 9716 in 2003 to 10,484 in 2004, representing an average increase of 7.9%. While the average in-patient occupancy level was 83.3%, occupancy was particularly low during the summer months and efforts are being made to improve occupancy during these months in 2005. Changes in case mix also had an impact on length of stay and occupancy levels. Theatre activity fell by 1.4%, although the endoscopy unit experienced an overall increase of 10.6% in the number of treatments provided. A summary of in-patient and out-patient activity is set out in the Corporate Services Division Report.

Consultant's Forum

The following consultants were nominated and elected to the forum during the year – Professor Diarmuid O'Donoghue, Mr. Enda McDermott, Dr. Diarmuid O'Shea, Mr. Donal Maguire and Dr. Karen Murphy.

Ms. Margaret O'Donnell, Dr. Donal O'Shea, Dr. Charles Gallagher, Dr. Tim McDonnell and Dr. Tom Crotty stepped down as members of the Forum. Their contribution to the work of the Forum is very much appreciated. The role of the Forum in providing advice and support on a range of matters including clinical policies and procedures, debtors days, clinical service developments is very much central to the work of the private hospital. Dr. J. Griffin and Mr. E. Kelly were nominated to the Hospital's Executive Committee. Professor D. O'Donoghue was appointed to the Best Practice Committee and Mr. S. Dudeney, Dr. T. Crotty and Dr. S. Skehan will continue to participate in the IT Strategy Committee.

The Consultant's Forum and the Finance Team continued to monitor debtor days, which dropped from 68 days in 2003 to 67 days in 2004. The admitting privileges protocol was revised and updated during 2004. Thanks are also due to Ms. Yvonne Farnan who provided secretarial support to the Forum during the year.

Management Team and Executive Committee

The Management Team and Executive Committee of the private hospital continued to meet throughout the year to review and monitor progress in relation to the budget and service plans.

Regular meetings were held with the Team Leaders/Heads of Departments during the year. At these meetings the Management Team provided updates on the hospital's clinical and financial performance and on relevant internal and external factors affecting the hospital.

The Management Team provided an induction programme for new staff to familiarise them with the hospital and its policies and procedures. They also produced a number of Newsletters to keep all staff updated on developments during the year. General Staff briefings will be provided in 2005.

Website

The hospital's website (www.svph.ie) continues to be updated regularly and now includes a distinct section for staff. The site is used to assist in Recruitment and Selection and also has a section covering questions and answers and news for patients. The site is linked to St. Vincent's University Hospital and provides a general description of the hospital services and insurance arrangements.

Staff Pension Fund

Pension Fund investments have generally dissimproved in the recent past and the hospital's own Fund is no exception. Following advice from our pension advisers, Mercer Human Resource Consulting, the hospital increased the employer contribution paid into the staff pension





fund. The reasons for this extra payment included a) the poor performance of investments generally b) increased future liabilities due to increases in pay and c) the projected number of staff due to retire over the next five years. A decision was also taken to introduce a new Defined Contribution Pension Scheme in 2005 for new staff.

Two staff/member Trustees were appointed during the year in respect of the Defined Benefit Pensions Scheme. Staff were encouraged also to plan for retirement and to give serious consideration to the payment of 'Additional Voluntary Contributions' in order to supplement retirement benefits.

Quality Improvement – Accreditation

Another milestone was reached in the Accreditation process with the submission of self-assessment documentation to the Irish Health Services Accreditation Board (IHSAB). Fourteen teams comprising staff from St. Vincent's University Hospital, St. Vincent's Private Hospital and St. Michael's Hospital prepared the documents and submitted same on behalf of St. Vincent's Healthcare Group.

The teams also prepared quality improvement plans to address areas for improvement in individual hospitals and on a Group basis.

The formal peer review by surveyors representing IHSAB took place during the week commencing 10th May 2004.

Smoking Regulations

Following the introduction of new legislation on smoking, a new 'No Smoking Policy' was developed for the hospital and an external smoking area was established in the car park opposite the main entrance. The main entrance to the hospital was re-designated as a no smoking area.

Risk Assessments

The hospital Safety Statement is an important document for the organisation as it allows us to identify hazards and outline the systems in place to eliminate or minimise risks such hazards pose to staff, patients or visitors.

In order to facilitate the update and continuing maintenance of our Safety Statement in line with legislative requirements, we updated the risk assessments for all areas of the hospital. Training was provided for the in-house risk assessors. The information gathered was made available to the Health & Safety Sub-Committee, which in turn was used to generate initiatives and policies to enhance work environments and practices.

Finance / HR Offices

On 2nd July 2004, staff from the Human Resources Division and the Creditors, Payroll, Patient Billing sections of the Finance Division relocated to new offices at the rear of the radiotherapy building. This move was necessitated by the expansion in the Radiotherapy Department.

Radiotherapy Project

The first of two new Linear Accelerators was delivered at the end of the August. Following installation and commissioning it is envisaged that the new equipment will be in use by the end of the first quarter in 2005.

Infection Control

Ms. Una Nicholson was selected for the position of Infection Control Clinical Nurse and is due to take up her appointment in early 2005. Una is currently Clinical Nurse Manager in TSSD having established the service, and has a keen interest in the Infection Control area. She will be responsible for Infection Control in St Vincent's Private Hospital and will work in conjunction with the Infection Control Team within St. Vincent's Healthcare Group.

Group Participation

Hospital representatives participate on the Group's Executive, Finance and Audit Committees and a number of other fora designed to harness synergies with the Healthcare Group. The hospital also supported the efforts of the Group's Internal Audit Department in improving standards generally. The hospital continues to support the Breast Screening Programme and other joint initiatives in conjunction with St. Vincent's University Hospital.

Michael Redmond,
Chief Executive, SVPH

Finance Division

Staff

The Finance Division comprises 21 staff. Ms Mary Kelly joined the department as Financial Accountant on 10th May 2004 and Ms. Phyllis McCarthy retired from the Debtors Department on 31st December 2004.

Service Activities

- Provide financial information to Management and Board of Directors.
- Ensure the hospital has sufficient cash flow to operate.
- Ensure that an adequate margin is generated over the services we provide.
- To submit prompt claims and collect cash from insurers.
- To pay our suppliers and our staff.
- To monitor and review all expenditure.

Achievements

- Net surplus generated of €812,000 from €110,000 in 2003.
- Debtors days at an average of 67 for year from 68 in 2003.
- Capital spend of €2.9m of which €1.8m is accounted for by the new Linear Accelerator in the Radiotherapy Department.
- Establishment of a Defined Contribution Pension Fund for new staff.
- New purchasing system introduced.

Future Plans

- To continue the growth in surplus achieved in 2004 with a goal of generating a surplus of €2m in 2006.
- To reduce debtors days to an average of 62 for year.
- To provide sufficient cash flow to fund a capital expenditure programme of €4m in 2005.
- Introduction of a new stock system.

James Crowe
Financial Controller

Nursing Division

The Nursing Division has made substantial progress in 2004 in the area of quality improvement by enabling personal and professional development and by focusing on new initiatives in service delivery. In tandem with efforts to achieve nursing objectives, a multidisciplinary team approach was taken to develop excellence in practice through the accreditation process.

The new ward names were introduced in February. The staff decided to name them after trees.

- 1st Floor, Redwood 25 bedded ward
- 1st Floor, Maple unit 15 bedded unit
- 2nd Floor Hawthorn ward remains 41 bedded ward
- 3rd floor, Rowan ward 25 bedded ward
- 3rd Floor, Elm 16 bedded unit
- 4th Floor. Cedar, 25 bedded ward
- 4th Floor, Cara - 15 bedded unit

Academic Achievements

Clare Jordan	Masters of Science Environmental Health Risk Management

Higher Diploma in Oncology Nursing

Suzanne Flynn	Elaine McNamara
Ellie Ryan	Eleanor Sheehy (Higher Diploma in Breast Care)
	Rachel Algar (High Diploma in Palliative Care)

Attending Higher Diploma of Oncology Nursing

Susan Cairney	Elaine Corcoran
Sharon Doyle	Grainne Griffin (Higher Diploma in Breast Care)

Elaine Farrelly	(Nursing Admin) – Diploma in First Line Management
Mary Hayden	Masters of Science - Nursing Education
Mary Connolly	Masters of Business in Health Services Management

Education & Training

Our active programme of events are organised by Nora Dwan, Education Officer and a professional development committee. She continues to assess new nursing staff and agency staff for their intravenous therapy skills. A two-day orientation programme for all new staff is facilitated on a regular basis. Nora continues to provide ongoing education in CPR for all staff.

Recruitment & Retention

Appointments in 2004

Assistant Director of Nursing: Barbara Murray,
Care Assistant Ciara Caffrey, Ramona Lupei, Stephanie Khelassi, Roisin Walker
Chaplaincy Elizabeth Coyle,
Clinical Nurse Manager I Paula Ryan, Bernadette Brooks
Clinical Nurse Manager II Doreen Martin
Portering Paul Stone, Patrick Cunningham
Staff Nurse: 40 Staff Nurses recruited
Theatre Attendant David Ormonde

Critical to service provision was the threat of a shortage of nurses in 2005. Much energy was expended in the recruitment of staff nurses with effects culminating in the very successful open day on September 25th 2004.

The Clinical Nurse Managers prepared ward profiles of their respective areas. Interviews were scheduled for people interested in coming to work in the hospital. The presentation and information packs used to promote the profile of the hospital will be used for orientation of future staff.

Resignations

Fr. Frank Downes-Chaplaincy Department
Ms. Mary Redmond - Chaplaincy Dept.
24 Staff Nurses resigned, many relocating to country hospitals.

Mission Effectiveness Committee

Ms Gretta Colbert, Chair
Ms Margaret Barry, Mr Eddie Hartland, Ms Alice Cromien, Vincent Lane, Orla FitzGibbon, Deirdre Behan, Mary Donnelly, Sr. Sheila Wall, Ms Catherine Collins.

Mission Liaison appointees are Sr. Sheila Wall and Ms. Ann Cavey. Mission Effectiveness Programme – Unit 1 and Unit 2 were well attended despite staff shortages.

I would like to thank all staff for their commitment and dedication in providing a quality patient service.

Ms Gretta Colbert
Director of Nursing

Corporate Services Division

The Corporate Services Division encompasses a number of major elements of hospital activity: - Quality & Risk Management, Information Technology, Capital Development and Statistics. During 2004 progress and improvement took place in each of the areas.

Quality and Risk Management

Across St. Vincent's Healthcare Group, the Accreditation process was adopted as the platform for quality improvement. At St. Vincent's Private Hospital, the Corporate Services Division provided the central point for coordinating the Accreditation work of the staff and quality team members. Application to the Accreditation Board (IHSAB) had been made in 2003 and was followed by the establishment of quality improvement groups who commenced the self-assessment process. The self-assessment process was completed in February and submitted to IHSAB. During May a peer review survey of the organisation took place with seven surveyors from IHSAB. The survey included team meetings and visits to many of the clinical and non-clinical departments of the Hospital.

The Healthcare Group received the award of 'Pre Accreditation Advanced'. Quality teams have continued to operate post survey to progress the quality improvement plans identified during the self-assessment process.

The Best Practice Group, which was established in 2003, continued the development of quality and risk management at the Hospital in 2004. The Group also acts as an umbrella for the Health & Safety Committee, the Radiation Protection Committee and the Infection Control Group. Elements of the work programme of the Best Practice Group in 2004 included: -

- Introduction of Grading Process for incidents reported.
- Introduction of monthly risk management information report for divisional managers and staff.
- Development and approval of clinical policies.
- Progression of occupational health and infection control.

The Health and Safety Committee work in 2004 included the following items: -

- Introduction of No Smoking policy and legislation.
- Improvements in the infrastructure and operation of Chemotherapy Preparation in pharmacy.
- Risk assessment training programme and establishment of Risk register.
- Continuation of fire training programme for all staff.
- Upgrade and extension of Fire Alarm system.
- Audit of Health & Safety Manuals.
- Provision of flu vaccine for staff.
- Improvement in security procedures with ID badges and CCTV.
- Environment Awareness Week.

Information Technology

IT development in 2004 concentrated on expansion of the hospital network.

The priority identified was the coordination of the Finance function and all members of the Finance team were joined to the network. The patient billing system, payroll & HR systems, together with the 'Pegasus Opera' finance system have all been linked through the network. Work commenced to improve the efficiency of the systems by networking printers and developing the purchase ordering and stock control modules of the finance package. The finalisation of the Finance networking was achieved with the move of a major section of the Division to the new accommodation block adjacent to the Radiotherapy unit. Cabling and additional hardware were installed to link the unit with the central servers.

Plans were developed to expand the network to encompass the growth in the Radiotherapy Department. The IT development in Radiotherapy will include e-mail facilities and the ability to transfer images from the diagnostic imaging department for treatment planning.

The hospital continued its investment in hardware and software to enhance the security and reliability of the system. New UPS equipment was installed and further e-mail security software will be put in place in early 2005.





Capital Development

Capital development at the hospital is spread across all divisions. A number of 2004 projects are highlighted below.

- Commencement of 2 year Radiotherapy project to introduce new Linear Accelerators, CT simulation and Treatment Planning system.
- Introduction of a new Chemotherapy Preparation Room.
- Replacement of passenger lifts.
- Development of office accommodation block adjacent to the hospital.
- Car park system.
- Accommodation enhancements including Board Room and Meeting Rooms.
- Enhancement of the emergency lighting system.
- Replacement of water tanks.
- New clinical waste storage facilities.

Statistics

The collection of statistical information and presentation of reports for decision making was augmented with the introduction of an additional suite of reports. Details of activity in clinical areas such as the theatre, minor theatre and endoscopy were reported. In addition, Allied Health departments such as Radiotherapy, Diagnostic Imaging and Respiratory Laboratory reported details of both outpatient and inpatient throughput in each modality.

Peter D. Sheehan
Head of Corporate Services

Hospital Activity 2004

	Jan to December	Jan to December	Variance	Variance %
Patient Discharges				
Inpatients	9,397	9,127	(270)	-2.9%
Inpatient Occupancy	84.40%	79.30%	-5.1%	-6.0%
Inpatient Bed days	49,904	47,018	(2,886)	-5.8%
Av. Length of Stay(Days)	5.31	5.15	(0.16)	-3.0%
Daycases	5,298	5,317	19	0.4%
Oncology Daycases	4,418	5,127	709	16.0%
Operating Theatres				
SVPH Theatres	3,864	3,808	(56)	-1.4%
Minor Operating Theatre	3,155	2,967	(188)	-6.0%
Endoscopy unit	4,703	5,203	500	10.6%
Diagnostic Imaging				
MRI Scans	4,689	5,169	480	10.2%
CT Scans	5,011	5,914	903	18.0%
Ultrasound	4,985	5,346	361	7.2%
Mammography	1,962	2,163	201	10.2%
Other	11,651	10,732	(919)	-7.9%
Total	28,298	29,324	1,026	3.6%
Radiotherapy				
No of Treatments	11,531	11,594	63	0.5%
Cardiology				
No of Procedures	6,491	6,465	(26)	-0.4%
Respiratory Medicine				
No of Procedures	2,636	2,312	(324)	-12.3%
Pathology Tests				
No of Procedures	388,369	380,444	(7,925)	-2.0%



Allied Health Division

Overview

The Allied Health Division had a busy and challenging 2004. Departments continued to develop and work towards their annual action plans and this led to a number of service developments / improvements.

As part of the Accreditation survey, a number of Allied Health representatives took part in various Care Teams and the external surveyors visited Diagnostic Imaging and Pharmacy. Participation in the process promoted collaborative working with colleagues elsewhere within St. Vincent's Healthcare Group and facilitated a mechanism for all areas to review policies and identify opportunities for improvement.

From a practical perspective, departments have been working with the Support Services Department in the establishment of an Asset Register, containing details of clinical equipment that has been purchased and service contract arrangements.

During 2004, two divisional representatives sat on the hospital's Health & Safety Committee and assisted in events held during the year including the 'Environment Awareness Week' in March and the 'Bullying & Harassment Awareness Week' in November (see Corporate Services section for further information).

Staff also took part in mandatory Induction, Fire, Manual Handling and Bullying & Harassment Training.

A wider range of staff from across the division were identified and trained as risk assessors. This involved working with a range of departments to identify and manage risks to patients and staff.

Lack of space and IT infrastructure continue to impede the growth of some departments and their ability to meet the needs of inpatient and outpatient populations.

Cardiology

Staffing difficulties were experienced with a limited supply of qualified staff and this led to some operational shortages during the period. Staff retention remains a challenge given the limited range of services currently provided by the department.

However, activity remained broadly in line with 2003 levels (see Corporate Services Section). Some services such as Echo and Stress tests were effected by changes in demand from other parts of the group (in 2003, the department provided support to SVUH and SMH).

Investment in equipment included an additional holter monitor, two E.C.G. machines and an Echo probe.

Diagnostic Imaging

Diagnostic services continue to make-up a significant proportion of outpatient services provided by the hospital. Increased competition is emerging in this market with the introduction of new providers and the use of competitive tendering by healthcare insurers.

Despite this, overall activity is up on 2003 levels with significant increases in MRI and CT (see Corporate Services Section).

The main equipment purchased in the year was an EZEM PROTCO2L Colon Insufflator for CT Colonography. The department was also redecorated.

Extensive work was carried out in conjunction with the finance department on the development of a comprehensive outpatient price list for 2004.

Recruitment and retention remained a challenge and efforts turned to overseas to provide required locum cover. MRI provided work experience placements for UCD and Erasmus Overseas Students.

During the year, a number of staff obtained postgraduate qualifications in First Line Management, Magnetic Resonance Imaging (MRI) and Computed Tomography (CT).

Conferences attended included, The Faculty of Radiology Meeting, the Joint BMRI and MRI users meeting, the 5th International MDCT Seminar and the Radiological Society of North America Annual Meeting.





Dietetics

Dietetic services are offered to both outpatients and inpatients. 2004 saw an overall decrease in numbers of patients seen and this is attributed to the overall reduction in bed occupancy, the suspension of sleep studies and a notable change in the complexity of cases (and in particular TPN) referred to the department.

The department is a member of the Irish Nutrition and Dietetic Institute (INDI) and has participated in a range of study days, conferences and interest groups (oncology and haematology). Staff also attend the SVUH Nutrition Committee.

In addition to the review of policies and procedures as part of the Accreditation process, outpatient and inpatient information leaflets were developed.

Pharmacy

Demand of pharmacy services continued to increase in 2004, particularly with the growth in oncology services.

A new aseptic preparation area was developed in 2004 and has led to a significant improvement in the overall aseptic facilities. The purchase of made-up chemotherapy increased to meet demand and has enabled the department to remain flexible in providing services.

During the year, one of the pharmacists joined SVUH's Drugs and Therapeutics Committee and this has proven beneficial to the department.

Recruitment of staff continued to be a challenge for the department and the development of technician roles is continuing.

In conjunction with the Finance Department, a number of purchasing agreements were negotiated and new stock control procedures were introduced. A further development was the establishment of a process with VHI for the re-imbursement of new drugs to the market. To date this has proven to be successful with a number of new products now available to patients.

Physiotherapy

Services are provided to a range of inpatient and outpatient orthopaedic (sports injuries, back care, rehabilitation) and breast care (post operative, reconstruction, plastic surgery) and more recently, neurology patients.

Activity for the period is down on 2003 levels and this can largely be attributed to the inpatient occupancy.

Space continues to be a critical issue for the department.

One member of the department trained to become a non-patient Manual Handling Instructor and is now involved in providing mandatory training for staff. The head of the department also successfully completed the First Line Management course.

Radiotherapy
(including Medical Physics)

Activity for the period remained broadly in line with 2003 levels (see Corporate Services section) despite the breakdown of existing linear accelerator due to the age of the equipment and difficulties with obtaining parts.

In February 2004, the Group Board sanctioned the expansion of radiotherapy services. This project involved the acquisition of two linear accelerators, a CT simulator and a record and verification system. The contract was awarded in April and building works to accommodate phase one of the new equipment began in May. The first of the linear accelerators was delivered in August and commissioning began in November. It is anticipated that the first patients will be treated in March 2005. Phase two of the project will involve the installation of the CT simulator and the replacement of the existing linear accelerator.

New administrative and medical physics staff were employed as part of the expansion of services.

One member of the Radiotherapy Department trained as a patient manual handling instructor and is now involved in the provision of training to staff.

Respiratory

(Pulmonary Function and Sleep Lab)

The respiratory service was effected by staff shortages in 2004 and this led to the suspension of sleep study services for approximately six months. Overall activity was therefore down on 2003 levels (see Corporate Services section).

Recruitment was successful and the department welcomed three new members of staff.

Staff are members of the Association of Respiratory Technology & Physiology (ARTP), the Association of Respiratory Technicians in Ireland (ARTI), the European Respiratory Society (ERS), the Irish Institute of Clinical Measurement Science (IICMS) and the British Sleep Society (BSS).

A patient support group (ISAT) invited staff from the sleep laboratory to host a help desk at their AGM in May.

Social Work

The Social Work Department was set up in 2000 and provides services to all inpatient areas. Referrals continue to increase (464 new referrals in 2004) and this has put pressure on the limited resources available.

Patients and families involved with oncology services are linked into the Social Work Department via regular multi-disciplinary ward meetings. Areas identified include practical and occupational difficulties, counselling and family issues. There is also close liaison with Palliative Care services and follow-up of bereaved families where appropriate.

Vulnerable elderly patients form a large part of the social work caseload. Most patients return to their own homes after their hospital stay but may require increased domiciliary support to sustain this. Liaison takes place with Public Health Nurses and other community support agencies. Difficulties remain in suitably placing patients, usually elderly, who no longer require an acute hospital bed but are unable to cope at home.

Ms. Gerada Warnes
Allied Health Manager



Human Resources Division

Mission / Vision

"The Mission of the HR Division is to design and deliver innovative HR services in partnership with the hospital to ensure a progressive, equitable and challenging environment for staff, and a quality service for patients."

Our Vision is to lead the way in HR expertise, creating a unique environment for our people that will generate success for the hospital.

Our Values include:

- Being the guardian of fairness and equity
- Valuing all our staff
- Listening and responding appropriately
- Balancing people and business needs
- Learning from our successes and our mistakes
- Communicating intentions and expectations clearly
- Advising managers on how to manage performance fairly and firmly.

Developments to date in the HR Division.

The HR Division was set up in June of 2000 with the appointment of the HR manager. The formation of the Division was necessary to centralise all of the activities relating to the personnel function, and to ensure that the hospital complied with all employment legislation enacted over the last number of years. The activities that derive from this role are manpower planning; organisation development; recruitment and selection; staff development and training; industrial relations and implementation of policies and procedures.

The post of Assistant to the HR manager was created in December 2000, and Ms Marian Murphy is the first holder of the post. I would like to thank Marian for her continued commitment and support during the year.

Accreditation

The HR Division took part in the Healthcare Group application for accreditation with IHSAB as part of the HR team. A considerable amount of work was carried out with our colleagues from St Vincent's University Hospital and St Michael's Hospital under the leadership of Mr Noel Cassidy. This work is ongoing. A number of Private Hospital staff were involved as part of the HR team including Ms Orla Fitzgibbon (Assistant Director of Nursing), Ms Annette O'Neill (Radiography Services Manager), Ms Niamh Cross (Catering Officer) and Ms Marian Murphy (HR Division). Sincere thanks to all for the substantial contribution made.

Medical Records

The HR Division also has responsibility for Medical Records (including Floor Secretary staff), and Patient Focus. I would like to thank Ms Caitriona O'Connor, Ms Ann Marie Kavanagh, Ms Joanne Clarke, Ms Geraldine Pender, Mr Ciaran O'Callaghan, Ms Ann Pender, Ms Katie Chadwick-Thompson and Ms Ann Cavey for the consistently high level of service they provide to patients and staff.

On the initiative of Mr Michael Redmond (CEO), a new medical records system was introduced on the wards. A new chart folder has been developed and modifications have been made to the nurses stations on the wards to accommodate a new chart storage system. The changes are aimed at improving the quality of the medical records system.

Training & Development

Induction training was provided on a number of occasions during the year for new staff. Training in the areas of employment law, conflict resolution, disciplinary procedure and motivation was provided for supervisors in co-operation with the HR Department in the University Hospital. I would like to thank Ms Sorcha O'Quigley (Deputy HR Manager SVUH) for her assistance in providing places on training courses held in the University Hospital.

Support was given to a number of staff for courses of study in their own time. This support took the form of financial support and study leave.

In order to increase the awareness of 'Bullying and Harassment' in the workplace a number of seminars were held for Team Leaders and a briefing session was also held which was open to all staff. I would like to thank Mr Ian Maguire (HR Manager, St Michael's Hospital) for his support in arranging information material for these seminars. It is hoped to continue with this programme in 2005 with additional topics including 'Equality and Diversity' awareness training.

Seminars on Pensions and AVCs were also held for staff and delivered by representatives from Mercer and Irish Life.





Occupational Health Service

Discussions took place with St Vincent's University Hospital regarding the purchase of Occupational Health Services for Private Hospital staff. It is hoped to provide the main elements of this service early in 2005.

Recruitment, Selection & Retention

Recruitment of staff is carried out on an ongoing basis. Difficulties continue in sourcing a number of grades of staff including Radiographers, Doctors, Nurses, Pharmacy and Administration staff. The HR Division assisted other divisions with the selection of candidates for internal promotion vacancies.

The Hospital website was adapted to facilitate 'on-line' applications by applicants. This has proven to be very popular and successful.

Policies & Procedures

The existing number of HR Policies and Procedures was expanded to include 'Recruitment & Selection' and 'Data Protection'. Changes in legislation are anticipated during 2005, which will necessitate continuous updating of our policies.

Staff Benefits

A number of staff benefits were put in place with 'The Mortgage Store' and discussions took place with 'AXA Insurance' with a view to introducing a staff scheme in 2005.

Employee Relations

Discussions on various issues took place with trade unions and staff representatives throughout 2004. A number of individual staff grievances were also resolved by agreement with the staff members concerned.

During the year we worked closely with our colleagues in the salaries office - Ms Dorothy Nolan and Ms Yvonne Casserly. I would like to thank them for their valued assistance.

Neil Twomey
Human Resource Manager

Support Services Division

During 2004 a restructuring of the Support Services Division took place. There are now two main areas, one led by Jennifer Calton that includes Reception, Household, Maintenance and Projects. Yvonne Byrne and Geraldine O'Nolan lead the second area, which incorporates Coffee Shop, Patient Catering, Main Kitchen and all food purchasing.

Reception

The reception has continued to evolve during 2004. The area now manages many important front of house operations. Some of the main ones are:

- 270 direct dial phone system, 30 incoming and 30 outgoing lines.
- All incoming and outgoing post.
- Managed car park system.
- Hospital fire register and main panel have both been updated to incorporate more areas of the hospital.

We have also reviewed and improved our security within Reception. We have installed the following systems:

- Visitor signing in and out book.
- Lone working register.
- Hospital key register.

Our head receptionist resigned in December and we will be recruiting a new staff member in 2005 to lead this team.

Household Services

Yvonne Gleeson is Household Services Officer with a team of 4 who have taken on many new challenges in 2004. This department provides assistance and support to all departments in the hospital.

In 2004, an inventory list was developed for all patient areas to enable household services to establish a refurbishment programme. The department now have 12 policies, 17 procedures and 27 forms that are used as records when completed which help us run a more efficient department.

The department deals directly with 29 service providers, some of the main ones are:

- Maybin Clean who provide the cleaning service in the hospital with 20 staff members working over 7 days
- Celtic Linen provide a laundry service throughout the hospital
- Noonans provide a window cleaning programme every four months

A hairdressing service is also provided

Maintenance Department

We have one full time maintenance operative, Eddie Hartland and nine part time operatives. We have a total of 66 external contractors who service and maintain equipment on a regular basis. Non-routine work and projects are organised separately by using reputable outside firms who complete the majority of our project work.

Projects

External contractors carry out projects. However, they also require assistance from our internal facilities staff in household and maintenance. Some of the projects completed last year include:

- Providing extra spaces and new signage in our car park.
- New directional signs are now in place.
- New storage container at the end of Bloomfield Avenue.
- Coillte carried out tree audit and de-crowning.
- Office enhancement in Radiotherapy.
- A new 1000 litre water tank was installed on the roof.
- Pharmacy upgrade.
- Refurbishment of patient bathrooms.
- New window curtains for a patient floor.
- Reorganisation of offices in Support Services.
- Ongoing painting programme.

Catering Department

The Catering Department provides patient and staff catering in line with our legal requirements under the ISO 9000:2000 and IS 340 HACCP Legislation.

Yvonne Byrne & Geraldine O'Nolan are the Catering Officers and there is a staff of 67 in the department.

Service Developments/ Activities

On the 3rd May 2004 we implemented our new menu. This concluded a project that was researched with involvement from our patients and our catering team as part of the patients review committee. We have a





comprehensive menu to meet our patient's needs and requirements. The new menu also caters for light meal options. We are continuously improving our service by carrying out surveys and documenting feedback from patients.

Significant Achievements

Internal auditing – 10 audits were carried out last year by Catering Officers.

External Audits

In June 2004 the Coffee Shop was re-certified with the Happy Heart at Work Healthy Eating Symbol. In September 2004, Cater Care Limited carried out a Food Safety Audit.

The Catering Department was audited twice by CVA International who has awarded the Catering Department with ISO 9000:2000 & IS 343.

Departmental Statistics

The Catering Department provides service to 164 patients, daycare & 450 staff daily. The department also caters for in house meetings and functions throughout the year. Our coffee shop seats 105 and is open from 7.00 a.m. to 5.00 p.m. daily providing service to both staff and visitors in a busy environment.

Minor Capital Developments/Improvements

A new blast chiller was purchased to ensure cold food is below 5^{0}c.
All departments are now approving invoices before payment is made.
The coffee shop purchased a new dishwasher and a double fridge.

General Support Services

Training

A training plan is devised annually in conjunction with mandatory training such as refresher food hygiene, manual handling and fire training. The following are courses attended by staff in the Catering Department in 2004.

Lorraine O'Neill completed her 2-year Day Release Course in professional cooking.

Marie Jordan completed the Manger of Food Hygiene Course.

Maxine Doran completed Trainers in Industry.

Denise Ryan completed her Supervisory Management Programme.

Marie Jordan and Helen Dowd attended a Bullying & Harassment Course with IBEC.

Five staff attended a Bullying & Harassment course in-house.

Maxine Doran attended an Introduction to Supervisory cover with Failte Ireland.

Yvonne Byrne attended a Performance Management Appraisal Skills Course with IBEC.

Geraldine O'Nolan attended an Introduction to Employment Law IR Procedures.

Lorraine O'Neill completed Apprentice Chef Course.

Staffing

Leona Dowd was appointed as Administration Assistant to the Catering Department.

Stephanie Watson was appointed as Administration Assistant to the Facilities/Maintenance Department.

Michelle Pounch was appointed Senior Chef in August.

Barbara McMullen was appointed as Catering Supervisor.

Kay Marks and Jane McFarlane both retired in February after 15 years service.

Future Plans

Continue to work on the quality improvement plans identified in our accreditation process.

Review on site staffing levels in our maintenance department.

Monitor maintenance reporting and prioritisation.

Complete hospital asset register.

Continue to work on the recommendations of our fire management programme.

Review the catering service to our patients in the Day-Care and Oncology Units.

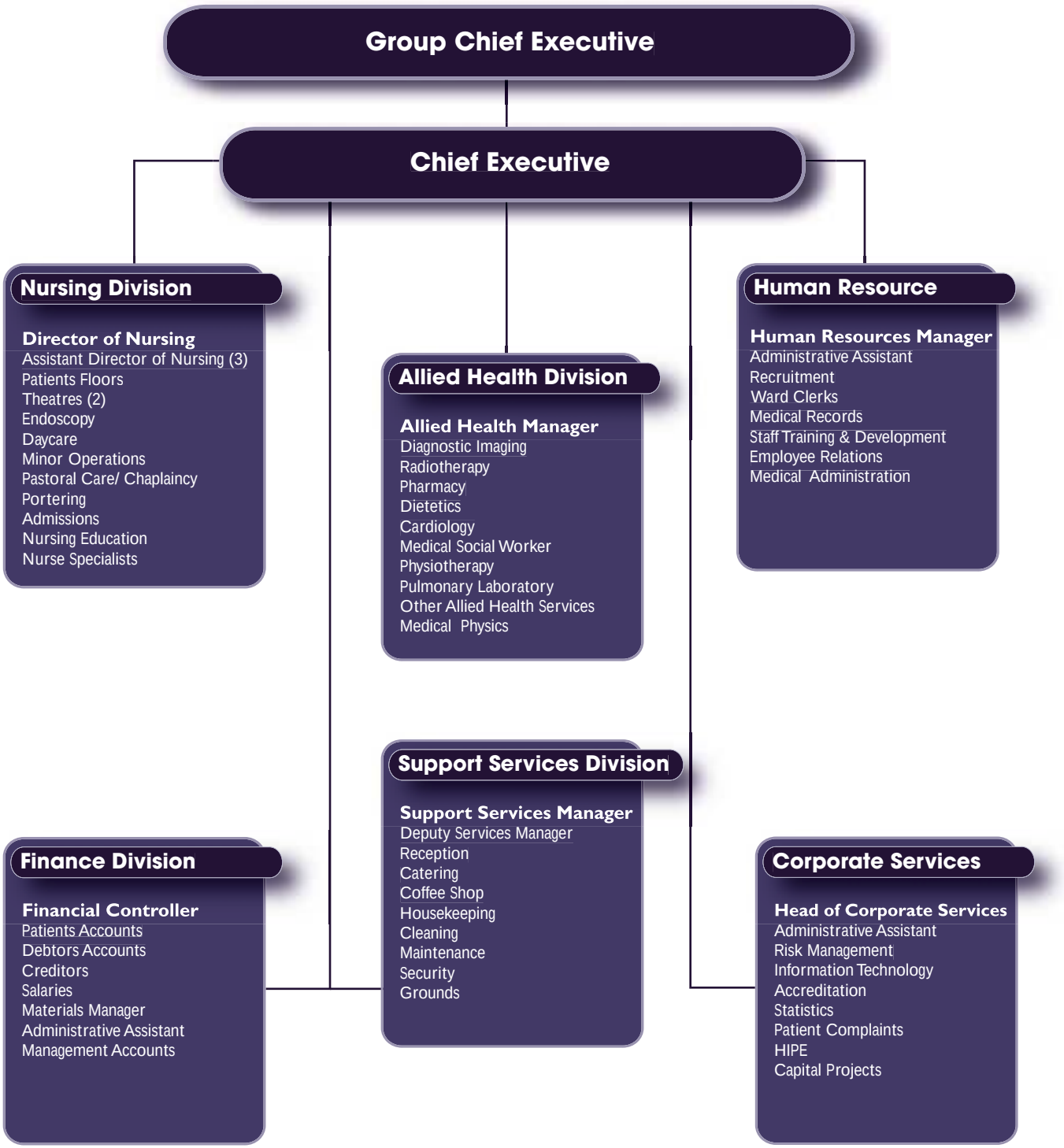
To revise our HACCP system in light of pending new legislation ISO 22000.

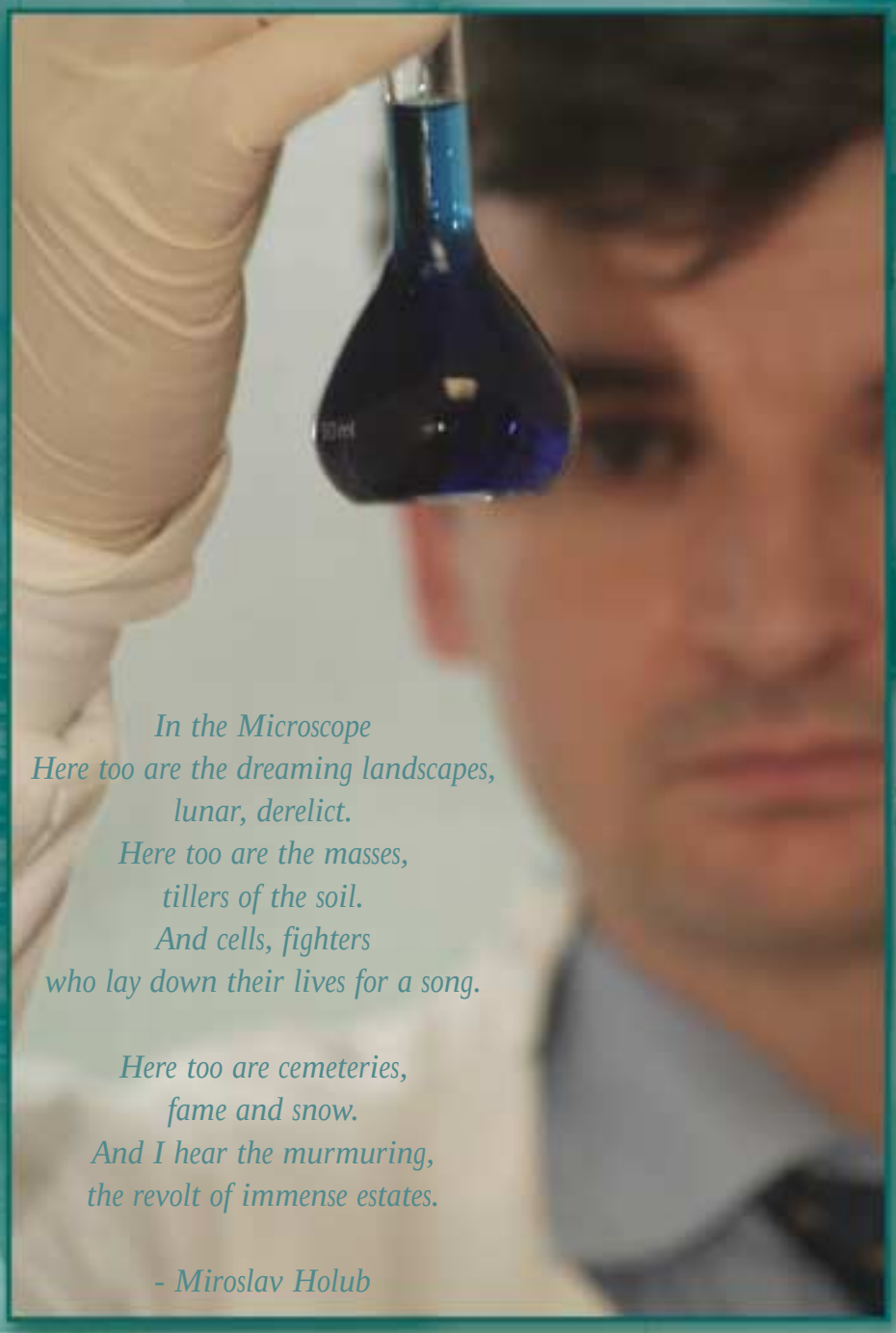
To carry out customer surveys to ascertain our customer needs.

Janet Murray
Support Services Manager

St. Vincent's Private Hospital

Organisation Structure





In the Microscope
Here too are the dreaming landscapes,
lunar, derelict.
Here too are the masses,
tillers of the soil.
And cells, fighters
who lay down their lives for a song.

Here too are cemeteries,
fame and snow.
And I hear the murmuring,
the revolt of immense estates.

- Miroslav Holub

Education & Research Centre

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Review by Medical Director
Prof. Oliver FitzGerald

Education & Research Centre

Review by Medical Director, Education & Research Centre, Prof. Oliver FitzGerald

Under the direction of 29 SVUH medical consultants and senior scientific staff, the ERC continues to thrive. In 2004 alone, grants worth over €3 million were awarded by a number of different agencies including the HRB, SFI, the Department of Agriculture, Enterprise Ireland, the EU and a number of pharmaceutical companies. More than 45 researchers access the facilities in the ERC and 28 researchers are registered or are in the process of registering for higher degrees. As a result of all these activities, there have been a number of significant publications in 2004 with the total number of publications exceeding 70. The highlight of the SVUH research year was the ERC symposium in November, which attracted both international and national speakers and allowed ERC researches to showcase their work.

The major challenge going forward relates to the lack of adequate space for all of the ERC related activities. Space is urgently required for offices, write-up and computer space, and specimen storage, in addition to core facilities. It is clear that the issue relating to space will need to be urgently addressed in 2005.

2004 saw the publication of the Research Strategy Report commissioned by the St Vincent's University Hospital Board of Directors and undertaken by Prof. Patrick Fottrell and Mr. Leo Kearns. The report has clearly set out a direction for SVUH research development going forward over the next 5-10 years. A key recommendation is the setting up of a Research Advisory Board (RAB), which is shortly to be established. The RAB will have overall responsibility for research at SVUH and the board will for the first time bring together the interests of both SVUH and UCD at a very high level. This development is warmly welcomed by all of the research community at SVUH.

No doubt with the establishment of the RAB and with the development of the Genome Resource Unit (GRU), 2005 will herald an era of progress and of change. I would like to thank the members of the ERC management grouping for their ongoing dedication and support. I would also like to acknowledge and thank the research administration team, which is ably directed by Ger Lanigan Ryan.

Introduction

Education & Research Centre

St.Vincent's University Hospital (SVUH) is renowned in Ireland for the excellence of clinical care given to its patients. decades, this excellence has been supported by a commitment to research. In more recent years, SVUH has extended this reputation with the development of the Education and Research Centre as one of its key initiatives for increasing the knowledge base in the hospital and improving patient care. With dedicated laboratory and clinical research facilities at the ERC, research programmes have developed which complement the primary clinical, diagnostic and treatment challenges faced by healthcare workers at SVUH. In particular, clinical and basic research into arthritis, liver disease and breast cancer are now integral components of Ireland's premier Early Arthritis Clinic, the National Liver Transplant Programme and the National Breast Cancer Unit which are all based at SVUH. A major clinical and basic research initiative in Respiratory Medicine over the years has also resulted in the development of expertise in integrative biology, respiratory physiology and sleep disorders as well as a nationally recognised expertise in the management and research of Cystic Fibrosis. Continuous clinical research over two decades in Endocrinology has made significant contributions to clinical management of Diabetes and Infertility at SVUH. More recently, an ambitious combined clinical and basic research programme in Colorectal Disease is contributing to the emerging research profile of SVUH.

SVUH researchers from clinical and scientific backgrounds collaborate with clinicians and researchers from leading national and international institutes to address major questions in the diseases that afflict the Irish population including rheumatoid arthritis, hepatitis C, cancer and diabetes. A common theme of the research programmes is the investigation of molecular mechanisms of inflammatory and malignant disease with a particular focus on gene expression and its regulation. Identification of specific changes in gene expression will lead to the development of novel diagnostic and prognostic applications, insight in to failure of current therapeutic strategies as well as identification of new therapeutic targets for arthritis, liver disease and malignancy.

A strong feature of the ERC research is the collection, over many years, of tissue samples from well-defined cohorts of patients and from normal donors. The clinical data on these cohorts is collated and stored in databases, constituting a valuable research resource. The combination of these clinical data with state-of-the-art molecular techniques for the analysis of disease markers and mechanisms places the E.R.C. in a position to make singular, clinically relevant contributions to biomedical research that will have implications worldwide.

A major development was the confirmation of funding to proceed with the development of a Genome Resource Unit (G.R.U.) based beside the E.R.C.The G.R.U., which is linked to the Dublin Molecular Medicine Centre (D.M.M.C.), will allow for the co-ordinated clinical and genetic phenotyping of large cohorts of patients attending S.V.U.H.This in turn will undoubtedly open up new avenues for treatment in conditions as divergent as prostate cancer and rheumatoid arthritis. With plans for development at an advanced stage, it is expected that the G.R.U. will be operational in 2005.



E.R.C. Principal Investigators 2004

Education & Research Centre

Principal Investigators 2004

Prof. Barry Bresnihan

Dr. Derek Doherty

Dr. Lynda Fenelon

Dr. Charles Gallagher

Prof. John Hegarty

Dr. Andrew Lloyd

Dr. Ken McDonald

Prof. T.J. McKenna

Prof. Walter McNicholas

Dr. Evelyn Murphy

Prof. Cliona O'Farrelly

Dr. Jacintha O'Sullivan

Dr. Tom Smith

Dr. James Tharappel

Dr. Doug Veale

Dr. Lucy Golden-Mason

Dr. Donal O'Shea

Prof. John Crown

Prof. Joe Duffy

Prof. Oliver FitzGerald

Mr. Justin Geoghegan

Mr. Arnold Hill

Dr. Donald McCarthy

Mr. Gerry McEntee

Prof. Paul McLoughlin

Mr. Hugh Mulcahy,

Prof. Diarmuid O'Donoghue

Prof. Niall O'Higgins

Dr. Kieran Sheahan

Prof. Oscar Traynor

Dr. Leonie Young

Prof. Kevin Malone

Dr. Ursula Fearon

Dr. Niall Tubridy

Research Laboratories – Personnel 2004

Education & Research Centre

Comhordaitheoir na Saotharlainne: Michelle Ipadeola

Senior Scientists & Post Doctoral Fellows

Jimmy Tharappel
Andrew Lloyd
Lucy Golden-Mason
Ursula Fearon
Margaret O'Brien
Jacintha O'Sullivan
Kirsty O'Brien
Louise Flanagan
Catherine Sweeney

Research Assistants

Susan Gaines
Martina Gogarty
Laura Greenan
Marie Elena Ruiz Zamora
Jessica McMahon
Sarah Cahalane
Emma McGrath
Miriam Tosetto
Shane O'Sullivan

M.D.

Trevor Markham
Ronan Mullen
Lorraine Power
Diarmuid Manning
Ronan Ryan
Terence Rooney
Alan Coss
Juliette Sheridan
Tom Cawood
Clare Matthews
Ceara Walsh
Deirdre Waterhouse
Eoin Judge
Wahlid-Al-Shehi

MCh.

Caitriona Canning
Mohammad R. Zaman

MSc.

Shane O'Sullivan
Mekdess Asamnew

PhD Students (Post Grads)

Rowan Higgs
Lydia Lynch
David Lynn
Patricia McGowan
Neil O'Brien
Jane Culleton
Brid Ryan
Susan Behan
Jonathan Dean
Tony Kenna
Lucille Kavanagh
Alison Beirne
Paul Cormican
Mary Connolly
Aoife O'Donovan

Under Graduate Students

Helena Grant
Ashling Quinn
Karina Kelly

Review by Director, Research Laboratories

Research at St.Vincent's University Hospital: People, Progress and the Future.

Prof. Cliona O'Farrelly,
Director Research Laboratories,

There are now 10-12 active Research Groups at SVUH availing of the research facilities at the Education and Research Centre and all have strong collaborative links with research groups on the UCD campus. Research areas complement the fields of clinical expertise that give SVUH its reputation for clinical excellence and include: -

Arthritis
Breast Cancer
Colorectal Disease
Liver disease, Transplantation and Hepatitis C
Mental Health
Endocrinology
Respiratory Physiology and Lung Disease

Research groups include several hospital consultants and senior scientists collaboratively involved in the planning, direction and implementation of research programmes that dovetail with ambitious clinical plans. Research teams also include pot-doctoral researchers and several research assistants, NCH doctors registered for higher degrees (MD, MCh or PhD), as well as science graduate students also registered for higher degrees. Several SVUH PIs (Principle Investigators) are core Conway Investigators, and promote extensive collaborations with researchers based at the Conway Institute on the UCD campus and use of the core technologies available there. Each group has large biobanks of DNA and other patient material with matched clinical data bases which, in addition to the clinical expertise of the researchers, are the key research resources of the hospital and will form the nucleus of the Genomic Research Unit planned to come on stream in 2005.

The availability of fully sequenced genomes which can be down loaded to computers in the Research Laboratories and in *silico* technologies for data analysis, together with the sophisticated technologies for studying gene expression and proteomics now available at the Conway Institute require, sophisticated levels of collaboration if the potential is to be harnessed and translated into better patient care.

Translational Research: The Future for Better Patient Care at SVUH

A successful future for biomedical research in SVUH and in Ireland will depend on co-operative collaboration and focus on excellence. Traditionally, SVUH research groups functioned as separate units. However the beneficial experiences of each group sharing core technologies, as practised in the Research Laboratories for over a decade and now at the Conway Institute, is stimulating appreciation for the advantages of integration of thinking, technologies and layers of expertise to develop comprehensive research programmes that are internationally competitive. The Annual Symposium which has had as its theme 'Common Molecular Mechanisms of Inflammation and Malignancy' for the past three years is symbolic of an emerging consensus.

The arrival of Prof Cecily Kelleher Professor of Preventative Medicine at UCD and SVUH provides a similar opportunity to build a collaborative philosophy that links Dept of Health aspirations into our Translational Research Programmes. Development can now happen in parallel with the emerging awareness of the influence of the mind on disease being brought to our attention by Prof Kevin Malone, Prof of Psychiatry. The arrival of Helen Colhoun the Professor of Medical Genetics and Epidemiology also adds a critical interlocking dimension to research programmes at SVUH and provides natural links and collaborative opportunities with other researchers in the Conway and the DMMC.

The Challenge for the Future

We are now at an important crossroads: if we are to develop improved patient care and SVUH competitiveness through translational research success, we require significant investment on several fronts, in particular in resources and collaborative efforts.

Research at the E.R.C

Education & Research Centre

The principal investigators at the ERC include physicians and scientists whose collaboration is key to the developing profile of cutting edge biomedical research. National and international reputations have developed as evidence by publications in international peer-reviewed journals, and by grants and awards.

Over the years ERC Principal Investigators have raised several million Euro in research funds, from governmental and charitable sources. New facilities for laboratory and clinical research have been designed. On a regular basis, new equipment is purchased, technologies introduced, and research staff recruited – critical for continued development of the ERC as a premier Biomedical Research Centre exploring key genetic, molecular, clinical and therapeutic features of some of the most important diseases suffered by adults in Ireland today.

Research Group

Research Group	Principal Investigator	Research Staff
Bioinformatics	Dr Andrew Lloyd Prof. Cliona O'Farrelly	David Lynn Paul Cormican
Respiratory/ Comparative Physiology	Dr Charles Gallagher	Sinead Barry Deirdre Waterhouse Eoin Judge
Sleep	Prof. Walter McNicholas	Dr. Silke Ryan Dr. Liam Doherty Dr. Ronan Ryan
Endocrinology	Prof. T.J. McKenna Dr. Donal O'Shea Dr Leonie Young Dr Tom Smith	Lucille Kavanagh Tom Cawood
Innate Immunity	Prof. Cliona O'Farrelly Dr James Tharappel Dr Andrew Lloyd	Susan Gaines Rowan Higgs Sarah Cahalane
Liver Research /Hepatitis C	Prof. John Hegarty Prof. Cliona O'Farrelly Mr. Justin Geoghegan Mr. Oscar Traynor Mr. Gerry McEntee Prof. Aidan McCormick	Dr Lucy Golden-Mason Susan Behan Tony Kenna Dr Conor O'Brien Dr Lorraine Power Dr. Diarmuid Manning Dr. Caitriona Canning Dr. Margaret O'Brien Alison Beirne Marie Elena Ruiz-Zamora Dr. Mohammad R. Zaman Jessica McMahon Emma McGrath
Haematology	Prof. Cliona O'Farrelly Dr Donald McCarthy	Dr Lucy Golden-Mason Jonathan Dean
Gastrointestinal Research & Colon Cancer	Prof. Diarmuid O'Donoghue Dr. Kieran Sheahan Mr. John Hyland Dr. David Fennelly Prof. Cliona O'Farrelly Dr. Jacintha O'Sullivan	Shane O'Sullivan Lydia Lynch Miriam Tosetto Alan Coss Juliette Sheridan
Mental Health Research	Prof. Kevin Malone	Aoife O'Donovan
Reproductive Immunology	Dr. Colm O'Herlihy Prof. Cliona O'Farrelly	Dr. Lucy Golden-Mason Lydia Lynch
Rheumatology	Prof. Oliver FitzGerald Prof. Barry Bresnihan Dr Doug Veale Dr. Ursula Fearon	Dr Ronan Mullan Dr Wahlid-Al-Shehi v Laura Greenan Dr Terence Rooney Dr Trevor Markham Martina Gogarty Catherine Sweeney Mary Connolly Dr Ceara Walsh Dr. Clare Matthews
Breast Cancer Research	Prof. Joe Duffy Mr. Arnie Hill Prof. John Crown Prof. Niall O'Higgins Dr. Enda McDermott Dr. Kirsty O'Brien Dr. Louise Flanagan	Neil O'Brien Brid Ryan Jane Culleton Patricia McGowan

Research Activities

Education & Research Centre

Department of Surgery

Principal Investigators

Mr Arnold Hill, Professor Joe Duffy, Mr Enda McDermott, Dr. Leonie Young, Professor Niall O'Higgins

Research team 2004

Marie McIlroy, Post Doc Yvonne Buggy, Post-Doc.
Sinead Kelly, PhD student. Niamh Cosgrave, PhD student.
Eddie Myers, MD student. Dara Kavanagh, MCh student.
Ruth Pritchard, MCh student. May Cleary, MCh student.
Julie Watson, Research Assistant.

Intracellular signalling and transcriptional regulation in human breast cancer

The clinical use of an antiestrogen for the treatment of breast cancer was first reported by Cole et al in 1971, who described the potential use of tamoxifen (ICI 46,474). Since then tamoxifen has become the most widely prescribed anti-cancer drug in the world. However, while most estrogen receptor (ER) positive patients will initially respond to tamoxifen, approximately one third relapse within a period of 18 months. While initial response rates are high among ER-positive tumours, most breast cancers that acquire tamoxifen resistance do so while continuing to express functional ER. Despite extensive clinical use of tamoxifen, several aspects of its mechanism of action and acquired resistance remain unclear.

The ER is encoded for by two genes, ER- α and ER- β, both can function as transcription factors to modulate target gene expression. ER- α and ER-β interact with a number of nuclear proteins *in vitro*. These 'co-activator/co-repressor' proteins interact directly with steroid receptors at their response element in the promoter region of target genes to enhance or inhibit transcription. Co-activators such as SRC-1 are thought to facilitate the activity of the ER whereas co-repressors such as SMRT maintain transcriptional silence. We have described a significant association between expression of co-activators SRC-1 and AIB1 in human breast cancer. At a molecular level we have observed distinct transcription factor- co-regulatory protein interactions and protein DNA interactions in endocrine sensitive cells compared to those that are resistant, in response not only the steroid environment, but also growth factors.

Co-regulatory proteins interact with nuclear receptors, including ER, at a conserved LXXLL motif within the receptor interacting domain of the protein to drive target gene expression. Although the steroid co-regulatory proteins were previously thought to exclusively associate with nuclear receptors, we have identified MAP kinase effector transcription factors as novel co-activator protein targets. These new interacting partners may provide the basis for a new model of endocrine resistance.

We feel that co-regulatory proteins are pivotal to the development of endocrine resistance and may ultimately be responsible for the development of steroid independent tumours.

Tumour Biology Group

Principal Investigator

Professor Joe Duffy

Researchers

Dr Louise Flanagan,
Dr Kirsty O'Brien, Dr Neil O'Brien,
Brid Ryan, Brigid Browne,
Jane Culleton, Patricia McGowan

Collaborators (Local)

Professor John Crown,
Mr Arnold Hill, Mr Enda McDermott,
Professor Niall O'Higgins.

Collaborators (International)

Drs Denis Slamon/Gottfried Konecny,
University of California at Los Angeles;
Dr Stan Krajewski, Burnham Institute,
La Jolla; Prof Fred Sweep, Nijmegen;
Prof Nils Brunner, Copenhagen,
Prof Manfred Schmitt, Munich.

Research Focus

The overall aim of the Tumour Biology Group is to develop new molecular markers to aid the early diagnosis of breast cancer, help with prognosis and predict likely response or resistance to specific therapies.

Dr Louise Flanagan and Dr Neil O'Brien's research is focusing on mammaglobin, a recently identified gene that appears to be expressed almost specifically in breast tissue. Based on its almost exclusive expression in breast tissue, mammaglobin is currently one of the most promising new markers in breast cancer. Neil was also one of the first to show that mammaglobin exists in multiple molecular forms in breast cancer. These different forms of mammaglobin appear to contain different amounts of carbohydrate. Despite considerable efforts, Neil was unable to identify any factor capable of regulating mammaglobin expression in breast cancer cell lines. One

of the aims of Dr Flanagan's work is to identify a possible biological function for mammaglobin in breast cancer. This project involves collaboration with Professor T. Fleming, University of Washington, St Louis.

In breast tumour cell, mammaglobin exists in a complex with another protein known as lipophilin B. Jane Culleton has shown that while lipophilin B is preferentially expressed in breast tissue, limited expression also occurs in other steroid-dependent tissues. In breast cancers however, a significant correlation was found between mammaglobin and lipophilin B suggesting co-ordinate regulation of these genes. Currently, Jane is investigating lipophilin B at protein level using Western blotting, immunohistochemistry and tissue arrays. This last part of the project is being done in collaboration with Professor G. Landberg, Malmo, Sweden.

Aberrant gene methylation is now recognized as one of the key events in cancer formation. Specifically, methylation in the promoter regions of specific genes has been shown to result in gene silencing or shutting down of gene expression. Many of the genes silenced by methylation are known as tumor suppressor genes, ie, genes that prevent the formation of cancer. Failure to express these tumor suppressor genes is one of the critical events in the transformation of a normal cell into a malignant cell. Dr Kirsty O'Brien is investigating the potential of methylated genes to determine prognosis and predict response to therapy in breast cancer. This project is part of an EU Consortium involving 8 different collaborating institutions in Europe.

Brid Ryan's work involves investigating survivin which is both an inhibitor of apoptosis (or cell death) and a regulator of cell division. Although not specific for breast tissue, survivin is one of the most pan tumour-specific genes identified to-date. Brid has shown that survivin is present in almost all breast cancers but is rarely detected in normal breast tissue. Surprisingly, levels of survivin in breast cancer do not appear to correlate with rates of apoptosis but do correlate with rates of proliferation. These finding suggest that the primary function of survivin in breast cancer may be to control proliferation rather than inhibit apoptosis

Brigid Browne is researching the mode of action of the therapeutic antibody Herceptin, as well as Taxanes in breast cancer cells grown in culture. Brigid has shown that taxanes both inhibit proliferation and induce apoptosis in cell culture. During the year, Brigid spent some time working in the Laboratory of Prof Denis Slamon at the University of California, Los Angeles. In his post-doctoral position, Neil O'Brien is also working on Herceptin. The aim of Neil's project in to investigate why certain women with breast cancer fail to respond to Herceptin, despite overexpressing HER-2.

Patricia McGowan is working on TACE (ADAM17), a multidomain protein possessing both a protease and an adhesion domain. It is thus potentially involved in cancer progression. Patricia's early results suggest that the TACE is significantly upregulated in breast cancer and furthermore that TACE protein exists in multiple molecular forms in breast cancer. The clinical significance of these different forms will now be investigated.

Bioinformatics Group

Senior scientist	Andrew T. Lloyd PhD
Graduate students	David Lynn, M.Sc., Paul Cormican M.Sc.

Research

The principal research project, funded by the Department of Agriculture, continues to investigate innate immunity in chickens. We were delighted to see the delivery into the public domain of the completed genome of the chicken in March 2004. One aspect of our research is the discovery, by sensitive genomic searches, of previously unknown, unannotated genes. Another involves investigating the rate of sequence evolution in those genes involved in the control of the immune system. We have identified sites within mammalian defensin sequences that are undergoing positive selection. We are also involved a project investigating whether and how mammalian genes move about the genome.

Education and Outreach

The Bioinformatics Group continues to consults with and advise other research groups in the ERC. We also advised a prize-winning entry to the Young Scientists' Competition.

Innate Immunity Group

Report of the Chicken Antimicrobial Peptide Group

Rowan Higgs, Sarah Cahalane, David J. Lynn, Susan Gaines, Jessica McMahon, Tharappel James, Andrew T. Lloyd and Cliona O'Farrelly.

In silico and in *vivo* characterisation of β-defensin type novel antimicrobial peptides and their bactericidal activity against intestinal pathogens

There is concern that the extensive use of extraneous antibiotics may be causing increased resistance among pathogens. Furthermore, certain residual antibiotics in carcasses may have a negative effect on the consumer flora and fauna. However, there exist in most if not all organisms innate antimicrobial molecules [such as the antimicrobial peptides (AMPs) which are generally thought to act by forming pores in the membranes of microbes], which are essential components of primary and routine host defense. There is considerable

interest in the identification of novel innate antimicrobial agents perhaps as natural replacements and/or supplements for antibiotics. In the past three years we have pursued a combined bioinformatic/ molecular biological approach to identify antimicrobial peptides in chicken.This approach has been immensely helped by various whole genome sequencing initiatives to design and discover novel conserved sequence motives for AMPs. Thus, prior to the release of the draft sequence of the chicken genome, only four such AMPs were known in chicken. They were of the class β-defensins [epithelial cellular peptides with up to six cysteine amino acids (see Figure 1 below) orderly placed to form select S-S linkages] known as gallinacins (Gal-1 to Gal-3 and Gal 1) with homologous peptides found also in the turkey. These peptides are active against a range of Gram-positive and Gram-negative bacteria and the yeast Candida albicans (Gal-1 and Gal-1α). Subsequently, when 420,000 chicken expressed sequence tags (EST) became publicly available, we used bioinformatics algorithms to identify a further seven gallinacins (Gal-4 to Gal-10), a novel cathelicidin and a liver-expressed AMP (LEAP-2), and characterised their tissue-specific mRNA-expression pattern (reported previously). In 2004, the draft sequence of the chicken genome was released (http://www.genome.wustl.edu/projects/chicken/), which provided us with the opportunity to exhaustively search the whole-genome for every possible gallinacin-like sequence in chicken. Because of the presence of conserved motifs in β-defensins, it is possible to use hidden Markov model (HMM) profile searching to identify peptides with similar profiles (Fig. 1A). With this approach we identified two additional chicken AMPs, Gal-11 and Gal-12, their chromosomal location (Fig. 1B), investigated their mRNA expression in a panel of healthy chicken tissues (Fig. 2), examined the bactericidal activity of one of them (chemically synthesized Gal-11) and showed that it has a broad range antimicrobial activity (Fig. 3).

Personnel

David Lynn graduated with a PhD this year. After a successful viva in March, he went to work with Professor Bradley's group in TCD. This is the first purely bioinformatic molecular evolution PhD to graduate from UCD. His place has been taken by Paul Cormican, with a BSc in Pharmacology in UCD and an MSc in Bioinformatics from Coleraine. Susan Gaines, our invaluable research assistant, left the group for a post in UCD Vet School. Sarah Cahalane replaced her with effect from October 2004.

Invited talks

7th April 2004, David Lynn - "Novel Antimicrobial Peptides in the Chicken - A Bioinformatics Approach". Current Topics in Veterinary Science 58th Annual Conference, Scarborough, U.K.

May 2004, Andrew Lloyd - "Antenologs and syntenologs – genes lost and wandering". Irish Bioinformatics Meeting, Maynooth.

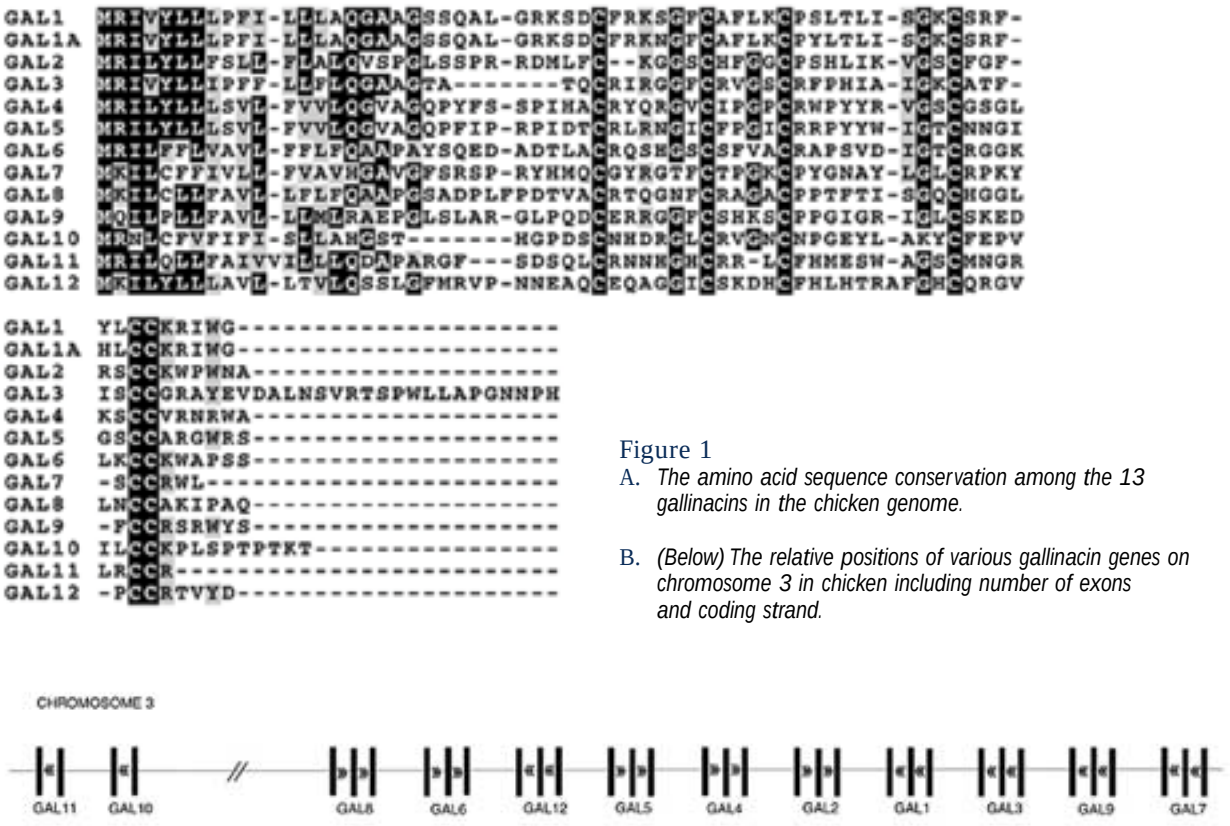


Figure 1

A. *The amino acid sequence conservation among the 13 gallinacins in the chicken genome.*

B. *(Below) The relative positions of various gallinacin genes on chromosome 3 in chicken including number of exons and coding strand.*

24th June 2004, David Lynn - "Selection in Bovine Immune Genes - Candidates for Disease Resistance?" Immunogenomics Workshop, Education & Research Centre, St.Vincent's University Hospital, Dublin 4, Ireland.

Posters

16th Sept 2004, The Chicken TLR Pathway. Irish Society for Immunology. Paul Cormican.

Liver Research Group

Cliona O'Farrelly, John Hegarty, Lucy Golden-Mason, Margaret O'Brien, Lorraine Power, Diarmuid Manning, Susan Behan, Catriona Canning, Alison Beirne, Zaman Badar, Jessica McMahon, Elena Ruiz, Niamh Nolan, Gerry McEntee, Oscar Traynor, Justin Geoghegan.

The Liver Research Group, led by Professors Cliona O'Farrelly and John Hegarty, has had a long-standing interest in the immunology of the normal and diseased adult human liver.The group has gained an international reputation in this area and members of the team are constantly in demand to present their work at National and International meetings. The group's activities cover many aspects of both basic and translational research including Hepatic Stem Cells; Hepatitis C Viral (HCV) Infection; Hepatic Malignancy; Liver Transplantation and Immuno-suppression. Important areas of research developed by the group over the past year include: the role of Nitric Oxide in donor organ injury and, the effect of the cytokine micro-environment of the liver on hepatic malignancy.

Nitric oxide in early ischaemia-reperfusion injury during human liver transplantation:

(Varadarajan *et al.,* Transplantation 2004, 78:250-6).

The condition of the donor organ contributes significantly to outcome post liver transplantation. Prior to transplantation the donor organ is subjected to a period of cold ischaemia during transport which can cause some damage to the organ. After implantation the liver is reperfused this may also cause some injury to the new liver. Characterisation of the pathways contributing to liver damage in humans will help identify specific therapeutic targets and new strategies to limit damage caused by ischaemia-reperfusion during liver transplantation. Altered nitric oxide (NO) metabolism has been shown to contribute to ischaemia-reperfusion injury in animal models. However, similar studies had not been performed to date in human liver transplantation.

In this study we examined nitrate, nitrite and nitrosothiols (NOx), NO synthases (*e*NOS and *i*NOS) and nitrotyrosine in early IR injury following human LT. Paired biopsies were obtained from nine donor livers before cold ischaemia (retrieval biopsy) and following reimplantation (reperfusion biopsy). Sections were graded for reperfusion injury using the Suzuki score. NO was detected by chemiluminescence after reduction of NOx. Expression of *e*NOS and *i*NOS was by Western Blot and RT-PCR and peroxynitrite by immuno-detection of 3-nitrotyrosine. Reperfusion biopsies showed histological evidence of injury (median Suzuki score: retrieval 2; reperfusion 6, p=0.008) and neutrophil infiltration. NOx was reduced following reperfusion, from 5.41µM/100mg (median, range 2.17-13.39µM) to 3.51µM (1.45-5.66µM, p=0.05). *e*NOS protein was reduced following reperfusion from 0.6 units (median, range 0.45-1 unit) in retrieval biopsies to 0.39 units in reperfusion biopsies (range 0.2-0.79 units, p=0.007, figure 1). We demonstrated increased nitrotyrosine formation following reperfusion, median 0.42 units (range 0.16-0.87 units) in retrieval biopsies and 0.68 (0.29-1.06 units) in reperfusion samples (p=0.007) and localised to peri-portal regions.

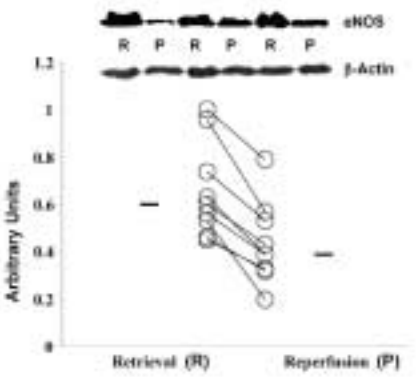


Figure 1: Reduction of eNOS protein in early reperfusion.

This study suggests that reduced NO bioavailability due to reduced eNOS may contribute to early ischaemia-reperfusion injury during human liver transplantation. We are currently investigating the expression of tissue repair genes in collaboration with Cormac Taylor (Conway Institute) which may prevent or limit organ damage.

Transplantation for Hepatitis C viral (HCV) infection:

(Golden-Mason *et al.,* Hepatology 2004, 40:163A).

Severe recurrent HCV infection post liver transplantation is fast becoming a major problem. Identification of predictive indicators of severe recurrent HCV in the allograft is critical to the management of these patients. In collaboration with Professor Hugo Rosen (Portland VA Medical Centre, Oregon, USA) we aim to unravel the immune mechanisms involved in the prevention or the promotion of

severe recurrence. Preliminary results from this study were presented at the plenary session of the American Association for the Study of Liver Disease (AASLD) annual meeting in Boston this year.

The innate immune response provides a critical first line of defence against viral infections through rapid and potent cytotoxic activity and production of inflammatory cytokines such as IFN-γ. Following liver transplantation for HCV infection, the allograft is acutely infected but the role of the innate immune response in this setting has not been defined. We hypothesized that the innate immune response pre- and early after liver transplantation would correlate with the severity of HCV recurrence. In order to evaluate the role of natural killer (NK) cells in the outcome of liver transplantation for HCV, we prospectively collected peripheral blood mononuclear cells (PBMC) pre-, 1 and 2 months post-liver transplantation from 10 HCV-positive recipients who developed divergent HCV recurrence (5 with mild HCV recurrence, 5 with severe HCV recurrence).

Among PBMC, the % of NK, NKT, and T cells and their phenotype was determined by flow cytometry. The production of IFN-γ and IL-2 was evaluated by intracellular cytokine staining. Natural cytotoxicity (lytic activity against the K562 cell line) and lymphokine activated killing (LAK; following stimulation with IL-2) were also assessed pre-transplantation. Prior to transplantation, the mild group demonstrated higher total $CD56^+$ populations and NK ($CD56^+CD3^-$) cells but not $CD3^+$ populations including NT cells ($CD56^+CD3^-$). However, the % of total $CD56^+$ and NK cells that produced IFN-γ following stimulation was significantly higher in the severe group prior to transplantation ($p<0.05$). The mild group showed enhanced natural cytotoxicity and LAK activity pre-transplantation. At one month post-transplantation, the trend in % of circulating NK cells was reversed, with lower $CD56^+$ populations in the mild group as compared to the severe group. In the severe group, NK cells were also more activated as evidenced by the higher expression of CD161. At two months post-transplantation, there was no difference in $CD56^+$ population in the mild vs. severe groups. In summary, pre-liver transplantation circulating NK cells are more frequent and demonstrate enhanced cytotoxicity in patients who ultimately develop mild HCV recurrence within their allografts. In contrast, after transplantation, higher frequencies of activated NK cells at one month correlate with the development of severe recurrence.

These data suggest dual roles for the innate immune response in HCV-infected liver transplant recipients, i.e., protection versus injury of the allograft, depending on the time point examined (figure 2).

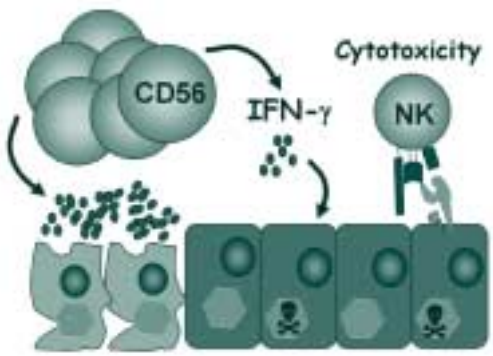


Figure 2: *CD56+ cells control HCV infection but over-activation may cause liver damage.*

Liver cytokine micro-environment in hepatic malignancy:

(Kelly *et al.*, Cytokine 2004, 25:273-82).

The adult human liver is constantly exposed to a large antigenic load which includes pathogens, toxins and tumour cells. Because of its location and rich blood supply, it is the primary site of metastases from colorectal carcinoma. Adult human liver is enriched with $CD8^+$ T- and $CD3^+CD56^+$ natural T- (NT) lymphocytes, important anti-tumour effectors. These cells display phenotypic and functional similarities to murine NKTs. IL-12 promotes the anti-tumour function of NKTs. We quantified IL-12 levels and CD56+/$CD8^+$ T lymphocytes in normal and tumour-bearing human liver. We also examined the effect of IL-12 on the expansion/activation of human peripheral blood cells (PBMCs) *in vitro* using flow cytometry (figure 3).

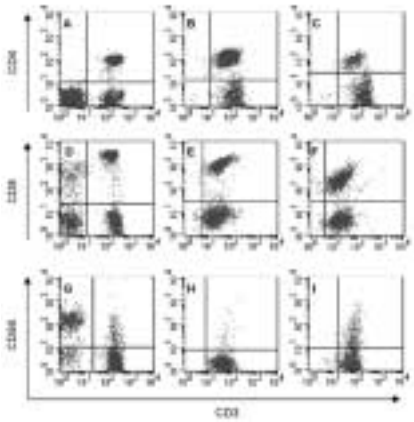


Figure 3: *Flow cytometric analysis of cells cultured in IL-12.*

Significant levels of IL-12 protein were detected in normal liver (n=13, median 2032pg/100 mg protein). IL-12 was increased in tumour-bearing liver (n=9, 3678pg, $p<0.01$). Infiltrating monocytes appear to be the principal producers of IL-12. Culture with IL-12 selectively expanded CD8+T cells and NT cells and polarised their cytokine responses to Th1 type. However, there was no in vivo expansion of these cells in tumour-bearing liver. The changes observed in culture required the

addition of IL-2. We therefore quantified IL-2 levels in hepatic tissue. IL-2 was detected in normal liver at a median concentration of 4700pg/100 mg protein. Surprisingly, there was no significant increase in levels in tumour-infiltrated liver.

The presence of IL-12 may create an environment in the healthy liver that promotes the accumulation of $CD8^+$ T and NT cells. Therefore, the development of metastases in the presence of high levels of IL-12 may be due to an insufficient IL-12 response or to a breakdown in the IL-12/ NT/CD8/IFN-γ pathway. Alternatively, lack of IL-2 rather than a defect in IL-12, may be responsible for insufficient expansion/ activation of tumour specific cytotoxic T lymphocytes. We are currently investigating the expression of a range of cytokines in hepatic malignancy which may prove to be useful therapeutic targets.

Haematology Group

Principal Investigators Dr. Donald McCarthy, Prof. Cliona O'Farrelly

Researchers Jonathan Dean, Dr. Lucy Golden-Mason

NKR^+ Cells in Human Bone Marrow

The involvement of the immune system in attacking cancer of various types has been attracting increasing interest over the last few years. The ability of immune cells, and T cells and Natural Killer (NK) cells in particular, to eliminate cancerous cells offers a number of routes to follow in the development of novel anti-cancer therapies. In particular, researchers have identified a subset of T cells that appear to be capable of not only attacking the cancer cells directly, but are also implicated in the initiation of anti-cancer immune reactions by other cell types. Identified on the basis of simultaneous cell-surface expression of protein marker molecules normally associated with either T cells (CD3) or NK cells (CD56), these "NKR-positive" (NKR+) T cells are present at a number of sites in the human body, and analogous cells can be detected in the mouse.

Local, specialised populations of immune cells, and T-, NK and NK receptor (NKR) positive T-cells in particular, reside within a number of organs, including the gut, liver, lung, skin and uterus. These are all tissues that experience site-specific immunological demands, being exposed to externally-derived (or "non-self") antigen molecules and/or harmful "self" antigens on an ongoing basis. The distinct compositions of immune cells found at these sites is believed to reflect these locally-specific immunological demands, the composition altering to meet the specific needs of the tissue. While, unlike these other organs, bone marrow is not directly exposed to foreign/quasi-foreign antigens, it is the main site of production of both red and white blood cells in the adult human body. With more than 10^{10} cells being produced every day, there is an increased risk of mutation and subsequent transformation into a cancerous/malignant state. It is, therefore, likely that local mechanisms exist for identifying and suppressing neoplastic cells before they can become established.

The focus of this group has primarily been to examine the bone marrow and ascertain whether it may represent an additional site of "regional immunity" in the adult human, with particular focus on $NKR^{+/-}$ T cells. Initial investigations showed that the composition of the marrow cell subsets was not directly comparable to any one of the other regional immune tissues thus far identified, instead sharing characteristics with several such organs, and perhaps reflecting the unique immunological needs of the marrow environment. Having established the existence of these cell populations, and knowing of their involvement in anti-cancer functions at other sites and in the bone marrow of the mouse, the investigation was extended to look at bone marrow from patients suffering from, or in remission following, haematological malignancies. Distinct differences in the makeup of the $NKR^{+/-}$ T cell and NK cell populations in donor (normal) and patient (malignant) bone marrow were, indeed, identified.

Furthermore, evidence from the study of mice, and from examining the human liver and gastrointestinal tract, suggests that the specific immunological needs of some organs may be met by a pool of immune cells that, rather than maturing else where in the body, as they are traditionally believed to do, arise and mature locally, within the organ itself. Therefore, our group has also been exploring the possibility that T cells and/or NK cells in human bone marrow may be developing locally and, while no direct evidence has yet been obtained, the presence of a number of T/NK cell maturation-specific molecules has been detected in immature cells isolated from healthy bone marrow.

In conjunction with the investigation of T/NK cell maturation in the marrow, we have also collaborated with the Gastrointestinal Research Group by performing an immunophenotypic characterisation of $CD34^+$ stem/progenitor cells within the bone marrow. This investigation has provided a background against which similar analysis of haematopoietic stem/progenitors in the gut can be compared. As part of this joint study, we have also established a collaboration with Miltenyi Biotec, who have provided us with a product sample to allow us to isolate progenitor cells on the basis of cell-surface expression of an alternative stem cell marker molecule, c-kit.

Communications

Oral presentation by Jonathan Dean: "$NKR^{+/-}$ T cell Subsets in Human Bone

Marrow: Implications for Cancer Immuno-surveillance" Irish Society for Immunology Annual Congress, September 2004

Poster presentation

"NKR$^{+/-}$ T cell Subsets in Human Bone Marrow: Implications for Cancer Immuno-surveillance" Haematology Association of Ireland Annual Meeting, November 2004

Awards

Jonathan Dean was awarded his Ph.D. degree for the thesis entitled "NKR+ Cells in Human Bone Marrow"

Respiratory Sleep Research Laboratory

Director	Prof. Walter McNicholas
Research Lecturer	Dr. Silke Ryan
Chief Technician	Ms. Geraldine Lawless

Summary of Research Programme

The Respiratory Sleep Research Laboratory at St. Vincent's University Hospital continues a varied clinical and basic science research programme that has been evolving over several years. Ongoing collaboration exists with the research group of Dr. Cormac Taylor in the Conway Institute at UCD and the Department of Electronic and Electrical Engineering (Dr. Conor Heneghan and Prof. Mark O'Malley), also in UCD, in addition to the Department of Physiology, R.C.S.I. (Dr. Aidan Bradford).

Together with Dr. Taylor we have developed a translational research programme that is examining the molecular consequences of intermittent hypoxia (IH), which is a hallmark of obstructive sleep apnoea (OSA), and particularly the role of IH in the pathogenesis of cardiovascular disorders in OSA. In studies of OSA patients and also a unique cell culture model of IH, we have demonstrated that IH selectively activates inflammatory over adaptive molecular pathways through the selective activation of the inflammatory transcription factor NFkB, whereas the adaptive factor HIF-1 is not activated by IH. This finding is very different from sustained hypoxia, which is associated with a brisk activation of HIF-1. NFκB has been shown to predispose to atherogenesis.

In collaboration with Dr. Heneghan, we have studied the ability to identify sleep apnoea from digital signal processing of the ECG signal, based on characteristic changes in the ECG that accompany apnoea. We are working towards the development of a simplified ambulatory diagnostic system for OSA based on detailed analysis of the ECG, oxygen saturation and related derived signals such as pulse transit time. In collaboration with Prof. O'Malley, we have developed new surface electrode configurations to record genioglossus EMG with a view to studying the role of skeletal muscle fatigue in the pathophysiology of OSA.

We have also undertaken several clinical studies that have been completed during 2004 comparing standard CPAP therapy for OSA with the more sophisticated auto-adjusting positive pressure system (APAP).

Presentations of original research were given by members of the research team at the annual meeting of the American Thoracic Society in Orlando in May and the annual meeting of the European Respiratory Society in September.

Centre for Colorectal Disease

Clinical & Research Personnel

Diarmuid O'Donoghue
Consultant Gastroenterologist
(Director of Centre for Colorectal Disease)

John Hyland	Consultant Surgeon
Kieran Sheahan	Consultant Pathologist
David Gibbons	Consultant Pathologist
Hugh Mulcahy	Consultant Gastroenterologist
David Fennelly	Consultant Oncologist
John Armstrong	Consultant Radiation Oncologist
Michael Moriarty	Consultant Radiation Oncologist
Denise Keegan	Genetic Nurse Specialist
Anne White	Cancer Nurse Coordinator
Nikki Smith	Nurse Specialist
Grace McEvoy	Nurse Specialist
Jacintha O'Sullivan	Senior Research Scientist
Dermot Leahy	Senior Research Scientist
Miriam Tosetto	Research Assistant
Juliette Sheridan	MD research fellow
Alan Coss	MD research fellow
Edward Fox	Ph.D student
Shane Sullivan	MSc. student
Robert Geragthy	Senior Medical Scientist

The Centre for Colorectal Disease is an umbrella for clinicians and diagnosticians with an interest in Colorectal Cancer, Inflammatory Bowel Disease and Functional bowel disorders. The aims of the centre are the delivery of state of the art services for our patients, research into causes and treatment and the education of the public.

The Centre for Colorectal disease has, for the past 25 years, had an active background in both basic gut immunology and clinical and basic colorectal cancer research and has been responsible for bringing on a new generation of basic science and clinical researchers. Higher degrees written up in 2004 include those of Sean Cochrane (MD) and Anne Marie Lennon (PhD). In addition, the arrival of Dr Jacintha O'Sullivan, Senior Scientist, has especially

helped push the research we perform to a higher plane. New immunological, molecular and cell culture techniques have been introduced to our scientific armamentarium allowing us to extend the scope of the research we perform. This has also been helped by additional research funding and we would like to acknowledge the generous donations for research and education from Mr Bob Joyce, the Anglo Irish Bank and the family and friends of the late Ellie Brady.

The scientific, clinical and financial changes to the Centre also allowed us to expand our personnel during 2004. A very necessary appointment was that of a Scientific Research Assistant in the Centre, Ms Miriam Tosetto. In addition, two new MD thesis students joined the Centre. Dr Juliette Sheridan is now conducting research into colorectal cancer in young patients and Dr Alan Coss is exploring the role of proteases in the progression and outcome of early colorectal cancer. As always, our collaborations with other research departments both within the UCD campus and farther afield have added considerable value to our research and we would like to thank Professor Cliona O'Farrelly and Professor Alan Baird, amongst others, for their continued collaborations.

Other highlights of 2004 included the election of Mr. John Hyland to President of the Coloproctology Association of Great Britain and Ireland. Additionally, the ever-popular International Colorectal Meeting was once again held before a large audience in the Education and Research centre in September. It was a particular pleasure to welcome Oscar Traynor as the state of art lecturer at our eleventh meeting

Overall Research Theme

According to the National Cancer Registry in Ireland, approximately 2,000 new Colorectal Cancer (CRC) cases are diagnosed each year. Survival rates for CRC improve with early detection, therefore efforts are needed to provide an "early warning system" for such individuals. The knowledge of cancer risk would profoundly affect the clinical management of such patients. Our research project summaries listed below are designed and conducted using our extensive colorectal cancer and inflammatory bowel disease databases and research findings are correlated with patient clinical and pathological information. The main research goals of our centre are as follows: -

- To achieve a more complete understanding of which biological factors drive the growth of colorectal cancer.
- To determine if the use of different biological markers can help distinguish people at high risk of this disease.
- To work out which subset of patients will respond to particular treatment regimes.

These translational research projects will be of major importance to the Irish population and worldwide resulting in early detection of people at high risk of colorectal cancer, early medical intervention and more effective treatment of the disease.

Invited Talks

Garrett Cullen:	Irish Society of Gastroenterology, November 2004
Jacintha O'Sullivan:	ERC Research Symposium, November 2004
	Colon Study Day, September 2004.
	Irish Society of Gastroenterology, June 2004.
Edward Fox:	Irish Society of Gastroenterology, June 2004.

Project Summaries

Project 1: Role of genomic instability in promoting CRC development in Ulcerative Colitis (UC)

The overall goal of this project is to determine if quantitative measurements of early genomic instability can stratify UC patients on the basis of cancer risk and thus allow for improved & focused medical management. There is a great need for the development of sensitive biomarker assays that can help stratify which UC patients are at greatest risk for colorectal cancer. Our sophisticated IBD electronic database established 25 years ago (1,500 cases) allows us to track the progress of UC patients through their initial and subsequent treatment offering an excellent resource of tissue material to conduct these longitudinal research studies. We are currently determining (1) if telomere dysfunction is the initiating factor in UC tumourigenesis (2) if chromosomal instability is a consequence of telomere shortening in UC (3) if the frequency of bridge breakage fusion cycles increases with neoplastic progression and if this is accompanied by telomerase activation. This study will allow us to get a better understanding if these biomarkers can be use to classify UC patients at greatest risk of UC. This study will ultimately increase our understanding of the molecular mechanisms involved allowing progression to malignancy in UC.

Project 2: Role of telomeres and telomerase in CRC development, progression & metastasis

Telomerase and telomere length is increasingly studied as prognostic markers in malignancy. Telomerase expression is repressed and re-activated in cancer allowing continued proliferation of tumour cells. Our extensive CRC database (over 2000 cases) allows us to closely examine the role of telomeres and telomerase in

colorectal adenocarcinoma. We currently grow primary cultures from tumours with different Dukes staging. These cultures will be valuable in understanding the basic biology of how telomerase re-activation fuels cell immortalization. Using these cultures, we will explore the possibility that telomerase up-regulation may be a function of increased tumour cell invasion and metastatic potential. Telomerase possesses an RNA template necessary to bind and extend telomere ends making, it an ideal target for inhibition by oligonucleotides. We will test the effect of telomerase inhibitors (2'-O-methyl-RNA) on telomerase expression, telomere lengths, rates of cell proliferation and decipher if telomerase inhibition causes immortal colon cancer cells to undergo apoptosis. It is thought that telomerase inhibitors will be most efficient when combined with conventional chemotherapeutics. We will investigate the effects of telomerase inhibition in the presence and absence of different chemotherapeutic drugs and angiogentic inhibitors in our primary culture systems. These studies will be pertinent in understanding the pathogenesis and treatment of CRC.

Project 3: Role of proteases in early Colorectal Cancer

This project involves screening over 600 CRC patient samples for expression of various protease expressions. These include Cathepsin B, L, D, H. Levels of MMP 2, 7, 9 and uPA are also been examined. These immunohistochemistry studies are been carried out using tissue microarrays whereby 4 cores per case is examined. In this project, we are also screening early colorectal neoplasms for gene amplifications and deletions in a panel of specific cancer related genes using degenerate oligonucleotide primed-polymerase chain reaction (DOP-PCR) based array comparative genomic hybridisation (array-CGH). We will correlate array-CGH, immunohistochemical and anatomical data with case evolution and long-term (up to 20 years) survival to identify biomolecular markers associated with clinical progression of early stage CRC.

Project 4: Microsatellite instability (MSI) and Methylation in familial & sporadic colorectal cancer

This project involves examining MSI and methylation of different mis-match repair genes in sporadic CRC and in HNPCC patients. MSI profiles have been assessed to date in over 110 sporadic CRC cases and in samples from 11 HNPCC families. Expression of mis-match repair proteins, MLH1, MSH2, MSH6 and MGMT are being screened and quantified in all the above cases. Preliminary results indicate that cases with high MSI demonstrate loss of expression of one of more mis-match repair proteins.

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Project 5: Investigate the biochemical, molecular, clinical & pathological changes occurring in colorectal cancer in young individuals compared to the older patient cohort.

Colorectal cancer in young patients (40 years of age or younger) are considered to have a worse prognosis than in older patients. Changes in certain genes may increase the risk of colorectal cancer in these young patients. We are currently examining the expression of reactive oxygen species, telomere lengths and markers of oxidative damage. These biological markers are hypothesised to drive the growth of cancer. We are also examining the expression of a large panel of tumour suppressors (genes that suppress tumour formation) and oncogenes (predisposes cell to tumour formation) and mis-match repair proteins. Data will be correlated with extensive clinical and pathological information. Stratification of the biology of tumours with age may reveal which aspects of tumour biology and tumour growth vary in different age groups. Also, the age-related factors that contribute to tumour growth could provide insights that could lead to tailored therapeutic approaches. Patients with strong family history will be stratified into a separate group and compared. Physcological grade of CRC patients will be assessed prospectively along with the immunological status of these patients.

Project 6: Role of Glut 1 & HIF-1 in early colorectal cancer progression

GLUT-1 is an intrinsic membrane protein responsible for basal glucose uptake. Cancer cells show increased glucose uptake and glycolytic metabolism compared to normal cells. GLUT-1 is upregulated in cancers, including colorectal cancer. To further elucidate the role of HIF-1 and Glut-1 expression in Dukes A and B colorectal carcinoma (CRC) and its prognostic significance, we are investigating the expression of HIF-1 and Glut-1 protein immunohistochemically using tissue microarrays. Preliminary results indicate a significant association between Glut-1 expression and pathological stage, peritoneal involvement, tumour margin, vascular invasion and neural invasion. In addition, preliminary data indicates that survival rates of patients with Dukes B tumours are significantly lower than Dukes A. We are further elucidating if the level of Glut-1 expression may be a useful marker, which can provide relevant information on tumour aggressiveness and metabolism in patients with Dukes A and B colorectal carcinoma.

Project 7: Pathological response following long course chemoradiotherapy for locally advanced rectal cancer.

Total mesorectal excision for rectal carcinoma has been augmented by the use of neoadjuvant chemoradiotherapy to reduce local recurrence for locally advanced disease (T3/T4). Between 1997 and 2003, 186 patients were diagnosed with rectal cancer from this unit, of whom 51 underwent total mesorectal excision following long course combined chemoradiotherapy. Tumour regression is

scored by 2 pathologists using a 5 point tumour regression grade (TRG). This grading system has now been revised. A stronger association between tumour regression and both local recurrence and cancer specific survival was found for the 3 point score compared to the 5 point score. For TRG 1 and 2, no local recurrences or cancer-related deaths have occurred. This tumour regression grade is a reproducible method of scoring tumour response to chemoradiotherapy. The modified 3 point score has the advantage of better reproducibility, while achieving greater prognostic significance. All tissue samples from this study are been screened for the expression of the following genes p53, COX2, VEGF and HIF1.

Department of Rheumatology

Staff

Consultant Staff	Professor Barry Bresnihan, Professor Oliver FitzGerald, Dr Douglas Veale
Senior Scientists	Dr Ursula Fearon
Post-Doctoral Scientists	Dr Catherine Sweeney
Graduate Students	Jennifer Ralph, Martina Gogarty, Mary Connolly
Research Assistants	Laura Greenan, Keith McQuillan, Catherine Sweeney
Clinical Research Fellows	Wahlid Al-Shehi, Trevor Markham, Julia Martin, Ronan Mullan, Bea Radovits, Terence Rooney, Ceara Walsh

Research Activities

The principle focus of research in the Department of Rheumatology is understanding mechanisms of inflammation and tissue damage in inflammatory arthritis, particularly rheumatoid arthritis (RA) and psoriatic arthritis (PsA). We have previously demonstrated that acute phase serum amyloid A is abundantly produced by inflamed synovial tissue. Ronan Mullan has extended this work and demonstrated that A-SAA enhances expression of adhesion molecules on rheumatoid synovial fibroblasts and on endothelial cells. Moreover, A-SAA enhances functional adhesion of PBMC to fibroblasts and endothelial cells. A-SAA promotes angiogenesis. Finally, A-SAA-induced adhesion molecule and matrix metalloproteinase expression was shown to be mediated through nuclear factor kappa B (NFκB) dependent pathways. These observations may be important as they suggest that targeting A-SAA, or its signalling pathways, may represent a realistic therapeutic approach in the treatment of inflammatory arthritis. Mary Connolly started a PhD in Oct 2004 and is funded by Science foundation Ireland. This project further examines the role of A-SAA as a key inflammatory mediator in RA. Specifically, this project examines the effects of A-SAA on cell migration, cartilage metabolism and the transcriptional pathways mediating this process. Mary's preliminary work has demonstrated that A-SAA stimulates chemokines in synovial cells and induces EC cell invasion and migration. In addition, preliminary results suggest that A-SAA alters cytoskeletal assembly/ disassembly with RA synoviocytes producing filopodia following stimulation with A-SAA. Finally, Mary is examining the effects of A-SAA on cartilage invasion and developing models of synovial tissue/ cartilage co-cultures.

Ursula Fearon's main research interest is the role of angiogenesis in driving the pro-inflammatory response, with a specific interest in dissecting out the pathways involved in blood vessel maturity and survival. Angiogenesis is new blood vessel (BV) formation by endothelial cell (EC) proliferation from sprouting capillaries. Specific interactions between Vascular endothelial growth factor and Angio poietins are thought to be critical to blood vessel structure and function, resulting in either new blood vessel formation, vessel stabilisation/maturity or vessel regression. We have previously described distinct vascular morphology which was associated with over-expression of Ang1/2 and VEGF in the inflamed SM, indicating that co-expression of VEGF and Ang2 is critical in the early stages of angiogenesis. In addition, we have shown a synergistic role for VEGF and Ang2 in promoting cell cycle turnover, migration and matrix in synovial cells. These observations suggest that VEGF and Ang2 are key players in both angiogenic and cartilage invasion processes in inflammatory arthritis. More recently however, it appears that this system is more complex and it is thought that TNFα can directly regulate angiogenesis specifically through the Angiopoietin receptor Tie-2, modulating vascular survival and maintenance. Consistent with this, Dr Trevor Markham's work demonstrated a significant downregulation in Tie2 gene and protein expression in psoriasis skin biopsies in-patients undergoing anti-TNFα therapy and this was associated with inhibition of more downstream events critical to EC survival such as Sox 18 and Survivin. Trevor has submitted this work for MD Thesis UCD.

Dr. Clare Matthews undertook a visiting research fellowship from Northern Ireland where she is in the 4th year of her specialist training. Dr Matthews performed studies of survivin and expression of STAT

transcriptional proteins in RA synovial tissue, this work was completed and presented at the Irish Society for Rheumatology in October 2004. Dr. Catherine Sweeney joined the team as a HRB funded postdoctoral scientist in October 2004. The aim of the project is to further dissect out the pathways involved in the EC survival machinery. Specifically the project will examine specific interactions between VEGF, Angiopoietins and TNFα regulate (1) vascular morphology and blood vessel maturation, (2) EC survival /stability and (3) transcriptional regulation. Using confocal microscopy, Catherine has been determining the maturity of blood vessels in different arthropathies and in RA patients pre/post anti-TNFα therapy. Preliminary data has also demonstrated expression of Notch/hedghog signalling proteins in synovial cells which are pivotally involved in vascular cell fate. Currently, Catherine is using laser caprture technology to examine expression of ket vascular protein involved in inflammatory vasculature and is using mecahnsitic studies to determine their key upregulators.

Ursula Fearon with Lucy Golden Mason (Liver Immunology group) are directing studies of natural killer cells that may be important in perpetuating the inflammatory response in PsA. Recently, the importance of natural killer (NK) and NKT cells in the pathogenesis of psoriasis skin has been highlighted. Control of NK cell activity is provided principally by inhibitory signals through cell-surface receptors (NKRs). Therefore, NK cells, through their ability for self-cell lysis and cytokine secretion, may be implicated in tissue destruction in the joints of PsA patients and/or exacerbate T-cell mediated pathology through production of cytokines. NKT cells are also a potentially auto-reactive population. Little is known about NK/NKT/NKR+ populations in the PsA joints.

Data from our initial studies suggests that NK cells may be important in the pathogenesis or perpetuation of PsA, demonstrating that markers of activation of NK cells are significantly upregulated in the joint compared to the peripheral circulation. Our data also suggests a role for the Vα24Vβ11 NKT cell subset in the control of PsA. To further explore this, current studies are examining the following: -

(a) The differential distribution of NK/NKT/NKR+ cells at the sites of active inflammation (synovial fluid/membrane) compared to the systemic circulation in PsA.
(b) That there is a direct role for these cells in inflammatory response regulation within inflamed tissue.

Continuing research led by Evelyn Murphy, undertaken by Jennifer Ralph, identified the nuclear orphan receptor NURR1 as a molecular target of methotrexate action in human inflammatory joint disease and confirmed that the immunomodulatory actions of methotrexate are mediated through adenosine release. Thus, elucidating the molecular mode of action of effective therapeutic agents can provide novel information on disease-related pathways.

Laura Greenan completed her Masters project, and demonstrated that activated rheumatoid arthritis synovial fibroblasts are characterised by increased adhesion molecule expression and production of matrix-degrading enzymes, characteristics that are consistent with their known potential for tissue invasion and cartilage and bone destruction. Additional studies suggested an important dissociation between invasion and proliferation in rheumatoid synovial fibroblasts. Laura has started a new research project led by Dr Douglas Veale examining the effects a novel potential anti-inflammatory cytokine therapy.

A number of fascinating clinical studies are nearing completion. Firstly, Waleed Al-Shehhi prospectively evaluated the burden of inflammation in a cohort of patients with early inflammatory arthritis. He hypothesised a correlation between the inflammatory burden and carotid intimal medial thickness. His findings confirmed that patients with inflammatory arthritis (RA and PsA) are at greater risk of developing atherosclerosis than normal subjects. This study is ongoing and further analysis may demonstrate an association between incompletely controlled inflammatory mechanisms and the development of atherosclerosis.

In a collaboration by a team from SVUH, University Hospital Geneva and Amgen, Thousand Oaks, California, Terence Rooney evaluated the effects of targeting IL-1 and TNFβ in-patients with severe rheumatoid arthritis. His preliminary findings demonstrated that serum levels of proinflammatory cytokines, metallo-proteinases and their inhibitors reflected clinical and radiographic responses to cytokine targeted treatments. Further studies evaluating the effects on tissue cell populations and expression of proinflammatory mediators are ongoing.

An important development in rheumatology research is the increasing volume of research being undertaken by allied health professionals. Norelee Kennedy evaluated the clinical benefits and economic impact of inpatient and outpatient rehabilitation programmes. This work resulted in the award of a PhD. Patricia Minnock studied pain outcome and fatigue levels reported by women with established RA. The majority of women reported poor pain outcomes and high fatigue levels. The association between pain outcome and fatigue levels is the subject of further work. Marie O'Rourke investigated the level of knowledge of disease in-patients with PsA and how this related to quality of life. The findings supported the hypothesis that patients with higher levels of knowledge of disease have better quality of life than patients who are less well informed.

Signal transduction pathways involved in the pathogenesis of inflammatory diseases

Dr Evelyn Murphy, Dr Kimberlee Mix, Postdoctoral Fellow
Dr Darren Ennis, Postdoctoral Fellow
Jennifer Ralph, PhD student Carol Aherne, PhD student

Long-term physiologic stress responses are achieved by integration of the autonomic nervous, HPA, cardiovascular, metabolic and immune protective responses. Achieving this dynamic stability requires considerable "tissue wear and tear" resulting from sustained overactivity (or underactivity) of these systems. Disturbances in endocrine-immune interactions upset the normal regulatory homeostatic balance and may alter susceptibility to a variety of disease states associated with immune dysregulation. Inflammatory stimuli reliably elicit HPA activation, and it is now established that the immune and HPA systems are mutually regulatory and that their interactions partly determine stress effects on immune function (Figure 1). What has not been extensively studied, however, is the interactive nature of the endocrine and immune systems and whether this might alter predisposition to inflammatory disease or stress-related pathologies.

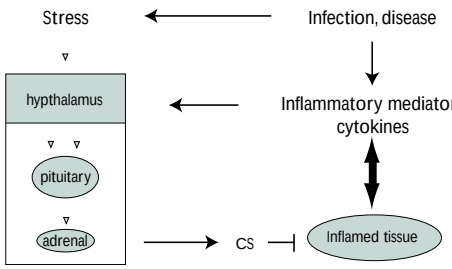


Figure 1. The hypothalamic-pituitary-adrenal (HPA) axis plays an important regulatory and modulatory role in normal immune function, permitting adequate responses to invading organisms and other inflammatory stimuli. The HPA axis potently modulates the inflammatory response through the actions of corticosteroids (CS). Pro-inflammatory cytokines (including IL1, IL6 and TNF) can activate the HPA axis. Dysfunction of this bi-directional communication has been implicated in the development and pathogenes of chronic inflammatory disease.

Recent advances have highlighted the efficacy of IL1 and TNF blocking strategies in the treatment of inflammatory diseases, including rheumatoid arthritis (RA), psoriasis and colitis. However, it is now clear that redundancy of cytokine actions will necessitate combination therapy to interrupt positive feedback loops. The identification of signal transduction pathways involved in the pathogenesis of these diseases has sparked a search for novel therapeutic strategies. These intracellular signalling systems transduce extracellular signals from the cell surface to the nucleus, where they are integrated at the level of transcription factor activity. The aberrant functions of downstream transcription factors have been implicated as critical regulators of gene expression involved in chronic inflammation. Therefore, specific signal transduction targets or transcription factor targets may interrupt the perpetuation mechanisms in RA and help re-establish homeostasis. We have evidence to support the conclusion that modulation of the nuclear transcription factor NURR1 by pro-inflammatory mediators is an important component of the inflammatory process in human arthritis. Our data demonstrates that NURR1 induction occurs downstream to TNFα and IL-1β signalling and may therefore be an effective target for anti-cytokine therapy in inflammatory conditions. The ongoing aim of our research is to gain further insight into the cytokine signalling pathways regulating NURR1 expression and to understand the regulatory targets of NURR1 tran-scriptional activity. These studies will provide important information on disease mechanisms and pathways involved in the inflammatory process.

Other Activities

Reviewer for Funding Agencies:

Wellcome Project Grant Applications
Health Research Board Post-Doctoral Fellowships

Reviewer for Journals:
Journal of Arthritis Research and Therapy
Oncogene

Arthritis and Rheumatism

Cell and Molecular Letters



Department of Endocrinology & Diabetes Mellitus

Consultants	Professor T.J. McKenna, Dr Malachi McKenna, Dr Donal O'Shea.
Lecturers	
Department of Medicine, U.C.D.	Dr James Gibney and Dr Diarmuid Smith.
Specialist Registrars	Dr Kevin Moore, Dr Chong Yew Tan.
Registrars	Dr Montasir Hammam, Dr Sally Daly.
Research Registrars	Dr Twani Tuthill, Dr Tom Cawood.
Endocrine and Diabetes Research Nurse	Ciara O'Dwyer.
Endocrine Laboratory	Dr. Tom Smith.
Research-Graduate Students	Sinead Kelly and Lucille Kavanagh.

Research Projects

Control of adrenal steroidogenesis: (Ms Sinead Kelly)

Classically, the adrenal cortex consists of three morphologically distinct layers, the zonae glomerulosa, fasciculata and reticularis. The zona glomerulosa cells produce aldosterone and the zona fasiculata and reticularis secrete both glucocorticoids and adrenal androgens. The capacity of the adrenal to produce steroids is controlled in part through the transcriptional regulation of steroid enzymes. The orphan nuclear receptor SF-1 is central to the transcriptional regulation of all steroid hydroxylase enzymes, whereas nur77 can preferentially regulate steroid enzyme genes relevant to cortisol production. Sinead Kelly is currently investigating the role of these orphan nuclear receptor and their co-activator proteins in regulating key steroidogenic enzymes and subsequent production of cortisol and adrenal androgens.

Macroprolactinaemia: (Ms Lucille Kavanagh)

(a) Macroprolactin is frequently described as a complex of monomeric PRL bound by an autoantibody. The presence of such autoantibodies may reflect an underlying autoimmune condition in those individuals producing the macroprolactin complex. We compared a number of markers of autoimmunity (anti-nuclear antigen autoantibody, anti-thyroglobulin autoantibody, anti-thyroid peroxidase autoantibody and CD5+ B cells) in groups of normal subjects, macroprolactinaemic patients with a group of patients exhibiting true-hyperprolactinaemia. CD5+ B cells occurred in the macroprolactinaemic patients in a higher frequency than in the normoprolactinaemic patients. These findings suggest an autoimmune susceptibility in macroprolactinaemic patients.

(b) The bioactivity of macroprolactin is brought into question by the clinical observation that the classic signs and symptoms of true hyperprolactinaemia, are absent or less severe in-patients with macroprolactin to explain hyperprolactinaemia. Preparations of macroprolactin and of monomeric prolactin were obtained by gel filtration chromatography of macroprolactinaemic sera. Using a lymphoma cell line that exhibits a mitogenic response to prolactin, we demonstrated that macroprolactin is bioactive *in vitro*, though in general it exhibits somewhat reduced bioactivity relative to monomeric prolactin. The much reduced bioactivity apparent *in vivo* may be due to non-bioavailability rather than intrinsic inactivity of prolactin in macroprolactin.

Androgen-secreting tumours: (Dr D Smith)

A clinical description of androgen secreting tumours: Androgen excess is a frequent disorder in women and 95% of such patients have polycystic ovary syndrome as the underlying disorder. Recognition of the rare more sinister disorders associated with androgen excess including androgen-secreting tumours is of vital importance to patients. The department has previously described clinical presentations that require intense evaluation. The description includes clinical presentation of irregular menstruation and excess body hair growth in women presenting for the first time outside the decade 15-25 years, progressing rapidly so the patient presents for medical attention within one year of the first symptom, the presence of virilisation and the finding of testosterone levels more than twice the upper limit of normal. The recognition of 5 androgen-secreting tumours in 2004 has been reviewed against the background of previous experience with these disorders. In each case, the possibility of an androgen-secreting tumour was raised by features of the presentation, which have been described above. The importance of these findings is that it supports the policy of restricting comprehensive evaluation of patients presenting with signs of androgen excess to a relatively small percentage of patients with a high likelihood of identifying a disorder other than polycystic ovary syndrome.

Bone Disease: Thyroid Disorders: (Dr D Smith, Ms Barbara Murray, Metabolic Laboratory)

Bone turnover and bone remodelling balance has been assessed prior to and following treatment of overactivity of the thyroid gland. This generally under appreciated feature of hyperthyroidism is now clearly documented. Over activity of the thyroid glands is associated with increased bone turnover and decrease in bone remodelling balance which will lead to osteoporosis. The department is

currently examining the possible impact of mild forms of overactivity of the thyroid gland e.g. sub-clinical hyperthyroidism, on bone disease. The usefulness of indices developed in the Metabolic Laboratory for the assessment of bone turnover and bone remodelling balance have also been assessed and applied to patients with Paget's Disease, hyperparathyroidism and eating disorders.

Bone Biomarkers in the assessment of Breast Cancer Patients:

(Dr J Gibney, Ms Barbara Murray, Metabolic Laboratory, and Dr D Smith)

Over 100 patients have entered in a study whereby the sensitivity of bone biomarkers for the detection of bone metastases in breast cancer patients with normal bone scan is ongoing. This study will take several years to yield a result.

Diabetes

Clinical trials are currently being conducted to assess the efficacy of newly introduced long-acting anolog insulins which have reported the value in reducing the frequency of hypoglycaemia, particularly through the night, reduced weight gain, improved metabolic control with more reproducible glycaemic profiles. A study is also ongoing to assess the impact of a loner insulin in combination with fish oils, or fish oils alone or neither treatment in-patients with mild diabetes and evidence of vascular risk.

Thyroid Eye Disease (Dr. T Cawood)

Thyroid eye disease is a disfiguring, painful and occasionally sight threatening condition that affects about 40% of those with Graves' thyrotoxicosis. Dr. Cawood will examine the inflammatory cytokines that mediate the early stages of this condition in the hope that more targeted treatments can be developed to interrupt the process.

Achievements and Distinctions

Dr Tom Smith is Chairman of the Association of Clinical Biochemists, Republic of Ireland Region.

Dr James Gibney was appointed Consultant Endocrinologist, Adelaide & Meath Hospital incorporating the National Children's Hospital, Tallaght.

Sinead Kelly submitted a thesis entitled "Transcriptional regulation of key steroidogenic enzymes in the human adrenal gland" for consideration for the PhD degree to The Department of Pharmacology, UCD.

Tom Cawood was awarded the Ritchie Research Scholarship by the Royal College of Physicians and Surgeons of Glasgow to study "The roles of new therapies for thyroid eye disease".

Dr Abdulwahab Suliman received the Irish Journal of Medical Science Doctor Award for excellence in Clinical Research for the best paper published in the field of Endocrinology during 2003. The paper was entitled "Frequent misdiagnosis and mismanagement of hyperprolactinemic patients before the introduction of macroprolactin screening: Application of a new strict laboratory definition of macroprolactinemia". Suliman AM, Smith TP, Gibney J and McKenna TJ. Clin. Chem. 2003; 49(9) 1504-9. (The research reported was undertaken when Dr Suliman was Lecturer in Medicine, U.C.D., attached to this department).

Abstracts

The 12th International Congress of Endocrinology: Lisbon, 31st August - 4th September 2004.

1. Kavanagh L, Smith TP, Gibney J, McKenna TJ. Specificity and Clinical Utility of Methods for the Detectionof Macroprolactin. Proceedings, p364.

European Association of Radiotherapists: Nice, April 2004.

1. O'Shea D, Cawood TJ, Moriarty P. Thyroid Eye Disease – approaching targeted therapy. Radiotherapy and Oncology; April 2004: Vol. 71, Supp. 1, S2.

The 23rd Joint Meeting of British Endocrine Societies: Brighton, England, 22nd -24th March 2004.

1. Smith TP, Gibney J, Kavanagh L, Fahie-Wilson MN, McKenna TJ. Specificity and Clinical Utility of Methods for the Detection of Macroprolactin. Endocrine Abstracts (2004) 7; p173.
2. Kelly SN, McKenna TJ and Young LS. Secretagogues induce transcription factor SF-1 and co-activator, SRC-1 activity in the human adrenal gland. Endocrine abstracts 7 P220.
3. Hoashi S, O'Connell J, Kelly SN, Young L, Fitzpatrick J, O'Keane C and Kinsley BT. Human Chorionic gonadotrophin responsive adrenal cortical carcinoma and Cushing's syndrome in pregnancy. Endocrine Abstracts 7, P299.





4. Cawood TJ, Smith D, Wen YC, Gibney J, Au-Yeoung ML, Dodd JD, McKenna M, Fitzgerald O, Gallagher C, O'Shea D. Oral bisphosphonates may reverse the decline in bone mineral density in adults with cystic fibrosis. Endocrine Abstracts, March 2004; Vol. 7, P3.

Irish Endocrine Society 29th Annual Meeting: Malahide, 5th-6th November 2004.

1. Gibney J, Smith TP, McKenna TJ. The Impact on Clinical Practice of Routine Screening for Macroprolactin. Ir. J. Med. Sci. 173; (Suppl. 1), 9.
2. Kavanagh L, Smith TP, Gibney J and McKenna TJ. The Value of Ultrafiltration in the Detection of Macroprolactin. Ir. J. Med. Sci. 173; (Suppl. 1), 20-21.
3. Kavanagh L, Smith TP and McKenna TJ. Reduced Bioactivity of Macroprolactin *in vitro*. Ir. J. Med. Sci. 173; (Suppl. 1), 23.
4. Smith D, Tan CY, Murray BF, Doyle M, Malone K, Darby M, O'Shea D, McKenna M, McKenna TJ. Analysis of bone turnover in patients with anorexia nervosa. Ir. J. Med. Sci. 173; (Suppl. 1), 10-11.
5. Lacey LA, Wolf A, O'Shea D, Ruof J. Cost-effectiveness of orlistat for the treatment of overweight and obese patients with type 2 diabetes in Ireland. Ir. J. Med. Sci. 173; (Suppl. 1), 20.
6. Cawood TJ, McKenna MJ, Gallagher CG, Smith D, Wen YC, Gibney J, Dodd JD, O'Shea D. Oral bisphosphonates improve bone mineral density in adults with cystic fibrosis. Ir. J. Med. Sci. 173; (Suppl. 1), 22.
7. Cawood TJ, McKenna MJ, Gallagher CG, Smith D, Chung WY, Gibney J, O'Shea D. Cystic fibrosis-related diabetes in adults in Ireland. Ir. J. Med. Sci. 173; (Suppl. 1), 22.
8. Smith D, Murray BF, Gibney J, Tan CY, McKenna M, O'Shea D, McKenna TJ. The effect of thyroid dysfunction on indices of bone activity and their response to thyroid treatment. Ir. J. Med. Sci. 173; (Suppl. 1), 25.
9. Moore KB, Daly S, Gibney J, McKenna MJ, O'Shea D, McKenna TJ. Audit of the accuracy of a clinical diabetes database. Ir. J. Med. Sci. 173; (Suppl. 1), 29.
10. Moore KB, Smith D, Tan CY, Gibney J, Smith T, McKenna TJ. Androgen profiles in patients with polycystic ovariansyndrome. Ir. J. Med. Sci. 173; (Suppl. 1), 29-30.
11. Tan CY, Smith D, Hall D, Gibney J, McKenna MJ, McKenna TJ, O'Sullivan T, O'Shea D. An evaluation of the East Coast Area diabetes shared care programme. Ir. J. Med. Sci. 173; (Suppl. 1), 41.
12. Cawood TJ, Moriarty P, Golden-Mason L, O'Farrelly C, O'Shea D. Anti-TNFα Therapy inhibits TNFα-dependent ICAM1 up-regulation in orbital fibroblasts from patients with thyroid - associated ophthalmology. J. Med. Sci. 173; (Suppl. 1), 11-12.

Department of Psychiatry & Mental Health Research Staff

Head	Prof. Kevin Malone
Senior Research Fellow	Dr. Martin Mahon
Research Co-Ordinator	Ms. Sharon Bolger
UCD Newman Scholar	Ms. Aoife O'Donovan
AFSP Neuroscience Research Fellow	Dr. Robert Whelan
Medical Education Researcher	Dr. Allys Guerendel
Research Assistant	Ms. Grace Armstrong
Senior Administrator	Ms. Fiona O'Brien Lavin

Summary

2004 was an exciting year for our Department, with the recruitment of new researchers, and the commencement of several new research projects. Ms. Aoife O'Donovan was appointed as Newman Scholar, and is embarking on a major collaborative research programme with clinicians and basic science. Prof. Malone was awarded a prestigious American Foundation For Suicide Prevention (AFSP) Distinguished Investigator Award to continue his work on functional brain imaging studies in suicidal depression. Dr. Martin Mahon's paper on Suicide in The Irish Armed Forces was accepted for publication in The American Journal of Psychiatry. The Department is closely affiliated with the recently founded Charity 3Ts (Turning The Tide of Suicide) (Prof. Malone Co-founder). Several successful awareness and education projects were completed by 3Ts in 2004, including an International Conference on Suicide in Modern Ireland: New Dimensions, New Responses, opened by President Mary McAleese in November 2004.

Active Research Projects 2004

Ireland North South Urban Rural Epidemiologic (INSURE) Collaborative Study of Suicidal Behaviour in Major Psychiatric Disorders (UCD & QUB) (Ms. Sharon Bolger)

Suicide in The Armed Forces, in the Garda Siochana, and Irish Doctors (Dr. Martin Mahon)

Suicide on Irish Railways (Ms. Sharon Bolger)

Health Psychology, Immune Function and Physical Disease (Ms. Aoife O'Donovan)

Functional Brain Imaging of Brain Pathways associated with Future Thinking in Healthy Volunteers and In-patients with Suicidal Depression (Dr. Robert Whelan)

Computer Assisted Learning in Psychiatry (ICALPSYCH) (Dr. Allys Guerendel)

Mental Health Policy for UCD (Ms. Grace Armstrong)






The INSURE Project: The Ireland North South Urban Rural Epidemiological (INSURE) Study of Suicidal Behaviour in Major Psychiatric Disorders is a North-South study of biological psychological and clinical factors associated with suicidal behaviour in major psychiatric disorders. We currently have research fellows in Dublin, the Midlands, Ballinasloe, Donegal, Omagh and Belfast. To date, we have conducted clinical interviews on over 500 consecutive referrals to the psychiatric services in these research areas. We have also conducted almost 2,000 clinical interviews on patients who have presented in suicidal crisis. We are currently in the process of conducting statistical analysis on this data set, which will make a significant contribution to our understanding of suicidal behaviour in psychiatric disorders within the Irish context.

Brain Imaging Studies in Suicidal Depression: We have established a brain imaging research programme in collaboration with the Mater Hospital and the Brain Imaging Laboratory in Dartmouth, New Hampshire, where we have developed a functional MRI paradigm to study brain activation during future thinking, which is a key component in suicidal depression. Our preliminary data from this study has received significant international attention and our work was recently awarded a Distinguished Investigator Award by the American Foundation for Suicide Prevention. We are about to now bring this research paradigm into patients with suicidal depression.

Suicide in the Irish Armed Forces: We have conducted a study of all cause mortality in the Irish Armed Forces over the last thirty years, with a particular focus on risk factors associated with suicide. Some data from this work is currently in press in the American Journal of Psychiatry and the work is ongoing.

Suicide in Irish Doctors: With some start-up support from the Irish Medical Council, we have begun to examine all cause mortality in Irish doctors over the past thirty years with a particular focus on risk factors associated with suicide. This work is ongoing.

Suicide in An Garda Siochána: With start-up funding from the National Suicide Review Group, we are establishing all cause mortality in the Garda Siochána over the past thirty years, with a particular focus on occupational-specific risk factors associated with suicide. This work is ongoing.

Suicide on Irish Railways: With start-up funding from Iarnród Éireann, we have conducted a study of all rail deaths on Irish railways over the past twenty years, with a particular focus on potential hot-spots for suicidal acts. This work is ongoing.

Reaching out to Young People: This study was being conducted in collaboration with the Department of Child and Adolescent Psychiatry in the Mater Hospital (Professor Carol Fitzpatrick) where we followed up young people who have presented to the emergency services following a suicidal crisis, to explore their experience of psychiatric service treatment in their time of crisis. The findings were recently published in the 'Irish Journal of Psychological Medicine'.

Molecular Genetic Factors Associated with Suicidal Behaviour: This work received start-up funding from the Conway Institute and we are examining potential genetic polymorphisms associated with suicidal behaviour in high-lethality suicide attempters. This work is ongoing.

Psychological Stress, Optimism and the Immune System: This work has received support from a Newman Scholarship Programme at UCD and is being conducted in collaboration with the Department of Immunology in St. Vincent's Education & Research Centre as well as the Conway Institute and the Dublin Molecular Medicine Centre.

Suicide in Ireland Survey: This survey is designed to conduct personal and clinical interviews with relatives in the aftermath of a suicide. This study is designed to get a greater understanding of the lives behind the statistics. It is intended that this study will be a national study, which will commence by middle of this year.



Postgraduate Department

Education & Research Centre

The Postgraduate Department once again had a very successful and busy year. The meetings organized at postgraduate level for the hospital staff still continue to attract a large attendance from St. Vincent's University Hospital and are a vital contact for all the NCHDs.

Annual Hospital Study Day

The Hospital Annual G. P. Study Day remains a very important event for the hospital staff and the general practitioners in the community. This year our Study Day was held on Saturday 6th November and was attended by 115 General Practitioners. The educational format was similar to that of 2003 and consisted of Poster Sessions as follows:

1. Current Guidelines in Hypertension Management - *Dr. Martin Quinn*
2. Current Issues in Breast Cancer - *Mr. Arnold Hill*
3. Update on Parkinson's Disease - *Dr. Niall Tubridy*
4. Polycystic Ovarian Disease - *Dr. Grainne Flannelly*

There were also two Practical Workshops:

1. Update on GP Relevant Orthopaedics - *Mr. Kieran O'Rourke*
2. Current Guidelines in G.I. Endoscopy - *Dr. Hugh Mulcahy*

As the new hospital is nearing completion it was agreed by the committee that a tour of the new facility would be of interest to the GP's. This commenced at 6.30pm and as anticipated was informative and appreciated by all. Later in the evening a reception and dinner was held in the hospital restaurant and the consensus was that this was a huge success both with regard to attendance and atmosphere.

Medical and Surgical Meetings

The Postgraduate Lunch Time Meetings include:

Respiratory	Gastroenterology	Neurology
Nephrology	Palliative Care	Endocrinology
Colorectal	Surgical Audit	Rheumatology

There are also early morning meetings as follows:

Thursdays at 8.00am	Medical Conference
Fridays at 7.30am	Cardiology Conference

All of the above meetings are organized by the Postgraduate Department. Lunch or breakfast is provided.

Intern Lunch Time Seminars

Monday Lunch Time Seminars for Interns consist of Clinical Skills Sessions and Data Interpretation.

The Tuesday Lunch Time Seminar is organized by the Medical Intern Tutor in conjunction with the Postgraduate Department. Each week an intern presents a case to the class with a discussion of same afterwards.

Thursday Lunch Time Lectures are on matters relating to all aspects of Surgery and are given by consultant surgeons within the Hospital.

Friday Lunch Time Seminars for the Interns are presented by guest speakers and include Career Guidance, Risk Management, Stress Management and a wide variety of other topics.

Due to demand it was decided to introduce further lectures for Interns on Wednesdays at lunch time which commenced in January 2003.

SHO Teaching

In response to requests from our SHOs we have recently re-organised their formal teaching programme. We now run a two-month intensive programme of bedside and data-interpretation tutorials three times per year (January/February, May/June, October/November). These courses are aimed primarily, but not exclusively, at the MRCPI examination.

Clinical Science Course

The above course is also organized by the Postgraduate Department, in conjunction with the Royal College of Physicians of Ireland, and takes place in teaching hospitals throughout Dublin. These meetings are aimed at candidates preparing to sit Part II of the MRCPI Examination. The course is run from this office bi-annually, ending before the June and November examinations. The course continues to run smoothly and the standard of teaching is excellent covering such specialties as Rheumatology, Geriatrics, Haematology/Oncology, Gastroenterology, Endocrinology, Cardiology, Nephrology, Dermatology, Respiratory and Neurology.




International and National Meetings

In addition to the large number of in-house meetings, the Postgraduate Department helped organise the Eleventh Colorectal Study Day which was held on Friday 3rd September 2004. This year's programme was as follows:

Session 1	The Biology of Metastatic Disease
	Tumour Invasion – Dr. Jacintha O'Sullivan
	Host Defence – Dr. Lucy Golden-Mason
Session 11	Metastatic Disease – A Standardised Approach
	Referral Patterns and Trends – Mr. Justin Geoghegan
	Modern Imaging for Metastatic Disease – Dr. Conor Collins
	Therapeutic Algorithms – Mr. Donal Maguire
Session 111	Therapies for Metastatic Liver Disease
	Stereotactic Radiotherapy – Prof. John Armstrong
	Radiofrequency Ablation : Current Status – Dr. David Breen
	Newer Chemotherapeutic Approaches – Dr. Richard Goldberg

The State of the Art Lecture was given by Prof. Oscar Traynor on "Hepatic Surgery in Colorectal Cancer". The organising committee included Dr. D. Fennelly, Mr. J.M. Hyland, Prof. D.P. O'Donoghue, Dr. H. Mulcahy and Dr. K. Sheahan. The meeting was extremely well attended and interaction was excellent. A dinner was held afterwards in the Conrad Hotel and was attended by fifty guests.

"Get to Know You Function" for Incoming NCHDs

Once again this year we had a very successful and enjoyable welcoming lunch for incoming NCHDs on Wednesday 7th July 2004. This event is an opportunity to give NCHDs the relevant information about ongoing lectures/conferences and welcome them to the hospital. This is now an annual event.

The Student Summer Project

This continues to be an important part of the hospital's academic year. The aim of the project for the student is to develop an interest in research so that he/she will continue to develop this interest at postgraduate level. The judging panel of physicians and surgeons select the students to present a formal evening presentation based on the original abstract submitted. This year the winner was Conor Lahiff whose project was entitled "Chemotherapeutic Responses and Cell Migration Rates in Human Colon Carcinoma Cells and their Lymph Node Metastic Derivatives". Along with the prestige of winning this award, the student also receives a monetary amount. The presentations were held on Wednesday 17th November 2004

The Library

The library is located on the first floor of the Education & Research Centre and is open to all staff of St Vincent's University Hospital Group. Its collection includes books, journals and audiovisual material covering all disciplines. Some journals are available online and many more are available via the UCD Library website.

There are 17 PCs in the library providing access to several databases including Medline, the Cochrane Library and Web of Science. Advanced training on these products is available free of charge by appointment only. Internet, document-creating, printing, scanning and multimedia projection facilities are also available.

Postgraduate Education Committee

This committee is now well established and meets biannually to discuss postgraduate education in the hospital. Its aim is to enhance the education environment of postgraduate doctors at St. Vincent's University Hospital.

Pharmaceutical Representatives

The link for the pharmaceutical companies here in St. Vincent's University Hospital is the Postgraduate Department. The companies sponsor the Medical and Surgical Lunch and Breakfast Meetings and, in turn, the medical representative meets the consultants and NCHDs to discuss their products. This is an important link for both the doctors and medical representatives.

Postgraduate Medical Training Booklet

A Medical Booklet is now being compiled and circulated bi-annually by the Postgraduate Department. It contains valuable information re contacts in various departments, dates and locations of tutorials and lecture programmes

Prof. Walter McNicholas
Co-ordinator of Postgraduate Education

Ms. Betty McArdle
Postgraduate Department

Academic Activities

Education & Research Centre

Annual Biomedical Research Symposium 2004
Education & Research Centre
St. Vincent's University Hospital & Conway Institute UCD
Friday 12th November 2004 9.00 a.m. – 6.00 p.m.

Common Molecular Mechanisms in Inflammation & Malignancy

Welcome by Prof. Hugh Brady, President, UCD

Session I: Basic Molecular Mechanisms in Malignancy
Chair: Prof. Diarmuid O'Donoghue, S.V.U.H.

Mutator Phenotype and Cancer.
Prof. Larry Loeb, University of Washington, Seattle Manipulating Prostate Cancer Resistance to Apoptosis.
Dr. Bill Watson, Conway Institute UCD

Modelling Breast Cancer Progression and Treatment in *vivo*.
Dr. Louise Flanagan, S.V.U.H.

Selected Oral Presentations

Cyclosporine A-induced epithelial-mesenchymal transition: a role of PKC-β
C. Slattery, T. McMorrow and M.P. Ryan Dept. of Pharmacology, Conway Institute of Biomedical Research, University College Dublin

Is telomere length shortening a predictive biomarker for Cancer Risk in Ulcerative Colitis?
J. O'Sullivan, C O'Riain, Julie Gorman, Denise Keegan, H. Mulcahy, J. Hyland, David Fennelly, K. Sheahan & D. O'Donoghue
Centre for Colorectal Disease, St Vincent's University Hospital, Dublin 4.

Session II: Potential Therapeutic Targets in Malignant and Inflammatory Disease
Chair: Prof. Bill Powderly, Medicine and Therapeutics Dept U.C.D. & Mater Misericordiae Hospital

CD44: A Central Player in Inflammation and Metastasis? Dr. Aideen Long, R.C.S.I.

Selected Oral Presentations

Pre-Transplant $CD56^+$ Lymphoid Populations Predict Outcome post Liver Transplantation (LT) for Hepatitis C Viral Infection
Lucy Golden-Mason, Hugo R. Rosen, John E. Hegarty, Cliona O'Farrelly, St. Vincent's University Hospital, Dublin 4 and Portland VAMC, Portland OR/Oregon Health & Science University

Gene Expression Responses to Endogenous Anti-Oxidant Activity in Human Vascular Smooth Muscle Cells.
Z. Martin 1,2,3 C. Costello 2, A. Duffy 1, P. McLoughlin 2, J.M. Fitzpatrick 3, F. Martin 2, M.K. O'Malley 3 and T. O'Brien 1. Dept. of Experimental Medicine & Pharmacology, Clinical Science Institute, University College Galway, Ireland 2. Department of Physiology & Pharmacology, Conway Institute for Biomedical and Biomolecular Research, University College Dublin, Ireland 3. Department of Surgery, Mater Misericordiae University Hospital, Dublin Ireland

Ligation of DCAL-1, a novel C-type lectin-like molecule induces partial maturation of dendritic Cells
Elizabeth J. Ryan[1] Immunology Research, Sanofi-Aventis, Campus Mérieux, 1541 Avenue Marcel Mérieux, 69280 Marcy l'Etoile, France.

Progress with Novel Therapeutic Strategies in Malignancy
Prof. John Crown, S.V.U.H. & D.C.U.

Session III: Regulation *of Molecular Mechanisms in Inflammatory Disease*
Chair: Dr. Doug Veale, S.V.U.H.

Angiogenesis in Rheumatoid Arthritis.
Prof. Alisa Koch, University of Michigan Medical School

Lipoxins: Proresolution Antifibrotics and Mediators in Inflammation.
Prof. Catherine Godson, Conway Institute, U.C.D.

New Pathways of IGF-1 Survival Signalling.
Prof. Rosemary O'Connor, Dept. Biochemistry U.C.C.

Selected Oral Presentations:

The Role of the Notch Signalling Pathway in Vascular Smooth Muscle Cell Apoptosis
Catherine Sweeney(1), David Morrow (1), Yvonne Birney (1), Dermot Walls (1), Eileen M. Redmond (2) and Paul A. Cahill (1). Vascular Health Research Centre, Dublin City University, Ireland University of Rochester, New York, U.S.A.

Session IV: Poster Discussion

Leader: Dr. Steve Simpson, SCIENCE
Co-Chair: Prof. Stephen Pennington, Conway Institute, U.C.D.

Panel Discussion: Development of Translational Research in Ireland.

Chair:	Dr. Risteárd Ó Laoide, Chairman, Medical Board, S.V.U.H.
Panel:	Prof. Hugh Brady, President, U.C.D.,
	Dr. Ruth Barrington, Chief Executive, H.R.B.,
	Dr. Pierre Meulien, Director, D.M.M.C.
	Prof. Larry Loeb
	Prof. Alisa Koch
	Mr. Eamonn Fitzgerald, Deputy C.E.O., St. Vincent's Healthcare Group
	Mr. Dave Shanahan, M.D. Pfizer Healthcare Ireland

Prizes given for the best oral presentation and the best poster.

The Meenan Prize awarded for the best research presentation by a UCD Medical Graduate

Research Workshops 2004

Education & Research Centre

Comparative Immunogenomics Workshop

St.Vincents University Hospital/UCD Veterinary School/
The Conway Institute
Workshop II, June 24th 2004

Setting the Scene: Immunology, Genes and Species Differences: Cliona O'Farrelly SVUH/Conway Institute.

The zebrafish: a model organism for gaining insight into the development of the immune system. Lisa Steiner, MIT.

Gene expression profiling of a bovine immune response to trypanosomiasis. Emmeline Hill, Department of Animal Science, UCD.

Detecting acceleration in the rate of nonsynonymous substitutions in bovine genes - candidates for disease resistance? David Lynn, Dept Genetics, TCD.

Antimicrobial peptides and TLR gene discovery in chicken genome: a bioinformatics approach. Andrew Lloyd/Paul Cormican, SVUH/Dept Genetics TCD.

Computational analysis of the potential immune-modulatory activity of Hsp90. Mario Fares, NUI Maynooth.

Discussion

DMMC

Immunology & Inflammation Course 2004

Location:	Education & Research Centre, S.V.U.H.
Dates:	18th February – 14th April 2004
Course Leader:	Prof. Cliona O'Farrelly
Contributors:	Lucy Golden-Mason, (SVUH)
	John Baugh PhD (Dept. of Medicine UCD)
	Prof Alex Whelan (TCD),

Introductory Overview
Organs, cells and molecules of the immune system

Innate Immunity 1
Toll like receptors; cytokines; chemokines; anti-microbial peptides

Innate Immunity 2
Complement; inflammation; anti-inflammatory mechanisms

MHC
Antigen processing and presentation; dendritic cell maturation
Cellular and Molecular Components of Adaptive Immunity
T and B lymphocytes; TCR; Immunoglobulins; regulatory cytokines

Lymphocyte Development
Haematopoietic stem cells; thymus; bone marrow; gene rearrangement; positive and negative thymic selection

Effector Mechanisms in Immunology
Cytotoxicity; apoptosis; immune responses to pathogenic challenge; tolerance induction

Immunological Dysregulation
malignancy, autoimmunity and allergy

Objectives of the Course

At the end of this course, participants should be familiar with the basic terms and concepts of innate and adaptive immunity. They hopefully will be conversant with the names and functions of the main organs, cells, molecules and genes involved in initiating and mediating immune responses. They should be able to explain fundamental immunological concepts and discuss the roles of functional components of the immune system including acute phase proteins, complement, haematopoietic cells, immunoglobulins and cytokines in protecting against disease. They should also understand some basic concepts of immunological dysregulation seen in inflammatory and autoimmune diseases, allergy, malignancy and immunopathogenesis.

Education & Outreach

Education & Research Centre

ERC Journal Club 2004

Schedule: Thurs mornings 9.00-10.00am
Venue: Seminar Room, E.R.C.

S.V.U.H. Schools One Day Seminars:

Laboratory staff in the E.R.C. contribute by giving talks on the workings of the research laboratories to secondary school students.

Healthwise Newsletter:

Articles from E.R.C. researchers appear regularly in Dept. Preventive Medicine Healthwise Newsletters.

The Irish Scientist Yearbook 2004:

The E.R.C. contributed two articles to the Irish Scientist Yearbook 2004 "Research at St. Vincent's University Hospital: improving patient care today and tomorrow." and "The search for antimicrobial peptides."

S.V.U.H. Innovation Award 2004:

E.R.C. GIR Group led by Team Leader Alison Beirne won a quality improvement initiative award for their project "Increase communication between HCV patients and the Education & Research Centre".

In 2004, several UCD undergraduates carried out research for their theses at SVUH, students from TCD Molecular Medicine Science course and several Secondary school students were also accommodated. A project devised by SVUH researchers and implemented by two secondary school students from the Loreto Convent in Wexford won first Prize in their category in the 2004 Young Scientists Exhibition.

Prestigious Invitations to E.R.C. Researchers

Education & Research Centre

Prof. Cliona O'Farrelly

Appointed member of Cancer Research Ireland (CRI) 2004-2006

Appointed to SFI Immunology Review Panel 2004

"The liver: an immunological battlefield or peace process?" Annual Congress of the British Society for Immunology, Harrogate Dec 2004

"Defend to the Death: Liver Immunology in Malignancy and Infection" Annual Isla Haliday Lecture for UIG Belfast Nov 2004

"In Silico Cloning of TLR Components and New Antimicrobial Peptides" 1st International Conference on Basic and Clinical Immunogenomics, Budapest, Hungary Oct 2004

Assessor, RIA Science Writing Competition 2004, June 2004

RAMI, Section of Biomedial Sciences, Summer Meeting 2004, University of Ulster, Jordanstown, Conway Review Lecture "A pattern recognition genomics approach to pattern recognition immune molecule discovery" June 2004

"NK Cells – Lessons from the Liver" at BSG Workshop on "Immune Epithelial Interactions in the Gastrointestinal Tract", Trinity College Dublin June 2004

"Immunoregulation in the liver and its extrahepatic relevance" European Society for Pediatric Gastroenterology Hepatology and Nutrition (ESPGHAN) meeting in Capri, Italy May 2004

University of Bath, Pharmacology Society "Work in Progress" presentation May 2004

"New Anti Microbial Peptides from Chicken Genome", Roslyn Institute, Edinburgh April 2004

Immune Regulation Workshop, DMMC Spring Research Symposium, Conway Institute UCD April 2004

Prof. Joe Duffy

XXXII International Society for Oncodevelomental Biology and Medicine (ISOBM): Helsinki. Title of presentation: Markers in colorectal cancer European Group on Tumor Markers Guidelines

5th Central European Conference on Human Tumor Markers: Prague. Title of Presentation: Markers in breast cancer: which ones are clinically useful?

Urological Club of Great Britain and Ireland Annual Meeting, Dublin.
Title of Presentation: Beyond PSA

Prof. Walter McNicholas

Annual meeting of the Japanese Respiratory Society in Tokyo in April.

Annual meeting of the European Respiratory Society in Glasgow in September.

Annual meeting of the Asian Pacific Society of Respirology in Hong Kong in December.

Prof. McNicholas was also a Visiting Professor at Brown University, Rhode Island, USA, in October 2004 during which he presented Medical Grand Rounds at the University.

Dr. Kieran Sheahan

Guest lecturer in Dept. Pathology, University of Washington, Seattle

Professor T J McKenna

President of the Royal College of Physicians of Ireland

Member of the National Taskforce on Medical Manpower and a member of its Medical Education and Training Group

On the Editorial Boards for the journals, Clinical Endocrinology (Oxford) and The Endocrinologist (U.S.).

Member of the Students Affairs Committee of the Endocrine Society (U.S.).

Professor McKenna was invited assessor of quality for the Department of Medicine, University College Galway.

Professor Barry Bresnihan

Professor Bresnihan was an invited speaker at the New Zealand Rheumatology Association and the Israel Society for Rheumatology.

Visiting Professor at the Rheumatology Update Programme at Georgetown University, Washington.

Chaired the satellite symposium on 'B Cells in Rheumatology' at the EULAR Congress in Berlin in June.

External examiner for PhD thesis at the University of Newcastle and a member of the Dutch Arthritis Association review committee

Professor Bresnihan continues to co-chair the European Synovitis Study Group. Chairman of the Subcommittee for Academic Support, Arthritis Research Campaign (UK).

Chairman of the Arthritis Ireland and a member of the Oliver Bird Programme Selection Committee.

Professor Bresnihan serves on the Editorial Board for the Journal of Rheumatology, Annals of the Rheumatic Diseases, Joint Bone and Spine, and Bailliere's Clinical Rheumatology.

Professor Oliver FitzGerald

Invited speaker both at the British Society for Rheumatology and at the American College of Rheumatology.

He was also invited to speak at the meeting on Breakthrough Therapy for the Treatment of Psoriasis.

External examiner for PhD theses in Queens University Belfast and at Oxford University

External examiner at the Trinity College final medical examinations

He is a Council Member of Arthritis Ireland and is interim Director of the SVUH Education and Research Centre.

Member on the Editorial Board of Rheumatology and of Current Rheumatology

Dr. Doug Veale

Dr Veale was an invited speaker at the EULAR Congress and the American College of Rheumatology Meeting.

Medical Director, Rheumatology Unit at Our Lady's Hospice, Harold's cross.

Chairman of Arthritis Action Ireland, a Council member of the Irish Society for Rheumatology and Editor E-Medicine.

Honours, Awards and Prizes 2004

Education & Research Centre

Professor Cliona O'Farrelly

Isla Haliday Memorial Lecture 2004
Conway Lecturer and Medallist 2004

Professor Kevin Malone

Prof. Malone was awarded a prestigious American Foundation For Suicide Prevention (AFSP) Distinguished Investigator Award to continue his work on functional brain imaging studies in suicidal depression.

Conor Lahiff

3rd Poster prize, Health Research Board Undergraduate Research meeting, Dec 2004.
First Prize for best undergraduate research project, St Vincent's University Hospital, Oct 2004.

Dr. Jacintha O'Sullivan

Joint first prize for best oral presentation, ERC Research Symposium, November 2004.

Travel Award to attend NCI cancer consortium conference, Baltimore, USA.

Garrett Cullen

3rd prize for best oral presentation, Irish Society of Gastroenterology, November 2004.

Dr. Kim Mix & Jennifer Ralph

1st prize for presentation of their research at the ERC Annual Symposium, St Vincent's University Hospital and the Irish Society of Rheumatology Annual Meeting.

Jennifer has also been awarded an Australian Endeavour Award to travel to Melbourne and engage in arthritis research for up to 1 year.

Jenny Ralph won First Prize Oral Presentation at the Irish Society of Rheumatology Meeting in Dublin in October.

Dr. Doug Veale

Awarded Best Oral Paper at the Irish Association of Dermatologists in 2004: Markham T et al, Anti-TNF α therapy in psoriasis and psoriatic arthritis:
Clinical and angiogenic responses.

Mr. John Hyland

President of the Coloproctology Association of Great Britain & Ireland

Degrees Awarded

Tony Kenna	awarded PhD
David Lynn	awarded PhD
Neil O'Brien	awarded PhD
Jonathan Dean	awarded PhD
Salih M. Bakhiet	awarded MSc

Publications

Education & Research Centre

Published or Accepted for Publication in International Peer Reviewed Journals 2004

Albores-Saavedra J, Sheahan K, O'Riain C, Shukla D. *Intraductal tubular adenoma, pyloric type, of the pancreas: additional observations on a new type of pancreatic neoplasm.* Am J Surg Pathol. 2004 Feb;28(2):233-8.

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Bresnihan B, Gogarty M, FitzGerald O, Dayer J-M, Burger D. *Apolipoprotein A-I infiltration in rheumatoid arthritis synovial tissue: a control mechanism of cytokine production?* Arthritis Research & Therapy 2004, 6: R563-R566.

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B.R. Celli, W. MacNee, A. Agusti, A. Anzueto, B. Berg, A.S. Buist, P.M.A. Calverley, N. Chavannes, T. Dillard, B. Fahy, A. Fein, J. Heffner, S. Lareau, P. Meek, F. Martinez, W.T. McNicholas, J. Muris, E. Austegard, R. Pauwels, S. Rennard, A. Rossi, N. Siafakas, B. Tiep, J. Vestbo, E. Wouters, and R. ZuWallack. *Standards for the diagnosis and treatment of patients with COPD: a summary of the ATS/ERS position paper.* European Respiratory Journal, Jun 2004; 23: 932 - 946. PMID: 15219010.

Cleary M, Coss A, Sheahan K, Mulcahy H, Hyland J. *Dieulafoy's lesion in a Meckel's diverticulum.* Gastrointest Endosc. 2004 May;59(6):747-9.

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Doherty LS, Nolan P, McNicholas WT. *Effects of topical anesthesia on upper airway resistance during wake-sleep transitions.* Journal of Applied Physiology (accepted for publication).

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Doherty LS, O'Connor C, O'Malley M, McNicholas WT, Nolan PJ. *Optimising the measurement of genioglossus fatigue using surface electromyography.* American Journal of Respiratory and Critical Care Medicine 2004; 169(suppl): 434.

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M. Garland, E. Lavelle, D. Doherty, L. Golden-Mason, P. Fitzpatrick, A. Hill, N. Walsh *Cortisol does not mediate the Suppressive Effects of Psychiatric Morbidity on Natural Killer Cell Activity: A cross-sectional study of patients with early breast cancer* Psychological Medicine 2004 34, 481-490 PMID 15259833

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L.Golden-Mason, A.Kelly, J.Hegarty, C.O'Farrelly *Hepatic Interleuklin 15 (IL-15) Expression: Implications for Local NK/NKT Cell Homeostasis and Development* Clin. Exp. Immunol 2004 Oct;138(1):94-101 PMID 15373910

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R. Higgs, D.J. Lynn, S. Gaines, J. McMahon, J. Tierney, J. Tharappel, A.T. Lloyd, G. Mulcahy, C. O'Farrelly. *The Synthetic Form of a Novel Chicken b-Defensin Identified in Silico is Predominantly Active against Intestinal Pathogens* Immunogenetics 2004 (in press)

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Ann Rheum Dis. 2004 Feb;63(2):162-9

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McNicholas WT. *Controlling passive smoking through legislation in Ireland: an attack on civil liberty or good public health policy?* European Respiratory Journal 2004 Sep; 24(3): 337-8. PMID: 15358684.

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American Journal of Psychiatry (In Press)

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The Irish Psychologist 2004, 30 (7), p.157.

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Transplantation 2004 July 27;78(2):250-6 PMID 15280686

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Journal of Psychology and Psychological Therapy 2004, 4, 285-302.

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Chapters in Books

Education & Research Centre

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Bresnihan B. Management of rheumatoid arthritis:synovitis. *In Practical Rheumatology* 3e. Edited by Hochberg MC, Silman AJ, Smolen JS, Weinblatt ME, Weisman MH. Elsevier Ltd 2004.

Invited Reviews

Education & Research Centre

Bresnihan B. Are synovial biopsies of diagnostic value?
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Ann Rheum Dis. 2004 Apr;63(4):333-4.

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Ann Int Med Jan 2004 page 152.

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Grants Active in 2004

Education & Research Centre

P.I.	Name of Study	Source of Grant	Fund Amount	Start Date	Finish Date
Dr. Evelyn Murphy	Pathobiologic function of Corticotropin-Releasing Hormone	HRB Research Funding	€110,000	2001	2004
Dr. Evelyn Murphy	Signal Transduction in inflammatory arthritis	HRB Research Project Grant	€150,000	2000	2003
Dr. Derek Doherty	The functional significance of diversity in human natural killer cell receptors	Enterprise Ireland Project Grant	€154,044	2000	2004
Dr. Derek Doherty	The role of intrahepatic gd T cells in immunity against hepatitis C virus	HRB Project Grant	€164,126	2001	2004
Dr. Derek Doherty	Activation and regulation of human NKT cells	Enterprise Ireland Project Grant	€72,616	2000	2003
Prof. Joe Duffy/Brid Ryan	Studies on survivin in breast cancer	Irish Research Council for Science, Engineering and Technology (IRCSET)	€38,000	2002	2004
Prof. Joe Duffy	Role of ADAM17(TACE) in breast cancer	Health Research Board	€107,600	2003	2006
Prof. Joe Duffy	Mamaglobin as a marker for breast cancer	Irish Cancer Society	€52,000	2003	2004
Prof. Joe Duffy/ Prof. John Crown	Mode of action of the therapeutic antibody Herceptin in breast cancer.	St. Vincents University Hospital Medical Oncology Fund	€100,000	2001	2004
Prof. Joe Duffy/ Jane Culleton	Studies on lipophilin B in breast cancer	Irish Research Council for Science, Engineering and Technology (IRSET)	€57,000	2002	2005
Prof. Joe Duffy/ Dr. W. Gallagher (Pharmacology Dept, UCD)	Functional genomics in bvreast cancer: a focus on metastasis.	Irish Cancer Society	€209,000	2002	2005
Prof. Joe Duffy	Epigenetic profiling of breash cancer: prognostic and therapeutic application	EU Sixth Framework Programme	€2,533,758 total €186,720 to SVUH/UCD	2004	
Dr. Leonie Young	Breast Cancer Research Portal	Health Research Board, Infrastructure	€187,000	2004	
Dr. Leonie Young/ Mr. Arnold Hill	Molecular mechanisms of survivin regulation in breast cancer	Cancer Research Ireland	€97,700	2002	2005
Dr. Leonie Young/ Mr. Arnold Hill/	Chromatin remodelling in estrogen receptor target genes in breast cancer	St. Lukes Institute of Cancer Research	€100,000	2003	2005

P.I.	Name of Study	Source of Grant	Fund Amount	Start Date	Finish Date
Dr. Leonie Young/ Mr. Arnold Hill	Role of co-regulatory proteins in estrogen receptor function in breast cancer	Presidents Award, University College Dublin	€10,000	2002	2003
Dr. Leonie Young/ Prof. T.J. McKenna	ACTH regulation of adrenal steroidogeneis	Health Research Board	€60,000	2000	2003
Dr. Leonie Young/ Mr. Arnold Hill	Estrogen receptor co-regulatory proteins in tamoxifen resistant breast cancer	Royal College of Surgeons in Ireland	€49,700	2002	2003
Prof. Cliona O'Farrelly		Westgate Biological Ltd.	€12,000	2002	2005
Dr. Margaret O'Brien (Prof. Cliona O'Farrelly)	Alternative splicing of CD1d: Generation of novel antigen presenting molecules	HRB Research Fellowship	€157,500	2002	2005
Dr. Conor O'Brien (Prof. Cliona O'Farrelly)	Cytokine Gene Polymorphisms and their association with the outcome of Hepatitis C viral infection	HRB Clinical Research Training	€89,500	2002	2005
Dr. Lucy Golden Mason/ Prof. Cliona O'Farrelly/ Prof. John Hegarty	Adult Hepatic Stem Cells	Health Research Board Post Doctoral Research Fellowship	€148,000	2001	2004
D Prof. Cliona O'Farrelly/ Prof. John Hegarty	Hepatitis C Viral Load Quantification in Lymphocyte Sub-Populations and Correlation with CD81 Expression: Implications for Treatment and Prognosis	Health Research Board Postgraduate Student Scholarship	€54,700	2001	2003
Prof. Cliona O'Farrelly/ Dr. Lucy Golden-Mason	Adult Stem Cells: Development of gdT cells in the human intestine	HRB Research Project Grant	€120,600	2001	2004
Prof. Cliona O'Farrelly/ Dr. Lucy Golden-Mason	Liver Immunology/Hepatic Lymphocyte Analysis	HRB Project Grant	€82,000		
Prof. Cliona O'Farrelly/ Prof. Dermot Kelleher/ Prof. Kingston Mills/ Dr. Derek Doherty	Characterisation Hepatitis C Induced Immunological Subversion and its Implications for Treatment Response	HRB Programme Grant	€552,000	2003	2006
Prof. Cliona O'Farrelly/ Prof. Colm O'Herlihy/ Dr. Lucy Golden-Mason	Cytokine Milieu, Natural Killer Receptor Positive Cells, Haematopoietic Progenitors in Human Endometrium	Enterprise Ireland Basic Research Grant Programme	€149,293	2003	2006
Prof. Cliona O'Farrelly	A combined bioinformatic, molecular and biochemical approach to the identification and analysis of antimicrobial peptides in Hen Eggs	Dept. Agriculture FIRM Grant	€671,437	2001	2005
Prof. Cliona O'Farrelly	Antimicrobial activity in Hen Eggs, a Combined Bioinformatic, Biochemical and Molecular Biological Approach	Dept. Agriculture Extension to FIRM Grant	€55,000	2004	2005
Prof. Cliona O'Farrelly	Functional immunogenomics in the chicken: relevance to avian influenza virus infection	Dept. Agriculture FIRM Grant	€691,286	2004	2007

P.I.	Name of Study	Source of Grant	Fund Amount	Start Date	Finish Date
Prof. Cliona O'Farrelly/ Prof. John Hegarty	Educational Grant	ROCHE Pharmaceuticals	€50,000	2004	
Prof. Cliona O'Farrelly (with S. Pennington & M. Dunn, Conway Institute)	Biomarkers of drug responsiveness in chronic hepatitis C virus infection	InVitrogen Life Technologies Research Grant	€96,000	2004	
Prof. Cliona O'Farrelly	Organ-specific molecular and cellular pathogenic mechanisms	Health Research Board Equipment Grant	€54,450	2004	
Prof. Cliona O'Farrelly/ Dr. B. Watson/Prof. D. Reen	Flow Cytometry: applications for cellular function and differentiation	Health Research Board Equipment Grant	€180,518	2004	
Prof. Cliona O'Farrelly	Identification and characterisation of hepatic features prognostic of disease progress in transplant rejection and Hepatitis C	Liver Trust SVUH	€170,000	2004	
Prof. Cliona O'Farrelly/ Prof. Hugo Rosen	Hepatitis C Immunity in the Human Liver Transplant Model	U.S. Dept. of Veterans Affairs	€580,000	2004	
Dr. Tom Smith/ Prof. T.J. McKenna	Structure and Etiology of Macroplactin	Health Research Board	€110,000	2002	2005
Prof. W. McNicholas/ Dr. A. Bradford/ Dr. P. Nolan	Pathophysiology of obstructive sleep apnoea	European Union	€280,000	2000	2003
Prof. W. McNicholas/ Dr. Philip Nolan	Pathphysiology of sleep apnoea	Health Research Board	€105,000	2000	2003
Prof. W. McNicholas/ Dr. Geraldine Connolly/ Dr. Peter McKenna	Sleep disordered breathing in pre-eclampsia	Friends of the Rotunda Hospital	€25,000	2001	2003
Prof. W. McNicholas	Sleep studies in COPD	Glaxo Smith Kline	€105,000	2001	2004
Prof. Diarmuid O'Donoghue/Alan Baird	Colonic Adhesion and Inflammation	Health Research Board	€74,000	2001	2003
Dr. Jacintha O'Sullivan/ Prof. Diarmuid O'Donoghue	The role of genomic instability in promoting colorectal cancer development in Ulcerative Colitis patients	Health Research Board	€185,000	2004	2007
Dr. Jacintha O'Sullivan/Prof. Diarmuid O'Donoghue/ Dr. Kieran Sheahan	The role of telomeres and telomerase in colorectal cancer growth, progression and metastasis.	Cancer Research Ireland	€123,000	2004	2007
Colorectal Group		Fundraising monies for research studies	€100,000	2004	
Dr. Hugh Mulcahy	Protease profiling in early Colorectal Cancer and its association with chromosomal abnormalities, anatomical stage and clinical progression.	Cancer Research Ireland	€135,000	2003	2006

P.I.	Name of Study	Source of Grant	Fund Amount	Start Date	Finish Date
Dr. Kieran Sheahan	Molecular analysis of colorectal cancer: its role in detecting familial cancer and predicting response to chemotherapy	Health Research Board	€122,000	2001	2004
David Gibbons	Predicting Response to Neo-Adjuvant Therapy in Rectal Cancer Using Multiple Biomarkers	St. Lukes Institute for Cancer Reserach	€90,000	2002	2004
Dr. U. Fearon/Dr. D. Veale/ Prof. B. Bresnihan	The mechanistic role of A-SAA in regulating angiogenesis and cartilage metabolism in Arthritis	SFI Project Grant	€122,000	2004	
Dr. D. Veale/Dr. U. Fearon/ P. Cahill	The role of angiopoietins in inflammation	HRB Project Grant	€165,000	2004	
Dr. R. Mullan/Dr. U. Fearon/Dr. D. Veale	The clinical applications of anti-TNF therapy and cartilage Metabolism	HRB Project Grant	€109,000	2004	
Dr. C. Walsh/ Prof. B. Bresnihan	Withdrawal of anti-TNF therapy in patients with rheumatoid arthritis in clinical remission: can maintenance of remission be predicted?	HRB Project Grant	€160,000	2004	
Dr. U. Fearon/Dr. D. Veale	Cytokines in rheumatoid arthritis patients treated with anti-TNF therapy	Glaxo Smith Kline plc	€150,000	2004	2005
Dr. U. Fearon/Dr. D. Veale	The effect of cytokine therapy in endothelial cell function	Glaxo Smith Kline plc	€100,000	2004	2005
Dr. D. Veale	Studies of patients undergoing anti-cytokine therapy	Pfizer Clinical Fellowship	€50,000	2004	
Prof. O. FitzGerald	Differential expression of VEGF and PlGF and VEGF receptors in inflammatory arthritis	HRB Project Grant	€165,000	2004	2007
Dr. D. Veale	Protein analysis equipment	HRB Equipment Grant	€7,000	2004	
Prof. O. FitzGerald	Gel documentation equipment	HRB Equipment Grant	€7,000	2004	
Prof B. Bresnihan	Role of cytokines and growth factors in cartilage destruction in osteoarthritis	EU Fifth Framework Programme Research Grant	€190,460	2000	2003
Prof B. Bresnihan	Synovial pannus evaluation and Cytokine-targeted therapy in Rheumatoid arthritis	Amgen	€655,208	2001	2004
Prof O. FitzGerald	Kineret in psoriasis and paoriatic arthritis: a single center, open label pilot study	Amgen	€90,000	2002	2005
Dr. D. Veale	Angiogenic growth factors in Psoriasis before and after treatment with Anti TNF Therapy	Health Research Board	€129,640	2002	2004
Prof. O. FitzGerald	North/South Grant	Health Research Board	€15,655	2002	2003
Prof. O. FitzGerald	The profibrotic potential of ankylosing spondylitis synovial membrane	Health Research Board	€100,000	2001	2003

P.I.	Name of Study	Source of Grant	Fund Amount	Start Date	Finish Date
Prof. O. FitzGerald	T cell receptor analysis in seronegative Arthritis: evidence for persistence of oligoclinal T cells in remission tissue	Health Research Board	€148,560	2000	2004
Prof. O. FitzGerald	Etanercept in psoriasis and psoriatic arthritis: a single center open label pilot study	Wyeth	€120,000	2004	2006
Prof. O. FitzGerald	The "Switch" Study	Abbott	€60,000	2004	2005
Dr. D. Veale	The role of cytokine regulation of RA and the effect of TNF therapy	Pfizer	€155,000	2001	2003
Dr. D. Veale	Etanercept in psoriasis and psoriatic arthritis: a single center open label pilot study	Wyeth	€112,00	2002	2004
Prof. B. Bresnihan	Withdrawal of anti-TNF therapy	Health Research Board	€175,428	2004	2006
Dr. C. Gallagher	Continuous positive airways pressure (CPAP), Intrinsic PEEP and exercise tolerance in cystic fibrosis	CF Association of Ireland	€46,472	2000	2003
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Psychological Stress, Optimism and Immune System	Craig Dobbin Newman Scholarship	€85,000	2004	2006
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Brain Imaging Studies in Suicidal Depression	AFSP Distinguished Investigator	€100,000	2004	2006
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Brain Imaging Studies in Suicidal Depression	National Neuroscience Network	€160,000	2004	2006
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Suicide and other causes of mortality in Medical Practitioners in Ireland 1978-2005	HSE/National Suicide Review Group & Irish Medical Council	€30,000	2004	2005
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Suicide and other causes of mortality on Irish Railways	Iarnrod Eireann	€30,000	2004	2005
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Suicide and other causes of mortality in Medical Practitioners in Ireland 1978-2005	Irish Medical Council	€20,000	2004	2005
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Suicide and other causes of mortality in An Garda Siochana	Health & Safety Authority/ National Suicide Research Review Group	€15,000	2004	2005
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Molecular Genetic Factors Associated with Suicidal Behaviour	Astra Zeneca Suicide Research Fellow	€30,000	2004	2005
Prof. Kevin Malone Dept. Psychiatry & Mental Health Research	Turning the Tide of Suicide	3T's Suicide Research Fund	€20,000	2004	2005



Departmental Review

Department of Anaesthesia, Intensive Care and Pain Medicine

Departmental Review

Staff

There are currently 19 Consultant Anaesthetists working in SVUH, 12 of these consultants have significant commitments to other hospitals. Consequently, there are only 12 whole time equivalents to staff services in the group. Among the services requiring cover are 15 operating theatres, including 1 in St. Michael's Annexe, 2 in St. Michael's Hospital and 2 in St. Vincent's Private Hospital. As was the case in 2003, staffing levels remain a problem for the department in 2004.

We would like to congratulate and welcome Dr Ingrid Browne and Dr Ola-Petter Rosaeg to the department. They have replaced Dr Edward Gallagher, who retired in 2004, and Dr Kevin McKeating, who was appointed to a new National Maternity post. The occasion of Dr Gallagher's retirement was marked by a departmental night out. We wish him and his wife Eileen, a happy retirement and thank him for his contribution over the years. We are currently waiting for approval from the HSE regarding the academic post and will be advertising as soon as possible.

Dr Slazenger also retired from St. Michael's Hospital at the end of 2004. Again we would like to thank him for his contribution to the department and extend our best wishes to him and Noreen during his retirement. We will shortly have interviews for a replacement post (2 posts) with an interest in ICU.

Service Development / Activities

The amalgamation of the two pools of NCHD for the purpose of staffing St. Michael's Hospital has worked well. Further issues, with respect to on-call cover at St. Michael's remain to be resolved.

A module of regional anaesthesia training has started under the supervision of Dr Gabrielle Iohom. This has been a great success and we look forward to the development of this much neglected aspect of anaesthesia in the future.

Activity Report – Operating Statistics 2004

Category	SVUH		St. Michael's Annexe		St. Michael's Public		St. Vincent's Private	
Dental		42		118				
Dermatology		130						
E.N.T			290	161		136		
General	2928	520	1018	1583				
G.U Endo	1623			585				
G.U Open		161	250	223	158			
Gynaecology		271	69	78	351			
Ophthalmology		296			419			
Orthopaedic		1837		270	171			
Pain			538	21				
Plastic			1041	156		157		
Thoracic	309							
Vascular	394							
Totals			9860	1177	1589	3678		




Emergencies

A total of 2,339 emergencies were carried out in SVUH and 1,298 of these were performed outside the normal working day. In 2004, 45 liver transplants were carried out in SVUH.

Outstanding / Significant Achievements

Congratulations to the following individuals who obtained their exams in 2004.

Dr Brian Alyward	Fellowship
Dr Michelle Roets	Primary
Dr Mary White	Primary

A revised series of resident merit awards is to be introduced at the end of 2005.

Two former transplant/research fellows, Dr. D.M. Honan (2001-2) and Dr. C.L. McCaul (1999-2000), were appointed to consultant posts in 2004.

Dr. J.G. Laffey, a previous fellow (2001-2), received the 2004 JMS Anaesthesia award for research conducted while based at SVUH.

Dr. John Boylan was appointed as honorary lecturer in clinical physiology at the Department of Physiology, UCD.

Significant Publications / Academic Activity

Peer-Reviewed Publications

J.G. Laffey, D. Honan, N. Hopkins, J-M. Hyvelin, J.F. Boylan, P. McLoughlin.
Hypercapnic acidosis attenuates endotoxin-induced acute lung injury.
American Journal of Respiratory & Critical Care Medicine 2004: 169: 46-56.
(This article was the subject of an accompanying editorial - Erik R. Swenson: Therapeutic hypercapnic acidosis: pushing the envelope.
American Journal of Respiratory & Critical Care Medicine 2004; 169: 8-9.)

Academic Activity

Labour Analgesia – What's New?
College of Anaesthetists Obstetric Anaesthesia Study Day - December 2004

Anaesthesia for Liver Disease
Final Fellowship Course, College of Anaesthetists - January 2004

Influence of the introduction of guidelines for the prescribing of medications pre-operatively
I. Brady, I. Browne, J. Boylan
College of Anaesthetists Annual Scientific Meeting, Dublin - May 2004

Current practice patterns in teaching hospitals for the discontinuation of OCP/HRT prior to surgery
F. Naughton, I. Browne, T. Owens
College of Anaesthetists Annual Scientific Meeting, Dublin - May 2004

Pre-operative herbal medicine uses and the anaesthetist's opinion
K. Tan, D. Harmon, I. Browne
College of Anaesthetists Annual Scientific Meeting, Dublin - May 2004

Resuscitation Training/ Developments

In 2004, education and training was provided to 1,046 staff members in the form of in-service, lectures, ward-based training and formal workshops and courses. 687 of the staff were nurses, 177 were doctors, 60 were paramedical staff, 68 were non-clinical staff and the remainder were students.

87 staff trained as ACLS Providers.
471 staff trained as Basic Life Support Providers.

An Intern Orientation Programme was presented for the second time and included Adult Heartsaver AED training. This is one of the pre-requisites for ACLS training.

Other courses include Basic Rhythm Recognition workshop, Cardiac Arrest Management workshop. Numerous lectures and in-service were presented on a variety of topics associated with resuscitation.

Ward-based training proved most popular and it is hoped to increase the number of sessions provided in 2005.

In addition:

- SVUH renewed its status as a training site with the Irish Heart Foundation.
- A database of those trained was maintained.
- A database of cardiac arrests was maintained.

- Obsolete defibrillators continue to be replaced.
- Contents of the cardiac arrest trolleys were reviewed and significant improvements made relating to equipment and drugs.
- A new cardiac arrest trolley content check list was piloted and introduced to the wards.
- Additional Laerdal pocket masks were placed in strategic locations around the hospital.
- SVUH collaborated with St Michael's Hospital to provide both ACLS and BLS Instructor training.
- Training equipment was purchased through the ACLS Account.
- Resuscitation guidelines have been reviewed and are being updated.
- Do Not Attempt Cardiopulmonary Resuscitation (DNACPR) draft guidelines are in the final stages of review.
- Cardiac arrest trolleys and defibrillators were assessed for the tender for the new building.
- A Resuscitation web page has been developed by the IT Department.
- Ongoing risk assessment and 'trouble shooting' takes place.

Cardiac Arrests

In 2004, 283 cardiac arrest calls were received by switch. Of these calls, 111 were to the Emergency Department. From these 283 cardiac arrest calls, 94 audit forms were received. The breakdown of the cardiac arrest calls detailed on the audit forms was as follows: -

- Asystole (59 cases)
- Pulseless Electrical Activity (18 cases)
- Ventricular Fibrillation / Pulseless Ventricular Tachycardia (13 cases)

Risk Management

Dr Richard Assaf has been appointed for an initial period as Clinical Risk Facilitator. He is involved in liasing with the Medical Board and Hospital Management on Clinical Risk Management issues, and teaching the NCDH staff. His role also includes reviewing all adverse drug reactions submitted to the Ethics Committee.

Vascular Access

Director Dr Alan McShane
Consultant Dr Neil McDonald

2004 was another busy year for the Vascular Access Service, with procedure numbers increasing to 421, an increase of 59 cases. Despite the huge numbers being processed, there has been little progress in the provision of dedicated theatre sessions for the service. This is far from satisfactory from a patient and doctor point of view. It can lead to anxiety, unnecessary fasting, interference with drug administration and sometimes cancellation of cases. The service needs a dedicated slot each day to continue to provide consultant delivered care.

On a positive note, an ultrasound device was purchased in 2004 to aid line placement. This should make the procedures safer and will greatly aid in the teaching of the technique.

At the request of the College of Anaesthetists, Dr McShane organised a Vascular Access workshop. This was well attended and proved to be very popular. Dr Neil McDonald was part of the faculty. A study was also undertaken to evaluate the safest and most useful methods of identifying correct venous line placement. This should be published in 2005.

South East Dublin Department of Anaesthesia (SEDDA) Report

Co-ordinator Dr Alan McShane

The South East Dublin Department of Anaesthesia was delighted to welcome the appointment of two new consultant colleagues, who will work between St. Vincent's University Hospital and the National Maternity Hospital. Dr Ingrid Browne took up her post in November 2004 and Dr Ola-Petter Rosaeg is due to commence in April 2005. A new consultant locum position was also filled by Dr Gabrielle Iohom, working between St. Vincent's University Hospital and St. Michael's Hospital Annexe. This appointment has greatly aided and increased the throughput of elective surgical patients from St. Vincent's University Hospital. The popular rotation of anaesthesia trainees has continued between the constituent hospitals. Attendance at the organised teaching sessions has improved.

Looking to the future of SEDDA, serious effort is to be expanded on rationalising the multiple night duty rosters. The busy areas will have to be better staffed and in light of recent proposals, the less busy locations will probably be best closed at night.

Finally, I would like to thank Dr Edward Gallagher for his professionalism and collegiality over the years. I wish him many happy years of retirement.

Intensive Care Unit

Medial Director	Dr Kieran Crowley
Consultant	Dr Pat Benson
Clinical Nurse Manager II	Ms Geraldine Carey

Service workload

516 patients were admitted to the ICU in 2004, an increase on recent years. 45% of patients stayed 1 day, while 39% stayed 3 days or more, similar to recent years. Average length of stay was unchanged at 6 days. Bed occupancy of 92.7% remains above internationally recommended levels.

Severity of illness has plateaued over recent years: average APACHE II score was 19, similar to last year. Similarly organ failure rates and organ support rates seem to have stabilized: for example, 69% of patients received mechanical ventilation. Overall ICU survival was 78.5%, with hospital survival of 75%.
St. Vincent's supplied an ICU nurse and registrar to staff the transfer of critically ill patients by MICAS (mobile intensive care ambulance service) on a 1:4 roster.

Staffing

Nursing staffing remained at or near to its full complement during 2004, allowing utilisation of all beds for almost all of the year.

There are two care attendants; one shared orderly, one shared technician and a shared ICU secretary. A registrar or specialist registrar rotating from the Department of Anaesthesia, available 24 hours a day staffs the ICU.

During 2004 Dr. Cathal Nolan joined the consultant staff in a locum position.

Developments

The structure of the new ICU is now completed and planning is at the phase of preparing tenders for equipment. There will be 16 beds in the new development.
During 2004, the ICU team successfully underwent peer review as part of the process of hospital accreditation.

For the majority of 2004, the ICU provided the critical care component of the Liver Transplant Programme. The year saw a record 45 transplants, with a 100% 60-day survival for elective transplants.

ICU Activity

	2004	2003	2002	2001	2000	1999	1998	1997
Admissions	516	450	471	421	469	556	551	537
Average length of stay (days)	6	6	6	5	5	3.4	3.9	4.8
Average occupancy	93%	90%	85%	89%	85%	64%	75%	92%
Elective surgical	36%	27%	30%	32%	35%	36%	43%	43%
Emergency surgical	30%	39%	31%	31%	35%	31%	30%	30%
Medical	34%	34%	39%	37%	30%	33%	27%	28%
Mechanical ventilation	69%	70%	75%	68%	67%	58%	57%	69%
Central venous access	64%	73%	81%	69%	78%	75%	65%	74%
Vasoactive infusions	20%	18%	25%	33%	33%	26%	22%	27%
Acute renal failure	10%	17%	25%	23%	24%	14%	13%	16%
Contin. renal repl. therapy	10%	16%	21%	16%	14%	10%	8%	12%
Tracheostomy	6%	13%	11%	13%	6%	6%	9%	9%
Unplanned ICU discharges	21%	25%	1.5%	11.4%	6%	5%	4%	5%
Readmissions	8.3%	2.9%	0.9%			6.3%		9.6%
APACHE II score (mean)	19	20	19	20	18	14	15	16
ICU survival	79%	82%	75%	73%	75%	81%	82%	83%
Hospital survival	75%	78%	70%	68%	70%	71%	75%	73%

Pain Medicine

Director	Dr Declan O'Keeffe
Consultants	Dr Ray Victory
	Dr Hugh Gallagher
Clinical Psychologist	Dr Rosemary Walsh
0.5 wte Physiotherapist	Mrs Martine Darcy (Mat Leave March 2004)
	Miss Jane Dawson
0.5 wte Occ. Therapist	Miss Katie Robinson (to August 2004)
	Miss Amanda Wallace (from Nov 2004)
Pain Management Programme Co-ordinator/PA to Director	Ms Judy Kelly (to May 2004)
	Ms Laura Hamilton (from July 2004)
Clinical Secretary and PA to	Ms Laura Hamilton (to July 2004)
Clinical Psychologist	Ms Sarah Brady (from July 2004)

Activity

Outpatient Clinics

Day	Consultant	Number of Sessions	New Patients	Return Patients	Total
Tuesday	Dr O'Keeffe	48	192	1,956	2,148
Tuesday	Dr Victory	4	4	5	9
Wednesday	Dr O'Keeffe	61	61	808	869
Total		113	257	2,769	3,026

Inpatients

Admissions	101
Discharges	106

Day Care

St. Mark's Day Centre	106
Extra Mural Theatre	256
Total	362

Pain Management Programmes

Type Programme	No of courses in 2004	Total Number of Patients on course
3 Week	6	57
1 Day	4	39
Pain School	1	9

Education and Training

The first chronic pain service information day for staff was held at St. Vincent's University Hospital on Global Pain Day, October 11th 2004. Lunch was provided and the Pain Team made a series of very informative presentations. Dr O'Keeffe chaired this meeting. Dr Declan O'Keeffe was President of the Irish Pain Society in 2004.

Meetings Attended by Members of the Department

(Dr Declan O'Keeffe, Dr Hugh Gallagher, Dr Rosemary Walsh)

- Site visit to Refractory Angina Clinic in Liverpool.
- Site visit to the Integrated Pain Centre in Denver, Colorado in December 2004.
- Attendance at a one-day workshop on PASHA CATH in Hamburg.
- Attendance at 'Basic Science Meeting in Pain Management', Haifa, Israel July 2004.
- EFIC Barcelona September 2004.
- Irish Pain Society Meeting, Croke Park, Dublin October 2004.
- Implantation Workshop, Brussels, November 2004.
- British Pain Society Meeting in Glasgow March 2004.
- Pan European Pain Specialists (PEPS) Sweden May 2004.

Papers from the Pain Service were presented at:

- The Irish Pain Society.
- The Greek Pain Society.
- Association of Anaesthetists of United Kingdom and Ireland.
- Lecture to Marrion ICGP Faculty March 2004 on Opioid Management of Chronic Pain.
- Lecture to Higher Diploma Peri-anaesthesia Nursing, UCD, December 2004.
- Annual Research Conference for the Psychology Society of Ireland (November 2004).

Publications

O'Connor TM, Swords R, O'Keeffe D, McNicholas WT.
Improved ventilatory function associated with relief of chest pain in a patient with Kartagener's syndrome treated by subcutaneous analgesia

Future Plans

The new building is expected to open at the end of 2005. Following decanting to the new ICU and theatres (8), phase 1A will begin. This will result in a new day surgery ward and 2 day theatres (12 theatres in total). Additional staff are required to facilitate these developments. The day ward is expected to have a new pre-operative assessment clinic, with 2 assessment areas side by side. Again, additional appointments will be required to staff the assessment clinic and day theatres. Finally, it is hoped to have an acute pain nurse appointed in the coming six months. This development will represent a huge step forward for SVUH, in its dealing with acute pain in the hospital.



Department of Cardiology

Departmental Review

Overview

2004 was a busy year for the Cardiology Unit. The long awaited Catheterisation Laboratory development was started at the end of the year and should be completed by April 2005. The clinical service continues to make significant demands on facilities. The ongoing problem with shortage of Coronary Care Nurses resulted in continuous closure of three beds within the Unit. Fortunately, these are being very efficiently used as day care beds for transfer of patients from St. Columcille's and St. Michael's Hospital.

Service Developments/Activities

Heart Failure Unit

3417 patients visited the outpatient department based in St. Michael's Hospital, Dun Laoghaire.The service provides structured out-patient follow up of patients discharged from hospital. In addition, a new patient clinic is run weekly to allow for immediate assessment of new patients diagnosed in the community. There is also a continued expansion of a community direct access programme where patients can be seen immediately if clinical deterioration is suspected. This now comprises approximately 20% of the unit workload.

GP Open Access Clinic

The Open Access service continues to be a success. This is an out-of-hours non-invasive diagnostic service for GPs. Statistics for this service are as follows:

	No of Procedures
Echocardiogram	118
Exercise Stress Test	110
Blood Pressure	72
Holter Monitor	101
Event	1

The development for chest pain evaluation is ongoing. Protocols for chest pain evaluation have been developed in conjunction with the Emergency Department.

The non-invasive service continues to work at 110%. The acquisition of a new Echo machine at the end of 2004 has improved diagnostic abilities. Thanks and recognition must be given to Siobhan Toomey, Department Secretary and Liz O'Reilly, Cardiac Technician who left this year. Statistics for the Cardiac Department are as follows:

	No of Procedures
ECG	8093
Exercise Stress Test	1273
Echocardiogram	3144
Pacemaker Check	1479
Event Monitors	59
Holter Monitors	1525
Transoesophageal Echocardiogram	357

Cardiac Catheterisation Laboratory

	No of Procedures
Angiography	
Coronary and left ventricular (including grafts)	1565
Coronary Angioplasty	412
Pacemakers	201
ICD and biventricular pacemakers	36
Other Procedures*	53
Total Procedures	2267

*includes 37 right heart catheters for Liver Transplant Service.

Plans for 2005

Plans for 2005 will see the opening of the new Catheterisation Laboratory. The Chest Pain Evaluation Unit should be fully functional. The further development and integration of information technology in the Department will also proceed and plans are in development for the introduction of a full audit of the clinical duties within cardiology. Direct angioplasty service for in-patients with acute myocardial infarction is operating on a limited basis within normal working hours and it is hoped to provide a 24 hour service by the end of the year.

Publications

Aspirin dose and six-month outcome after an cute coronary syndrome.

Quinn MJ, Aronow HD, Califf RM, Bhatt DL, Sapp S, Kleiman NS, Harrington RA, Kong DF, Kandzara DE, Topol EJ. J Am Coll Cardiol.

2004;43(6):972-8.





Effect of clopidogrel pretreatment on inflammatory marker expression in patients undergoing percutaneous coronary intervention.
Quinn MJ, Bhatt DL, Zidar F, Vivekananthan D, Chew DP, Ellis SG, Plow E, Topol EJ.
Am J Cardiol. 2004;93 (6):679-84.

Molecular and functional differences induced in thrombospondin –1 by the single nucleotide polymorphism assoiciated with the risk of premature, familial myocardial infarction.
Narizhneva NV, Byers-Ward VJ, Quinn MJ, Zidar FJ, Plow EF, Topol EJ, Byzova TV.
J. Biol Chem. 2004;279(20):21651-7.

The prevalence and natural history of anaemia in an optimally treated heart failure population.
Enda Ryan, Maeve Develin, Terence Prendiville, Mark Ledwidge, Kenneth McDonald.
British Journal of Cardiology. 2004, Vol. 11 (5), 369-375.

Poor sensitivity of weight and brain natriuretic peptide changes in pre-clinical deterioration of Heart Failure.
J. Lewin, C. O'Loughlin, M. Ledwidge, K. McDonald.

Telemonitoring in a Heart Failure Unit: the nature and value of patient-initiated contacts.
C. Conlon, C. O'Loughlin, M. Ledwidge, K. McDonald.

Abstracts and Presentations

- Poor sensitivity of weight and BNP changes in predicting clinical deterioration of Heart Failure. J. Lewin, C. McNally, C. O'Loughlin, M. Ledwidge, K. McDonald.
- Fluid restriction does not alter time to clinical stability following emergency admission for NYHA class IV heart failure. B. Travers, C. O'Loughlin, M. Ledwidge, K. McDonald.
- BNP and ECG are essential tools for General Practitioners to enable more effective triage in the assessment of potential new diagnosis of Heart Failure. B. Boilson, P. Chakravarthi, C. O'Loughlin, M. Ledwidge, K. McDonald.
- Emerging trends in patients profile in a hospital – Based Heart Failure Unit: Increasing Burden of Unscheduled Clinical Review. P Chakravarthi, C O'Loughlin, M Ledwidge, K McDonald.
- The Practice of 2 hours observation following escalation of beta blocker therapy is not necessary: a review of 198 titration. B. Travers, C O'Loughlin, C, Conlon, M Ledwidge, K. McDonald.

Department of Dermatology

Departmental Review

The Department of Dermatology at St Vincent's University Hospital together with Hume Street and St Michael's Hospitals provide dermatology services for patients living in Dublin South-East and Wicklow.

Staff

Consultants	Dr Paul Collins and Dr Sarah Rogers.
Specialist Registrar	Dr Sinead Langan (SpR3/4) January-December (St Vincent's/St Michael's)
Registrar	Dr Rupert Barry January-April
(Hume St/St Vincent's)	Dr Khasif Ahmed July-December
Senior House Officer	Dr Debbie Fitzgerald May-June (locum)
Clinical Nurse Specialist	Ms Sheila Ryan
Administrative Assistants	Mrs Eileen Cusack, Mrs Brenda Harte (both half time)

Clinical Service

Inpatients

Eleven patients were admitted as urgent cases. Twenty patients with severe dermatoses remained under the care of the admitting physician but required significant dermatology care. Patients with chronic stable skin conditions, including psoriasis and atopic eczema, requiring five day elective inpatient dermatology nursing, were admitted to Hume St Hospital (250 patients).

288 patients at St Vincent's University Hospital and 50 patients at St Michael's Hospital had inpatient dermatology consultation.

Day-Care

Extramural theatre

Day case surgery, predominantly for non-melanoma skin cancer and atypical melanocytic naevi, was performed for 129 patients at extramural theatre.

St Mark's Ward

There were 59 patient episodes for infusions of infliximab and immunoglobulin as day case patients.

Dermatology day-care unit

Narrowband UVB phototherapy, PUVA therapy, patient education, cryotherapy, dressings including scalp therapy were provided by Ms Sheila Ryan and 2,672 episodes were recorded.

Outpatient Clinics

There were 99 St Vincent's dermatology clinics at which 2,333 patients (751 new referrals) were treated. There were 49 St Michael's dermatology clinics at which 782 patients (310 new referrals) were treated. Outpatient investigation and therapy including cryotherapy, diagnostic punch biopsy, four layer bandaging, wart dressings/treatments, wound care and light cautery for macrocomedones was provided for 492 patients at SVUH and 224 patients at SMH.

Patients were referred to Hume St Hospital for photodynamic therapy, patch testing and phototherapy/PUVA and there were 10,760 patient episodes. There were 461 new and 2,723 return patients seen at clinics in the outpatients and in daycare.

Postgraduate and Undergraduate Education

Higher Specialist training for specialist registrars in dermatology and General Professional training for senior house officers was provided at St Vincent's and Hume St hospitals. There was a weekly postgraduate dermatology session with dermatologists from St James's and Adelaide/Meath hospitals comprising journal club and combined clinical case meeting at Hume Street on Wednesday mornings. The fortnightly histopathology meeting took place at St Vincent's University Hospital.

Fifth year medical students joined the Dermatology team for two-week rotations between St Vincent's and Hume St Hospitals. Ms Sheila Ryan taught third year student nurses on atopic eczema, psoriasis and skin cancer.

Conferences/Courses attended

Dr Rogers

British Association of Dermatologists Annual Meeting, Belfast.
Irish Association of Dermatologists meetings, Craigavon and Cavan.
American Academy of Dermatology, Washington.






Dr Collins
Irish Association of Dermatologists meetings, Craigavon and Cavan.
British Association of Dermatologists annual meeting, Belfast.
European Academy of Dermatology and Venereology, Florence.

Ms Sheila Ryan:
British Dermatology Nursing group annual meeting, Belfast.
Irish Red Cross annual meeting re cosmetic camouflage service.

Publications

Modified surgical excision for the treatment of chondrodermatitis nodularis.
Ormond P and Collins P. Dermatol Surg 2004;30:1-3.

Brachioradial pruritus – an enigmatic entity.
Barry RBM and Rogers S., Clin Exp Dermatol 2004;29:637-638.

A guide to cryotherapy Ryan S., Irish Nursing Journal 2004

Presentations

British Association of Dermatologists Annual Meeting, Belfast 2004.

Vascular endothelial growth factor and angiopoietins stimulate endothelial cell proliferation and matrix degradation:pathogenic role in psoriasis.
T Markham, R Mullan, L Golden-Mason, S Rogers, B Breshihan, O Fitzgerald, DJ Veale and U Fearon. (Registrar's symposium) Br J Dermatol 2004:151 (Suppl 68):12-13.

Nodular extramammary Paget's disease of the penis.
SM Langan, A O' Brien, D Fennelly, D Mulvin and P Collins.
(Clinicopathological case) Br J Dermatol 2004:151 (Suppl 68):17.

Topical anaesthesia is effective for pain of topical 5-aminolaevulinic acid photodynamic therapy. M O' Kane and P Collins. (poster) Br J Dermatol 2004:151 (Suppl 68):25.

Brachioradial pruritus: a heterogenous condition that is difficult to treat.
RBM Barry and S Rogers. (poster) Br J Dermatol 2004:151 (Suppl 68):32.

Royal Society of Medicine, Dermatology section, London.

Cutaneous sarcoid with granulomatous vasculitis.
RBM Barry and S Rogers. January 2004

Variegate Porphyria in an Irish family with a new genetic defect.
S Langan and S Rogers. December 2004

Irish Association of Dermatologists meeting, April 2004, Craigavon.

Staphylococcal scalded skin syndrome (poster) S Langan and S Rogers.

Royal Academy of Medicine in Ireland, Dermatology Registrars clinical case prize meeting. February 2004.

Nodular extramammary Paget's disease of the penis.
SM Langan, A O' Brien, D Fennelly, D Mulvin and P Collins.

Pyoderma gangrenosum responds rapidly to infliximab.
RBM Barry, K Sheahan, D O' Donoghue, M Barry and P Collins.

Notalgia paraesthetica
M Alwari and S Rogers

Royal Academy of Medicine, Dermatology section clinical meeting

Sixteen clinical cases were presented.

Achievements

Dr Rogers and Dr Collins hosted the Royal Academy of Medicine, Dermatology section clinical meeting in November at St Vincents University Hospital. Dr Sean Whittaker Consultant Dermatologist, St John's Dermatology Unit at St Thomas's Hospital, gave the guest lecture on Cutaneous Lymphomas.

Ms Sheila Ryan has been elected secretary of the Irish Dermatology Nursing group and Irish representative on the Council of the British Dermatology Nursing group. Sheila also started a Masters of Nursing Science (advanced practice) at University College Dublin.

Dr Sinead Langan (SpR4) has gone to the Queen's Medical Centre Nottingham to undertake a clinical fellowship in dermatology with Professor Hywel Williams.

We have received approval to advertise a new Consultant Dermatologists post at St Vincent's University Hospital.

Department of Emergency Medicine

Departmental Review

Staffing

During 2004, the Emergency Departments in St.Vincent's University Hospital (SVUH) and St. Michael's Hospital (SMH) were dominated by plans to deal with patient overcrowding, and the challenges of peer review accreditation, quality improvement plans and performance indicators. The regional approach to emergency care incorporating St. Columcille's Hospital was further enhanced. Trauma by-pass in conjunction with the in house trauma team continues to prove its value as does the direct access availability for emergency vascular patients. A number of new staff joined the Department; Margaret Boland was appointed Divisional Nurse Manager for Ambulatory Day Care, Emergency Department and Infection Control and Mary Hayes appointed Clinical Nurse Manager 3 for the Emergency Department. Louise Morgan was appointed as a social worker dedicated to the Emergency Department.

Service Developments

The accurate streaming of patients underpins the future development of Emergency Medicine and allows rapid and appropriate patient assessment and supplies a more customer orientated health care service. This is made possible by Triage and Rapid Assessment and Treatment.

The Rapid Assessment and Treatment service (RAT), having been formally piloted in December 2003 got underway in January 2004. With its introduction came an improvement in patient streaming, a reduction in the patient to health care professional waiting time and also a reduction in the number of written complaints. Margaret Boland and Allen Doyle were awarded a poster presentation prize at the National Council for Professional Development for Nurses and Midwives in November 2004, for their poster entitled "Proactive and Professional Development to Support Change: Reorganisation of Service Delivery in an Emergency Department, Rapid Assessment Treatment". A second poster was also presented on 'Reorganisation of Service Delivery in an Emergency Department' by Allen Doyle, and Margaret Boland.

The Minor Injury Unit is a separate operational stream with its own dedicated staff. It deals with approximately 40% of department attendees. Patients are seen and assessed rapidly with an overall admission rate of 5%. Having a dedicated radiological unit will increase efficiency in the future.

A Clinical Decision Unit is planed for the assessment of patients with rule in rule out complaints according to agreed multi-disciplinary protocols. This should allow certain patients to be treated on an outpatient basis obviating the need for inpatient investigation. Other protocols have already been developed in conjunction with colleagues to allow outpatient management of thrombo-embolic disease, cellulitis and transient ischaemic attacks. Respiratory care pathways and close liaison with St Michael's Hospital respiratory unit allows the admission of certain respiratory patients from the emergency department to the St. Michael's respiratory unit.

A Chest Pain Evaluation Unit has been further developed with the Department of Cardiology. This allows the protocol driven management of chest pain patients with same day exercise stress testing where indicated. Combined with the continuing monitoring of departmental thrombolysis and increasing access to cardiac catheterisation, there has been an improved outcome for cardiac patients and a shorter hospital stay.

We are grateful to our colleagues who have allowed ready access to diagnostic imaging and laboratory services. This combined with near patient testing for blood gasses, cardiac markers and drugs of misuse has improved patient care where time critical interventions are indicated. A departmental ultrasound machine has been purchased for focused abdominal sonography in trauma and will further improve timely patient care.

The I.T. Department has further enhanced the computerisation of the department and improved networking with radiology, laboratory and patient administration systems. This will allow order communications and improve the collection of information and facilitate the planning of services. New medical records notes have been introduced, these are patient centred and allow integration of detailed medical and nursing notes and facilitate co-ordinated patient handover to speciality teams. Document imaging will further enhance record keeping.

Mr. John Ryan has the responsibility for medical education. The mandatory two-day skills based induction course for new SHOs has been of immense benefit. This is supported by continuing formal on site education and off site education at





www.emed-induction.ie. We continue our commitment to evidence based medicine and risk management with a robust multi-disciplinary audit programme and a critical incident monitoring programme. The two specialist registrars have contributed significantly to the functioning of the emergency department and have participated in valuable secondments in cardiology and orthopaedics. We are grateful to our colleagues in these specialities for their support. In September, Mr. Derek Barton directed a very successful RCSI accredited Advanced Trauma Life Support course in SVUH. He has also being involved in many ACLS courses.

Mr John Ryan was a member of a Department of Health Influenza Pandemic Expert Group which helped develop a national strategy for managing a Pandemic outbreak of Influenza.

Professional Nursing Development, underpinned by the Scope of Professional Practice continues with Venepuncture and Cannulation Training, and Manchester Triage. Jackie Brennan and Joanne Farren attended the Leading and Empowered Organisation Programme. Trauma Nursing Care and Advanced Cardiac Life Support courses were attended by nursing staff during the year.

Accreditation

The Emergency Departments at SVUH and SMH have explored new ways of providing emergency health care. These are not once off initiatives, but part of a concerted forward thinking plan to provide optimal emergency care with the patients' needs central to the plan. A number of these initiatives have been developed in close liaison with other specialties. The emphasis has been upon providing the right care for the right patient, at the right time and by the right person. These initiatives have been predicated upon rapid access to diagnostic tests, introduction of senior staff earlier in the decision making process, balancing variations in patient attendance with medical and nursing rosters and avoiding unnecessary return visits.

These initiatives have been recognised in the Accreditation Peer Review Survey carried out in May. This survey identified strengths and weaknesses within the department and allowed us to develop Quality Improvement Plans based on Performance Indicators. Among these Performance Indicators would be Door to Catheter Laboratory time, Registration to Health Care Professional time, Door to Analgesia time and prioritising use of the Rapid Assessment Team. Margaret Boland and Derek Barton are co-team leaders of the Emergency Medicine/General Internal Medicine Quality Improvement Team.

Significant Achievements

Jackie Brennan was awarded the Cecil King Memorial Prize for outstanding contribution to Emergency Nursing in February 2004.

Jerry Kearns commenced an MSc in Nursing Advanced Practice in UCD in September 2004

Brian Hayes, a 4th year medical student, won the Watts medal for the best Health Research Board funded student project in 2004. His project involved developing a computerised database for the real time collection of data following out of hospital cardiac arrest using the Utstein criteria.

Mr John Ryan has been awarded an EEC grant to develop a distance learning prototype, using virtual reality training in disaster management, in collaboration with a number of other European countries.

Publications

Ryan J,
Emergency Department overcrowding.
Canadian Journal of Emergency Medicine 2004;6:395

McKenna A, Nordt SP, Ryan J. Acute *Nutmeg Poisoning*
Eur J Emerg Med. 2004; 11(4):240-1

Presentations

Ryan J Clinician Impairment. Invited Presentation at the 10th International conference on Emergency Medicine Cairns June 2004

Ryan J, Barton D, Boland M. Emergency Department Accreditation: A framework for quality improvement. Presented at the 10th International conference on Emergency Medicine Cairns June 2004

Ryan J, McGrath L. Brain Natriuretic Peptide as a Near Patient Test for Distinguishing Cardiac from Non-Cardiac Causes of Dyspnoea in an Emergency Department. Presented at the 10th International conference on Emergency Medicine Cairns June 2004

Ryan, S.Feeney. Cocaine Associated Chest Pain, Prevalence in an Urban Irish Emergency Department. A Poster Presentation presented at the 10th International conference on Emergency Medicine Cairns June 2004

J Ryan, J Curran, M Stewart, 'Tag Rugby' Injuries - Presentations from a Non-contact Sport to an Emergency Department. A Poster Presentation presented at the 10th International conference on Emergency Medicine Cairns June 2004

O Sullivan JM, Ryan J, Boilson B, Cunningham S. Ischaemic modified albumen in the emergency department. Does it have a role in determining the disposition of patients presenting with cardiac sounding chest pain ? A Poster Presentation presented at the 10th International conference on Emergency Medicine, Cairns June 2004

Roe B, Ryan J. Cardiac Arrest Numbers – A National Lottery ? A Poster Presentation presented at the 10th International conference on Emergency Medicine Cairns June

Statistics

There were 33,407 new attendances and 4546 review attendances during 2004. Of the attendances, the triage categories were I (1%), II (18%), III (47%), IV (33%) and V (0.5%). There were 7984 admissions an admission rate increase of 23.9%. As evidence of overcrowding a new statistic was collected during the year, "patients awaiting beds following completion of assessment".

Future Plans

The main focus for 2005 will be the move to the new building and reorganising services within the Emergency Department. Service development will be introduced as per provider plan for 2005 –2007.

It is hoped to introduce a Clinical Decision Unit adjacent to the ED early 2005. Jerry Kearns commenced an MSc Nursing Advanced Practice in UCD on Chest Pain Evaluation will focus on further developing the Chest Pain Evaluation Unit.

With the reorganisation of Mental Health Services within the area it is planned to introduce a Liaison Psychiatric Nurse Service as part of the service development. As part of an overall improvement in community liaison and strengthening links with outside providers we will be improving bi-directional flow with the local GP Co-operative, enhancing the role of the GP Liaison Nurse and introducing a Care of the Elderly Nurse.

The Regional approach to Emergency Medicine, incorporating a local response for local needs, will be enhanced with the development of a revised role for St. Michaels Hospital including a CDU, dressing clinic and nurse led minor injury unit. A telemedicine service linking the three hospitals will allow senior support and specialty advice for doctors and nurses in St. Michael's and St. Columcille's Hospitals.

Expanding the role for nursing should include nurse x-ray requesting and nurse prescribing for certain medications. There is also a Continuing Management Development Programme for all Shift Leaders and CNM 2 Staff.

Department of Endocrinology and Diabetes Mellitus

Staff

Consultants Professor T.J. McKenna, Dr Malachi McKenna, Dr Donal O'Shea Lecturers, Department of Medicine,U.C.D.: Dr James Gibney and Dr Diarmuid Smith

Specialist Registrars Dr Kevin Moore, Dr Chong Yew Tan

Registrars Dr Montasir Hammam, Dr Sally Daly

Research Registrars Dr Twani Tuthill, Dr Tom Cawood

Diabetes Centre Nurses Ms Deirdre Gleeson, Ms Moira Haran, Ms Nora Collis, Ms Phil Sharkey, Ms Joanne O'Neill, Ms Maureen Quill,

Primary Care Liaison Ms Deirdre Hall

Dieticians Ms Maeve Moran; Primary Care Liaison, Ms Yvonne Ryan

Diabetes Liaison Psychiatrist Dr. Eamon Kenny

Secretarial Staff Ms Geri Daly, Ms Patricia Sinnott, Ms Brenda Sullivan, Ms Marie McCarrick, Ms Joyce Doyle, Ms Catherine Brady, Ms Fiona Whelan, Ms Michelle Keane, Ms Heather Bailey.

Endocrine and Diabetes Research Nurse Ciara O'Dwyer.

Endocrine Laboratory Dr. Tom Smith, Principal Biochemist; Anne Dickinson,

Senior Medical Scientist Anne-Marie Glynn, James Connolly, Martina Cassidy, Paddy Doran, Sheila McKeon

Research-Graduate Students Sinead Kelly and Lucille Kavanagh.

Weekly Departmental Academic Events

Monday lunchtime	–	Diabetes Team Meeting
Monday - 2.00 p.m.	–	Laboratory/Research Meeting
Tuesday Noon	–	Endocrine Conference
Friday – 11.00 a.m.	–	Departmental Journal Club

Outpatient Clinics

3 Diabetes Clinics per week

1 Joint Endocrinology Clinic (Prof. McKenna and Dr. O'Shea)

Weekly multidisciplinary group education sessions for newly diagnosed patients with Type 2 diabetes.

Monthly group education session on cholesterol management and diet.

Quarterly group education sessions for patients with newly diagnosed impaired glucose tolerance.

Service Developments/ Activities

Diabetes Service

The Diabetes Service is based on the combination of formal clinical consultation, one to one and group problem solving, education and counselling sessions delivered by Diabetes Nurse Specialists and the Diabetes Centre Dietician, Maeve Moran. This system is further supported by a telephone contact service which has grown in response to need over several years. During this year an audit of this service was set in place and there are plans to develop a more formal and efficient telephone-based service. Dr Kevin Moore and Dr Sally Daly undertook an audit to assess the service performance with regard to achieving treatment targets for diabetic patients.

During the year continuous blood glucose monitoring was introduced which will be helpful in assessing the possibility of silent hypoglycaemia inpatients with type 1 diabetes particularly.

While there has been a general increase in Diabetes Centre activities, patient encounters have increased by 25% over the last year. Clearly, additional personnel will be required to serve continued growth at this level.

Endocrine Service

The Endocrine Service is delivered on an almost exclusively out-patient basis in line with international trends. Particular expertise exists in the area of thyroid disorders, disorders of reproductive endocrinology, disorders of the adrenal glands and pituitary. Disorders of metabolism, including obesity, metabolic bone disease and calcium disorders all enjoy a national profile. With the critical co-

operation of the Radiology Department and the Endocrine Laboratory, investigation of hypoglycaemia, and particularly the search for insulin-secreting pancreatic tumours, has been developed. This essentially involves arterial catherisation of blood vessels feeding the various sections of the pancreas with infusion of calcium and simultaneous sampling of blood from the portal venous system insulin secreting tumours too small for detection by conventional imaging may be localised when calcium stimulates the insulinoma to secrete insulin. This investigation has attracted national attention.

The Endocrine Laboratory experienced an overall 9% growth rate. The Endocrine Laboratory continues to provide HbA1c measurement for diabetes patients, in addition to a full array of steroid and glycoprotein measurements.

Research Projects

1. *Control of Adrenal Steroidogenesis: (Ms Sinead Kelly)*
   Sinead Kelly is currently investigating the role of these orphan nuclear receptors and their co-activator proteins in regulating key steroidogenic enzymes and subsequent production of cortisol and adrenal androgens.

2. *Characterisation of Macroprolactin: (Ms Lucille Kavanagh)*
   (a) Findings suggest an autoimmune susceptibility in macroprolactinaemic patients.

   (b) The much reduced bioactivity apparent in vivo may be due to non-bioavailability rather than intrinsic inactivity of prolactin in macroprolactin.

3. *Androgen-secreting Tumours: (Dr D Smith)*
   The importance of the findings in this study is that it supports the policy of restricting comprehensive evaluation of patients presenting with signs of androgen excess to a relatively small percentage of patients with a high likelihood of identifying a disorder other than polycystic ovary syndrome.

4. *Bone Disease: Thyroid Disorders:*
   (Dr D Smith, Ms Barbara Murray, Metabolic Laboratory)
   The usefulness of indices developed in the Metabolic Laboratory for the assessment of bone turnover and bone remodelling balance have also been assessed and applied to patients with Paget's Disease, hyperparathyroidism and eating disorders.

5. *Bone Biomarkers in the Assessment of Breast Cancer Patients:*
   (Dr J Gibney, Ms Barbara Murray, Metabolic Laboratory, Dr D Smith)
   Over 100 patients have entered in a study, whereby the sensitivity of bone biomarkers for the detection of bone metastases in breast cancer patients with normal bone scan is ongoing. This study will take several years to yield a result.

6. *Thyroid Eye Disease: (Dr T Cawood)*
   Dr Cawood will examine the inflammatory cytokines that mediate the early stages of this condition in the hope that more targeted treatments can be developed to interrupt the process.

7. *Diabetes*
   Clinical trials are currently being conducted to assess the efficacy of newly introduced long-acting anologue insulins which have reported the value in reducing the frequency of hypoglycaemia, particularly through the night, reduced weight gain, improved metabolic control with more reproducible glycaemic profiles. A study is also ongoing to assess the impact of analogue insulin in combination with fish oils, or fish oils alone or neither treatment in-patients with mild diabetes and evidence of vascular risk.

Achievements and Distinctions

Professor T J McKenna is President of the Royal College of Physicians of Ireland. Professor McKenna is a member of The National Taskforce on Medical Manpower and a member of its Medical Education and Training Group. He is also on the Editorial Boards for the journals, Clinical Endocrinology (Oxford) and The Endocrinologist (U.S.). He is a member of the Students Affairs Committee of the Endocrine Society (U.S.). Professor McKenna was invited assessor of quality for the Department of Medicine, University College Galway.

Dr Donal O'Shea is a member of the Eastern Regional Health Authority Diabetes Review Committee. Dr O'Shea is Secretary of The Irish Endocrine Society. He is also a member of the National Taskforce on Obesity and Chairman of the Detection and Treatment Subgroup. Dr O'Shea is Chairman of the Nutrition Council of the Irish Heart Foundation and Chairman of the St Vincent's Healthcare Group Drugs and Therapeutics Committee. He received an under-graduate-nominated Premier Award for Teaching Excellence from the Faculty of Medicine, UCD.

Dr Tom Smith is Chairman of the Association of Clinical Biochemists, Republic of Ireland Region.

Dr James Gibney was appointed Consultant Endocrinologist, Adelaide & Meath Hospital incorporating the National Childrens Hospital, Tallaght.

Ms Maire Culliton was appointed Chief Medical Scientist, National Maternity Hospital, Holles Street.

Martina Cassidy was awarded MSc in Biomedical Science by the University of Ulster, Coleraine, Co Derry.

Sinead Kelly submitted a Thesis entitled "Transcriptional regulation of key steroidogenic enzymes in the human adrenal gland" for consideration for the PhD degree to The Department of Pharmacology, UCD.

Tom Cawood was awarded the Ritchie Research Scholarship by the Royal



Collegeof Physicians and Surgeons of Glasgow to study "The roles of new therapies for thyroid eye disease".

Dr Abdulwahab Suliman received the Irish Journal of Medical Science Doctor Award for excellence in Clinical Research for the best paper published in the field of Endocrinology during 2003. The paper was entitled "Frequent misdiagnosis and mismanagement of hyperprolactinemic patients before the introduction of macroprolactin screening: Application of a new strict laboratory definition of macroprolactinemia". Suliman AM, Smith TP, Gibney J and McKenna TJ. Clin. Chem. 2003; 49(9) 1504-9. (The research reported was undertaken when Dr Suliman was Lecturer in Medicine, U.C.D., attached to this department).

Conferences/Courses/Meetings Attended

DPC Immulite Training Course, Wales: 2nd- 4th March 2004.
Tosoh HPLC Training Course, Brussels: 10th March 2004.
The 23rd Joint Meeting of British Endocrine Societies, Brighton, England: 22nd-24th March 2004.
Association of Physicians of Great Britain and Ireland, Trinity College Dublin: 25th-26th March 2004.
Diabetes Federation of Ireland Study Day for Health Professionals: Dublin 26th March 2004.
American College of Physicians Annual Meeting: New Orleans, 22nd - 25th April 2004.
Designing and Conducting an Audit for Clinical Practice: Monaghan, 7th May 2004; Run by Professional Development Centre Irish Nurses Organisation
The Irish Endocrine Society/British Thyroid Association Study Day: Dublin, 13th-14th May 2004.
Public Health Nurses Education Day: Vergemount Hospital, Dublin, 10th June 2004.
Diabetes Federation of Ireland Kids Camp: Tain Adventure Centre, Co Louth, 16th-18th July 2004.
12th International Conference on Intelligent Systems for Molecular Biology (ISMB 2004) and *3rd European Conference on Computational Biology* (ECCB 2004); 31st July - 4th August 2004, Glasgow, Scotland.
T*he 12th International Congress of Endocrinology: Lisbon*, 31st August – 4th September 2004. Professor T J McKenna - Invited speaker: Meet-the-Professor: "Polycystic ovary syndrome".
Medisense Annual Diabetes Meeting: Kildare, 17th September 2004.
Diabetes Nurse Specialists Association A.G.M.: Cavan, 7th - 8th October 2004.
Irish External Quality Assurance Conference: Dublin: 14th October 2004
Mater Misericordiae Hospital, Dublin Annual Study on Contemporary Issues: 14th October 2004. Professor T J McKenna - Invited speaker: "The impact on training of the European Working Time Directive".
Medical Representative Institute of Ireland Education Evening: St Vincent's University Hospital, Dublin, 2nd November 2004.
Immulite 2000 Users Group Meeting: Kilkenny, 4th - 5th November 2004
Irish Endocrine Society 29th Annual Meeting: Malahide, Dublin, 5th - 6th November '04
Group Facilitation (Irish Management Institute): Dublin, 10th November 2004.
European Federation of Endocrine Societies: Postgraduate Course in Clinical Endocrinology, Sitges, Spain: 11 - 13th November 2004; Professor T J McKenna, invited speaker "Macroprolactin".
Academy of Medical Laboratory Science Conference: Killarney, Co Kerry: 12th November 2004
National Diabetes Awareness Day: Dublin, 14th November 2004.
Nutrition and Dietetic Institute of Ireland Meeting: Dublin, 19th November 2004.

Publications

Suliman AM, Freaney R, McBrinn Y, Gibney J, Murray B, Smith TP, McKenna TJ.
Insulin-Induced Hypoglycemia Suppresses Plasma Parathyroid Hormone Levels in Patients with Adrenal Insufficiency.
Metabolism 2004 53: 1251-1254.

Kelly SN, McKenna TJ, Young LS.
Modulation of steroidogenic enzymes by orphan nuclear transcriptional regulation may control diverse production of cortisol and androgens in the human adrenal.
J Endocrinol.;181:355-65.

O'Connell J, Green A, Crotty T, O'Shea D, McKenna TJ.
Li-Fraumeni Syndrome with Adrenocortical Tumors Producing Estrogens in a Young Man and Testosterone in his 18 Month-Old Daughter.
Endocrinologist 2004; 14:133-137.

Cawood TJ, O'Shea D. et al.
Update on insulin analogues.
Drug Ther. Bull. 2004; 42:77-80.

Cawood TJ, Moriarty P, O'Shea D.
Recent developments in thyroid eye disease.
British Medical Journal 2004; 329:385-90.

Department of Metabolism

Departmental Review

Staff

Medical Director	Professor TJ McKenna
Principal Biochemist	Barbara Murray
Senior Biochemist	Dr Jennifer Brady
Medical Scientist	Julie Riordan
Laboratory Aide	Mairead Moore
Secretary	Siobhan Kelly

Paula O'Shea who was Senior Biochemist in the Department has joined the Endocrinology Laboratory in Beaumont Hospital as Principal Biochemist since June 2004. Julie Riordan joined us as Medical Scientist in June 2004 and Dr Jennifer Brady joined the Department as the Senior Biochemist in August 2004.

Service Developments / Activities

The Metabolism Laboratory provides a comprehensive analytical and consultation service on metabolic problems for medical and surgical patients at St. Vincent's Healthcare Group, and acts as a referring centre for specialist markers of bone and renal disease nationally.

To comply with best practice guidelines recommended by the American Diabetes Association, the urinary Albumin/Creatinine ratio test (ACR) was introduced as a screening test for early indications of deteriorating renal function in diabetic subjects. The ACR is also used to monitor patients renal response to an ACE inhibitor. The provision of this service to the Diabetic Clinics has increased urinary Albumin testing by 67%.

The measurement of 25 Hydroxy Vitamin D has been added to the test profile (Parathyroid Hormone and Ionised Calcium) for renal dialysis patients with intact parathyroid hormone levels above a target range for the stage of chronic kidney disease. This decision was based on evidence and clinical practice guidelines for bone metabolism and disease in chronic kidney disease, produced by the American National Kidney Foundation. Vitamin D workload has increased by 27% in providing this important service.

The serum resorption marker C - terminal cross - linking telopeptide of type 1 Collagen(CTX-1) has proven to be a useful parameter to measure in conjunction with bone formation markers in the anuric patient.

There has been an increase in the requesting of bone biomarkers to monitor bisphosphonate treatment in an osteogenesis imperfecta group of children in Our Lady's Hospital for Sick Children in Crumlin.The measurement of bone biomarkers to monitor the effectiveness and dose response of this relatively new form of treatment in this specialist area has been acknowledged to be most useful clinically. Since the licensing of new osteoporotic treatments such as recombinant PTH and Strontium Ranelate, requests for the measurement of bone biomarkers prior to and at set points during treatment has increased. It is hoped that observed changes in bone marker levels during and after treatment will reflect longterm increased bone mineral density and strength.

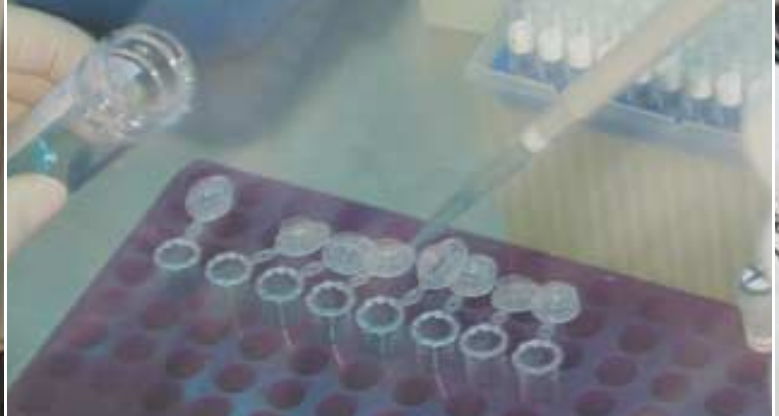
The laboratory is advancing the accreditation programme to ensure compliance with Clinical Pathology Accreditation (CPA) requirements.

Research / Future Plans

The study of changes in bone remodelling after a nutritional rehabilitation programme in Anorexia Nervosa patients has progressed. Our findings have demonstrated the significant impact of nutrition on bone formation.

Bone turnover is being studied in a group of sub clinical hyperthyroid patients.

New cellular markers of osteoblastic and osteoclastic activity are being studied in newly diagnosed breast cancer patients. It is hoped that some of these markers may identify patients most at risk of developing bone metastases.


Representative Functions

Barbara Murray is a member of the Executive Committee of Eurachem Ireland.

Meetings attended by Department Personnel

Focus 2004 Birmingham	17th – 20th May 2004
European Calcified Tissue Society	Nice, France 4th – 9th June 2004
Irish Endocrine Society	Malahide , 5th – 6th November 2004

Publications

Suliman AM, Freaney R, McBrinn Y, Gibney J, Murray B, Smith TP, McKenna TJ.
Insulin-Induced Hypoglycemia Suppresses Plasma Parathyroid Hormone Levels in Patients with Adrenal Insufficiency.
Metabolism 2004;53 (10) : 1251-1254

Oral Presentations

Irish Endocrine Society - Malahide , 5th – 6th November 2004
Smith D, Murray BF, Tan CY, Doyle M, Malone K, Darby M, O'Shea D, McKenna M, McKenna TJ.
Analysis of Bone Turnover in Patients with Anorexia Nervosa.
Ir.J.Med.Sci 2004;173:4(Supple 1)2

Poster Presentations

Irish Endocrine Society Malahide , 5th – 6th November 2004
Smith D, Murray BF, Gibney J, Tan CY, McKenna M, O'Shea D, McKenna TJ.
The effect of Thyroid Dysfunction on Indices of Bone Activity and their response to Thyroid Treatment.
Ir.J.Med.Sci 2004;173:4(Supple 1)5



Department of Medical Oncology

The Medical Oncology Unit of St. Vincent's University Hospital underwent considerable growth and evolution during 2004, reflecting international changes in oncology. There is an increased focus on ambulatory treatments, and an increasing numbers of patients being treated in the oncology day centres of the University and Private hospitals. Our move towards a new patient-friendly flexible oncology outpatient/consultation model continued, with the re-development of the old oncology day centre as a new outpatients clinic.The type of treatment we give has also changed with an increasing focus on new molecular therapies e.g. Avastin and Erbitux in colo-rectal cancer.

Research

We continued to develop our translational, and laboratory research activities in collaboration with Professor Joseph Duffy in St. Vincent's, Professor Martin Clynes, in DCU and Professor Denis Slamon in UCLA. In addition, several of our clinical studies reached fruition.The Anglo-Celtic Study, one of the first large-scale trials embarked on in the Unit, was published in the Journal of the National Cancer Institute. The first of our translational clinical trials, in which laboratory observations which were made by our international collaborators, were married with clinical trials performed in St. Vincent's University Hospital, was published in the JNCI.

Another of our studies, the International Breast Cancer Dose Intensity Study, a wholly St. Vincent's led initiative, was accepted for a major presentation at meetings in the United States and Europe.

We were delighted to host Dr. Giuseppe Gullo as a visiting oncology fellow from Milan. We also hosted the 4th International Translational Meeting in Dublin in July 2004.

Enrolment to clinical trials continued to pick up during 2004.
The total number accrued was 54.

Publications

Crown J.
Smart bombs versus blunderbusses: high-dose chemotherapy for breast cancer. Lancet. 2004 Oct 9;364(9442):1299-300.

Leonard RC, Lind M, Twelves C, Coleman R, van Belle S, Wilson C, Ledermann J, Kennedy I, Barrett-Lee P, Perren T, Verrill M, Cameron D, Foster E, Yellowlees A, Crown J;
Anglo-Celtic Cooperative Oncology Group. Conventional adjuvant chemotherapy versus single-cycle, autograft-supported, high-dose, late-intensification chemotherapy in high-risk breast cancer patients: a randomized trial.
J Natl Cancer Inst. 2004 Jul 21;96(14):1076-83.

Crown J, O'Leary M, Ooi WS.
Docetaxel and paclitaxel in the treatment of breast cancer: a review of clinical experience.
Oncologist. 2004;9 Suppl 2:24-32. Review.

Pegram MD, Pienkowski T, Northfelt DW, Eiermann W, Patel R, Fumoleau P, Quan E, Crown J, et al.
Results of two open-label, multi-center phase II studies of docetaxel, platinum salts, and trastuzumab in HER2-positive advanced breast cancer.
J Natl Cancer Inst. 2004
May 19;96(10):759-69.

Schoemaker NE, Kuppens IE, Moiseyenko V, Glimelius B, Kjaer M, Starkhammer H, Richel DJ, Smaaland R, Bertelsen K, Poulsen JP, Voznyi E, Norum J, Fennelly D, et al.
A randomised phase II multicentre trial of irinotecan (CPT-11) using four different schedules in patients with metastatic colorectal cancer.
Br J Cancer. 2004 Oct 18;91(8):1434-41.

Department of Medicine for the Elderly

Departmental Review

Departmental Statistics

	2002	2003	2004
Inpatients			
Admissions	425	317	405
Discharges	443	403	429
Outpatients			
New	280	277	252
Review	500	543	546
Carew Day Hospital			
New	420	523	571
Review	1395	824	774
Stroke Inpatient Service	194	255	256
Total Attendance			
New	700	800	823
Review	1895	1367	1320

The number of in-patients treated has remained unchanged on previous years, though out-patient attendances have increased. New patient attendance is up from 665 in 2001 to 823 in 2004. This is reflected in the throughput in both outpatients and Carew House Day Hospital. Great credit is due to Ms Angela Moriarty, CNM II, and her multidisciplinary team for the throughput and quality of work there.

Service Developments / Activities

The department's clinical services provide an in-patient and out-patient consultation service, treatment and rehabilitation, primarily for elderly patients from the hospital's catchment area in South East Dublin and East Wicklow. Issues continuing to challenge the service include the lack of resources to maintain people in their home with dignity and a consequent increase in the number of people awaiting long-term continuing care.

In Patient Consultations

In 2004 over 900 inpatient consultations were seen in St Vincent's University Hospital by the Elderly Care team. Meetings are held weekly between Dr. O Shea, the elderly medicine registrars and the Medical Social Work Department to facilitate planning of discharge of those elderly patients on whom consultant colleagues have requested advice.

Stroke Service

Weekly meetings are held and chaired by Dr. Crowe. Representatives of the nursing, physiotherapy, occupational therapy, speech and language therapy and other medical departments attend. There are now 8 dedicated beds, on Our Lady's Ward, for patients with stroke. Ms. Imelda Noone, Stroke Nurse Specialist, is involved in further development of this unit and the introduction of set guidelines and protocols. An active stroke follow-up programme is now up and running in Carew House Day Hospital. A once weekly, dedicated TIA clinic was initiated in January 2003, to facilitate assessment and treatment of patients with TIAs, who do not require hospital admission.

Falls and Syncope Service

Space has been provided on Our Lady's Ward for a falls and syncope service for the hospital and the neurocardiovascular service, which is being run by Dr. Diarmuid O'Shea. In total 120 patients were seen this year (of all age groups). Some progress has been made but the hoped for introduction of a Falls Nurse Specialist will be crucial to the further development of what is currently a rudimentary service.

The Royal Hospital Donnybrook

The Royal Hospital Donnybrook makes a very important contribution to the rehabilitation of our elderly patients. There are 40 rehabilitation beds in this unit, of which 12 are now dedicated to stroke rehabilitation. The Day Hospital at the Royal in Donnybrook continues to provide outstanding care and outpatient rehabilitation services to those with multi-disciplinary needs. The long established links with long-term care facilities continue to evolve through the dedicated work of Dr Mary Deane, Medical Director.





Staffing

Close liaison is maintained with St. Columcille's Hospital, Department of Medicine for the Elderly under the direction of Dr. Morgan Crowe and with St Michael's Hospital under the direction of Dr JJ Barry. Dr. Rachel Doyle has been appointed as a Consultant Geriatrician to St. Columcille's Hospital. Additional consultant posts are being applied for, to improve the service delivery in St. Vincent's University Hospital.

Congratulations to our SpR, Dr. Jose Miranda, who was appointed as Consultant Geriatrician in Letterkenny General Hospital. He is due to commence work there in July 2005.

Achievements / Conferences Attended

The multidisciplinary team members at Carew House Day Hospital attended The Stroke Information Day, Annual Stroke Study Day, The Irish Gerontological Society Conference, Parkinson's Association Meeting, National Council on Ageing and Older Peoples Conference. Ms Imelda Noone was an invited speaker at the Irish Heart Foundation Study Day.

The first Annual Elderly Care Study Day took place on 28th January, 2004, in the Education & Research Centre, St. Vincent's University Hospital. This year it focused on Stroke Medicine and was attended by 140 people. This will now be a regular study day in the teaching calendar.

Publications / Abstracts

Should a Question on Caring for the Younger Child become part of the Routine Assessment of the Older Patient?
C. O'Dwyer, A. Moriarty, C. Collins, D.O'Shea

Nutritional Status of Patients on a Care of the Elderly Ward- a pilot study
C. Fallon, R. McNamara, S. O'Hanlon, D. O'Shea.

Predicting The Outcome of Acute Stroke Using a Prognostic Score
I. Noone, A. Murphy,D. O'Shea, M. Crowe

Stroke in Patients with Atrial Fibrillation on Warfarin Therapy
C. O'Dwyer, I. Noone, B. Daly, D. O'Shea, M. Crowe.

Stroke Survivors Require Multidisciplinary Assessment after Hospital Discharge.
M. Cagney, A. Moriarty, S. Noelle, I. Noone, M. Crowe.

Department of Nephrology

Departmental Review

The haemodialysis service has further developed and extended as per population needs, with the unit also providing an increasing number of treatments for those patients with acute renal failure in hospital.

The peritoneal dialysis programme is now well established with the number of patients availing of this treatment option increasing to nineteen. A total of four patients from this patient population were successfully transplanted in 2004.

The Continuous Ambulatory Peritoneal Dialysis (CAPD) nurse specialist continues to work as part of the multidisciplinary team in the pre dialysis setting within the nephrology clinic, monitoring those individuals with progressive chronic renal failure and ensuring a smooth transfer to dialysis when necessary.

Service Developments

A designated nurse works within the haemodialysis unit managing the Erythropoeitin programme to control and eliminate renal anaemia. This service also extends to the pre renal nephrology clinic. This allows for close monitoring, which can help to ensure prevention of renal anaemia in the pre dialysis setting.

Conferences

Miss Emer Kenny, CAPD nurse specialist, and Mr. Frank Kelly, Senior Clinical Engineer, attended the European Dialysis Transplant Nurses Association conferences in Geneva in September 2004. Emer Kenny also attended the Pre-Dialysis Forum in June 2004. Dr. Aishling O'Riordan attended both the American Society of Nephrology and the Irish Society of Nephrology in 2004.

Dr. Vincent Wong attended the European Dialysis Transplant Association conference in 2004.

Publication and Presentations

Mr. Frank Kelly, Senior Clinical Engineer, presented in Tullamore on *"Renal Technology a Students Prospective"* to the Biomedical / Clinical Engineering Association of Ireland.

Ms Linda Parke, Renal Dietician, presented *"Investigation of the effects modifying food intake during dialysis of the incidents of Intradialytic Complications"* at the Irish Nephrology Society Conference and the British Renal Society Conference 2004.

Dr. Aishling O'Riordan presented a pilot study [poster presentation] on *Renal Failure in Liver Patients* to the Irish Society of Nephrology at the Irish College of Physicians in 2004.

Training

A series of short talks were presented by Frank Kelly, Clinical Engineering, on "Implications of Poor Water Quality" to the dialysis Nurses as part of Continuous Professional Development.

Achievements and Appointments

Ms Tara Carneigie, St. Peter's Ward, was appointed Clinical Nurse Manager 1.

Dr. Aishling O' Riordan was awarded the Newman Scholarship in Nephrology 2004.

Department of Neurology

Departmental Review

Overview

2004 was a very significant year for Neurology at St. Vincent's University Hospital for a number of reasons. Most importantly, Dr Niall Tubridy took up the position of Consultant Neurologist in January, with immediate benefit in reducing waiting times for OPD attendances and in-patient consultation and increasing the breadth of expertise available to patients and colleagues. Since we have been able to set up two OPD clinics for new patients in St Michael's Hospital, there has been an immediate impact on waiting times reduced from 6 months to less than 3 months.

Last year over 4,000 people with neurological problems were booked to the neurology clinics. This number will only increase with the gradual expansion of the Department and the addition of extra specialist staff. Our thanks to Bernadette Howard and her staff for all of their hard work in making a difficult situation somewhat manageable.

Another important event was the move to the basement of Carew House which, thanks to the efforts of Administration and Support Services, particularly Cecily Dawson, Larry Clancy, Pat McDonnell and the IT Department, was completed in October. This means that we have excellent space for departmental staff and are able to assess patients in a well-equipped clinical room. We also welcomed our new MS Research Nurse Lisa Vahey and our new secretary Paula Halpin. Both have had an immediate impact in helping to generate an effective Neurology Team and in increasing the numbers of Clinical Trials available to our patients.

Service Developments/Activities

Initiatives for 2004 included reduction of the OPD waiting list both by new clinics and by a pilot study of an e-mail assessment/referral service. This "Teleneurology" project has been introduced by Dr Tubridy and should be shortly available in 2005. If successful, we plan to apply it to the East Coast area. A significant proportion of neurology referrals can be dealt with by this method; some patients do not need to see a neurologist. We are looking forward to improving the service to outpatients in the new ADCC, which will allow patients more privacy and more time. Although the Neurology Department has two areas of special expertise recognised internationally, multiple sclerosis and dystonia; we have been unable to treat these patients in the ideal setting of a special multi-disciplinary clinic due to space limitations in the old OPD building.

The research registrars, Drs John O'Dwyer (Dystonia) and Killian O'Rourke (MS) are due to finish their research periods in June 2005 and both have been successful in obtaining platform presentations at the Association of British Neurologists (Beijing), the American Academy of Neurology (Miami)(both JOD) and the European Neurological Society (Vienna) (JOD and KOR). During the summer of 2004, we had 3 medical students, Kinley Roberts, Jennifer Williams and Hugh Kearney doing short research projects during which they gained some insight into research methods. Jenny was short-listed for the UCD Student Prize for her work on cognitive function in people with MS. Kinley has submitted her work to the Irish Neurological Association in 2005. Dr Iftikhar Sheikh has been working on an interesting dystonia project as part of his M Med Sci degree.

The ward, as ever, has been busy under the guidance of Clinical Nurse Manager Heather Kevelighan. The main difficulty has been in getting female patients admitted due to pressures on bed occupancy. Waiting list initiatives tend to improve the situation in the summer but it worsens again in the winter months.

Conferences Attended

Association of British Neurologists London April 2004, Movement Disorders Society, Rome, June 2004, Association of British Neurologists Blackpool September 2004
ECTRIMS Vienna, October, 2004





Publications in Peer –reviewed International Journals

Prof. Michael Hutchinson & Dr Niall Tubridy (5 of 14 published)

O'Riordan S, Raymond D, Lynch T, Saunders-Pullman R Bressman SB, Daly L, Hutchinson M.
Age at onset as a factor in determining the phenotype of primary torsion dystonia.
Neurology 2004;63:1423–1426

Vukusic S, Hutchinson M, Hours M, Moreau T, Cortinovis-Tourniaire P, Adeleine P, Confavreux C,
The Pregnancy In Multiple Sclerosis Group; Pregnancy and multiple sclerosis (the PRIMS study): clinical predictors of post-partum relapse.
Brain. 2004;127:1353-60.

McGuigan C, McCarthy A, Quigley C, Bannan L, Hawkins SA, Hutchinson M.
Latitudinal variation in the prevalence of multiple sclerosis in Ireland, an effect of genetic diversity.
J Neurol Neurosurg Psychiatry. 2004;75:572-6.

McMonagle P, Byrne P, Hutchinson M.
Further evidence of dementia in SPG4-linked autosomal dominant hereditary spastic paraplegia.
Neurology. 2004;62:407-10.

Rubio JP, Bahlo M, Tubridy N, Stankovich J, Burfoot R, Butzkueven H, Chapman C, Johnson L, Marriott M, Mraz G, Tait B, Wilkinson C, Taylor B, Speed TP, Foote SJ, Kilpatrick TJ.
Extended haplotype analysis in the HLA complex reveals an increased frequency of the HFE-C282Y mutation in individuals with multiple sclerosis.
Hum Genet. 2004, May;114(6):573-80.

Department of Clinical Neurophysiology

Staff

Consultant — Dr Sean Connolly, MD, FRCPI

Neurophysiology Measurement Technicians at SVUH

Ms Brigid Clark (full-time senior)
Ms Birgitta Levick (part-time senior)

SVUH Departmental Secretary — Ms Lesley Bergin

The Department of Clinical Neurophysiology is part of the South Dublin Clinical Neurophysiology Service, which also includes units at St James' Hospital and the Adelaide & Meath Hospitals, incorporating the National Childrens' Hospital (AMNCH) at Tallaght. As this service is currently run by one consultant, it is best to consider this service as a whole.

The range of investigative/diagnostic services provided include routine Nerve Conduction Studies (NCS), Electromyography (EMG), Quantitative Sensory Testing (QST), Electroencephalography (EEG) and Visual Evoked Responses (VERs).

Applications for three additional consultant posts had been submitted to the ERHA prior to December 2004:

(a) SVUH 5 sessions, Beaumont Hospital 6 sessions
(b) AMNCH (Tallaght) 6 sessions, Beaumont Hospital 5 sessions
(c) St James's Hospital 6 sessions, Beaumont Hospital 5 sessions

Dr Connolly conducts regular group sessions for neurophysiology measurement technicians approximately monthly at the AMNCH department, with the aim of teaching, and improving methods and standards of the neurophysiological investigations carried out within the service. Technicians from departments in Dublin, Limerick and Cork usually attend.

Research, currently being carried out mainly at the AMNCH and St Vincent's departments, includes a transcranial magnetic stimulation study in dystonia. There are also research collaborations with the department of electrical and electronic engineering at UCD and with Dr Geraldine Boylan in the department of paediatrics and child health, UCC.

Recent Developments

The addition of an experienced, albeit part-time, technologist with new evoked potential and nerve conduction equipment has had a significant positive impact on the number, range and quality of investigations being carried out in the St Vincent's University Hospital department. Investigations being added include more sophisticated assessments of visual function. More technologists are required and additional departmental space is necessary.

Recent Publications

Fitzsimons M, Ronan L, Murphy K, Browne G, Connolly S, McMenamin J, Delanty N. *Customer needs, expectations, and satisfaction with clinical neurophysiology services in Ireland: A case for tele-neurophysiology development.* Irish Medical Journal 2004;97(7):208-211

Ronan L, Murphy K, Browne G, Connolly S, McMenamin J, Lynch B, Delanty N, Fitzsimons M. *Needs analysis for Tele-neurophysiology in the Irish North-Western Health Board.* Irish Medical Journal 2004;97(2):46-49

Department Of Ophthalmology

Departmental Review

Staff

Consultant Staff	Dr Geraldine Kelly Mr Peter Barry Mr William Power
St James' Ward	Ms Colette Kingston, CNM2 Ms Sandra Murphy, CNM1
Ophthalmic Theatre	Ms Maeve Williams, CNM2
Surgical Registrar	Mr Patrick Talty/Dr Susan Kelly
Medical Registrar	Dr Magdy Nasralla
Lecturer in Ophthalmology	Dr Moji Oshodi
Orthoptist	Ms Nanno Fitzsimons

Clinical Activities

The department's clinical activities are summarised in the table on next page.

Its principle surgical activity is small incision cataract surgery with intraocular lens implantation specialising in the very elderly i.e. over the age of 80. We are grateful to the Department of Anaesthesia for their specialist contribution facilitating this surgery on these patients. The post of surgical registrar is highly sought after due to the training opportunities this permits. Ms Maeve Williams, CNM2 in theatre, ensures that everything runs smoothly.

The unit is recognised for both basic and higher specialist surgical training by the Royal College of Ophthalmologists.

The unit continues to become more centralised with an ever-expanding Day Care Unit in conjunction with the in-patient beds in St James' Ward. The number of daycare patients has increased from 788 to 1729 in the decade between 1994 and 2004. This busy unit is directed by Ms Colette Kingston, CNM2 and Ms Sandra Murphy, CNMl.

Staff nurses Esther McCarthy, Sheila Querubin, Gail Anderson, Carol Murphy, Gerry Porlante and Lulu Abellera each have specialist interests within the unit permitting its participation in clinical research and in-service education. Mr William Power conducts a weekly teaching session in which the entire unit staff participate.

All of this activity is catered for by the administrative skills of Jane Caulfield and Margaret Daly.

The department continues its involvement (with the seventh consecutive year) in the European Cataract Outcome Study – an unique system of audit for consultants and trainees whereby each surgeon can compare his/her performance and that of his/her department, with those of 52 other participating clinics in Europe. This activity was highly praised by the Accreditation Peer Review team.

Presentations

Department members made presentations and conducted instructional courses at the European Society of Cataract and Refractive Surgeons meeting in Paris and the equivalent American Society's meeting in San Diego. They also presented at the Association of Research in Vision and Ophthalmology (ARVO).

Future Plans

The department looks forward to 2005 when it hopes to begin the process of integrating its specialist nurses to a rotation through the ward, the day care unit, the outpatients and the operating theatre. This will hopefully become a reality when phase one of the new hospital development is complete.



Opthalmology St. James' Ward

		2000	2001	2002	2003	2004
St James' In-Patients	Admissions	443	461	458	458	442
	Discharges	436	457	456	451	434
St James' In-Patients	Cataract Procedures	308	330	324	310	299
St James' Day Care	Admissions	1526	1611	1846	1994	1729
	Discharges	1526	1611	1846	1994	1729
St James' Day Care Orthoptic	Total Attendances	573	607	502	927	777
St James' A/E Ophthalmic Patients	Total Attendances	1130	1153	1261	1016	1064
St James' +A2 In-Patient Referrals	Total Attendances	361	299	309	327	342
Ophthalmology Out-Patients	New	702	745	773	781	729
	Total Attendances	2651	2372	2398	2475	2245

Department of Orthopaedic Surgery

Departmental Review

Staff

Consultants	Mr. Sean Dudeney
	Mr. Brian Hurson
	Mr. Eamonn Kelly
	Mr. William Quinlan
	Mr. S. Kieran O'Rourke
Administrative Staff	Ms. Jenni Cross
	Ms. Frances Doyle
Orthopaedic Ward	CNM2: Ms. Gillian Fields
	CNM1: Ms. Rachel Hayden
	CNM1: Ms. Andrea Marnell
Orthopaedic Theatre	CNM2: Ms. Katrina Lennon
	CNM1: Ms. Anna Marie Bos

Service Developments/Activities

The Orthopaedic Department, as part of the South East Department of Surgery (SEDDS), is based in St. Vincent's University Hospital, but also delivers the service through the facilities at St. Columcille's Hospital, Loughlinstown and St. Michael's Hospital, Dunlaoghaire. The service caters for the area including Dublin City South, South County Dublin, County Wicklow and the adjacent cross over areas, giving a local population of between 350,000 and 400,000. In addition, the individual speciality interests of the surgeons in the unit draw from a much wider, and in some cases, national catchment area.

Outpatient clinics are carried out on all sites. Daily clinics are held in St. Vincent's University Hospital. Subspeciality clinics in joint replacement and upper limb are held in St. Anthony's on the St. Vincent's campus. Mr S. K. O'Rourke and Mr. E. Kelly attend St. Columcille's Hospital, where there are twice weekly clinics, while Mr Quinlan, Hurson and Dudeney run clinics in St. Michael's Hospital . There is a large orthopaedic oncology referral to St. Vincent's University Hospital. While some of this work is dealt with through the Outpatient Department in St. Vincent's University Hospital, the bulk is seen in speciality clinics in Cappagh National Orthopaedic Hospital. For many years, Mr. Brian Hurson has shouldered this demanding service. However, latterly Mr. Sean Dudeney having returned from training in the United States has also become involved, and there are plans to increase the service further.

A total of 13,150 patients were seen as outpatients through the St. Vincent's campus, with a further 9,066 seen at other clinics. Therefore, a total of 22,216 patients attended the unit over the course of the year.

The bulk of orthopaedic surgery carried out in St. Vincent's University Hospital is trauma or trauma related. For all patients presenting to the unit through any of the portals of entry viz. the Emergency Department in St. Vincent's University Hospital, St. Michael's Hospital or St. Columcille's Hospital, it is endeavoured to deal with them, where possible, within a twenty-four hour period. Such aspiration to efficiency requires maximum co-operation between the hospitals and staff involved.

The main impediments on an ongoing basis are the number of elderly patients waiting for long term placement and the dependence on an already stretched ambulance service for inter-hospital transfer. The demographics of this region are such that it has the highest percentage of over 65 year olds in the country. If the population forecasts of gradual increase and ageing are correct, it is vital for the future that we move to a proactive orthogeriatric concept which will allow these patients to be dealt with in a more appropriate and co-ordinated fashion than heretofore.

Patients for elective surgery, once seen at outpatient level, have surgery at Cappagh National Orthopaedic Hospital. This is a stand alone orthopaedic facility, which provides elective surgery for a number of Dublin Teaching Hospitals involving twenty orthopaedic surgeons, including the recently appointed Chair of Orthopaedics, Prof. John O'Beirne and as such is an important academic/teaching hub. From the St. Vincent's campus, there were approximately 1,000 cases operated on in Cappagh Hospital in 2004.

Education and Training

The orthopaedic staff are involved in teaching at both undergraduate and postgraduate levels. In combination with Cappagh Hospital and the other teaching

hospitals, St. Vincent's University Hospital is part of the Higher Surgical Training Programme in Orthopaedic Surgery, on behalf of the Irish Institute of Orthopaedic Surgery. Mr. Quinlan is currently the President of the Irish Institute of Orthopaedic Surgery and, as such, chairs a number of sub committees including the training committee. He has been on faculty of a number of academic meetings including the A.O. Instructional course in fracture management and the Annual Orthopaedic Updates Meeting. Mr. Quinlan is currently a Director on the Board of Directors of St. Vincent's Healthcare Group.

Mr. Dudeney is Secretary of the A.O. group Ireland. He was chairman of the A.O. instructional course in March. He was a member of the visiting faculty for the A.O. instructional course in fracture management in Portugal. He currently sits on the St Vincent's Healthcare Group Ethics Committee. Mr Dudeney was the Co-ordinator of the Intercollegiate Fellowship Exam held in Cappagh National Orthopaedic Hospital.

Mr. O'Rourke was a faculty member of the Second International Hip Resurfacing Meeting in November.

Mr Kelly is currently the Unit Director and the hospital representative to the training committee. He is also a member of council of the Institute of Orthopaedics.

Presented Work

1. *"Outcome following use of the "S" Quattro dynamic external fixation device in the management of fractures of the first ray."*
   AM Byrne, SR Kearns, S Morris, EP Kelly.
   Irish Hand Surgery Society Meeting, Dublin, May 2004.

2. *"Outcome following use of the "S" Quattro dynamic external fixation device in the management of fractures of the first ray."*
   AM Byrne, SR Kearns, S Morris, EP Kelly.
   Irish Orthopaedic Association Meeting, Killarney, June 2004.

3. *"Morbidity and mortality following hip fracture in the over 90's: A cohort study."*
   SR Kearns, C Ridge, K Cahill, P Moroney, EP Kelly, SK O'Rourke, S Dudeney, B Hurson, WB Quinlan.
   Irish Orthopaedic Association Meeting, Killarney, June 2004.

4. *"Comparison of open and modified percutaneous repair of acute tendo achilles ruptures."*
   S Brophy, SR Kearns, WB Quinlan, SK O'Rourke.
   Irish Orthopaedic Association Meeting, Killarney, June 2004.

5. *"Combined ankle and sub-talar arthrodesis: A novel application for the supracondylar femoral nail."*
   M Cleary, SR Kearns, WB Quinlan.
   Irish Orthopaedic Association Meeting,Killarney, June 2004.

6. *"Haemoatogenous pyogenic spinal infection."*
   B O Daly, S Morris, SR Kearns, P Moroney, P Kiely, SK O'Rourke
   Britspine Meeting, 28-30th April 2004.

7. *"Morbidity and mortality following hip fracture in the over 90's: A cohort study."*
   SR Kearns, C Ridge, K Cahill, P Moroney, EP Kelly, SK O'Rourke, S Dudeney, B Hurson, WB Quinlan.
   Freyer Meeting September, Galway.



Department of Palliative Medicine

Departmental Review

Staff

Consultant	Dr Eoin Tiernan
Clinical Nurse Specialist	Ms Millie Devenish, Olga Price Ms Siobhan Hollingsworth, Ms Barbara Whyte
Specialist Registrar	Dr Sinead Kelly - 01/01/04 to 30/06/04 Dr Ita Harnett – 01/07/04 to 31/12/04
Registrar	Dr Carol Stone – 01/01/04 to 30/06/04 Dr Aoife Gleeson – 01/07/04 to 31/12/04
Senior Social Worker	Ms Susan Cox, Ms Aine Canny
Administrative Support	Ms Joan Stokes, Mr Olajide Ogidan

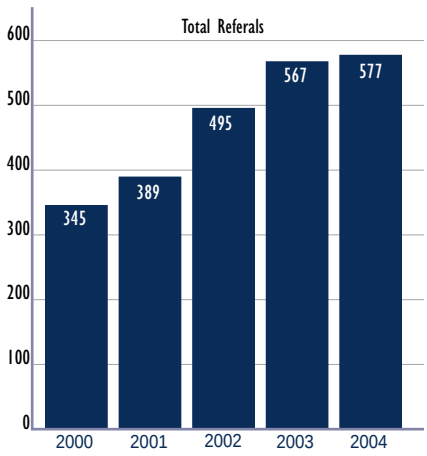
Service Development

2004 was a busy year for the Palliative Medicine Department. The team launched a major project in the hospital entitled "Care of the Dying Patient". The purpose of this initiative is to review the current standards of care given to dying patients in the hospital, and aims to improve all aspects of care to patients and support to families around time of death. The team received one of the Innovation Awards for Quality Improvement Initiatives presented for the project proposal in October. In summary, the project includes a survey of the experiences of staff nurses in caring for dying patients and their families, a survey of bereaved next-of-kin on their experiences of care given around the time of death, and a pilot project looking at the feasibility of introducing an established quality initiative/integrated care pathway from the UK, namely, the Liverpool Care Pathway for the Dying. Additional input and support for the Project is being provided by Ursula Bates, Senior Clinical Psychologist (Oncology Services), with methodological and statistical support from Dr. Barbara Dooley, Department of Psychology, UCD. The Project is being overseen by a Steering Group, which comprises representation from Hospital Management, the Medical Board, Nursing Administration, Social Work Department, Chaplaincy, and the Mission Committee.

We continued to receive rotating doctors from the Higher Specialist Training Scheme for Palliative Medicine, with Dr. Sinead Kelly and Dr. Ita Harnett each spending six months with the service. In July, Dr Aoife Gleeson replaced Dr Carol Stone as Registrar for a twelve-month period. Siobhan Hollingsworth joined the team in July as an additional clinical nurse specialist for a one-year period. Our Senior Social Worker, Susan Cox left St. Vincent's University Hospital to return to her native Australia, and was replaced by Aine Canny in March. Aine has taken up from where Susan left off; continuing to enhance the psychosocial and family care dimensions of our work. We continue to enjoy a close working relationship with the Clinical Psycho-Oncology Service, with Ursula Bates providing input to our multidisciplinary meetings, and also offering her expertise in team-work to facilitate some team-building exercises ourselves. With the ever-expanding remit of the Clinical Team, we continue to receive the excellent support of our administrative team comprising Olajide Ogidan and Joan Stokes.

Service Activity

2004 saw further increases in the clinical activities of the team – a total of 577 referrals were received (8% non-malignant diagnosis). The graph below illustrates the steady increase in referrals over the past 5 years.



St. Vincent's Private Hospital

Carmel Houlihan, Clinical Nurse Specialist, continues to provide a much-valued service, single-handedly, seeing overall referrals increase by 28% to 197. As with previous years, the majority of referrals continue to come from medical and radiation oncology (68%), with 14% of referrals from the surgical services and 18% of referrals from other medical services.

Education

The highlight of the Education calendar in 2004 was the hosting of a half-day symposium entitled "Care of the Actively Dying Patient" in the Education and Research Centre on 1st October. Approximately 180 people registered to hear eminent national and international speakers address various aspects of quality care of dying patients. The international faculty included Professor Declan Walsh

(Cleveland Clinic, Ohio), Dr. John Ellershaw (Marie Curie Centre, Liverpool), and Dr. David Carroll (Macmillan GP Facilitator, Aberdeen), with the local faculty comprising Dr. Sinead Donnelly (Milford Hospice, Limerick), Dr Ita Harnett, SVUH, and Ursula Bates, Clinical Psycho-Oncologist, SVUH.

The Nursing Team facilitated their Annual Palliative Care Study Day for registered nurses from around the country in December. The team continues to contribute to student and registered nurse education, ward attendant programmes and orientation programmes for overseas nurses, as well as facilitating clinical placements for nurses undertaking the Higher Diplomas in Palliative Care Nursing and Pain Management from UCD, and undergraduate medical students.

Audit and Research

Dr Carol Stone completed a research project validating a prognostic tool for patients with advanced cancer, which has been accepted for presentation at the European Association for Palliative Care's International Congress in 2005. The major team project initiated in 2004 was the Care of the Dying Project, previously described.

Courses and Conferences Attended by Members of the Team

A selection of the educational events attended by members of the team:

- Irish Association for Palliative Care Education and Research Seminar, Feb 2004;
- Liverpool Care Pathway Foundation Day, Feb 2004;
- 2nd Bristol Opioid Conference, Feb 2004;
- Multidisciplinary Approach to Pain Management (Irish Pain Society & IAPC), May 2004;
- 3rd Annual Kaleidoscope Conference (St. Francis Hospice), May 2004;
- 16th Annual Symposium of the Multinational Association of Supportive Care in Cancer, Miami, June 2004.
- "Palliative Medicine in the 21st Century – from Art to Science", Cleveland Clinic, Ohio, July 2004;
- Seminar – "Evaluation of the Effectiveness of the Role of the Nurse Specialist", Aug 2004;
- Advances in Radiation Oncology, Oct 2004.

Presentations

Dr Tiernan was an invited speaker at a number of international conferences during 2004:

- The Multinational Association of Supportive Care in Cancer, 16th International Symposium in Miami, June 2004, where he chaired a symposium entitled "Dying Patients: What Should be the Standard of Care? Consensus Development".
- Palliative Medicine in the 21st Century: From Art to Science, at the Cleveland Clinic, Ohio, where he presented a paper entitled "Setting Standards for Care of the Actively Dying Patient", July 2004.
- International Interim Meeting on Home Care & Hospice in Oncology, Istanbul, Turkey, October 7-9, 2004, where he was an invited international co-chair for the conference, and presented two papers, entitled "Quality of Life – A Perspective from Palliative Care" and "Organization of Home Care – An International Perspective".

A number of posters were presented by team members at the Irish Association for Palliative Care's Annual Research Seminar and at the Multinational Association of Supportive Care in Cancer's 16th International Symposium in Miami.

Publications

Abstracts –

Conroy M, Cunningham J, Tiernan E, Smyth P. *Issues for medical interns in providing end of life care.*
Supportive Care in Cancer 2004;
12(6): 405.

O'Leary N, Tiernan EJ, Stone C.
Activity of a hospital-based palliative medicine team.
Supportive Care in Cancer 2004;
12(6): 409.

Department of Plastic Surgery

Departmental Review

Staff

Consultants	Mr Tom O'Reilly Ms Margaret O'Donnell Mr Sean M Carroll
Registrars	Ms. D O'Keeffe Mr M Allam
SHO	Dr. D Knight
Occupational Therapist	Ms Aine O'Reilly
Hand Therapist	Ms Emma Cannon

Service Developments/ Activities

The Department of Plastic Surgery continues to provide a general plastic surgery service within the St. Vincent Healthcare Group with specialised interests in breast reconstruction, hand surgery, skin cancer, pressure sore surgery and head and neck reconstruction.

St Agnes' Ward remains at the centre of the service and we acknowledge the dedication and hard work of CMN Dolores O'Neill and Josephine Ryan and the highly competent ward staff. Expertise amongst the ward nurses ensures the professional and efficient postoperative monitoring of patients who have undergone microsurgery, without the need to utilise expensive ICU or HDU beds. Out-patient clinical activity remains high and the expertise and continued positive attitude of the OPD nurses in the face of large clinics helps all involved. Three OPD clinics are held per week in St. Vincent's University Hospital, two of which are general plastic surgery, and one Hand Clinic. A new OPD clinic on Monday afternoons in St. Michael's Hospital has increased OPD activity over the last year. A total of 4,450 out- patients were seen during 2004 between the two hospitals, in the four OPD clinics per week. The outstanding hard work of our unit secretary, Rachel Boyle, helps us to communicate effectively with our patients, referring doctors and hospitals and we thank her sincerely.

The breast reconstruction service continues to expand with over one hundred and twenty reconstructions being performed during the year. We have a close working relationship with the Surgical Professorial Unit from where a significant number of patients are referred. We also provide a reconstructive service to BreastCheck and many patients now undergo breast reconstruction at the time of their mastectomy. The majority of patients, however, are reconstructed sometime after their initial surgery. The service is complimented by the participation of the Breast Care Nurses and encompasses a Micro-pigmentation (Tattooing) Clinic to complete Nipple-Areolar reconstruction.

Activity within the hand surgery service is increasing and a lot of hard work has been done on the writing of hand therapy protocols for both elective and trauma cases. The professional dedication of our Occupational Therapist, Aine O'Reilly, and our Hand Therapist, Emma Cannon, is greatly appreciated by everyone. Congratulations are due to Emma who got married during the year and we wish her a long and happy marriage.

St Michael's Hospital has afforded our department many opportunities for expansion, with the addition of two extra operating lists a week, one of which is a dedicated regional anaesthesia hand surgery list. This allows for greater theatre time usage, less recovery time and quicker patient discharge. We acknowledge the expertise of the Anaesthetists providing this service and look forward to its further development within the new day care facilities. Development of a dedicated breast reconstruction service within St Michael's Hospital is envisaged, further helping the incorporation of the two hospitals.

A plastic surgical service continues to be provided for the National Rehabilitation Hospital (NRH). An outpatients and ward round at NRH takes place on alternative weeks. The further development of this service is envisaged but is dependent on the provision of protected beds into which our patients can be admitted.

Consultant staff numbers increased with the appointment of Sean Carroll to a permanent post in July of this year bringing the consultant body to three in number. He will continue to provide a general plastic surgery service with a special interest in hand surgery and microsurgery.





The future of the unit has been mapped out for the medium term. There is increasing demand, and need, for head and neck reconstruction, breast reconstruction and sarcoma surgery reconstruction. The ageing demographic and increasing incidence of skin cancer means that the pressures on this speciality will become even greater. It is apparent that the appointment of a fourth consultant is both appropriate and necessary. The accreditation of the unit for higher specialist training can now proceed with the appointment of the third consultant. This will enable us to attract high quality trainees, thus further improving our service potential, increasing our research output and contributing to the training of future plastic surgeons for Ireland.

Papers Published

The Two Finger Sign
Pashkumar K, Hanson R, Carroll SM
The British Journal of Plastic Surgery: 63: 45 - 46, 04

The Appropriate method of applying sterri strips
Pashkumar K, Hanson R, Carroll SM
Plastic and Reconstructive Surgery 113: 1106- 1107. 04

Submitted Papers

A complicated chest wall reconstruction
Pashkumar K, Hanson R, Chan K, O'Donnell M Tolan M
The British Journal of Plastic Surgery

Presentations

Compartment Syndrome and delayed decompression
Pashkumar K, Hanson R, Carroll SM
Irish Association of Plastic Surgery meeting Cork 04

Lectures, Teaching and Examinations

All three consultants are active in teaching both within the Hospital and the surgical skills course in the College. All three consultants examined in the final medical exams

Meetings Attended

British Burns Association Manchester April 2004 O'Donnell M

IAPS Annual Scientific Meeting May 2004 O'Reilly T, O'Donnell M

British Association of Plastic Surgery, Dublin June 2004
O'Donnell M, O'Reilly T, Carroll SM

American Association of Plastic Surgery, Philadelphia September 2004. Carroll SM

Birmingham Breast Reconstruction Meeting. December 2004:
O'Reilly T, Carroll SM , O'Donnell M.

British Association of Plastic Surgery London Dec 2004 O'Donnell M.



Department of Respiratory Medicine

Departmental Review

Activity

Inpatient Admissions	408
Discharges	378
St. Michael's Hospital Admissions	384

Out-Patient Acitivities

	New	Return	Total
Prof. McNicholas			
Tuesday (Respiratory)	314	1022	1336
Wednesday (Sleep apnoea)	0	357	357
Dr. McDonnell			
SVUH (Resp)	57	375	432
SVUH (TB)	149	683	832
SMH	68	712	880

Pulmonary Function Laboratory

Total Patients:	3138
Total tests:	6061

Sleep Laboratory

Total Admissions:	524
CPAP commencements:	159

Service Developments

Plans were developed during 2004 to establish a 6-bedded acute non-invasive facility in St. John's Ward, which is expected to come into operation in the first half of 2005. This facility will significantly improve the level of care for medical patients in acute respiratory failure requiring non-invasive ventilation.

Pulmonary Function and Sleep Laboratory

Three 8-week student placements were accommodated from DIT in the clinical measurement degree course. All technicians attended the Irish Thoracic Society meeting in Cork and two technicians attended the British Thoracic Society meeting in London. Technicians from St.Vincent's University Hospital provide Pulmonary Function testing service two days per week in St. Michael's Hospital. In St. Michael's a total of 449 patients had 750 tests.

The Sleep Research Laboratory continues a varied clinical and basic science research programme that has been evolving over several years. Ongoing collaboration exists with the research group of Dr. Cormac Taylor in the Conway Institute at UCD and the Department of Electronic and Electrical Engineering (Dr. Conor Heneghan and Prof. Mark O'Malley), also in UCD, in addition to the Department of Physiology, R.C.S.I. (Dr. Aidan Bradford).

Respiratory Education Centre

The Respiratory Education Centre received a total of 1800 consults during 2004, 650 of these were out-patients. Deirdre Donaghy, Patricia Jones and Lindsay Brown successfully completed a distance learning non-invasive ventilation module through University Hospital Aintree in Liverpool. Policy development has been this year's focus. The nebuliser policy was updated. An extensive tracheostomy care policy was devised collaboratively. Respiratory Nurse Specialists continue to devise audit tools to evaluate service provision, policy implementation and patient satisfaction.

On World COPD Day, 96 people visited the stall set up on the hospital concourse and educational support was well received. World Asthma Day was similarly promoted and well received.

St. Michael's Hospital.

The COPD Unit in St. Michael's Hospital continues to be busy accepting patients directly from the Emergency Department in St. Vincent's University Hospital when appropriate. The Pulmonary Rehabilitation Unit also remains very active with a total of 81 new patients to the programme in 2004, with reviews carried out on 2,452 patients. This represented a significant increase in activity over the preceding year. The patients are primarily referred from the respiratory services in St. Vincent's University Hospital as well as patients from St. Michael's Hospital. In addition to patients with COPD, a significant number of patients




with interstitial lung disease have been referred from the lung fibrosis clinic in St. Vincent's University Hospital. Finally, a nurse led COPD clinic has been initiated by Ms. Mary Frances O'Driscoll, Respiratory Specialist Nurse. It is run in conjunction with the respiratory clinic on a Friday morning. This has allowed more patients to be seen efficiently at the respiratory clinic

Honours and Distinctions

Prof. McNicholas was President of the European Respiratory Society during 2004, which is the principal scientific society in the area of respiratory diseases and one of the largest such societies worldwide. He also presided over the annual meeting of the society in Glasgow during September 2004, which was attended by over 14,000 delegates and represents the largest international respiratory meeting. During this congress, he presented a special award to Mr. Michael Martin in recognition of his successful introduction of the smoking ban in Ireland.

Invited Presentations during 2004

Prof. McNicholas gave invited lectures at the annual meeting of the Japanese Respiratory Society in Tokyo in April, the annual meeting of the European Respiratory Society in Glasgow in September, and the annual meeting of the Asian Pacific Society of Respirology in Hong Kong in December. He was also a Visiting Professor at Brown University, Rhode Island, USA, in October 2004, during which he presented Medical Grand Rounds at the University.

Presentations of original research were given by members of the research team at the annual meeting of the American Thoracic Society in Orlando in May (Dr. Silke Ryan, Dr. Liam Doherty and Dr. Sardar Ali), the annual meeting of the European Respiratory Society in September (Ms. Geraldine Nolan (Lawless)). Geraldine Nolan (Lawless) also gave a presentation at the Association of Respiratory Technology and Physiology meeting in the UK.

Selected Publications 2004

Prof. Walter McNicholas
From a total of 14 publications.
(Please see also ERC Publications listing.)

Martial FP, Dunleavy M, Nolan P, McNicholas WT, O'Regan RG, Bradford *A.*
Simultaneous recording of breathing and respiratory related neuronal activity in the brainstem of conscious rats.
Respir Physiol Neurobiol. 2005 Feb;145(2-3):301-6.
PMID: 15705544

Dr T McDonnell
Butler MW, O'Mahony MJ, Donnelly SC, McDonnell TJ.
Managing exacerbations in COPD; room for improvement.
Irish Medical Journal 2004; 97:108-110.

Armstrong J, Breathnach C, Carney D, Eakin R, Grogan L, McDonnell T, McManus K, O'Connell F.
Guidelines for the treatment of lung cancer.
Irish Medical Journal 2004 February Supplement.

3 Naji NA, Connor MC, Nagle MM, McDonnell TJ.
Modified BODE index predicts risk of death in chronic obstructive pulmonary disease.
Irish Journal of Medical Science 2004;173:S(2)20.

4 Naji NA, Connor MC, Nagle MM, McDonnell TJ.
Does a modified BODE index predict outcome of pulmonary rehabilitation?
Thorax 2004;59:ii60.

Department of Rheumatology

Departmental Review

Staff

Consultant Staff	Professor Barry Bresnihan Professor Oliver FitzGerald Dr Douglas Veale
Senior Scientist	Dr Ursula Fearon
Specialist Registrar	Dr Adrian Gibbs
Registrar	Dr Teck Loeng Tai
Research Staff	Mary Connolly, Martina Gogarty, Laura Greenan, Keith McQuillan, Trevor Markham, Julia Martin, Clare Matthews, Ronan Mullan, Bea Radovits, Jennifer Ralph, Terence Rooney, Catherine Sweeney, Wahlid-Al-Shehi, Ceara Walsh
Advanced Nurse Practitioner	Patricia Minnock
Clinical Nurse Manager	Catherine Slattery
Clinical Nurse Specialists	Alexia Grier, Miriam Molloy, Louise Moore, Madeleine O'Neill, Karen Reid, Susan van der Kamp

Service Developments/Activities

Service developments included specialist clinics for patients receiving biologic therapies (predominantly anti-TNF therapies). It is intended to maintain 3 additional weekly clinics supported by both medical and nursing staff. Other specialist programmes including the fast-track early arthritis clinic, the bone densitometry service and the arthroscopy programme continue to expand. Plans are nearing completion for the establishment of a diagnostic musculoskeletal ultra-sound facility within the department. Integration of the rheumatology service across the 2 sites (SVUH and Rheumatology Rehabilition, Our Lady's Hospice) continues.

Departmental Statistics

There were 643* acute medical admissions to St Vincent's University Hospital. In addition, 811 patients were admitted for evaluation and treatment to the Rheumatology Rehabilitation Unit, Harold's Cross. A total of 5,633 patients were seen in the Rheumatology clinics at SVUH, including the Early Arthritis and Juvenile Arthritis Clinics and of these 941 were new referrals.

*Breakdown of 643 accute medical admissions is as follows:-

A & E	436
Urgent	21
Elective	14
Day Care	172

Significant Achievements

Professor Bresnihan was an invited speaker at the New Zealand Rheumatology Association and the Israel Society for Rheumatology. He was a visiting Professor at the Rheumatology Update Programme at Georgetown University, Washington. He chaired the satellite symposium on 'B Cells in Rheumatology' at the EULAR Congress in Berlin in June. He was an external examiner for PhD thesis at the University of Newcastle and a member of the Dutch Arthritis Association Review Committee. Professor Bresnihan continues to co-chair the European Synovitis Study Group. He is chairman of the Subcommittee for Academic Support, Arthritis Research Campaign (UK), chairman of the Arthritis Ireland and a member of the Oliver Bird Programme Selection Committee. Professor Bresnihan serves on the Editorial Board for the Journal of Rheumatology, Annals of the Rheumatic Diseases, Joint Bone and Spine, and Bailliere's Clinical Rheumatology.

Professor FitzGerald was an invited speaker both at the British Society for Rheumatology and at the American College of Rheumatology. He was also invited to speak at the meeting on Breakthrough Therapy for the Treatment of Psoriasis. Professor FitzGerald was an external examiner for PhD theses in Queens University Belfast and at Oxford University. He was also an external examiner at the Trinity College final medical examinations. He is a Council Member of Arthritis Ireland and is interim Director of the SVUH Education and Research Centre. He is also a member on the Editorial Board of *Rheumatology* and of *Current Rheumatology*.


TEXTBOOK OF SURGERY

Dr Veale was an invited speaker at the EULAR Congress and the American College of Rheumatology Meeting. Dr Veale is Medical Director, Rheumatology Unit at Our Lady's Hospice, Harold's Cross. He was the chairman of Arthritis Action Ireland, a Council member of the Irish Society for Rheumatology and Editor of *E-Medicine*.

Prizes

Dr Veale was awarded Best Oral Paper at the Irish Association of Dermatologists in 2004: Markham T et al, Anti-TNFaα therapy in psoriasis and psoriatic arthritis: clinical and angiogenic responses.

Jenny Ralph won First Prize Oral Presentation at the Irish Society of Rheumatology Meeting in Dublin in October.

Publications

Benito MJ, Murphy EP, van den Berg W, FitzGerald O, Bresnihan B.
Increased synovial tissue NF-κB1 expression at sites adjacent to the cartilage-pannus junction in rheumatoid arthritis.
Arthritis Rheum 2004, 50:1781-1787.

Curran SA, FitzGerald OM, Costello PJ, Selby JM, Kane DJ, Bresnihan B, Winchester R.
Nucleotide sequencing of psoriatic arthritis tissue before and during methotrexate administration reveals a complex inflammatory T cell infiltrate with very few clones exhibiting features that suggest they drive the inflammatory process by recognizing autoantigens.
J Immunol. 2004 Feb 1; 172(3): 1935-44.

Kane D, Gogarty M, O'Leary J, Silva I, Bermingham N, Bresnihan B, Fitzgerald O.
Reduction of synovial sublining layer inflammation and proinflammatory cytokine expression in psoriatic arthritis treated with methotrexate.
Arthritis Rheum. 2004 Oct;50(10):3286-95.

McEvoy AN, Bresnihan B, FitzGerald O, Murphy EP.
Cyclooxygenase 2-derived prostaglandin E2 production by corticotropin-releasing hormone contributes to the activated cAMP response element binding protein content in rheumatoid arthritis synovial tissue.
Arthritis Rheum. 2004 Apr;50(4):1132-45.

O'Hara R, Murphy EP, Whitehead AS, FitzGerald O, Bresnihan B.
Local expression of the serum amyloid A and formyl peptide receptor-like 1 genes in synovial tissue is associated with matrix metalloproteinase production in-patients with inflammatory arthritis.
Arthritis Rheum 2004, 50: 1788-1799.



Department of Urology

Departmental Review

Staffing

Consultants	Mr David Quinlan, Mr David Mulvin Mr Gerald Lennon
Senior Registrars	Seamus Teahan (from 1/7/03) Richard Power (from 01/07/04)
Registrars	Stephen Connolly (from 1/7/03), Michael Floyd (from 01/07/04)
Senior House Officers	Ann Maria Byrne (from 1/01/04) Catherine Dowling (from 1/7/04)
Interns	Marie Therese Cooney, Eimear Halley, Evelyn O'Neill, Elizabeth Cummins (to 30/6/04) Christopher Hegarty, Fiona Magee, Deirdre Sweetman, Michael Boyle (from 1/7/04)
Clinical Nurse Manager 2	Ms Louise Hederman (St Charles')
Clinical Nurse Manager 1	Ms Mary Nevin, Ms Denise Murray (St Charles')
Clinical Nurse Manager Theatre	Ms Linda Mullen, Ms Breda O'Donoghue
Urology Cancer Nurse Coordinator	Ms Angela Kissane
Urodynamics/Urology Nurse Practitioner	Ms Siobhan Gardner
Inpatient Clinical Coordinator	Ms Nuala Kennedy
St Charles Ward Secretary	Ms Adelah O'Brien
Urology Secretaries	Ms Fiona Whelan, Ms Kirsty Whaley

Service Developments/Activities

2004 remained a busy year for the Department of Urology. As in 2003, St Charles' Ward was largely protected from admissions from other disciplines as of 1st March to 31st December 2004. This permitted patients on urological waiting lists to be given a date for admission, which in the majority of cases is honoured. The success of this strategy revolves around the dedicated office and admissions coordinator in St Charles' Ward to facilitate these patients' admission and associated investigations and procedures. Protection has been at the 85% level from the 1st March to 31st December. While protection is lost for the other two months, the Bed Manager has managed to secure admission to a high level during January and February for major oncological surgical cases.

While the Department of Urology has no dedicated 5 day beds on the campus of St Vincent's University Hospital (having relinquished St Kevin's Ward for five day activity in 2000), its five-day activity continues to occur off-site in the Annexe of St. Michael's Hospital. The provision of extracorporeal lithotripsy in the Annexe has been a particular success and benefit for Urological patients with renal stone disease.

Urodynamics has been very active. Most male patients presenting for the first time to outpatients with lower urinary tract symptoms arrive for their initial consultation with a packet of information from a pre-assessment with Siobhan Gardner consisting of urinalysis, renal profile, prostate specific antigen, symptoms score, uroflow and estimation of post void residual. This provides for instant decision making at the patient's first consultation.

Surgically, 2004 has seen the further development of Laparoscopic Urology under the direction of Mr David Mulvin. Mr Lennon has continued to expand Urodynamics and Female Urology.

The Urology Nurse Diploma programme at University College Dublin and St Vincent's University Hospital is now well established. This is the only programme of its type in the country and a most coveted development by the Department of Urology.

The Urological Club of Great Britain and Ireland held their annual meeting at St Vincent's on the 15, 16th and 17th of September. This meeting was well received by the club members and extremely well supported by the hospital and the university.

Departmental Statistics

During 2004, there were 10,329 patient presentations, which included:

- *Outpatients:* A total of 4,797 out patients were seen by the Urology Team. Three Urological outpatient clinics occur per week on the St Vincent's Campus with 3,965 attendances. Of these 940 were new patient attendances. There were 832

outpatient attendances in St Michael's Hospital seen in the once weekly outpatient clinic, of which 237 were new patient attendances.

- *Admissions:* Total admissions were 3,695 patients. There were 2,229 admissions in the year 2004 on the St Vincent's Campus, with an additional 914 in St Michael's Hospital and 552 daycare and 5-day patients through the St Michael's Annexe.
- *Theatre:* Surgical procedures totalled 3,250, with 1,784 surgical procedures performed on the St Vincent's Campus, 914 performed at St Michael's Hospital and a further 552 procedures performed in the St Michael's Annexe.
- *Consults:* 936 inpatient consultations in 2004.
- *Urodynamics:* There were 901 urodynamic procedures performed.

Teaching

For the urological trainees there is a major emphasis placed on teaching with academic conferences, teaching in the operating room and research endeavours. Each day starts with an organised teaching session with weekly conferences on:

- Research Conference and Journal Review (Monday 8:15a.m.)
- Irish Society of Urology Teaching Conference (Monday 7:00p.m.)
- Uroradiology Conference (Tuesday 8:15a.m.)
- Surgical Conference (Wednesday 8:15a.m.)
- Uropathology Conference (Thursday 8:15a.m.)
- Audit (Friday 8:15a.m.)

Monthly meetings occur as below:

- Multi-Disciplinary Team Meeting (Monthly, Monday 8:15 a.m.)
- University College Dublin Urology Conference (Bi-Monthly on Thursdays 6:30p.m. between Mater and Vincent's)
- Irish Society of Urology Clinical Case Conference (Monthly Wednesdays 7:00p.m.)

Ongoing Clinical Trials

- Casodex 150mg for non metastatic prostate cancer (Zeneca)
- Atrasentan for prostate cancer (Abbott)

Presentations

- Quinlan, DM: Course Director - Urological Trauma Course. British Association of Urology Annual Meeting, June.

Publications

O'Brien MF, Connolly SS, Kelly DG, O'Brien A, Quinlan DM, Mulvin DW. *Should patients with a pre-operative prostate-specific antigen greater than 15 ng/ml be offered radical prostatectomy?* Ir J Med Sci. 2004 Jan-Mar;173(1):23-6.

Galvin DJ, Mulvin D, Quinlan DM. *Thromboprophylaxis for radical prostatectomy: a comparative analysis of present practice between the USA, the UK, and Ireland.* Prostate. 2004 Sep 1;60(4):338-42.

Connolly SS, O'Toole GC, O'Malley KJ, Manecksha R, O'Brien A, Mulvin DW, Quinlan DM. *Positive apical surgical margins after radical retropubic prostatectomy, truth or artefact?* Scand J Urol Nephrol. 2004;38(1):26-31.

Connolly SS, O'Malley KJ, O'Brien A, Kelly DG, Mulvin DW, Quinlan DM. *Can prostate biopsies predict suitability for nerve-sparing radical prostatectomy?* Scand J Urol Nephrol. 2004;38(3):216-20.

Shaheen A, Quinlan D. *Feasibility of open simple prostatectomy with early vascular control.* BJU Int. 2004 Feb;93(3):349-52.

Koo V, Duggan B, Lennon G. *Spontaneous rupture of kidney with peri-renal haematoma: a conservative approach.* Ulster Med J. 2004 May;73(1):53-6.

Future Plans

The plans for 2005 will focus around continued improvements in ward accommodation for inpatients, particularly regarding privacy, shower and toilet facilities. Again, Urology needs to continue to improve the outpatient setting to provide a seamless transition into the new Ambulatory Day Care Unit. Information technology and secretarial infrastructures will need to be improved to provide a better interface with both referring doctors and their patients. Office space and secretarial support still remains an issue for development. A fourth Urological Consultant is required.

Department of Vascular Surgery

Departmental Review

Staffing

Consultants	Denis Mehigan, Stephen Sheehan, Mary Barry
Senior Registrars	Ronan Cahill, Eoghan Condon
Registrars	Asad Jawad, Mashood Ahmed
Senior House Officers	Shaf Rizan Tajri, Samer Saour Dermot McDowell, John Byrne
Interns	Richard Barry, Cathy Vaughan Deirdre Waterhouse, Deirdre Sweetman Fiona Magee, Ingrid Heaslip
Division Nurse Manager	Maureen Flynn
Clinical Nurse Managers	Niamh Kiely, Bernadine Cawley, Brid Murphy, Mary Leamy
Theatre Sisters	Bernadette Farrell, Maeve Williams Carmel Kelly, Annette Hughes
Plethysmographist	Catherine Kelly
Tissue Viability Nurse	Deirdre Cornally

Service Development/Activities

The Vascular Surgery Unit at St. Vincent's University Hospital continues to be a busy surgical department with good throughput of work despite restrictions imposed by bed shortages. The absence of dedicated 5-day surgical beds has been a particular problem during 2004 and has had an impact on throughput of elective cases in the unit. The High Dependency Unit, in particular, has proven invaluable in improving throughput in the department and has reduced the cancellation of moderate and major vascular surgical procedures. We hope that we may be in a position in the near future to take advantage of beds and theatre facilities in St. Michael's Hospital to improve access for our patients to timely elective 5-day medical care and surgical procedures.

Provision of emergency vascular services to the catchment area has been facilitated by an agreement with St. Michael's Hospital and St. Columcille's Hospital to transfer all vascular emergencies directly to St. Vincent's University Hospital. We provide a similar rapid-access service to our extended catchment area in the South-Eastern Region.

Staff

The Unit is happy to welcome our new nursing colleague, Mary Leamy (CNM1) to St. Joseph's Ward.

Service developments have been restricted again in 2004, but we hope that with the opening of the new hospital, along with improved imaging equipment, we will be able to provide expanded endovascular procedures, minimally invasive/laparoscopic vascular procedures and a comprehensive non-invasive vascular laboratory service.

As part of the Unit's commitment to ongoing quality improvement and clinical audit, patient information leaflets covering all aspects of vascular surgery have been produced. This initiative was led by Mr. Ronal Cahill, Specialist Registrar in Vascular Surgery who produced 15 booklets covering the topics of peripheral vascular disease, intermittent claudication, angiogram/angioplasty, femoropopliteal/ femorodistal bypass grafts, aorto/axillo-bifemoral bypass grafts, limb amputation, abdominal aortic aneurysms, abdominal aortic aneurysm surgery, transient ischemic attacks, carotid endarterectomy, varicose veins, venous leg ulcers, Reynaud's disease and associated conditions, lymphoedema and hyperhidrosis. These booklets will be updated and revised in the future as the need arises.

Department Activity 2004

Admissions	2004	2003	2002
Elective	227	245	330
7 day	216	191	151
5 day	11	54	179
Urgent/Emergency	228	197	179
Day Care	91	71	57
Total	546	513	566

Out Patient Attendances

Out patient sessions	117
New Patients	625
Return Patients	2,335
Total	2960





Department Activity 2004 continued

Operative Procedures	2004	2003	2002
Elective	316	222	243
Emergency	78	87	81
Total	394	309	324

Meetings attended by Members of the Department

European Society of Vascular Surgery, Innsbruck
Vascular Access for Renal Dialysis, London
Screening for Abdominal Aortic Aneurysm, London
Interventional Radiology Meeting, Sheffield
The American College of Surgeons, Chicago

Mr. Denis Mehigan is co-ordinator of the Basic Surgical Training at St. Vincent's University Hospital and is also a member of the Court of Examiners (FRCSI).
Mr. Stephen Sheehan is a Member of the Court of Examiners (FRCSI). He is an instructor on the Basic Surgical Skills Course and on the Advanced Trauma Life Support Course.

Ms. Mary Barry is Secretary of the Basic Surgical Training Committee, RCSI. She is also a faculty member on the Care of the Critically Ill Surgical Patient (CCRISP) Course. Ms Barry lectured on the Revision Vascular Course organised in conjunction with the European Society of Vascular Surgery Meeting held in Innsbruck, Austria in September 2004.

Academic Activity

Grants

Ms Vicki Fera, a third year medical student, received a Health Research Board Summer Student Grant in 2004 to continue work on the role of matrix metalloproteinases in ruptured and non-ruptured aortic aneurysms.

Presentations

1. Prospective Evaluation of the Efficacy of Dermal Botulinum Toxin for Primary Axillary Hyperhidrosis. L. Hanlon, R. Cahill, MC Barry.
   Sylvester O'Halloran Surgical Meeting, Limerick, March 2004.
2. Matrix Metalloproteinase activity in the thrombus of ruptured and non-ruptured abdominal aortic aneurysm. D. Kavanagh, H. Sliger, S. Kieran, JF Dowdall, L. Young, SJ Sheehan, D. Mehigan, MC Barry.
   Sylvester O'Halloran Surgical Meeting, Limerick, March 2004.
3. Impact of a Surgical High Dependency Unit on the Management of Patients with Abdominal Aortic Aneurysm. M. Cleary, RA Cahill, F. Younis, SJ Sheehan, D. Mehigan.
   Sylvester O'Halloran Surgical Meeting, Limerick, March 2004.
4. Utility of CT Angiography in a supraregional vascular surgical service. S. Kieran, RA Cahill, T. Owens, SJ Sheehan, D. Mehigan, MC Barry
   Sylvester O'Halloran Surgical Meeting, Limerick, March 2004.
5. Determination of the effects of peri-operative beta blockade in patients undergoing surgery for peripheral vascular disease. S. Kieran, RA Cahill, S. Saour, SJ Sheehan, D. Mehigan, MC Barry.
   Sir Peter Freyer Surgical Symposium, Galway, September 2004
6. Acute presentation of a popliteal artery pseudoaneurysm in a young patient
   J. Byrne, E. Condon, S. Sheehan
   Royal Academy of Medicine, Clinical Cases Competition December 2004.

Selected Published Papers

1. Young O, Neary P, Mehigan D. *Aorto-enteric fistula secondary to the cannabilisation of an aortic graft.* Eur J Vasc Endovasc Surg (Extra) 2004;8:17-19.

2. Kieran S, Neary P, Owens A, Mehigan. *Inferior vena caval agenesis with paravertebral muscle collateralisation.* Eur J Vasc Endovasc Surg (Extra)2004;8:123-25

3. Kelliher C, Kavanagh DO, Belton F, Dowdall JF, Barry MC. *Primary aorto-duodenal fistula following staphyloccal septicaemia.*
Eur J Vasc Endovasc Surg 2004;27:679-681..

4. Kieran S, Cahill RA, Sheehan S, Barry MC. *Radiation-induced femoral arteritis.* Ir Med J 2004;97:179-180.

5. Kavanagh DO, Richards S, Barry MC, Sheehan S. Aortic Saddle *Embolus causing Paraplegia.* Ir Med J 2004;97:117.

6. Kieran SM, Cahill RA, Sheehan S. Mycotic *peripheral aneurysms and intracerebral abscesses secondary to infective endocarditis.*
Eur J Vasc Endovasc Surg 2004;28:565-56

Liver Transplant Programme and Liver Unit

Staff

Surgical Professor Oscar Traynor, Mr. Justin Geoghegan, Mr. Donal Maguire, Mr. Gerry McEntee.

Medical Professor John Hegarty, Professor Aiden McCormick.

Registrars Dr. Fergal Donnellan, Dr. Asghar Qasim, Dr. Lorraine Power, Dr. Diarmuid Manning, Dr. Tariq Tajuddin, Mr. Emir Hoti, Mr. M. Zaman.

Transplant Co-ordinators
Ms. Aoife Coffey, Ms. Jennifer Fleming, Ms. Yvonne McGarry, Ms. Jane Howe.

Nursing Ms. Maureen Flynn, Ms. Margaret Cullen, Ms. Brenda O'Sullivan, Ms. Anne Brennan, Ms. Elizabeth Burke, Ms. Valerie O'Brien, Ms. Antoinette Flynn, Ms. Dolores Dempsey Ryan, Ms. Susan Patton, Ms. Ann Farrell, Ms. Concepta Molloy, Ms. Siobhan Cronin, Ms. Carol McNulty.

Administration
Ms. Rita Burns, Ms. Noreen Kelly, Ms. Aideen Holden, Ms. Georgina O'Reilly.

Liver Transplant Programme

The year 2004 has been the most successful year to date for the National Liver Transplant Programme. A total of 45 liver transplants were performed on 39 patients. The total number of liver transplants performed since the start of the programme in January 1993 is 339, and the overall success rate is just over 84% (one year survival), which is in keeping with the best rates achieved in large liver transplant units in Europe and the United States.

Organ donation continued at a very satisfactory level during 2004. A total of 82 livers were donated in Ireland during the year and this places us in an almost uniquely favourable position in relation to organ donation. Most transplant units throughout Europe and the United States are struggling to maintain adequate service in the face of reduced organ donation. We are extremely fortunate to have a population who are well disposed to the concept of organ donation.

The success of the National Liver Transplant Programme has resulted in a rapidly increasing referral pattern of patients with complex medical and surgical problems of the liver, which do not necessarily require liver transplantation, but who might be treated non-surgically or by types of surgery other than transplantation. This referral pattern has had highly beneficial effects on training for doctors, nurses and paramedical staff at St Vincent's University Hospital. At the same time however, it has stretched the resources of the Intensive Care Unit and the Department of Diagnostic Imaging. Furthermore, it has placed great strain on the bed availability in St Brigid's Ward. For this reason, the hospital has undertaken a major expansion and refurbishment of the liver unit during 2004.The bed complement has increased to 21, and this includes five HDU beds for liver transplant patients and patients who had major liver surgery. The expanded unit should be fully open by mid 2005 and will greatly enhance our ability to deliver a service that matches the requirements of the National Liver Transplant Programme.

A major milestone for the National Liver Transplant Programme during 2004 was the opening of the Halfway House. This facility provides accommodation for relatives of patients who are in St Brigid's Ward and also for some patients who have been discharged from hospital but are not yet ready to return to distant parts of the country. Great credit is due to Mr Gerry McEntee who first proposed the Halfway House and organised intensive fund raising for this venture and also to the Hospital Board of Management who made a major investment in the development of the Halfway House.

Liver Research Group

The Liver Research Group is currently concerned with a variety of research areas including Liver Transplant Immunology and the mechanisms of Ischaemia Reperfusion Injury following Human Liver Transplantation. Research activity continues in the areas of Interferon resistance in chronic Hepatitis C virus infection and the mechanisms underlying liver damage, which occurs after transplantation for chronic

Hepatitis C virus infection. Fifteen scientific papers were presented at National and International meetings. Dr. Aiden McCormick was in receipt of a Newman Chair in Clinical Research.

Professor Cliona O'Farrelly was in receipt of Isla Haliday Memorial Lecture and Conway Lecturer and Medallist. Additional funding for ongoing research activities was obtained from ROCHE Pharmaceuticals, InVitrogen Life Technologies and U.S. Dept. of Veterans Affairs.

Selected Publications

(See also ERC Publications listing)

Golden-Mason L, D.C. Doeuk, R.A. Koup, J. Kelly, J.E. Hegarty, C. O'Farrelly
Adult human liver contains CD8pos T cells with naïve phenotype but is not a site for conventional ab-T cell development.
J. Immunol 2004 May 15;172(10):5980-5 PMID 15128780

Kelly A, L.Golden-Mason,G. McEntee, O. Traynor, J.Hegarty, D.Doherty, C. O'Farrelly
Interleukin 12 (IL-12) is increased in tumour-bearing liver and expands CD8(+) and CD56(+) T cells in vitro but not in vivo.
Cytokine 2004 Mar 21;25(6):273-82 PMID 15036243

Barry J, Bourke M, Buckley M, Coughlan B, Crowley D, Cullen W, Dooley S, Keating S, Kelleher D, Moloney J, Murray F, McCormick PA, MacMathuna P, O'Connor J, O'Grady J, O'Sullivan C, O'Sullivan P, Quinn C, Smyth B, Sweeney B;
Dublin Area Hepatitis C Initiative Group. Hepatitis C among drug users: consensus guidelines on management in general practice.
Ir J Med Sci. 2004 July –Sep; 173 (3) : 145-50.
PMID: 15693384 [PubMed – in process]

McCormick PA, O'Rourke M, Carey D, Laffoy M.
Ability to pay and geographical proximity influence access to Liver Transplantation even in a system with universal access. Liver Transpl. 2004 Nov; 10 (11) : 1422 - 7.
PMID : 15497151 [PubMed – in process]

Bexis S, Vandeputte C, McCormick PA, Docherty JR.
Deletion of inducible nitric oxide synthase decreases mesenteric vascular responsiveness in portal hypertensive mice.
Eur J Pharmacol. 2004 Sep 24; 499 (3) : 325-33. PMID: 15381055
[PubMed – indexed for MEDLINE]

Kavanagh PM, Roberts J, Gibney R, Malone D, Hegarty J, McCormick PA.
Acute Budd-Chiari syndrome with liver failure: the experience of a policy of initial interventional radiological treatment using transjugular intrahepatic portosystemic shunt.
J Gastroenterol Hepatol. 2004 Oct; 19 (10) : 1135-9



Medical and Surgical Gastroenterology incorporating Centre for Colorectal Disease

Departmental Review

Staff

Consultant Medical and Surgical staff

Medical Gastroenterology	Professor DP O'Donoghue, Dr HE Mulcahy
Colorectal Surgery	Mr J Hyland, Mr D Winter
GI Pathology	Dr K Sheahan, Dr D Gibbons
GI Diagnostic Imaging	Dr C Collins, Dr R Gibney, Dr D Malone
Medical Oncology	Dr D Fennelly
Radiation Oncology	Dr M Moriarty, Professor J Armstrong

Non-Consultant Medical and Surgical staff

Specialist Registrars	Dr G Cullen: Dr J Canavan: Mr B Mehigan

Clinical Nurse Managers

St. Teresa's Ward	Ms A Smyth, Ms G O'Neill
St. Patrick's Ward	Ms A Kiely, Ms B Nolan
Endoscopy Unit	Mr R Marshall, Ms G Hickey

Research Staff

Senior Research Scientist	Dr J O'Sullivan
Senior Biochemist & Hon. Lecturer	Dr D Leahy
Research Assistant	Ms M Tosetto
Research Fellows	Dr A Coss, Dr J Sheridan
Research Nurse	Ms D Keegan
PhD student	Mr E Fox
MSc Student	Dr S Bakhiet

Dieticians

Medical Gastoenterology	Ms M Doyle
Surgical Gastroenterology	Ms C Lamont

Specialist Support Staff

Colorectal Pathology Technician	Mr R Geraghty
Cancer Nurse Co-ordinator	Ms A White
Colorectal Nurse Specialist	Ms G McEvoy
Colorectal Nurse Specialist	Ms N Smyth
Secretary	Ms M Dargan

Clinical Developments/ Activities

Medical and Surgical Gastroenterology continues to play a major role in the activities of St Vincent's Health Care Group. Close to one third of all hospital admissions fit into the category of acute or chronic gastroenterological disease and it is appropriate that the medical and surgical departments work so closely together to provide a patient oriented service. For some years now, Medical and Surgical Gastroenterology have run a combined Colorectal clinic which is probably unique in this island. This clinic, plus weekly Radiology and Pathology conferences run with our colleagues in Diagnostic Imaging and Pathology, allow for best practice in patient care and provide an excellent source of teaching for NCHDs. Further combined meetings include a monthly Audit of Colorectal Cancer and monthly research symposia in Inflammatory Bowel Disease and Cancer. We wish to acknowledge the close clinical and scientific support we receive from our colleagues in Medical and Surgical Oncology and Palliative Care.

The Centre for Colorectal Disease is an umbrella for clinicians and scientists with an interest in Colorectal Cancer, Inflammatory Bowel Disease and Functional bowel disorders. The aims of the centre are the delivery of state of the art services for our patients, research into causes and treatment and the education of the public. However, the multidisciplinary team would be unable to function successfully without the aid of nursing co-ordinators and nurse specialists such as Anne White, Grace McEvoy and Nikki Smyth, who lead patients through the varied and complex pathways of diagnosis, treatment and postoperative stoma care in a way that appears seamless. Grace and Nikki are to be congratulated on their production of an information book for patients undergoing total colectomy. This production has been enthusiastically received by many units throughout the country. A further major addition to the Centre in 2004 was the appointment of a second Colorectal surgeon, Mr Des Winter, to the hospital. Des is currently working in the Mayo Clinic and is due to return to join the unit in late 2005

We also wish to put on record our thanks to our nursing colleagues on St.Teresa's and St. Patrick's wards who manage our gastroenterology patients despite major involvement in general Medical and Surgical call. The arrival of Mr Richard Marshall to the post of CNM2 in the endoscopy unit

has proven very timely. His management skills will be of immense importance as we move into the new 3-roomed Endoscopy Department late in 2005. As usual, Ms Nuala Donnelly and staff in St. Mark's Day ward produce beds for our sick patients. Like us, they are looking forward to moving to a larger and purpose built ward adjacent to the new Endoscopy Department. This will facilitate the rapid transfer of patients between units.

Research

The Centre for Colorectal Disease has, for the past 25 years, had an active background in both basic gut immunology and clinical and basic colorectal cancer research and has been responsible for bringing on a new generation of basic science and clinical researchers. Higher degrees written up in 2004 include those of Sean Cochrane (MD) and Anne Marie Lennon (PhD). In addition, the arrival of Dr Jacintha O'Sullivan, Senior Scientist, has especially helped push the research we perform to a higher plane. New immunological, molecular and cell culture techniques have been introduced to our scientific armamentarium allowing us to extend the scope of the research we perform. This has also been helped by additional research funding and we would like to acknowledge the generous donations for research and education from benefactors.

The scientific, clinical and financial changes to the Centre also allowed us to expand our personnel during 2004. A very necessary appointment was that of a Scientific Research Assistant in the Centre, Ms Miriam Tosetto. In addition, two new MD thesis students joined the Centre. Dr Juliette Sheridan is now conducting research into colorectal cancer in young patients and Dr Alan Coss is exploring the role of proteases in the progression and outcome of early colorectal cancer. As always, our collaborations with other research departments both within the UCD campus and farther afield have added considerable value to our research and we would like to thank Professor Cliona O'Farrelly and Professor Alan Baird, amongst others, for their continued collaborations.

Other highlights of 2004 included the election of Mr. John Hyland to President of the Coloproctology Association of Great Britain and Ireland. During the year he gave the Freyer State of the Art Lecture 2004 *"The Evolution of Colorectal Surgery"* and also the Lockhart Mummery Lecture Basingstoke 2004 *"Laparoscopic Colorectal Cancer Surgery"*

Additionally, the ever-popular International Colorectal Meeting was once again held before a large audience in the Education and Research centre in September. It was a particular pleasure to welcome Professor Oscar Traynor as the state of Art lecturer at our eleventh meeting

Grants Active in 2004

(See ERC section)

Selected Peer Reviewed Research Publications in 2004 – Gastroenterology

(See also ERC Publications Listing)

O'Keeffe J, Doherty DG, Kenna T, Sheahan K, O'Donoghue DP, Hyland JM, O'Farrelly C. *Diverse populations of T cells with NK cell receptors accumulate in the human intestine in health and in colorectal cancer.* European Journal of Immunol, 2004; 34: 2110-9.

Troy AM, Sheahan K, Mulcahy HE, Duffy MJ, Hyland JM, O'Donoghue DP. *Expression of Cathepsin B and L antigen and activity is associated with early colorectal cancer progression.* European Journal of Cancer. 2004; 40: 1610-6.

Anker P, Zajac V, Lyautey J, Lederrey C, Dunand C, Lefort F, Mulcahy H, Heinemann J, Stroun M. *Transcession of DNA from bacteria to human cells in culture: a possible role in oncogenesis.* Annals of the New York Academy of Science 2004; 1022: 195-201.

Zierau O, O'Sullivan J, Morrissey C, Wünsche W, Schneider M, Tenniswood M and Vollmer G. *Tamoxifen exerts agonistic effects on clusterin and complement C3 gene expression in RUCA-I primary xenografts and metastases but not normal uterus.* Endocrine Related Cancer. 2004; 11: 823-30.

Schoemaker NE, Kuppens IE, Moiseyenko V, Glimelius B, Kjaer M, Starkhammer H, Richel DJ, Smaaland R, Bertelsen K, Poulsen JP, Voznyi E, Norum J, Fennelly D, Tveit KM, Garin A, Gruia G, Mourier A, Sibaud D, Lefebvre P, Beijnen JH, Schellens JH, ten Bokkel Huinink WW. *A randomised phase II multicentre trial of irinotecan (CPT-11) using four different schedules in patients with metastatic colorectal cancer.* British Journal of Cancer. 2004; 91: 1434-41.

Outpatient Department

Departmental Review

The following consultants commenced clinics in the Outpatient Department:

Dr. Niall Tubridy	Consultant Neurologist
Prof. John Armstrong	Professor of Radiation Oncology
Sean Carroll	Consultant Plastic Surgeon
Mr. Richard Mascarenhas	Locum Consultant General Surgeon

Staff

Margaret Boland	- appointed as Divisional Nurse Manager
Mary Farrelly	- joined the reception team as Grade IV Officer

Service Developments/Activities

The Outpatient Committee met regularly during the year. The main focus of the group was to participate in planning for the changeover to the new Ambulatory Day Care, which will happen in 2005.

Outpatient Department staff were involved with Quality Improvement teams as part of the accreditation process. The first quarter of the year focused on final preparation for the survey, which took place in May.

Neurology clinics for new patient referrals commenced in St. Michael's Hospital in October. This was organised in conjunction with Prof. Hutchinson, Dr. Tubridy and staff from St. Michael's Hospital. The overall aim is to reduce the waiting time for new referrals. All follow-up patient visits take place in St. Vincent's University Hospital.

The Liver Transplant Clinic changed from alternate weeks to weekly.

The Podiatry service was extended to a full day provided by Ms. E. O'Brien.

Procedures were put in place to allow for the discharge of outpatients on the Patient Administration System (PAS). By recording discharges on the PAS, inappropriate reviews without a proper referral letter are eliminated.

For clinics shared by more than one consultant, new computer templates were setup to allow for registration of patients to the appropriate named consultant.

Departmental Statistics

In the year 2004 a total of 3,942 sessions were held, which was an increase of 7.1% over the previous year. Total attendances were 90,879 of which 20,638 were new patients. Overall, attendances showed an increase of 4.2% from last year. DNA's (did not attend) for the year were 18,643 and walk-ins were recorded as 2,974.

Future Plans

- Planning for the move to the new building will be on going.
- Review of the DNA policy is taking place and will continue in the New Year.

Surgical Professorial Unit St. Raphael's Ward

Departmental Review

Staff

Consultants	Professor N. O'Higgins, Mr J. J. Murphy, Mr E. W. M. McDermott, Mr A. D. K. Hill, Mr R. Mascarenhas
Senior Registrars	Mr P. Ridgway, Mr Karl Sweeney
Adjunct Professor	Professor M. J. Duffy
College Lecturers	Dr P. P. A. Smyth, Dr Leonie Young
Special Lecturers	Mr Steve Richards, Ms Usha Shan
Registrar in Breast Diseases	Mr Athar Sheikh
Post-Doctoral Research Scientis	Ms Marie McIlroy
Research Fellows	Dr Claire Davidson, Dr Mary Dillon, Dr Dhafir Alazawi
PhD Student	Ms Niamh Cosgrove
Clinical Research Associate	Ms Trudy Roche
Clinical Nurse Manager	Ms Gillian Webster
Advanced Nurse Practitioner	Sr Mary Murray
Clinical Nurse Specialists in Breast Care	Ms Marina Nolan, Ms Gillian O'Donnell
Breast Cancer Co-ordinator	Ms Catherine Masterson
Senior Executive Assistant	(To be appointed)
Executive Assistants	Ms Elaine O'Sullivan, Ms Sinead Carey
Senior Laboratory Technician	Mr Dermot Carty

Undergraduate Activities

The enthusiastic teaching by the lecturers and consultant staff has been one of the outstanding features of St Vincent's University Hospital for many years. The strong support and interest of the consultant staff as well as the non-consultant Hospital doctors is greatly appreciated by the academic staff and by all the students. The recent arrangement whereby the Final Examination in Surgery takes the form of an Objective Structured Clinical Examination has been introduced and is under assessment.

Departmental Developments

Professor O'Higgins assumed office for a period of two years as President of the Royal College of Surgeons in Ireland. He has completed his three-year term as President of the World Federation of Surgical Oncology Societies and organized a meeting in Los Angeles between the WFSOS and the American Society of Surgical Oncology. He was Visiting Professor at the Royal Australasian College of Surgeons meeting in Brisbane and during 2004 was External Examiner in Kuwait and Malaysia.

Mr E. W. McDermott, in his role as President of the Irish Society of Surgical Oncology, organized a meeting of this society with the Sir Peter Freyer Symposium in Galway and continues his role as Chairman of the Cancer Care Committee of St Vincent's University Hospital.

Mr A. D. Hill delivered the St Luke's lecture entitled "Molecular oncology – current themes and future directions". He has been chosen as the James IV Surgical Traveller for 2005 by the James IV Association of Surgeons Inc. He was elected General Secretary of the Society of Academic and Research Surgery and is on the Scientific Program for the Association of Surgeons of Great Britain and Ireland. He continues as Secretary of the Royal Academy of Medicine in Ireland and of the Surgical Section.

Professor M. J. Duffy has been awarded a five-year programme grant by the Health Research Board.

Mr Denis Evoy has been appointed Consultant Surgeon with special interest in Breast Diseases with particular involvement in BreastCheck, the National Breast Screening Programme. He will take up his appointment on the 1st of April 2005. We look forward to him joining the Surgical Professorial Unit and wish him well.

Dr Leonie Young was successful in obtaining a grant for €200,000 from the Health Research Board to develop a Breast Cancer Research Portal, a singular achievement.

Sr Mary Murray was appointed the first Advanced Nurse Practitioner in Breast Care in Ireland in St Vincent's University Hospital and was accredited on the 1st of December 2004. We congratulate her on this achievement. The role of the Specialist Breast Care Nurse is now well recognized and is established throughout the country, mainly due to the Breast Services activity at St Vincent's University Hospital.



The year has been a particularly successful one for surgical trainees on the research front. Mr Paul Ridgeway was Invited Lecturer at the Society of Laproscopic Surgeons in Miami. His talk was on "Surgical training and simulation". Mr Karl Sweeney received the Bennett Medal in St James's Hospital for his presentation entitled "The value of adjuvant therapy in breast cancer".

Within the Surgical Professorial Unit Mr Eddie Myers from the Research Laboratory was awarded the Ronald Raven Prize for 1st place in the British Association of Surgical Oncology research meeting in London. He was also awarded 1st place at the William O'Keeffe Medal in October at the Surgical October Club in Waterford and finally won the prestigious Patey Prize at the Society of Academic and Research Surgeons meeting in the United Kingdom. Research theses for Higher Degrees were submitted by him and the other research fellows, Ruth Pritchard, Dara Kavanagh and May Cleary.

Clinical activity of the unit, including inpatient care and day care, saw a total of 2,537 admissions during the year 2004. In the Symptomatic Breast Clinic a total number of 7,186 attendances was recorded. This included 2,238 new patients and 4,998 return patients. The unit treated 422 newly diagnosed breast cancer patients, 257 directly from the Symptomatic Service and 165 from BreastCheck, the National Breast Screening Programme.

The Triple Assessment Clinic for Breast Diseases was relocated to be closer to the Radiology Department. This is much more convenient for the patients and represents a significant improvement in the services.

Publications

Of the 24 publications in peer-reviewed journals the following are representative:-

Myers E, Fleming FJ, Crotty TB, Kelly G, McDermott EW, O'Higgins NJ, Hill AD, Young LS.
Inverse relationship between ER-beta and SRC-1 predicts outcome in endocrine-resistant breast cancer.
British Journal of Cancer. 2004 Nov 1; 91 (9): 1687-93.

Buggy Y, Maguire TM, McGreal G, McDermott E, Hill AD, O'Higgins N, Duffy MJ.
Overexpression of the Ets-1 transcription factor in human breast cancer.
British Journal of Cancer. 2004 Oct 4; 91 (7): 1308-15.

O'Higgins N.
The world federation of surgical oncology: the global mission
Journal of Surgical Oncology. 2004 Sep 1; 87 (3): 109-15. Review.

Fleming FJ, Hill AD, McDermott EW, O'Doherty A, O'Higgins NJ, Quinn CM.
Intraoperative margin assessment and re-excision rate in breast conservation surgery.
European Journal of Surgical Oncology. 2004 Apr; 30 (3): 233-7.

Fleming FJ, Hill AD, McDermott EW, O'Higgins NJ, Young LS.
Differential recruitment of coregulator proteins steroid receptor coactivator-1 and silencing mediator for retinoid and thyroid receptors the estrogen receptor-estrogen response element by beta-estradiol and 4 hydroxytamoxifen in human breast cancer.
Journal of Clinical Endocrinol Metab. 2004 Jan; 89 (1): 375-83.

Duffy MJ.
The urokinase plasminogen activator system: role in malignancy.
Curr Pharm Des. 2004; 10 (1): 39-49. Review.

Department of Nursing

Departmental Review

Ms. Pauline Doyle retired as Director of Nursing in April 2004. During her tenure, Ms Doyle was responsible for significant achievements, including the restructuring of Nurse Management, the introduction of Pre-registration Diploma and Degree in Nursing and the development of Clinical Nurse Specialists. Pauline actively championed the continuing professional development of nursing at hospital and national level and never lost sight of the importance of patient care. Pauline's contribution to the DATHS group is widely acknowledged. Her kind and caring approach to patients, staff and their families will be missed. We wish her every happiness in her retirement.

The Nursing Department continues to develop and provide quality-nursing care to patients in line with the vision, ethos and mission of the hospital. The education and professional development of registered nurses, nursing students and nursing support staff is integral to the department's philosophy. Ms. Margaret Moran, Director of the Centre of Nurse Education, worked closely with the School of Nursing and Midwifery UCD to develop, organise and deliver a high standard of education and support for nurses. Ms Catherine Walshe, Post Registration Course Co-ordinator, Clinical Course Facilitators, Nursing Practice Development and nursing staff in the clinical areas, all play a pivotal part in achieving these aims. In addition to the BSc in Nursing programme, St Vincent's University Hospital provides clinical placement to ten post registration higher diploma nursing courses in partnership with UCD, as well as supporting a number of research projects in nursing at Masters level.

Clinical Nurse Specialist Roles

Anticoagulant	Transfusion Surveillance/Haemovigilance
Cancer Nurse Co-ordinator	CAPD
Coloproctology	Cardiac Rehabilitation
Dermatology	Cystic Fibrosis
Haematology Liaison	Diabetic
Heart Failure	Health Promotion
Hepatitis C	Infection Control
Liver Transplant Coordinator	Lung Transplant Coordinator
Multiple Sclerosis	Occupational Health
Oncology Liaison	Osteoporosis
Palliative Care	Orthopaedic Casting and Splinting
Respiratory	Sleep Apnoea
Rheumatology	Tissue Viability
Stroke Care	Urodynamics

Service Developments and Activities

External study is actively supported by a comprehensive study leave policy, which is availed of by an increasing number of nursing staff. A total of 466 nurses received support for a variety of courses over the year.

The National Council for Professional Development of Nursing and Midwifery granted funding for a number of projects for nursing:

- Training for two facilitators to run the Leading an Empowered Organisation (LEO) three-day course for the Health Care Group. It is envisaged that this training will be rolled out to CNM grades in 2005.
- To purchase software for clinical audit and train nursing staff in it's application.
- Additional funding was granted for Advanced Cardiac Life Support (ACLS) training for nurses working in critical care areas.
- A facilitation programme was sponsored in conjunction with the ERHA for Clinical Placement Co-ordinators and Practice Development Staff.
- Funding to provide a series of lectures on the legal aspects of documentation.

Nursing Divisions	Areas of Responsibility
Cardio Respiratory	Coronary Care Unit, Cardiology Ward, St. John's Ward, St. Paul's Ward, Cardiac Rehabilitation, Respiratory, Cystic Fibrosis and Heart Failure.
Critical Care	Intensive Care Unit, St. Brigid's Ward, St. Joseph's Ward, High Dependency Unit, Liver Unit (Liver Transplantation and Hepatitis C) and Tissue Viability Services.
General Medical	St. Kevin's Ward, St. Michael's Ward, St. Patrick's Ward, St. Mark's Ward, St. James' Ward, Haemoviligance, Diabetes Centre, Anticoagulant Services and Occupational Health.
Neurology, Medicine of the Elderly and Psychiatry	St. Agnes' Ward, St. Vincent's Ward, St. Monica's Ward, Our Lady's Ward, St. Camillus Ward, Day Hospitals (Psychiatry of Old Age, Medicine for the Elderly and Camillus), Stroke Rehabilitation, Infection Control and Multiple Sclerosis.
Oncology, Palliative Care and Professorial Surgical	St. Anne's Ward and Day Centre, St. Raphael's Ward, St. Teresa's Ward, St. Catherine's Ward, Palliative Care, Breast Care, Haematology and Cancer Co-ordinators.
Theatre	Operating Department, Cardiac Catheterisation Laboratory, Endoscopy and Radiology
Urology, Gynaecology and Nephrology	St. Charles' Ward, St. Peter's Ward, Dialysis Unit, St. Laurence's Ward and Urodynamics, Continuous Ambulatory Peritoneal Dialysis
Emergency Department, Ambulatory Day Care Infection Control	Emergency Department, Ambulatory Day Care Infection Control

Cardio Respiratory Division

Ms. Anne Henley, Divisional Nurse Manager

Coronary Care Unit

Thirteen students undertook the Higher Diploma in Nursing Studies Critical Care (Cardiovascular Pathway) year 1 and year 2. CCU facilitated thirty students from other hospitals on placements and also facilitated the placement of a number of Emergency Medical Technicians from Dublin Fire Brigade.

St. Paul's Ward

Three staff members completed their Bachelor of Science Nursing from UCD.
Ms. Orla Finnegan attended the North American Cystic Fibrosis conference in St. Louis Missouri in October 2004.

Cystic Fibrosis

Catherine Carroll was appointed as the third Clinical Nurse Specialist in Cystic Fibrosis.

St. John's Ward

Plans for the development of a 6-bedded non-invasive ventilation unit (NIV) continue. Eight nurses have completed an accredited nursing module on non-invasive ventilation. Equipment for the unit was procured. A multidisciplinary working group is in the process of developing policies and guidelines for the unit.

Cardiac Rehabilitation

The Cardiac Rehabilitation Service continues to offer four phases of rehabilitation to patients recovering from a Myocardial Infarction, Angioplasty, and Coronary Artery Bypass Surgery/Valve Surgery. There has been a notable increase in the number of patients seen over the last year. 460 patients were seen at Phase I. At Phase II, all patients were phoned post their event and 269 patients were met on an individual basis. 141 patients attended our six-week Phase III programme and 100 attended our Phase IV refresher sessions.

The Monthly Lifestyle Information Sessions continue for patients and their families, 199 people attended during 2004.

The Cardiac Rehabilitation database, which commenced at the beginning of 2003, is ongoing.

Developments

With the increasing number of patients being referred, a group rather than individual approach at Phase II was instigated. This involves giving preliminary education and lifestyle advice, which is delivered by a Cardiac Rehabilitation Co-coordinator and a Physiotherapist.

This year saw the introduction of obesity management in the cardiac rehab groups. This allows obese patients to exercise individually in the gym and to join the rest of the group for group therapy and education sessions.

Phil Pyne, Psychotherapist, also presented a paper on "Cognitive Behavioural Psychotherapy-useful in the Management of Anxiety and Depression in Cardiac Rehabilitation".

The members of the cardiac rehabilitation team, along with Sinead Keogh CNM1 CCU, compiled patient information booklets on discharge advice post angioplasty and myocardial infarction. These should be available early in 2005.

Equipment

A patient kindly donated funds for the purchase of a bicycle and rower in the cardiac rehabilitation gym.

Appointments

Lorna Timmons and Niamh O'Sullivan CNS (Job-Sharing) in Cardiac Rehabilitation resigned during the summer of 2004. Kathryn O'Sullivan, who is currently studying for an M.Sc. in Cardiac Rehabilitation, was appointed to the position.

The Heart Failure Unit

The Heart Failure Unit, located at St. Michael's Hospital, had 2,684 patients attend the Out patient Department. The number of re admissions for heart failure to St. Vincent's University Hospital has significantly reduced due to the excellent service delivered by the multidisciplinary team.

Critical Care Division

Ms. Maureen Flynn, Divisional Nurse Manager

One hundred and fourteen (wte) nurses and 12 (wte) support staff work across the Critical Care Division. In the later part of the year, the Endoscopy Department was temporarily included within the division pending the appointment of the Divisional Nurse Manager for the Operating Department.

The effectiveness of the Critical Care Nurse Leaders Group established in 2003 was positively evaluated and continued to meet monthly throughout 2004. Educational presentations were given on the Report of the National Taskforce on Medical Staffing - *The Challenge for Nursing and Midwifery by Ms. Aileen Lynch; Professional Development Planning an Introduction,* by Ms. Christine Hughes; and the *Role of the Occupational Psychologist at St. Vincent's University Hospital* by Ms. Nuala Gannon.

The Clinical Nurse Managers Group established for the Intensive Care Unit was positively evaluated and continued to meet on a monthly basis. The group prepared a high quality leaflet and poster *Thinking About a Career in Intensive Care Nursing*, which was widely circulated across the country and resulted in maximum uptake of places on the Higher Diploma in Critical Care Nursing programme.

Developments

- Nursing staff across the division actively participated in the i) Liver Transplant and Hepatitis C; ii) Theatre, Critical Care and Anaesthesia; and iii) Cardiology, Respiratory, Thoracic and Vascular, Accreditation Quality Improvement Teams.
- In May 2004, St. Brigid's Ward moved to the 11 bedded refurbished area (formally known as St. Clare's Ward). This area provides greatly enhanced facilities for patients and staff including wheelchair accessible en-suite bathrooms; clinical room for treating liver transplant patients requiring Hepatect infusion on an out patient basis; and heptafiltration for the entire ward and day room.
- The nursing team actively participated in the planning for the structural redevelopment and refurbishment of the St. Brigid's Ward, which commenced in August 2004.
- In July 2004, the High Dependency Unit closed for three days for the installation of air conditioning to enhance the care environment and minimise risk.
- Following an evaluation of a range of disposable chest drains and portable chest suctions (by the nursing and medical teams) a new system of care was introduced for the High Dependency Unit and St. Joseph's Ward.
- Guidelines for arranging admission and continuing medical care for patients in the Thoracic/Vascular High Dependency Unit were prepared by the Nursing and Medical Team and authorised by the Medical Executive in October 2004.
- The nursing team in St. Joseph's Ward/HDU provided input in the development of patient information leaflets on a range of vascular and thoracic conditions.
- The ICU Nursing team prepared the *Intensive Care Unit – Visitor Information Booklet.*
- The Clinical Nurse Specialists and the Divisional Nurse Manager represented the Hospital at the ERHA Hepatitis C Forum.

Education

- The Critical Care Clinical Facilitators and the Tissue Viability CNS, in association with Nurse Practice Development, prepared fourteen new evidence based Policies Procedures and Guidelines for nursing care launched in December 2004.
- Seven nurses successfully completed the Higher Diploma in Critical Care Nursing and twelve nurses commenced the Higher Diploma in Critical Care Nursing in September 2004.
- The two-week placement in the High Dependency Unit (HDU) was evaluated and continued for the Higher Diploma in Critical Care Nursing students.
- A two-week orientation to critical care nursing practice in Ireland was arranged for four new members of nursing staff (St. Brigid's Ward) in the General ICU, during 2004.
- For the first time, a clinical placement was facilitated in St. Brigid's Ward for the BSc first year nursing students and the third year rostered placement. This initiative was warmly welcomed by the staff of St. Brigid's Ward.

- A self directed learning pack incorporating liver disease and the transplant process was developed by Ms. Brenda O'Sullivan and is being used by all nursing team members.
- Commencing in 2004, links were developed with University of Central England, Birmingham to facilitate staff from St. Brigid's Ward in completing the *Principles of Hepatology and Transplant Care Course.* Ms. Brenda O'Sullivan and Ms. Anne Brennan successfully completed the programme. It is intended to facilitate two members of staff to complete the course on an annual basis.
- In 2004, all nursing staff across the Critical Care Division were actively encouraged and supported to complete the Advanced Cardiac Life Support (ACLS) two-day course.
- A number of nurses across the division successful completed Degrees in Nursing and Management.

In addition to this report the Liver Transplant Co-ordinators, Hepatitis C Clinical Nurse Specialists, Tissue Viability Clinical Nurse Specialist, and the Clinical Facilitators from ICU and St. Brigid's Ward each prepared and circulated detailed reports of their activities for 2004.

General Medical Division

Ms. Margaret Murphy, Divisional Nurse Manager

Comprising 74 in-patient beds, 9 ophthalmology in-patient beds and day care service, the diabetes centre, haemovigilance, and anti-coagulant service, the Occupational Health Department and 12 ambulatory day care beds are also incorporated in this division.

Throughout all areas, high standards of patient care were delivered while operating to almost 100% occupancy. The Anti Coagulant Service continued to be developed throughout 2004 by the CNS, Ms Elaine Tallon, who was awarded post graduate diploma in Clinical Practice by Trinity College Dublin.

By the latter half of the year, the Occupational Health Department had a full compliment of Clinical Nurse Specialists and was looking forward to rolling out the service to St. Vincent's Private Hospital. The Diabetes Centre continued to be very busy with the Clinical Nurse Specialists seeing in excess of 8,500 patients. The initial Accreditation process was completed in May and the division was pleased with the outcome and look forward to implementing the quality improvement plans.

Neurology, Medicine of the Elderly and Psychiatry Division

Mr. Patrick Gargan, Divisional Nurse Manager

Together with its core departments, St Agnes' Ward, St Monica's Ward and Infection control are also incorporated in this division, which comprises 102 in-patient beds and three day hospitals. Preparatory work on the merging of our Department of Psychiatry services with those of Vergemount Clinic, Clonskeagh continued with the establishment of a joint working group to review and merge existing policies in both centres. St. Monica's Ward became part of the newly established division under Margaret Boland in November. The Clinical Nurse Specialists in Infection Control developed an education programme for the Infection Control Liaison Nurses. The Department of Neurology held two study days, which were well attended. Plans are at an advanced stage for the development of a Parkinson's Nurse Specialist.

Developments in the Department of Medicine for the Elderly included the up grading of a nursing post to Clinical Nurse Manager 1 grade. It is hoped that this will provide further scope for the on-going development of Our Lady's Ward / Stroke Care Unit. We welcomed many new staff to the division from a variety of different nationalities and we look forward to the coming year with renewed enthusiasm.

Oncology, Haematology, Palliative Care and the Surgical Professorial Division

Mrs. Tanya King, Divisional Nurse Manager

The Oncology/Surgical division comprises 101 in-patient beds and a combination of out-patient and day care services. The nursing staff in the division continued to showadaptability and commitment in dealing with the pressures created by the many emergency admissions. This was particularly apparent in St Catherine's Ward, where the profile changed from a 5-day to a 7-day service. Throughout the division, high standards of nursing care were maintained and many of our nurses were members of the Accreditation teams.

Developments:

Ms Mary Murray, Advanced Nurse Practitioner Candidate, completed an outstanding report on site preparation. This resulted in St Vincent's University Hospital receiving approval as a site for an Advanced Nurse Practitioner in Breast Care from the National Council for Nursing and Midwifery. This is the first such post in Ireland.

Following redevelopment of the area where St Anne's Day Centre was previously located, a new dedicated Outpatient area was opened. This will accommodate Oncology, Haematology, Breast, Palliative Care and Nurse-led clinics.

Education

Ongoing further education is facilitated for the following strands of the Higher Diploma in Nursing Studies: Cancer Nursing, Palliative Care, Breast Care. Work



continued on the development of a specialist strand in Colorectal Nursing as part of the Higher Diploma in Gastroenterology. This will be facilitated in St Vincent's University Hospital.

Seven nurses successfully completed the Higher Diploma in Cancer Nursing, two of which completed the specialist Breast Care strand. Other courses that nurses are currently undertaking are:

- BSc. Nursing
- MSc. Nursing
- Diploma in Management

Theatre Division

Ms. Anne Grumley Divisional Nurse Manager

During 2004, The Theatre Division maintained a high standard of patient care. This was made possible through the commitment and dedication of the nursing staff, as part of the multi-disciplinary team.

Through the process of Accreditation, the value of multi-diciplinary team approach to service delivery and quality care was realised. In undertaking this process, specific quality initiatives have commenced.

The division welcomed colleagues from overseas to the team and staff were involved in the planning process for the new development due for completion in 2005.

The Cardiac Catheterisation Laboratory provided both diagnostic and interventional procedures. Plans for its refurbishment were at an advanced stage by the year-end. The Endoscopy Unit had a busy year and radiology nursing continues to offer many new challenges, with the increases in new techniques and procedures.

Staff Development and Education

Nurse education and the commitment to staff development is an ongoing process. Through formal educational sessions and attendance at conferences, nursing staff are maintaining their levels of competencies and keeping up-to-date with what is best practice. Staff within the department undertook the education programmes or attended the conferences as listed below.

- Higher diploma in Peri-Operative and Peri-Anaesthesia. Staff Nurses successfully graduated from these courses.
- Irish Operating Room Nurses Conference.
- National Association of Theatre Nurse (NATN).
- European Operating Room Nurses Association.

Staff Retirement

Ms. Ann Grumley retired from her position as Divisional Nurse Manager in the Operating Theatre Department after fifteen years in charge. During this time she saw many changes through the development of technology, coupled with the increasing complexity of surgery. Her commitment and dedication to her work was exemplary to her colleagues. All her colleagues wish Ann a long and healthy retirement.

Staff Appointments 2004

- Ms. Sheila Breiden as Clinical Nurse Nurse Manager 2
- Ms. Antoinette Cullen as Clinical Nurse Manager 2

Presentations

Tracking Surgical Instruments - *Dream or Reality* - Ita Balfe
Compliance with Standard Precautions among Peri-operative Nurses - Sheila Breiden

Poster Presentations

The Role Massage can have in the Operating Theatre - Lucy O'Connell
Complications from Tourniquet Use- Deirdre Morris, Michelle Singsby Smith

Future Plans

The new Operating Theatre Department will be the main focus of attention in 2005. What seemed a lifetime away is now imminent. There has been an enormous amount of work and commitment from all staff towards realising this state of the art Operating Theatre facility. People have committed time and energy to drafting operational policies and preparing presentations.

There will be enormous challenges in the process of moving, while at the same time maintaining the current level of services and quality of care for patients. Further to this is the development of a Bi-located Theatre Department and the logistics of developing and running such a service.

These are the challenges going forward. However, there is enthusiasm, energy and determination to deliver. Every part of the development would appear to be on schedule and by the time the next annual review is published it should reflect the ongoing process.

Statistics

In total, there were 9,860 theatre procedures carried out in 2004.

Urology, Gynaecology, and Nephrology and Orthopaedic Division

Ms. Therese Carey, Divisional Nurse Manager

Together with its core departments, the Orthopaedic Department is incorporated into this division. This division comprises sixty-seven inpatient beds, Continuous Ambulatory Peritoneal Dialysis (CAPD), Haemodialysis and Urodynamics units.

Nursing staff from all areas in the division actively participated in the Accreditation process. This undertaking offered the opportunity for staff from all three hospitals in the Healthcare Group to work together in successfully participating in this process for the first time. Members of the Renal Multidisciplinary Team participated in the preparation of Standard Operational Policies for Haemodialysis for the move to the new building planned for 2005.

Staff in the division continue to commit to ongoing education and professional development. A number of staff participated in Higher Diploma and Degree courses. Three members of staff successfully completed the first ever Higher Diploma in Nursing Studies (Clinical Practice) Urology Nursing Specialist Strand run in conjunction with UCD. While four members of St Peter's staff successfully undertook the Higher Diploma in Renal Nursing.

Ambulatory Day Care, Emergency Department and Infection Control

Ms. Margaret Boland, Divisional Nurse Manager

2004 continued to be both exciting and challenging for the Emergency Department. A number of new staff joined the Department. Margaret Boland was appointed Divisional Nurse Manager for Ambulatory Day Care, Emergency Department and Infection Control and Mary Hayes appointed Clinical Nurse Manager 3 for the Emergency Department. The environment has on occasions been challenging due to overcrowding, although a number of initiatives have helped to lessen the impact of overcrowding.

Service Developments

Close liaison with St Michael's Hospital Respiratory Unit continued with admission of respiratory cases from the Emergency Department to the St Michael's Respiratory Unit.

Professional development, underpinned by the Scope of Professional Practice, continues with Venepuncture and Cannulation training, and Manchester Triage. Jackie Brennan and Joanne Farren attended the Leading an Empowered Organisation Programme. Trauma Nursing Care and Advanced Cardiac Life Support courses were attended by nursing staff during the year.

The Biologics Clinic was started in November 2004 in St Anthony's Rheumatology Clinic (SARC). The clinic was set up to assess and review patients at the pre-treatment i.e. screening stage and ongoing course while on biologic treatment (see below). It is held two mornings a week, but will shortly need to expand to three mornings as numbers grow. The clinic caters for planned review appointments and emergency referrals. Three members of the nursing team in SARC commenced the Higher Diploma in Nursing (Rheumatology) in UCD in September 2004.

The Outpatient Committee met regularly during the year. The main focus of the group was to participate in planning for the changeover to the new Ambulatory Day Care, which will happen in 2005. This included the development of operational policies in preparation for the move to the new department.

Accreditation

Identifying and improving the quality of patient care and services was a significant development in 2004. The first quarter of the year was focused on final preparation for the survey, which took place in May 2004.

Significant Achievements

Margaret Boland and Allen Doyle were awarded a poster presentation prize at the National Council for Professional Development for Nurses and Midwives in November 2004, title "Proactive and Professional Development to Support Change: Reorganisation of Service Delivery in an Emergency Department, Rapid Assessment Treatment".

Jerry Kearns commenced an MSc in Nursing Advanced Practice in UCD in September 2004. The main focus for this advanced practice will be on Chest Pain Evaluation.

Future Plans

The main focus for 2005 will be the move to the new building and reorganising services within the Emergency Department and Ambulatory Day Care Services. Service development will be introduced in line with the provider plan for 2005 –2007 Management Development will continue for all shift leaders and CNM 2 Staff

It is hoped to introduce a Clinical Decision Unit adjacent to the Emergency Department early in 2005.

With the reorganisation of Mental Health Services within the area, it is planned to introduce a Liaison Psychiatric Nurse Service as part of the service development.

Nurse Education Centre

Staff

Director	Ms Margaret Moran
Registered Nurse Tutor	Ms Catherine Walsh
CNM3 (January to August 2004)	Ms Margaret Conway

The Registration/Diploma programme for the 2000 and 2001 nursing groups continued to be managed and delivered in the Nurse Education Centre throughout 2004. Fifty-eight nursing students from the 2000 Registration/Diploma group were presented with St Vincent's University Hospital badges in February 2004. Three UCD college lecturers delivered the theoretical component for Year 3 of the programme for the 2001 group. Fifty–seven students from the 2001 group completed the programme by December 2004. Three Proficiency Assessment seminars in the Art of assessing Registration Diploma student nurses were held in 2004

In-Service Education

Mary Killeen, Registered Clinical Nurse teacher, continued to co-ordinate the monthly 5-day orientation programme for newly appointed registered nurses until her retirement in August 2004. We wish Mary a long, happy and healthy retirement. Sinead Murphy, (A/CNM2) Nursing Practice Development, co-ordinated the programme from October 2004 to December 2004. A total of 59 registered nurses completed the programme in 2004.

Induction Programme for Newly Registered Diploma Nurses

A 4-day induction programme, in line with the DATHS recommendations for newly qualified nurses, was co-ordinated by Mary Killeen, Registered Clinical Nurse teacher for the 2000 Registration/Diploma students. A total of 54 newly registered nurses completed the programme.

Monthly Administration of Intravenous Medications Study Day

The Intravenous Study Day is incorporated into the 5-day orientation programme. Registered nurses from the clinical areas, who are obligated to attend an Intravenous Study day every 2 years, are accommodated on the IV Study Day with the newly appointed staff to St Vincent's University Hospital. In 2004, a number of registered nurses from St Michael's Hospital also attended the Intravenous Study Day in the Nurse Education Centre. A total of 147 nurses completed the Intravenous Study Day in 2004.

FETAC Health Care Support Certificate

The (Further Education and Training Awards Council) FETAC Health Care Support Certificate training programme for healthcare assistants continued throughout 2004. The 2003 intake continued until November 2004, under the co-ordination of Ms Liz Tuohy, Nursing Support Service Manager.

In September 2004, the Nursing Midwifery Planning and Development Unit appointed a part time co-ordinator for the FETAC programme for 2004/2005. The course commenced in September 2004. Five of the twenty-five healthcare assistants undertaking the 2004 course are from St Vincent's University Hospital. The programme is delivered from St Vincent's University Hospital, Nurse Education Centre for the East Coast Area.

Preceptorship Course

The Nursing Midwifery Planning and Development Unit appointed a registered nurse teacher to co-ordinate twelve 1-day Preceptorship programmes from April to June 2004. The programme is delivered from the Nurse Education Centre in St Vincent's University Hospital. Organisations which come under the remit of the Nurse Education Centre and who accommodate BSc degree student nurses were allocated a number of places on the programme. 73 registered nurses from St Vincent's University Hospital availed of a place on one of the Preceptorship courses.

A further series of twelve 1-day Preceptorship programmes commenced in December 2004.

Throughout 2004, the Nurse Education Centre was the venue for the delivery of a number of courses and study days (which came under the remit of the Personnel Department) in St Vincent's University Hospital.

Post Registration Nurse Education

The Post Registration Nurse Tutor is a member of the Centre for Nurse Education staff. The role involves the planning, co-ordination, facilitation, evaluation and development of post registration nurse education. The principal focus is co-ordinating the Higher Diploma in Nursing Studies programmes which are run in conjunction with School of Nursing & Midwifery, University College Dublin and partner hospitals. Students undertaking a H. Dip. in Nursing Studies programme are nursing in St. Vincents University Hospital, Mater Misericordiae University Hospital and hospitals throughout the country. Ms. Jennifer Carthy provides secretarial support for the Higher Diploma in Nursing Studies programmes.

Gastro-Intestinal Working Group

A working group was set up to continue the development of a Higher Diploma in Nursing Studies – Gastro-intestinal Strand. This programme is scheduled to commence in September 2005.

Assessment Committee

A committee, with representation from key stakeholders undertook a review of assessment procedures across the spectrum of the Higher Diploma in Nursing Studies programmes. There had been a wide variation of assessments. A standardized framework was agreed and implemented for all programmes commencing in September 2004.

Clinical Competency Assessment Tool Committee

A working group, with representation from UCD, St. Vincent's University Hospital, Mater Miseircordiae University Hospital and Paediatric Nursing, commenced evaluation of Clinical Competency Assessment Tool. A questionnaire was devised and will be sent to both assessors and students in January 2005, to determine the effectiveness of the tool. The group will then evaluate responses and develop the document accordingly.

Open Day

An in-house Open Day was held on 1st April, 2004 in the hospital boardroom to provide information and advice on educational opportunities for qualified nursing staff within St. Vincent's Healthcare Group.

Library & Information Services Committee

The Post Registration Nurse Tutor represents the Department of Nursing on this committee. The committee meets on a bi-monthly basis. Library and information technology issues, which are pertinent to all members of the St. Vincent's Healthcare Group, are dealt with at this forum.

Nursing Practice Development Department

The department has continued its committed approach to the support and delivery of nursing care through the development, implementation, monitoring and evaluation of nursing practices throughout the hospital. A significant achievement of this department in 2004 was the launch of the Nursing Policies Procedures and Guidelines 2004.

Other achievements included ratification of the revised Policy for the Administration of Intravenous Medications by Nurses and the development of a Protocol for the Intravenous Administration of opiates in the Post Anaesthetic Care Unit.

In collaboration with the Department of Physiotherapy, an assessment tool for patient mobility was also introduced (The Easy Move Form). On-going projects include a multidisciplinary policy for the care of the patient with tracheostomy, care of the patient requiring nebuliser therapy and the update/ revision of Discharge Co-ordination Guidelines for Nurses.

Programme	Number Attending
Assessing Adult Learning	147
Reflective Practice (undergraduate nursing students)	594
Nursing Care Conference (18 courses)	400
Best Practice discussions in clinical areas	195
Overseas Orientation Programmes x 3	56

The Nursing Practice Development Department continued to facilitate the Competency Assessment programmes designed to equip clinical staff with the knowledge and skills required for teaching and assessing degree student nurses and overseas pre-registration nurses. There are now three years of the BSc Nursing in progress. September saw the successful introduction of the BSc Nursing Rostered Service Year. The Clinical Placement Co-ordinators work in partnership with UCD developing assessment tools and improving the programme, supporting students and staff and facilitating learning in the clinical area.

The Clinical Facilitator for overseas nurses continues to lead the co-ordination, placement and assessment of overseas nurses with the support of registered nurses across the hospital. We welcome all our new colleagues who joined us during 2004.

Presentations

Irish Nurses Practice Development Association (INPDA) Annual conference - Poster presentation *"Developing a competency based framework for Registered Nurses"*. R. Mc Mullin Nursing, Practice Development Co-ordinator

Nursing Human Resources

All registration Diploma Student Nurses graduating in 2004 took up staff nurse positions at St Vincent's University Hospital. Selection and recruitment of staff, in collaboration with the six major Dublin Academic Teaching Hospitals (DATH's) continues both nationally and internationally. Senior nursing staff from St Vincent's University Hospital participated in three overseas recruitment campaigns in India and the Middle East. The Clinical Facilitator for overseas nurses leads the co-ordination, placement and assessment of these nurses with the support of registered nurses across the hospital. A total of 72 nurses were facilitated for the purpose of adaptation placement and assessment in 2004. We welcome all our new colleagues who joined us during 2004.147





Senior Nursing Appointments

Divisional Nurse Manager

Margaret Boland (Ambulatory Day Care/ Emergency Department)
Mervyn Hollywood (Operating Department)

Clinical Nurse Manager 3

Mary Hayes (Emergency Department) Ann Breen (Out of Hours)
Geraldine Culloty (Bed Management)
Margaret Cullen (National Liver Transplant Unit)

Clinical Nurse Specialist

Carol McNulty (Hepatitis C) Sheila O Toole (Hepatitis C)
Catherine Carroll (Cystic Fibrosis) Kathryn O'Sullivan (Cardiac Rehabilitation)
Asiling Purcell (Occupational Health) Marguerite Duggan (Multiple Sclerosis)

Liver Transplant Co-ordinator

Yvonne McGarry, Jane Howe

Clinical Nurse Manager 2

Sinead Brennan (Nurse Practice Development)
Fiona O'Keefe (Night Duty) Annette Cullen (Theatre)

Clinical Placement Co-ordinator

Patricia Hogan

Clinical Facilitator

Sheila Breedin (Operating Department)

Clinical Nurse Manager 1

Joan Killeen (ICU) Susan Patton (St Brigid's Ward)
Concepta Molloy (St Brigid's Ward) Mary Brady (CCU)
Olivia Gannon (St Teresa's Ward) Susan O'Grady (ICU)
Mary Leamy (St Joseph's Ward)

Awards

Presentation of Prizes and Hospital Badges

Fifty-eight newly qualified nurses were presented with St.Vincent's University Hospital Badge by the Department of Nursing. Special awards were made to the following members of staff.

Award	Recipient	Area
Gold Badge	Mary Killeen	Clinical Nurse Teacher
Mother Mary Aikenhead Medal	Jean Mc Carth	CNM2 Anaesthetics
Mother Mary Bernard Medal	Rachel Hayden	CNM1 St Laurence's Ward
Nuala Deeny-Brennan Memorial Prize	Elizabeth Tuohy	Nursing Support Service Manager
The Cecil King Memorial Prize	Jackie Brennan	CNM2 Emergency Department
RCSI Bursary	All Nursing Staff	St Peter's Ward

Retirements

- Miss Pauline Doyle, Director of Nursing, retired in Spring 2004.
- Mary Killeen, Clinical Nurse Teacher, retired in August 2004

We all wish our colleagues happiness in their retirement.

Future Plans

An education programme to develop the Clinical Nurse Specialist role will be developed for all CNSs in the Healthcare Group.

Development of frontline nursing staff will continue through the provision of training and facilitation.

The Hospital Redevelopment Programme requires the development of Standard Operating Procedures for the new building and the Department of Nursing continue to have an important role to play in this area.

The Accreditation Programme has yielded Quality Improvement Plans that will be developed and implemented over the coming year.



Department of Psychiatry and Mental Health Research

Departmental Review

The Department of Psychiatry and Mental Health Research has continued its clinical mission in 2004 with ongoing inpatient, day hospital care and outpatient activity in the existing St. Camillus' Ward and outpatient services. In parallel with this service, the department is in the final stages of preparing its exciting move to the recently completed state of the art inpatient 54-bed unit on the St. Vincent's University Hospital campus. This new unit will incorporate the existing psychiatric service at St. Vincent's University Hospital, as well as those of the acute community inpatient service currently in Vergemount Hospital.

Negotiations with regard to occupation of the new unit are near to completion in 2004 and the unit should be fully operational by early to mid-2005.

Nursing

2004 was another busy year for the nursing staff in St Camillus' Ward. In November 2004, the ward had its first visit from the Mental Health Commission.

An ECT nursing record form was passed for approval by Nurse Practice Development.

March brought the introduction of the smoking ban and this was introduced on St Camillus' Ward. Thanks to all patients and staff for their co-operation in this matter.

Meetings commenced with Vergemount Hospital staff pertaining to work on policies, procedures and guidelines for the new psychiatric unit in April 2004.

We would like to thank all staff for their participation in the Accreditation process.

Congratulations to Sinead Smyth on the birth of baby Cathy and to Sharon Byrne on the birth of baby Alannah. We said goodbye to Neena Harrypassad who returned to South Africa and welcomed Ciara Dunne.

Psycho-Oncology

The Psycho-Oncology service has had a very active year, contributing to guidelines for psycho-oncology support for primary care within the HSE East Coast region. The service was delighted to be part of the opening of the new Cancer Care Support House, 'Lios Aoibhinn', which is a drop-in centre for patients who are suffering from cancer.

Mental Health Research

The department is engaged in over eight different projects in relation to the study of suicidal behaviour, including suicide and major psychiatric disorders. The department is also studying suicide in high-risk occupational sub-groups of society with a view to encouraging earlier intervention and prevention across all communities in society. Together with the charity 3Ts (Turing the Tide of Suicide), the department organised a very successful international conference titled *Suicide in Modern Ireland – New Dimensions, New Responses* which was opened by President Mary McAleese on the first day of her second term in office. This conference received significant media attention and is bringing increasing pressure to bear on both government and society to seriously face up to this modern tragedy. Further important research projects are planned for 2005.

Departmental Statistics

Total Admissions:	198
Old Age Psychiatry Admissions accounted for	30%;
Overdose/Self Harm and Eating Disorders	15% 8.6%.

Day Hospital Attendances: 1,231
of which 38% were attending for treatment of Eating Disorders.

47% of all new patients referred to OPD were suffering from Eating Disorders.

Day Psychiatric Treatment Programme

The Day Psychiatric Treatment Programme has developed several specific programmes relating to Eating Disorders, which is particularly important as the hospital is the National Referral Centre for Eating Disorders.

Geraldine Carroll, CNM2, graduated in November 2004, obtaining an MSc in Cognitive Behavioural Therapy. On foot of this, the SAIF group therapy programme for the treatment of Bulimia Nervosa was re-evaluated. A new 8-week therapy group was devised and will be launched in 2005.





Psychotherapy

The School of Psychotherapy at St. Vincent's University Hospital continues to play an important role in the training of psychotherapy clinicians. 2004 was a particularly important year for the School of Psychotherapy as four students have undertaken a PhD thesis project which will contribute significant added value to the School of Psychotherapy work.

Selected Publications

(See also ERC publications listing)

Bolger, S., O'Connor, P.O., Malone, K., Fitzpatrick, C., (2004)
Adolescents with suicidal behaviour: attendance at A&E and six month follow up.
Irish Journal of Psychological Medicine, 21:3, 78-84

Mahon, M., Tobin, J., Cusack, D., Kelleher, C., Malone, K., (In Press)
Military suicide: occupational specific risk-factors for workplace suicide; a retrospective case-control study.
American Journal of Psychiatry

O'Donovan, A. & Hughes, B.M. (2004).
Buffering cardiovascular stress: Is psychometrically assessed social support as effective as a laboratory analogue?
Paper presented at the annual conference of the British Psychological Society, Health Division, Edinburgh, Scotland, September 2004.

O'Donovan, A. & Hughes, B.M. (2004). *The relative effects of everyday social support and laboratory support availability on cardiovascular reactivity to a reading task.*
Paper presented at the first annual conference of the Psychological Society of Ireland, Health Division, Galway, Ireland, April, 2004.

O'Donovan, A. & Hughes, B.M. (2004). *Are the effects of social support availability on cardiovascular reactivity moderated by psychometrically evaluated social support?*
Paper presented at the annual Psychological Society of Ireland Student Congress, Dublin, Ireland, March 2004.

Whelan, R. & Barnes-Holmes, D. (2004). *Empirical models of formative augmenting in accordance with the relational frames of Same, Opposite, More-than and Less-than.*
International Journal of Psychology and Psychological Therapy, 4, 285-302.

Department of Old Age Psychiatry

Departmental Review

Departmental Review

The Department of Old Age Psychiatry had its busiest year to date in 2004. Levels of service activity increased significantly. We provide a community-orientated, hospital-based service for patients aged 65 and over, living in Dublin South-East, with new onset mental health problems. The aim of the service is to provide a comprehensive, accessible service to patients and their families, responding rapidly where required to crisis referrals. The major challenge facing us is to continue to provide a prompt service to patients and their families in the context of ever increasing rates of referral to the service.

Staff

The department welcomed Dr. Leone Judge, Senior Registrar in Old Age Psychiatry. Congratulations to Mary Finn who has taken up post as Assistant Chief Nursing Officer and has already made a significant contribution to the co-ordination of nursing services across the Old Age Psychiatry Service.

Service Developments and Activities

There was a 20% increase in the number of new patients seen in 2004, with over 600 new referrals assessed. New referrals include patients seen at home and in hospital settings within the catchment area, including St. Vincent's University Hospital, St. Michael's Hospital, The Royal Hospital Donnybrook, Loughlinstown Hospital and nursing homes in the area.

The Day Hospital had 1,118 attendances in 2004, its second year of activity. The day Hospital has provided an invaluable service for elderly patients with psychiatric illness while inpatients, to prepare them for discharge, and, for patients recently discharged, to provide support for them in the community.

An unmet need was identified for patients with enduring mental health problems for regular appropriate day care in the day hospital. An "Active Rehabilitation Group" for these patients was set up and meets every Thursday, with regular input from Senior Occupational Therapist, Senior Psychologist and Senior Nursing Staff.

The Community Psychiatric Nurses continue to provide an invaluable service to patients and their families providing seamless follow-up of patients across many different settings.

The team has been in negotiation with Hospital Management and Medicine for the Elderly service regarding the development of an Institute of Health Care for the Elderly.

Significant Achievements

Dr. Freyne, consultant psychiatrist, was awarded an MD by UCD/RCSI for her thesis on the prognosis of depression in older people and has been invited to present her findings to the All Ireland Institute of Psychiatry in Spring 2005.

Dr. Aideen Lewis, Senior Psychologist, developed The Nursing Home Educational Project which took place in January 2004 and October/November 2004. The Old Age Psychiatry multi-disciplinary team contributes to the project and it has received positive feedback from its target audience. The team won a Hospital Innovation Award under the category of Alternative Service Delivery Initiative.

Martina Dolan, Senior Social Worker, completed data collection for the Camberwell Needs Assessment Study of elderly people referred to the Old Age Psychiatry Service. In addition, she has developed and implemented an information session for carers of dementia sufferers, which is run regularly in the department.

Catherine Keogh, Senior Occupational Therapist, together with a Community Psychiatry Nurse, provides a very successful anxiety management programme (4 groups facilitated in 2004) at the Day Hospital. In addition, she has been invited to lecture on Occupational Therapy and Dementia at Trinity College Dublin and has facilitated a one-day conference with the Dementia Services Information and Development





Centre on *"Occupational Therapy and Person Centred Care in Dementia Care"*. She has been invited onto the Executive Committee of the Mental Health Commission to consider a working definition of severe dementia for the Mental Health Act 2001.

Dr. Judge has developed a lithium clinic in the department, which takes place every 3 months and she has also organised the monthly Old Age psychiatry lecture programme at the Education and Research Centre.

Research and Academic Activities

There is a very busy research programme at the department, which includes the following: -

- An exploration into the perceived factors influencing the implementation of person centred care in long-term settings: a qualitative study by Catherine Keogh, Senior Occupational Therapist.
- A survey of benzodiazepine use by patients referred to the service by Dr. Stoffels, Dr. Cooney and Dr. Freyne.
- Shouting in Dementia Sufferers – a case series and review.
- Health needs of patients referred to the Old Age Psychiatry service – Martina Dolan, Dr. Freyne and Dr. Cooney.
- Carers awareness of legal issues regarding older patients – Dr. Halley, Dr. Cooney.
- A review of psychotropic medication in delirium – Dr. Ni Chorcorain and Dr. Freyne.

Department of Pathology

Departmental Review

Service Development/Activities

The major developments in 2004 were:

- Commencement of the Health Link project to provide pathology results electronically to General Practitioners.
- Detailed planning exercise for the commissioning of the new laboratory to ensure that the pending transfer of services to the new building is seamless.

Quality Improvement/ Accreditation

After a number of years of preparation, two assessments of the Pathology Laboratories were carried out by Clinical Pathology Accreditation (UK) Ltd (CPA) in 2004. The Histopathology laboratory was assessed on the 19th/20th May and the Haematology/ Blood Transfusion laboratories were assessed on the 18th/19th November. The CPA team of assessors, which included medical consultants and senior medical scientists, spent 2 days on site per visit. The assessment involved audits of all aspects of the above services against the CPA standards for laboratories and included meeting with users of the services. Both assessments were very successful with favorable comments from the assessors on the quality of the services provided.

The report of the Histopathology assessment was issued on September 27th 2004 and was 'conditionally approved incorporating ISO 15189'. The report of the Haematology/ Blood Transfusion assessment is due for issue in March 2005. The above outcomes could not have been achieved without the commitment and enthusiasm of all laboratory staff for the accreditation project. The Pathology Department also participated in the Hospital Accreditation project, with a number of staff contributing to the care/service teams.

Departmental Statistics

Pathology activity increased by 2.3% in 2005. The increase in pathology activity can be attributed to the increase in workload from General Practitioners, which increased by 15% during the period.

Future Plans

The main priorities for pathology in 2005 are:

- Commissioning of the new Pathology Department.
- Continuation of the CPA accreditation assessments with the chemistry laboratories i.e. Clinical Biochemistry, Endocrinology, Nuclear Medicine and Metabolism scheduled for assessment during 2005.

Administration

In 2004, Niamh Devereaux, Marian Purcell and Siobhan O'Grady joined our staff as Grade III Clerical Officers. Louise Sweeney was promoted to Grade IV Officer.

Laura Conroy, Grade IV Officer in Biochemistry successfully completed Year 1 and commenced Year 2 of the Diploma in First Line Management held in St. Vincent's University Hospital by the National College of Ireland and Catherine Kavanagh, Grade IV Officer, Anticoagulant Monitoring Service commenced Year 1.

Theresa Ward, Sandra Devlin and Maureen O'Brien participated in modules of the Management Education Programme provided in SVUH for middle managers.

The following clerical staff registered for the e-learning Personal Development Programme on The Office for Health Management website, Louise Sweeney, June Donnelly, Laura Conroy and Niamh Lyons.

Phlebotomy

Outpatient phlebotomy activity increased by 3.9% in 2004. Despite the increasing workload the Phlebotomy team continue to provide a quality service both to the hospital and the community. The hours of the GP access service were extended (8.00am–12 noon) with a further extension of hours (8.00am–2.00pm) planned for 2005.

Clinical Biochemistry Department

Staffing

Ms Mairead O'Leary, Senior Medical Scientist, left Biochemistry at the end of October to take up the position of IT Coordinator/Quality Manager in Pathology. Ms Geraldine Scaife commenced as Lab Aide.

Service Developments/Activities

The workload of the Biochemistry Department increased by a further 5% in 2004, reaching a total of 1.75 million tests per annum.

Participation in External Quality Assurance Schemes was significantly extended, including responsibility for the External





Quality Assessment of Hospital Point-of-Care (POC) Testing for Blood Gas Analysis, Blood Glucose and Cardiac Markers. A new Therapeutic Drug Monitoring Analyser, the Abbott FLX, was installed during the year.

In collaboration with the Emergency Department and the Cardiology Department/Heart Failure Unit, the Biochemistry Department has been involved in ongoing evaluation of Ischaemia Modified Albumin (IMA), an early marker of Cardiac Ischaemia and B Natriuretic Peptide (BNP), a diagnostic test for Heart Failure.

Future Plans

In 2005, it is anticipated that the Biochemistry Department will be required to take responsibility for the maintenance and quality assurance of additional Blood Gas Analysers in ICU, the Emergency Department and St. Brigid's Ward. We plan to continue our evaluations of new Cardiac Markers and to introduce BNP into routine service as a new test.

The Biochemistry Department will undergo an assessment for Clinical Laboratory Accreditation in mid-year by CPA UK Ltd.

Achievements

Mr. Rowland Reece is Hon. Secretary of the Association of Clinical Biochemists in Ireland (ACBI) and was appointed to the Irish Expert Body on Fluoride & Health. *Dr Sean Cunningham* is Chairman of the Clinical Biochemists Registration Board. *Ms Myra O'Keane* is Chairman of the Safety Advisory Body of the Academy of Medical Laboratory Scientists.

Conferences Attended

Focus 2004, The National Meeting of the Association of Clinical Biochemists, Birmingham 18 – 20th May. Ms G Collier gave a presentation entitled *"An Audit of the Performance of Point-of-Care Blood Glucose Monitoring Using a Data Management System"*.

ACBI 2004, 27th Annual Conference of the Association of Clinical Biochemists in Ireland, 24-25th September, Cork. A presentation entitled " *Evaluation of the Beckman Coulter Total IgE Assay on the Immage Immunochemistry System"* was given by G Collier, L Healy, O Maguire and S Cunningham.

Waterford Regional Hospital Biochemistry Seminar *"Implications of New Diagnostic and Cardiac Marker Guidelines in Acute Coronary Syndromes"* presented by Dr S Cunningham, 27th May.

Beckman Coulter Meeting, Galway, 28th April. Mr. Rowland Reece gave a presentation on *'The Importance of Quality in Biochemistry Analyses'*.

Blood Transfusion Department

Service Developments

The focus for the year was the preparation of the blood transfusion laboratory for accreditation by CPA.

Outstanding Achievements

Deirdre O'Neill completed her Masters in Biomedical Science, which included project work on *"Patient Response to Current Platelet Products"*. Mr. Paul O'Brien and Mr. Niall Wall (AMANCH) developed a web-based register for patients who develop red cell alloantibodies. This was presented as a poster at Biomedica 2004 and is presently under consideration as a pilot for national use. Paul O'Brien was a member of the National Blood User Group that published Guidelines for the Administration of Blood and Blood Products.

Meetings Attended

Paul O'Brien presented at the National Haemovigilance Annual Meeting, Lesley Hopkins presented at the IEQAS annual meeting.

Future Developments

EU Directive 2002/98/EC of the European Parliament on setting standards of quality and safety for the collection, testing, processing, storage and distribution of human blood and blood components will influence the way forward. This is currently under consideration by the Department of Health and Children and will pass into law on November 9th 2005. The most likely implication for hospital blood banks will be accreditation to ISO Standard 15189 and also compliance with 100% traceability of all transfused blood and blood products.

Haematology Department

Service Development/ Activities

There was an increase of approximately 4% in haematology tests over 2003. The specialised tests for Thrombophilia and Immunophenotyping continue to increase every year. The main focus during the year was Accreditation (see above).

The Anticoagulant Monitoring Service continues to be very busy, with the number of referrals increasing every year. The RAID software was updated during the year and this has been a useful improvement. The Anticoagulant Nurse Specialist, Ms Elaine Tallon, and the Haematology Medical team now carry out calculation of Warfarin dosage.

During the year the D-dimer assay was introduced on a 24-hr basis for St. Vincent's University Hospital and for St Michael's Hospital. This is in line with recommendations from the Venous Thromboembolism (VTE) Group.

Staff – New Appointments

Mr. Brian Philbin took up a permanent position as Medical Laboratory Scientist.

Achievements/ Conferences Attended

Dr. Donald McCarthy is the President of the Haematology Association of Ireland (HAI).

Dr. Karen Murphy is the National Specialty Director for Haematology and Education Secretary to the Irish Haematology Society/Haematology Association of Ireland.

Mr. Ivan Shirley is a member of the Steering Committee of the Irish External Quality Assessment Scheme.

Invited Lecture: *Dr Karen Murphy* gave a lecture titled *"Thrombotic Risks in Pregnancy"* at Anticoagulant Course – University of Hertfordshire Faculty of Health. August 2004

Dr Jonathan Dean was awarded with a PhD during 2004 and the title of his thesis is *"NKR+ Cells in Human Bone Marrow"* – Dr DonaldMcCarthy was one of the supervisors.

Future Plans

The main plan for this year is the move to the new building.

Clinical Haematology

This year two registrars were employed to cope with the increased clinical workload, one of which is a Specialist Registrar post.

Histopathology

Staff

Julie Gorman was appointed as a Medical Scientist.

Terry Hegarty was appointed to a postgraduate Trainee post.

Outstanding/Significant Achievements

Shay Caffrey, Senior Medical Scientist, completed the Diploma in Healthcare Management with the Institute of Public Administration. Ca*therine Geoghegan* and *Clara Myers*, Senior Medical Scientists, completed the MSc degree in Biomedical Science by distance learning with the University of Ulster at Coleraine. *Dr. Kieran Sheahan* is currently President of the Pathology Section of the Royal Academy of Medicine of Ireland.

Service Development/ Activities

The workload in 2004 was largely unchanged from 2003, apart from a 26% rise in the number of (Coroner) post mortems performed.

Future Plans

The introduction of Fluorescence In Situ Hybridization (FISH) for analysis of predictive markers including HER2 in breast cancer awaits approval of a Senior Medical Scientist with special responsibility to the symptomatic breast service.



Immunology Laboratory

The total number of tests performed in 2004 was 64,067, an increase of 2% over 2003.

Staff

Kira Ulso Sax started as a medical scientist in March.
Cathal Prendergast worked in Immunology from June to September 2004.

Outstanding /Significant Achievements

Paul McArdle successfully completed the Trainee Medical Laboratory Scientist programmed and attained Membership of The Academy of Medical Laboratory Science.

Microbiology

Staff

Ms. Jean Power was promoted to Senior Medical Scientist. Ms. Heidi Anderson took up a permanent medical scientist post in January.
Ms. Angela Ormond was appointed as Laboratory Aide.

Service Developments/ Activities

Outbreaks of norovirus infection brought its own workload increase as did the MRSA screening programmed, which was tailored to maximize the yield of this organism.

Future Plans

The adoption of antibiotic sensitivity methods to meet NCCLS standards will begin immediately.

Microbiology – Professorial Unit

Professor William W. Hall.

Outstanding/Significant Achievements

Adjunct Professor, The Rockefeller University, New York, U.S.A.
Elected to the Board of the Institute of Human Virology, Baltimore, U.S.A.
Elected to the Membership Committee, Kunkel Society, Rockefeller University, New York, U.S.A.

Editorial Boards

AIDS Research and Human Retroviruses.
Neuropathology.
Journal of Neurovirology.
Journal of the Acquired Immunodeficiency Syndrome.

Nuclear Medicine Laboratory

The number of tests performed was 105,135, an increase of 9.4% over the previous year.

Staff

Isabelle de Gardelle was made acting Senior-Grade Biochemist.
Geraldine Murphy, Basic-Grade Biochemist resigned and was replaced by Louise Kelly.

Outstanding/Significant Achievements

Professor Joe Duffy continued to act as Chairman of the National Academy of Clinical Biochemistry (USA) Panel to establish new guidelines for the clinical use of markers in breast cancer. Professor Duffy was also invited to be the lead-author on new European Group on Tumor Markers (EGTM) guidelines for use of markers in ovarian cancer.

Invited Presentation at International Meetings

Professor Duffy was invited to speak at the following Internal Conferences:

International Society for Oncodevelomental Biology and Medicine (ISOBM) Meeting: Helsinki,
Title of Presentation: Markers in Colorectal Cancer European Group on Tumor Markers Guidelines.

Central European Conference on Human Tumor Markers: Prague,
Title of Presentation: Tumor Markers in breast cancer: which ones are clinically useful ?

Urological Club of Great Britain and Ireland Annual Meeting: Dublin.
Title of Presentation: Beyond PSA.

Selected Publications

(See also ERC Publications Listing)

Buggy Y, Maguire TM, McGreal G, McDermott E, Hill AD, O'Higgins N, Duffy MJ. *Overexpression of the Ets-1 transcription factor in human breast cancer.* Br J Cancer 2004;91(7):1308-15.

Dillon MF, Hill AD, Quinn CM, O'Doherty A, Crown J, Fleming FJ, et al. *Surgical intervention in screen-detected patients versus symptomatic patients with breast cancer.* J Med Screen 2004;11(3):130-4.

McCormick PA, Nolan N. *Palpable epigastric liver as a physical sign of cirrhosis: a prospective study.* Eur J Gastroenterol Hepatol 2004;16(12):1331-4.

McDonald DG, Kelehan P, McMenamin JB, Gorman WA, Madden D, Tobbia IN, Mooney EE. *Placental fetal thrombotic vasculopathy is associated with neonatal encephalopathy.* Hum Pathol 2004;35(7):875-80.

McDowell A, Mahenthiralingam E, Dunbar KE, Moore JE, Crowe M, Elborn JS. *Epidemiology of Burkholderia cepacia complex species recovered from cystic fibrosis patients: issues related to patient segregation.* J Med Microbiol 2004;53(Pt 7):663-8.

McKiernan SM, Hagan R, Curry M, McDonald GS, Kelly A, Nolan N, et al. *Distinct MHC class I and II alleles are associated with hepatitis C viral clearance, originating from a single source.* Hepatology 2004;40(1):108-14.

O'Connell MP, O'Leary M, MacKeogh L, Murphy K, Keane DP. *Is the monitoring of anti-Xa activity necessary in pregnant women undergoing thromboprophylaxis?* Eur J Obstet Gynecol Reprod Biol 2004;114(1):12-4.

O'Driscoll L, Cronin D, Kennedy SM, Purcell R, Linehan R, Glynn S, *et al. Expression and prognostic relevance of Mcl-1 in breast cancer.* Anticancer Res 2004;24(2A):473-82.

O'Keeffe C, Baird AW, Nolan N, McCormick PA. *Cholestatic pruritus - the role of cutaneous mast cells and nerves.* Aliment Pharmacol Ther 2004;19(12):1293-300.

Todd S, Xu J, Millar BC, Moore JE, Crowe M, Raoult D, et al. *Culture-negative Bartonella endocarditis in a patient with renal failure: the value of molecular methods in diagnosis.* Br J Biomed Sci 2004;61(4):190-3.

Xu J, Crowe M, Millar BC, Moore JE. *Non-culturable Haemophilus influenzae meningitis identified by 16S rDNA PCR and sequencing.* Ir J Med Sci 2004;173(1):57, 59.

Department of Radiology

Departmental Review

Radiology Management Team

Clinical Director of Radiology	Dr Stephen Skehan
Radiography Service Manager	Sharon Simpson
Clerical Supervisor Grade VI	Helen O'Reilly
Clinical Nurse Manager II	Sara Nicholson

Service Developments / Activities

New Development

Construction of the future Radiology Department within the new development forged ahead as steering committees continued to handle the procurement of new and exciting technology.

Clinical Service Developments

New interventional liver tumour treatment protocols such as chemoembolisation and radiofrequency ablation were implemented. Interventional activity increased with the increase in the numbers of hepatobiliary surgeons and gastroenterologists.

Staff Movement

Radiology

Dr Patricia Cunningham and Dr Jean Doyle left the department. Dr Susannah Harte and Dr Ronan Killeen joined the department as first year Specialist Registrars, with Dr Amjad Iqbal and Dr Aideen Larke joining as fifth year SpRs.

Radiography

The following Radiographers joined us during 2004:

Sinead Howard	Denise McDonnell	Mary Hinch
Lorna McEntee	Linda Menyo	Rebecca Hazlewood
Patricia Radebe	Maureen Minyuku	

Catherine Corr, Catherine Dullea and Tanja Van der Walt left the Radiology Department in 2004.

Mary-Pat Corridan and Caroline Gallagher were granted leave of absence.

Deirdre Scott-Hayward retired from the position of Radiography Service Manager after over 30 years of service. Toinette Tunney, Clinical Specialist Radiographer in Mammography, retired after 35 years of service. Best wishes for the future to both of our colleagues.

Lea Farrell became Acting Clinical Specialist Radiographer - Radiation Safety Officer.

We welcomed back S/N Loida Macalinao in 2004.

Outstanding Achievements

Administrative and Academic Achievements by Consultant Radiologists in 2004

Dr Risteard O Laoide was elected Chairman of the Medical Board in St. Vincent's University Hospital in 2004. Dr O Laoide resigned as Director of Radiology and was replaced by Dr Stephen Skehan. Dr Skehan also became Chairman of the Hospital Radiation Safety Committee.

Dr Conor Collins became the new Educational Co-ordinator for radiology trainees in St. Vincent's University Hospital.

Dr Dermot Malone became Chair of the Research Committee, Faculty of Radiologists, RCSI in October. He was nominated to the Editorial Board of Radiology during 2004. Dr Malone was a moderator at sessions of the ECR, RSNA and ESGAR. He was a Visiting Professor at the University of Toronto and McMaster University in May.

Specialist Registrar Achievements

Dr Eric Heffernan and Dr Fiona Hughes obtained their FFRRCSI, with Dr Heffernan awarded First Place in the examination. Dr David O'Donnell was successful in the primary examination for the Fellowship.

Radiology Section, RAMI: Annual Registrar's Prize 2004. Won by Dr. Lorna Browne for: Browne L, Staunton M, Heffernan E, Malone DE. Non-invasive imaging diagnosis of hepatocellular carcinoma: a review using 'Evidence-Based Practice' methods.

Journal of Medical Science Research Award (Radiology Section): Won by Dr. Michael Maher for:
Maher MM, McNamara AM, MacEneaney PM, Sheehan SJ, Malone DE. Abdominal aortic aneurysms: elective endovascular

repair versus conventional surgery--evaluation with evidence-based medicine techniques. Radiology. 2003 Sep;228(3):647-58.
Dr. Maher was subsequently appointed to the Chair of Radiology in Cork, where we wish him every success.

Selected Publications

Kealey SM, Dodd JD, MacEneaney PM, Gibney RG, Malone DE. *Minimal preparation computed tomography instead of barium enema/colonoscopy for suspected colon cancer in frail elderly patients: an outcome analysis study.* Clin Radiol 2004;59(1):44-52.

Dodd JD, MacEneaney PM, Malone DE. *Evidence-based radiology: how to quickly assess the validity and strength of publications in the diagnostic radiology literature.* Eur Radiol 2004;14(5):915-22.

Lim ET, O'Doherty A, Hill AD, Quinn CM. *Pathological axillary lymph nodes detected at mammographic screening.* Clin Radiol 2004;59(1):86-91.

Thirion P, Holmberg O, Collins CD, O'Shea C, Moriarty M, Pomeroy M, et al. *Escalated dose for non-small-cell lung cancer with accelerated hypofractionated three-dimensional conformal radiation therapy.* Radiother Oncol 2004;71(2):163-6.

Dutton JA, Bird NJ, Skehan SJ, Peters AM. *Evaluation of a 3-hour indium-111 leukocyte image as a surrogate for a technetium-99m nanocolloid marrow scan in the diagnosis of orthopedic infection.* Clin Nucl Med 2004;29(8):469-74.

Conferences

All of the Irish radiology conferences were attended, in addition to the following overseas conferences: RSNA, ESGAR, ICR, UKRC, EANM, BNMS and BMUS. Presentations were made at most of these meetings.

Post Graduate University Courses

Hilary Eustace received a Higher Diploma in Computed Tomography from UCD.
Emma Canniff received a Higher Diploma in Ultrasound from UCD.

The following radiographers received funding for University based courses:

Aine Kelly:	H.Dip. Ultrasound
Melanie Jackson:	H.Dip. Magnetic Resonance Imaging
Sonya Allen:	H.Dip. Magnetic Resonance Imaging

Short Courses

Several radiographers received funding to attend various short courses throughout the year:

QA and Radiation Protection:	Lea Farrell
PACS Course, Hammersmith:	Jason Groarke
Breast Imaging Update, UCD:	Carol O'Connor, Louise Venables
Scottish CT Course:	Susan Collins
Advanced Vascular Ultrasound Techniques:	Sarah Cullen
Musculoskeletal Ultrasound Techniques:	Esther Walsh
Fundamentals of MRI:	Sonya Allen, Melanie Jackson

S/N Mira Sta.Ana, S/N Vangie Tayao, Josephine Lynch, Donna Pili and Patricia Martinez participated in the "Second Chance - Key Skills Programme".

Ismay Crowley became a Grade IV Officer (Clinics) in November.

Helen O'Reilly was involved with the implementation of the Quality Improvement Plans of the Accreditation Information Management Team.

Samantha Alford successfully completed the Supervisory Management Course (IMI) and achieved a distinction.

Anne Sharkey obtained a Certificate in Patient Records & Health Service Management (IHSMI) and also achieved a distinction.

Ancillary Staff

The Radiology Aides have continued to contribute significantly to workflow. Their structured rota ensures that an Aide is present to assist a radiographer 24 hours a day.

Departmental Statistics

Statistics	2004
In-patients	34768
Out-patients	32212
Emergency Department	29018
General Practitioner	12799
Other	10613
Total	119410

Compared to the 2003 figures, this is an increase 7.6%.

Future Plans

The move to our new department, which is spread over three floors of the new building, will begin in late 2005 and will be completed early in 2006. Additional facilities will include MRI for the first time in St.Vincent's University Hospital.



School of Diagnostic Imaging

Departmental Review

Service Developments/ Activities

The year 2004 was challenging and exciting for the staff and students of the School of Diagnostic Imaging. The UCD Quality Assurance review of the school reached its peak activity in March 2004, when a team of external experts met with students and staff to discuss and report on potential improvements in the school's operation. A Quality Improvement Plan is developed following all such reviews and the staff and students of Diagnostic Imaging are currently involved in the plan's continued implementation.

Concurrently, the new Health Sciences complex on Belfield approached completion. The tendering process to equip the Diagnostic Imaging component of this facility was very effectively co-ordinated by Allison McGee and Mark McEntee. I would like to acknowledge the hard work of all staff involved in this exercise.

Of course, routine activity carried on regardless. 2004 saw fifteen new graduates in Radiography, as well as thirty five qualified staff enrolling for taught postgraduate programmes in specialised areas of Diagnostic Imaging, and ten postgraduate research students undertaking MSc or PhD in the School.

The number of ongoing education courses in 2004 was limited as a direct result of staff workload associated with the above activity. However, we were still able to accommodate 37 radiographers on short courses contributing to their CPD portfolio.

Finally, in November 2004, the School of Diagnostic Imaging became the first Irish organisation to our knowledge to exhibit at the prestigious RSNA conference in Chicago. The aim of our exhibit was to promote the school as a centre for international students to complete their postgraduate research, and this has proved extremely successful. The stand also provided a welcome meeting point for the many Irish delegates at the conference.

Achievements

The following prizes were awarded:

Karen Purcell	Second Year Scholarship Award (UCD)
Yvonne Corrigan	Third Year Scholarship Award (UCD)
Henry H Stewart	Scholarship (NUI)
Marie Louise Butler	Fourth Year Scholarship Award (UCD)
Marie Hanrahan	Siemens Medal & Prize (UCD)
John Tuffy	O Farrell Medal (UCD)
Bridget Furlong	Phillips Medal and Prize (UCD)
Aoife Neary	Olive Fleming Memorial Award (Irish Institute of Radiography)
Emma Canniff	Kodak Award (UCD)

Published Papers

Staff and students published widely, and only a selection of work is listed below.

Doolan A, Healy J, Brennan PC.
Gonad protection for the antero-posterior projection of the pelvis in diagnostic radiography in Dublin hospitals.
Radiography 2004; 10 (1): 15-21

Grondin Y, Matthews K, McEntee M, Rainford L, Casey M, Tonra M, Al-Qattan E, McCrudden T, Brennan PC.
Dose-reducing strategies in combination offers substantial potential benefits to females of childbearing age requiring X-ray examination.
Radiation Protection Dosimetry 2004; 108(2): 123-32

McEntee M, Brennan PC, O'Connor G.
The effect of kilovoltage peak on the image quality of PA chest radiographs when using digital image acquisition devices.
Radiography 2004; 10 (4); 287-292

O'Leary D, McDonnell, Brennan PC.
Increasing film focus distance (FFD) reduces radiation dose for X-ray examinations.
Radiation Protection Dosimetry 2004; 108(3): 263-8.

Wade C, Brennan PC.
An assessment of viewing conditions for the display of diagnostic images on workstation monitors in Dublin.
British Journal of Radiology 2004; 77 (918): 465-71.





Abstracts and Presentations

Doyle CA, Matthews K. Multifactorial influences on Pain during Mammography. *RSNA, Chicago.*

D'Helft CS, McGee AM, Rainford LA, McFadden SL, Hughes CM, Winder JR, Brennan PC. A survey of radiation dose and image optimisation for cardiac interventional procedures. *8th Annual Conference of the IIR*

Matthews K. Questioning females of reproductive capacity: implications of S.I.478 of 2002 European Communities (Medical Ionising Radiation Protection Regulations). *8th Annual Conference of the IIR*

Matthews K, Brennan PC. Paediatric patient dose variations in Irish X-ray Departments. *RSNA, Chicago.*

Matthews K, Furlong B. An analysis of the implementation of Irish legislation in establishing the pregnancy status of relevant female patients *RSNA, Chicago.*

McCrudden T, Brennan PC, Matthews K. The Calibration Accuracy of Dose-area Product Meters in Irish Imaging Departments performing Paediatric Radiography. *BJR Congress Series, Proceeding of UKRC 2004*

Future Plans

In May 2005, the School of Diagnostic Imaging will move from St. Anthony's to Health Sciences in Belfield, where state of the art premises will be shared with Nursing and Physiotherapy in Phase I, and also with Medicine in Phase 2. Over its lifetime, the school has occupied various premises, but has always been closely related to St. Vincent's University Hospital. Originally in prefabs behind St. Vincent's Hospital on St. Stephen's Green, the school moved to Elm Park with the hospital in 1971. The current building was donated in 1990 by Sr. Francis Ignatius, Mother General of the Sisters of Charity, and refurbished with Department of Health funding. The Herbert Avenue building has hosted many events in Diagnostic Imaging education, and will remain poignant in many memories when the school eventually moves to Belfield.

Throughout the long association with St. Vincent's, the education of radiographers has developed with the support of the Sisters of Charity and of the hospital. The school is also indebted for the contribution over the years of many "Vincent's radiographers". Radiography education in Ireland is entering a new era in a premises that can be ranked amongst the best in the world. However, the relationship with St. Vincent's University Hospital and the Sisters of Charity will not be forgotten. Radiography undergraduates will continue clinical placements in the excellent teaching environment of St. Vincent's University Hospital, and we aspire that the links forged in our past will be propagated through collaborative research between Radiographers.

Department of Information and Communication Technology

Allied Health Professional and Support Services

It was a busy year for the ICT department with the completion of existing projects and the commencement of new projects. We were sorry to say goodbye to Mary Ward and Paul Cosgrove, two valued members of staff, but were also very happy to welcome three new staff members Neal Mullen, Ailish Lawlor and Joe Ryan to our team.

Projects Completed in 2004

The Appointment Scheduling System, which we have developed in partnership with H2H Care, went live in St. Anne's Ward. This system has functionality to manage appointment scheduling, cancellations, rebooking and overbooking facilities and incorporates an interface to the Hospital Patient Administration System. The system also has the ability to produce management reports, statistical analysis and patient specific letters and documents. The system was very favourably received and we are now implementing it in other areas such as St. Mark's Ward. A number of other hospitals have expressed interest in the system.

We went live with our Group Intranet initially in St. Vincent's University Hospital but this will be extended to all areas shortly. This system will improve communications, allow departments to share information, reduce and eliminate paper and provide access to policies and procedures, manuals and forms online. The Intranet will incorporate links to current systems for enquiry, reporting and result reporting.

This year we implemented the first phases of our Management Information System and Data Warehouse drawing and purifying information from clinical and non-clinical systems into a single repository incorporating systems integration tools, on line analytical processing (OLAP), business intelligence software and desktop decision support. The system also contains a data-mining tool capable of providing comprehensive reports and adhoc queries. The solution enables the hospital to maximise the value of data already stored in our current IT systems, building an infrastructure that allows us to meet the current and increasing demands for management and clinical information. It is now envisaged that it will not only provide management and clinical reports, but will also help to support essential functions such as clinical audit.

We have initiated a project in conjunction with the Healthlink project team to provide data such as pathology and radiology data to GPs. We are piloting this project in four GP practices. We are evaluating the introduction of an online neurology referral module in this project.

Substantial upgrades of the Emergency Department and RIS systems have been carried out. A replication system for the Patient Administration System has been created incorporating an inquiry module for disaster recovery.

In 2004, we made a concerted effort to consolidate our infrastructure and enhance our security below is a list of our Infrastructural and Security Projects:

- We joined the E-Government Virtual Private Network (VPN) providing a 2MB to the Internet.
- 100% Academic Open Licenses : saving up to 80% on Microsoft products
- Campus agreement on servers : saving up to 90% on operating system licenses
- Implementation of ISA proxy server
- SurfControl e-mail filtering
- SurfControl web filter
- SurfControl IMI & Peer 2 Peer Filtering
- IP structure rationalization with a new design incorporating DHCP
- Documentation on all servers
- Central management of backups
- Contracts consolidated into one contract and one vendor
- Tendered for new Enterprise Storage Solution
- Implemented a single group Domain Structure
- Alpha Server Hardware Upgrade
- ePolicy Orchestrator anti virus software
- Migration from Linux and Pegasus mail to Exchange and Outlook
- Created Active Directory as part of a migration from Novell to Microsoft 2003
- Server Consolidation



- Microsoft SMS which will facilitate remote application installation, security and product updates and remote management of all PCs
- VPN Concentrator for remote access for vendors and hospital staff
- Netware SAN (File and Application Storage)
- Migration from Netware on all systems
- Rebuild of all servers to Windows 2003 Advanced Server
- Expansion of active network
- Removal of all modems
- Increased security throughout hospital
- E-mail and web monitoring for security

Projects that made significant progress in 2004 and to be completed in 2005:

We are currently preparing a requirements specification for an Order Communications System which will provide for the electronic raising / transmission of orders and the gathering / receipt of results between patient based departments or locations and ancillary departmental systems e.g. Pathology, Radiology etc.

The Nursing Personnel system will go live in January 2005. It records information on nurses relating to their demographics, nursing registration, contacts, career history, education, professional development activities, research, membership of professional organisations, skills and some details relating to leavers. A nurse-scheduling module will also be incorporated which will aid staff management and skill mix on the wards. The scope of this system has been extended to include Personnel and Medical Administration.

Our level of involvement with the new building project has continued to grow. We received substantial capital funding from the ERHA for the procurement of active network equipment, PCs and peripherals. We are working closely with the project office on the procurement of a number of clinical systems and on preparations for the commissioning of the new building.

Library & Information Service

Allied Health Professional and Support Services

The Library & Information Service (LIS) facilitates all staff of the three hospitals in St Vincent's Healthcare Group and all students from UCD who are resident in the hospitals.

Library and Information Services are located on two sites: at St Vincent's University Hospital (SVUH), in the Education & Research Centre; and at St Michael's Hospital (SMH), in the Annexe.

Service Developments/Activities

St Michael's Hospital

In 2004, St. Michael's Hospital appointed Breda Bennett, a qualified librarian, to develop library and information services. The Library is currently based in the former Nurses' Library. There is also a medical Reading Room in the Hospital stocked by UCD, for the sole use of its medical students and medical staff. Breda's remit is to pull these two resources together to create an integrated library and information service for all healthcare staff at St Michael's.

The two library sites – at SVUH and SMH - are working together to develop equal access across the Healthcare Group.

Informatics

HEA Net decided to install fibre optic networks to connect all Dublin hospitals. The work was to be completed by the end of the year but has been delayed.

A wireless local area network was installed in the ERC to provide mobile users with flexible connectivity to the UCD network.

In September, UCD agreed to install a VPN concentrator in SVUH to allow UCD staff on the hospital side gain access to UCD network services. This is a very significant development since it will be technically possible to roll out electronic resources to all healthcare staff, but the project will need significant backing from all stakeholders. Installation is due for early 2005.

Work also began to connect SVUH and SMH libraries for the purpose of sharing the Heritage IV software over two sites. This project is dependent upon an improved connection between the two hospitals. This should be achieved by early 2005.

Collection

Shelving: New shelving was added to SVUH Library at the end of 2003 and staff worked with contractors in the summer relocating and reorganising the shelves and stock.

Journals: SVUH Library subscribed to 106 journal titles in the areas of medicine, surgery, nursing and allied health, at a cost of nearly €50,000. This is funded jointly by the Hospital and UCD. Over 60 titles were also received on donation.

A review of journal subscriptions took place this year and, as a result, 12 new titles were subscribed to for 2005, ensuring that each healthcare department is now represented by a major journal in its field.

In St. Michael's Hospital a rationalisation project was begun to centralise all journal subscriptions. The Hospital now subscribes to 30 journal titles at a cost of €17,000 .

Books: €6637.16 was spent on 136 new books for SVUH Library. Of those, 24 books were purchased from the undergraduate medical textbook funds at a cost of €1540.70.

SMH Library purchased 27 new books in 2004 at a cost of €2000 approximately.

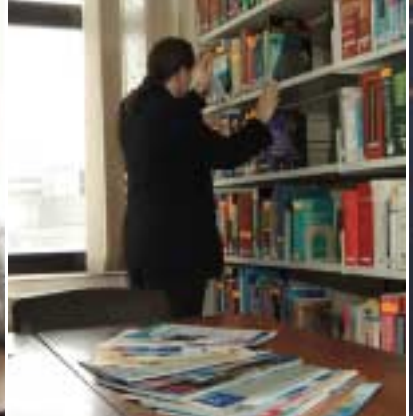
Heritage IV: All of the collections of SVUH Library were added to the Heritage system by the end of the year. The library in St Michael's Hospital received funding for an additional administrator licence for the system and for the WebOPAC module. This will make the catalogue available to all users in the Healthcare Group.

Electronic Resources

Through the UCD server SVUH Library had access to a number of databases including Cinahl, Web of Science and Medline. Over 15,000 journal titles are available through this server and other online resources for healthcare staff include Clinical Evidence, Books@Ovid and MD Consult.

The Library subscribed to two databases: PyscINFO and BNI Plus and provided online access to 60 of its journals. PubMed and the Cochrane Library continue to be freely available on the Internet.

A number of groups and individuals availed of the free tutorials provided by library staff on searching the electronic databases.


SMH Library now has 8 PCs, all connected to the Internet. Databases are available to individuals who have access to electronic resources from UCD, RCSI, DCU etc. Training is provided by the Librarian to individuals and small groups.

Interlibrary Loan

The number of interlibrary loan (ILLs) requests fell dramatically in SVUH this year, thanks to the increasing availability of online journals. 828 items were requested: 118 were from the British Library; 356 were from Subito; and 354 from the network of Irish Healthcare Libraries (IHL). The Library supplied 682 to other hospital libraries nationwide, proving how valuable our journal collection is.

The cost of ILLs dropped significantly this year too, to €2789.94. This was due not only to the fall in number of requests but also as a result of subscribing to Subito, a German-based document supply service, which provides material electronically at a fraction of the cost of the British Library.

SMH Library joined the IHL and the British Library. The numbers of requests outward were larger than the numbers of requests inward. Both were small.

Library & Information Services Committee

The Library & Information Services Committee met 6 times in 2004.
Its members were:

Aongus Curran, Medical Board *(Chair)*
Breda Bennett, St Michael's Hospital/Library
Salman Ghani/Huzaifa Adamali, UCD Medicine
Eamonn Fitzgerald, Group Executive
Saul Lugoye, UCD/ERC IT
Steve Richards/Usha Shan, UCD Surgery
Peter Sheehan, St Vincent's Private Hospital
Catherine Walsh, Nursing
Niamh Lucey, Library *(Secretary)*
Dermot Cullinan, IS/IT Department
Lloyd Felton, IS/IT Department
Pauline Leahy, Allied Health Professionals
Cliona O'Farrelly, ERC Research
Kieran Sheahan, Medical Board
Majella Tonra/Allison McGee, UCD Diagnostic Imaging

Outstanding/Significant Achievements

Greg Sheaf acted as head of the department, and secretary to the Committee, from January to August, to cover maternity leave.

Niamh Lucey co-edited the second edition of *Standards for Irish healthcare library and information* services, which, after a long delay, is due for publication in 2005.

Staff attended the following conferences and training days:

Advanced Internet Searching Techniques, Dublin – Greg Sheaf

Cochrane Library training, Dublin – Breda Bennett

Heritage IV training, Dublin – Breda Bennett, Niamh Lucey & Greg Sheaf

Library Association of Ireland Health Sciences Group Annual Conference, Tullamore – Breda Bennett & Niamh Lucey

Future Plans

2005 will be a crucial year in the development of an integrated healthcare library for St Michael's Hospital. Projects include: survey of all staff; development of ILL service; and the drafting of a new plan for the library service. Library staff members gratefully acknowledge the commitment of senior management at St Michael's to this project.

A combined catalogue of books, journals and other material held in both libraries will be launched on the Group intranet in 2005. This will allow users to access the catalogue from their workstations, to reserve and renew library material and, where possible, to link out to online journals.

Pending the completion of the IT networking project in St Vincent's Private Hospital, it is hoped to extend electronic access to library resources in 2005.

Many old journals will be removed from the shelves of St Vincent's University Hospital to make way for new material. However, lack of space continues to present a big problem. Plans for redeveloping the existing space will be drawn up and other options explored.

Medical Physics and Clinical Engineering Department

Allied Health Professional and Support Services

Overview

2004 was another busy year for the department. The continuing widespread digitisation and networking of medical equipment, while it brings immense benefits to the patient, presents great challenges in the areas of quality assurance and equipment maintenance. The department continued to be heavily involved in the commissioning of the new building. Staff members were also committed to the provision of significant medical physics and clinical engineering input in the specification of new equipment for the building. In addition to our many service responsibilities, staff were also involved in a variety of educational and training programmes within the Healthcare Group.

Medical Physics

The staff compliment remained unchanged in 2004. The four permanent staff members provided a wide range of routine medical physics services to the Group. These services included - radiation protection, image analysis, quality assurance and quality control, in-vivo and in-vitro testing, research support and teaching. Staff were also active in the compilation of submissions to regulatory agencies, such as The Radiological Protection Institute of Ireland, and in the preparatory work necessary to meet the inspection requirements of these bodies.

The professional training scheme being run in conjunction with St. Luke's Hospital absorbed a considerable amount of staff effort. The scheme was reassessed during the year and a greater emphasis placed on the candidate's training portfolio and on independent evaluation of competencies. These modifications necessarily involved a greater time commitment to the scheme from the four permanent staff members. One candidate successfully completed the training course in 2004.

There was also active participation of staff members in teaching at undergraduate and postgraduate level in U.C.D. (School of Diagnostic Imaging and Department of Physics) and also in the Faculty of Radiology of the Royal College of Surgeons. In recognition of this continuing ongoing commitment to teaching and research, Michael Casey was appointed honorary clinical lecturer in diagnostic imaging in the Faculty of Medicine in U.C.D.

Staff members were committed to professional development in the course of 2004. We wish to express our gratitude to hospital management for giving staff members the opportunity to attend the following conferences:

- British Nuclear Medicine Conference. Brighton. April 2004.
- Association of Physical Scientists in Medicine. Dublin. July 2004.
- European Nuclear Medicine Association. Helsinki. September 2004.
- Radiation Protection and Quality Control in Radiopharmacy. Slovenia. September 2004.

Clinical Engineering

Staff Members

Tom Smyth
Chief Clinical Engineering Technician

Richard Bergin AEng AMIEI
Principal Clinical Engineer

Derek Farrell
Senior Clinical Engineering Technician

Frank Kelly IEng.MIHEEM
Senior Clinical Engineer

John Harte
Senior Clinical Engineering Technician

Aidan O'Connor
Senior Clinical Engineering Technician

Declan Murray MSc
Principle Clinical Engineering Technician (Project)

We welcome the appointments of John Harte and Aidan O'Connor as part of the team.

Although Stephen McGrath has retired, the hospital has retained his valued service by re-hiring him on a part-time basis to work with the project team.

Service Developments/Activities

- Preparation for forthcoming project, equipping
- Compiling comprehensive equipment specifications
- Advising on and evaluating new equipment purchases
- Continuously updating information on EEC Directives
- Continuous liaison with health and safety including product alerts
- Quality control
- Technical specifications for all new electro-medical equipment

Outstanding/Significant Achievements/Presentations

F. Kelly achieved a Diploma (Hons) in Renal Technology City University London, in conjunction with School of Nursing St. Bartholomew's Hospital London. He lectured on the Higher Diploma in Renal Nursing UCD. He also lectured to Renal Nurses on dialysis water quality. He was a guest speaker at a Dialysis Technologist training course, Dublin. "Principles and Practice of Renal Technology". He is the National Treasurer for the Biomedical / Clinical Engineering Association of Ireland.

Academic Achievements

In September 2004, Richard Bergin achieved the title of Associate Engineer with the Institution of Engineers of Ireland. He was the first member of our staff to achieve the title. It is hoped that in the years to come more members of our staff would obtain the same title. The Institution of Engineers are the professional representative body for Clinical Engineering in Ireland and, as such, by act of the Oireachtas in 1969 are entitled to award the title of Chartered Engineer, Associate Engineer, and Engineering Technician of the Institution, confined to suitably qualified candidates.

Publications

"Renal Technology a Students Prospective"; BEAI Scientific Meeting Tullamore October 2004, F. Kelly

Departmental Statistics

The department has continuously succeeded in producing considerable savings in equipment repairs and planned preventative maintenance by adopting the policy of only outsourcing maintenance when absolutely necessary.

Future Plans

We plan to introduce a new I.T. solution to our method of equipment management. This will be in place by the end of April 2005 and will also serve to greatly assist the task of equipping the new building.

Also, an operational policy is being developed for the establishment of an equipment library.

Medical Social Work Department

Allied Health Professional and Support Services

Staff

In the past year two valued members of the team have left for various reasons.

Susan Cox, Senior Social Worker, returned to her native Australia. She had laid a very solid foundation for Social Work in the area of Palliative Care Medicine since February 2003.

Mary O'Connell took two years leave of absence in December 2004 in order to travel and work. She is presently in Vietnam and heading eventually to New Zealand. She was one of the founder members of the Bereavement Service that was launched by the Department in April 2003.

Kelle Mason took up a locum position at the beginning of June 2004 in the areas of Cystic Fibrosis, Pain Medicine, Gastroenterology and General Medicine.

Mairead Phelan provided a Social Work service during Rita Delaney's maternity leave from February to August 2004.

Enara Extebarria covered the remainder of the period until October 2004.

Louise Morgan obtained a permanent position as the Emergency Department Social Worker in September 2004 after doing four months as a locum in the same area. Finally, Aine Canny became Senior Social Worker in Palliative Care in June 2004.

Congratulations to Rita Delaney on the birth of baby Daniel in March 2004.

Service Developments/Activities

Bereavement Service

The Bereavement Service continues to develop and has completed a very successful year. Again, there were two blocks of seminars in April/May and October 2003. The speakers essentially remained the same while the format slightly changed. Two additional members of the team became involved, Teresa McDonnell, Senior Social Worker and Una Curtin, Senior Social Work Practitioner.

Information Evenings

Martina Dolan, Acting Senior Social Worker, has been running information evenings for relatives of patients who are known to the Old Age Psychiatry Service. The topics covered include community services, long term care options, health and social welfare entitlements. An evaluation questionnaire is completed by participants at the end of the session.

Emergency Medicine

Louise Morgan is the first Social Worker appointed in St Vincent's University Hospital designated solely to the area of Emergency Medicine. This is a vital appointment in that this is the first point of entry to the hospital service for many, usually a crisis situation, and the professional located here can focus immediately on the patients.

Professional Development

All staff had opportunities to advance their professional development during the year. Eileen O'Donnell and Aine Canny, Senior Social Workers, attended several courses pertaining to death and dying.

Margaret Cagney, Michelle Harte and Martina Dolan attended several seminars during the year concerning Older People and their care.

Victoria Richardson attended several study days run by the Social Work Interest Group on Mental Health.

Martina Dolan and Margaret Cagney were also active in their respective professional interest groups.

Rhona Sherlock and Rita Delaney attended a seminar on rehabilitation held in the National Rehabilitation Hospital, Dun Laoghaire.

Margaret Hunter was a participant in the International Conference on Cardiac Rehabilitation held in Dublin in May 2004.

There were also several in-house seminars attended during the year by Senior staff e.g. Employment Law, Teamwork, Recruitment, Diversity.

Several members of the Social Work Team were involved in the hospital Accreditation process during the year.

Significant/Outstanding Achievements

- In November 2004 our department received one of four awards for innovation from the hospital management. The award was for the work we have done in providing a bereavement service to relatives of patients who die in the hospital, as well as future plans for the project. We received a bursary of €2,000 that will be a useful addition to our budgetary resources for 2005.

Future Plans

- This year we hope to do some training as a team. We have chosen Brief Therapy as a pertinent topic for advancing individual skills. Hospital Social Work by its nature is short term and usually crisis orientated. We need to optimise our involvement with patients and families.

- We also have been focusing on improving our accommodation situation in the next few years. We need more space in order to interview clients/patients and families in privacy. We also need a staff/education room.

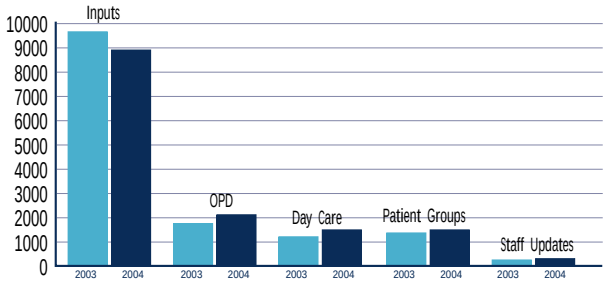
- We hope to look at more group work with patients and families in particular areas of medicine and surgery. It is a very practical way to encourage self-help amongst clients and is time effective.

Department of Nutrition and Dietetics

Allied Health Professional and Support Services

Departmental Statistics

The overall variance in consultations from 2003 to 2004 is down slightly (3.7%). However OPD consults have increased with the reinstatement of OPD clinics in 2004, which had been suspended during deferral of staff appointments in 2003. There has been a 9% increase in the number of patients attending group education sessions to facilitate the increased demand for services, as staffing levels have remained unchanged since 2001.



Staff

The staff compliment level of the Department is 12 WTE dietitians and one administrator. Yvonne Lydon and Eimear Fanning were successful in securing senior positions in St James' and Beaumont Hospitals respectively. Fiona Boyle, locum dietitian, obtained a permanent position in the National Children's Hospital, Temple Street in September. Julie Dagg, Senior Dietitian resigned her position in October. Dorothy Loane joined the department in April. Louise Walsh and Cathy Monaghan joined the department in locum capacity.

Service Developments/Activities

Five dietetic students successfully completed their clinical placement training with the department between February 2004 and February 2005. Each student completed 17 weeks of a 26-week Dietetics programme in this hospital.

In collaboration with the Nutrition Committee, the Annual Nutrition Seminar entitled *Nutrition and patients – a doctor's responsibility* was held in March with final year medical students as the target audience. 72 students attended and 68% completed the evaluation forms. While all attendees found the afternoon very informative and useful, the students identified areas that they required training on which included diabetes, weight and lipid management. These areas are beyond the scope of the Nutrition Committee. The evaluation summary has been sent to the Dr Seamus Donnelly and Barbara Cantwell so that the University can address these issues.

Maeve Moran, Senior Dietitian in Diabetes has collaborated with the Dietitians in St Michael's, St Colmcille's and the Community Dietetic service to devise diabetes diet sheets for use by all. This facilitates a consistent approach and content to the education of patients with diabetes. These diet sheets will be printed and in use by early 2005. The Dietitians and Clinical Nurse Specialists involved in the shared-care service for Diabetes have set up a quarterly education group for patients newly diagnosed with impaired glucose tolerance.

Vivien Reid, Clinical Specialist Dietitian, worked with Billy Kearns, Chef and Rosemary Hickey, Catering Manager, to revise the menu choices for patients in the Coronary Care Unit. Positive feedback has been received from patients and staff.

Dietitians participated either directly (as part of a team) or provided support material and information for a number of the quality improvement teams (QITs) leading up to the Hospital Accreditation process in May. We continue to participate in the quality improvement projects that have been sanctioned to date.

Significant Achievements

In collaboration with the Department of Speech and Language Therapy, we have undertaken a number of initiatives to address the gaps in care of patients with dysphagia. We have developed joint care plans, which are in use already. A further initiative was completed in 2004 with the development of an integrated educational tool for use with patients and/or carers. The booklet entitled *Swallowing Problems and Nutrition* was developed by a Dietitian and Speech and Language Therapist and approved by both Departments. It was piloted with patients and carers with modifications made on this feedback. The booklet complies with National Adult Literacy guidelines. It is a practical, user-friendly educational tool that can be used by therapists to facilitate the education and upskilling of patients and /or carers in relation to the safety and nutritional adequacy of a modified consistency diet. As an integrated tool, both professions can

support and reinforce appropriate and safe advice minimising the risk of respiratory infections, while maintaining or improving dietary intake and nutritional status. This department was delighted to share the Innovation Award allocated to this project.

Lorraine Sewell, Departmental Administrator was awarded her diploma in First Line Management in October 2004.

Significant Publications/Presentations

Presentations

Doyle, M. *"Imagining the body – anorexia of the mind"*. Delivered as part of SUVH Psychiatric team's presentation to the Health Conference organised by the Arts Council of Ireland as part of the closing of Ireland's Presidency of the EU. Dublin, May 04.

Doyle, M. *"Management of Eating Disorders"*. The Annual Study Day of Paediatric Interest Group of the Irish Nutrition and Dietetic Institute, Dublin Sept 04.

Moran, M. *"Nutritional management of diabetes – present and future"*. Diabetes Federation of Ireland Dublin Nov 2004.

Parke, L. *"Investigation of the effect modifying oral intake during dialysis and of other factors on the incidence of intradialytic complications"*. British Renal Society Annual Conference in Harrogate, May 2004

Reid, V. *"Influence of cardiac rehabilitation on lifestyle change – one year follow up"*. World Congress of Cardiac Rehabilitation, Dublin. May 2004.

Poster presentation

Barnes, EA, McAuliffe, E, *"Exploring the information needs of Irish patients with colorectal cancer"*. UICC Conference for Cancer Organisations in Dublin Nov. 2004.

Reid, V. *"Fruit intake of hospital staff – how does the data compare with national data?"* Nutrition Society, Dublin July 2004.

Publications

Moran, M. *"The evolution of the nutritional management of diabetes"*. Proceedings of the Nutrition Society Nov 2004 (in press).

Moran, M. *"Prescriptive education – Is this adequate for people with diabetes"*. Diabetes Wise Vol 1 (2) Summer 2004.

Future Plans

We will explore the use of group education for patients with coeliac disease. Best-practice guidelines indicate that patients should have an annual consultation with a Dietitian. However, with long waiting lists for OPD, group education may be more efficient. Patient evaluation of this initiative will be undertaken.

In collaboration with Speech and Language Therapy and Catering, we intend to explore the possibility of developing modified consistency diets for patients with dysphagia.

A pilot project of professional supervision will be undertaken for 2005. 7 members of the department will be involved in this process. If successful it will be rolled out to the remaining members of the department provided funding can be secured.

Occupational Health Department

Allied Health Professional and Support Services

Staff

Members of the Occupational Health Department (OHD)during 2004 were:

Occupational Physician		
	Dr. Paul Guéret	Dr. Jacintha More-O'Ferrall
	Dr. John Walsh	
Occupational Psychologist		
	Nuala Gannon	
Clinical Nurse Specialist (CNS)		
	Siobhan Bulfin	Ann O'Reilly
	Justine McGrane	Aisling Purcell (Acting CNS)
Clinical Nurse Manager (CNM)		
	Orla Seale	
Administrative Support	Áine Yap	Ciara McCarthy

Other hospital staff who contributed to the department include Ergonomists, the Health & Safety Coordinator, Infection Control, Health Promotion and the HR Department.

In June 2004 Susan Collum resigned from the position of administrative support.

Significant Achievements

Needlestick injury prevention activities

Medical and psychological assistance was provided for the 113 staff members who received inoculation injuries in 2004. This number is similar to the incidence of needle stick injuries in 2003 levels, which represented a reduction of 25% on the 2002 figures.

Needle stick injury prevention workshops continued throughout the year. 281 (14.5%) staff members attended these workshops. A concerted effort was made to include all categories of staff, including medical staff.

Seminars were provided on 'Management of Blood Borne Viruses' and 'Management of needlestick injuries'. Nuala Gannon presented a paper on the findings of the pilot phase of the needlestick injury prevention programme at the 12th Conference on Health Promoting Hospitals conference in November.

A needlestick injury prevention information stand was displayed regularly in different locations in the hospital.

'Working Backs Programme' for staff

A multidisciplinary care pathway was developed for staff members who require treatment for their back. Education sessions and Information Packs were sent to all Back Clinicians and Line Managers. Promotion stands were also held to advertise the programme to staff. The programme commenced in April 2004 and has been successful to date - 136 people have attended. There were 54 referrals to Physiotherapy. The average sick days lost if the employee attended the OHD was 1.8 days.

Stress Management Activities

The 'Managing Work-Related Stress' seminars continued. 218 staff members attended these sessions.

Other Seminars and workshops provided included:

- Well-being at work;
- Facilitating behavioural change;
- Understanding loss and bereavement amongst colleagues;
- Dealing with verbal and physical assault and aggression.

Influenza Vaccination Programme

All hospital staff were encouraged to avail of the influenza vaccination programme. Out-of-hours clinics were provided to staff resulting in 588 employees receiving the vaccine.

Infection Control

Follow up precautions were taken by the OHD following exposure to 3 cases of TB and 3 cases of shingles and Varicella.

SRSV: 450 staff contacted the OHD during 3 outbreaks in 2004, a considerable number of additional staff telephoned the department seeking advice.

MRSA: Student nurses attended the OHD for MRSA screening and ward staff were advised and screened as requested by Infection Control.

Education

The following seminars were provided:

- Pregnancy risk assessments and chemical risk assessment awareness
- Induction seminars
- Skin care information session ('The right glove for the right job')
- Use of Latex in Healthcare.

The department contributed to many other educational activities including:

- Health and Safety Week;
- Smoking Cessation programme;
- SVUH Adult education programme;

The OHD is continuing to review all policies and initiate setting up of standard operating procedures (SOPs) to standardise all our activities.

Audit

The following audits were carried out:

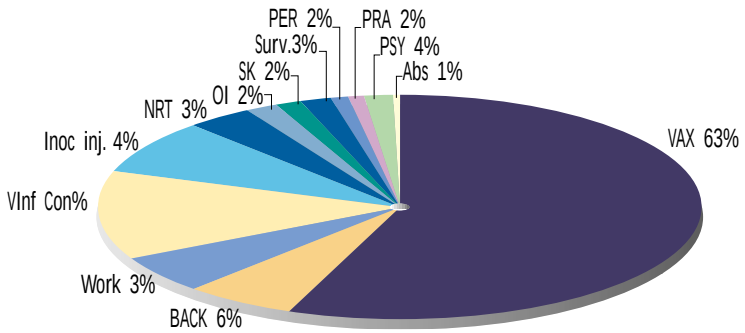
a) An evaluation of the needlestick injury prevention programme ("Be sharp, Be safe")
b) Needlestick injury rates
c) The effectiveness of needlestick equipment in use
d) Time lost from work due to back pain
e) Work absence in specific locations
f) Immune status evaluation was carried out on all staff in several areas.

These audits proved useful in informing the work of the department.

To comply with the Accreditation process, the OHD staff continue to monitor and improve the quality of its services.

Department Statistics

A total of 3,107 people attended; a significant number also availed of telephone consultations.



Reason	Number
Absenteeism	15
Vaccination	2002
Back	181
Workability	104
Infection Control	147
Inoculation Injury	113
Nicotine Replacement	83
Occupational Injury	41
Skin	53
Health Surveillance	84
Pre-Emp Review	68
Pregnancy Risk Assessment	45
Psychosocial	129

Reasons for attendance

Occupational Injuries

	2004	2003	2002	2001
Sharps/Inoculations	85	83	104	106
Blood splashes	28	18	24	9
Other injuries	245	189	252	235

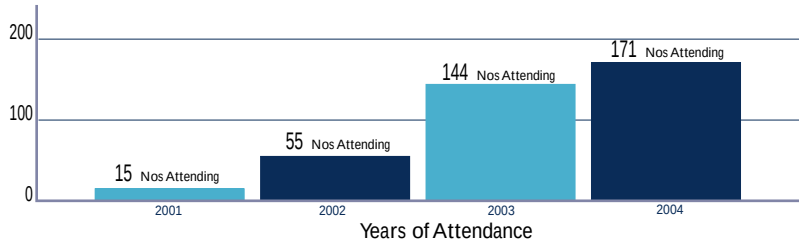
OHD attendance for certain occupational infections

	2004	2003	2002
SRSV	450	98	182

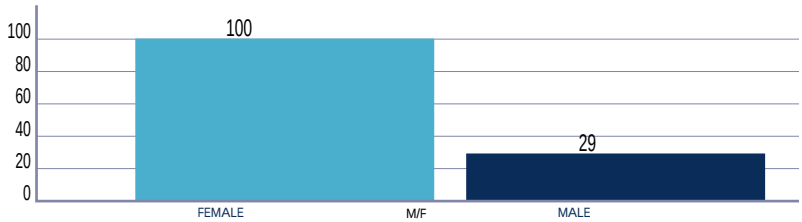
Visits to the OHD for non-vaccine consultations

	2004	2003	2002	2001
Seen by				
Occupational Nursing Team	1627	1247	993	617
Seen by				
Occupational Medical Team	1038	463	782	920
Seen by				
Occupational Psychologist	171	144	55	15

Attendance at Occupational Psychologist 2001-2003

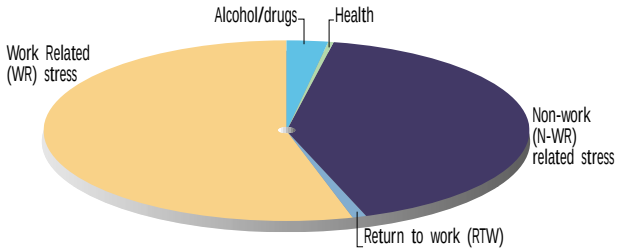


Breakdown by gender (n=129)



General classification of reasons for attendance at Occupational Psychologist

Reason for attendance



Alcohol/drugs	7.0%
Health	0.8%
N-WR stress	30.3%
RTW	1.6%
WR stress	30.5%
Total	100.0%

171 people attended the Occupational Psychologist, an 8% increase from 2003. Of these, 129 attended for stress management support; 42 people used the Managers advisory service.

In the majority of cases, the work-related stress was caused by interpersonal behaviours such as overt and covert criticism, isolation and/or exclusion of colleagues, discourtesy and disrespect, and the undermining of authority.

Priorities for 2005

- The provision of a limited service to St.Vincent's Private Hospital and development of closer links with St. Michael's Hospital Occupational Health Department.
- Continued focus on stress management activities.
- Contribution to the implementation of the HSEA 'Dignity at work' policy .
- An evaluation of the OHD services by examining:
  - The impact of the workshops on needlestick injury rates.
  - The effectiveness of 'Managing work-related stress' seminars.
  - The incidence and severity of assaults directed at staff.
  - The usage and uptake of the staff support service.
  - Varicella immune status of staff in high-risk areas.
  - The extent of latex glove use in certain departments.
  - The numbers of non-standard pre-employment medicals performed.
  - The Hepatitis B status of those carrying out exposure prone procedures.

Continuing Professional Development

Two members of staff commenced M.Sc. Degree programmes in Safety Health & Welfare – J. McGrane & A. Purcell.

O. Seale commenced a Diploma in Safety, Health & Welfare.

A. O'Reilly successfully completed the Manual Handling instructor's course.

Occupational Therapy Department

Allied Health Professional and Support Services

Staff

This year some new faces joined the Occupational Therapy Department. We welcomed Eileen du Plooy, Senior Occupational Therapist in General Medicine, and the following basic grade therapists, Áine Jackson, Amanda Wallace, Elaine Toner, Eilish Hogge and Freda Connolly, to the department.

2004 also saw two Occupational Therapy assistants join our team, Elizabeth Grant and Riona Nolan. There were also quite a lot of vacancies within the department, which also presented challenges in the area of recruitment. The Occupational Therapy Department looks forward to actively working on the area of recruitment and retention for 2005.

Service Developments/Activities

The departmental statistics convey that, despite the reduced numbers of new patients due to staff shortages, total treatment units are considerably up from both in and out patient perspectives in 2004. Outlined below are the specific developments/activities carried out within each service area during the year.

Old Age Psychiatry

- Involved in Nursing Home Education Programme.
- Ms. Catherine Keogh, Senior Occupational Therapist, and day hospital nursing staff developed the active rehabilitation group. This was facilitated weekly with good success.
- Anxiety management group continued. Four groups co-facilitated with Community Mental Health Nurses in 2004 with good success.

Hands and Plastics

- Flexor tendon injury audit completed with physiotherapy colleagues.
- Introduced use of Jobst garments in the treatment of patients for scar management and lymphoedema.

Rheumatology

- Multidisciplinary Education Group for Inflammatory Arthritis. Three groups facilitated by Occupational Therapy, Physiotherapy and Nursing in S.A.R.C.
- Ms. Carol O' Donnell, Senior Occupational Therapist, completed Rheumatology Splinting Guidelines as part of National Association of Occupational Therapists in Ireland.

Stroke/Neurology

- Mrs. Louise Sutton, Senior Occupational Therapist, formed part of clinical working group investigating best outcome measures for this service area.
- Founder members of national Occupational Therapy Stroke Special Interest Group.

Pain Management

- Programme re-commenced with great success following difficulties with recruitment.

Medicine for the Elderly

Research completed by Ms. Carmel Collins on driving and the elderly.

Significant Achievements

St. Vincent's University Hospital and St. Michael's Hospital Occupational Therapy Services formed part of steering group directing the pilot Slán Abhaile Equipment project.

SWOT analysis completed on the department.

Catherine Keogh awarded Post Graduate Fellowship Award from DSIDC.

Accreditation

Carol O' Donnell was team leader in the Bone and Joint Rehabilitation and Pain Management Team accreditation teams, while Catherine Keogh replaced Valerie Kelley on Old Age Psychiatry / Neurology /General Adult Psychiatry and Medicine for the Elderly team.

Presentations

Ms. Carmel Collins, Senior Occupational Therapist in Medicine and the Elderly and Ms. Freda Connolly, Basic Grade Occupational Therapist, presented Safety Awareness Management of Falls at the Care of the Elderly Annual Study Day.

Ms. Catherine Keogh, Senior Occupational Therapist Old Age Psychiatry was involved in the following presentations:

- Invited to lecture on Occupational Therapy and Dementia at Trinity College Dublin.





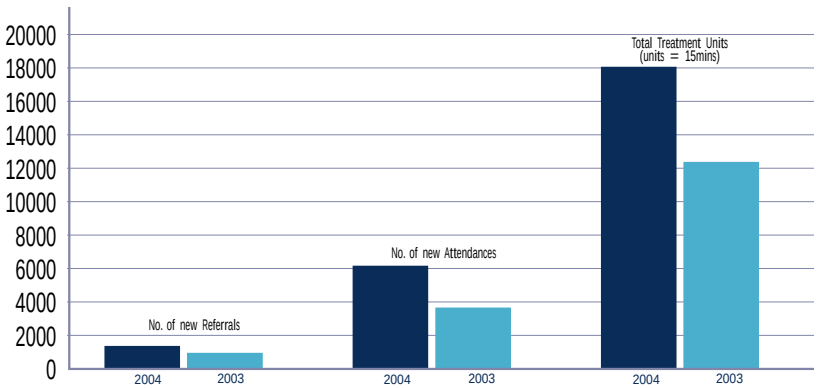
- Facilitated one-day conference in conjunction with the Dementia Services Information and Development Centre.
- Invited onto the Executive Committee of the Mental Health Commission to consider a working definition of severe dementia for the Mental Health Act, 2001.
- Facilitated one-day conference in conjunction with the Dementia Services Information and Development Centre (DSIDC) on "Occupational Therapy and Person Centred Care in Dementia Care".

Staff from the following areas continued active participation in professional Special Interest Groups including Rheumatology, Stroke and Neurology, Hands and Plastics Hand Therapy and Oncology.

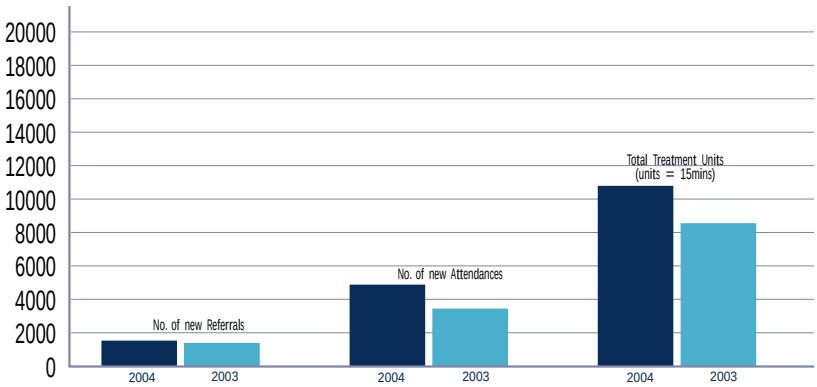
Departmental Statistics

Occupational Therapy In-Patient activity

Year	New Referrals	No. of Attendances	Total Treatment Units
2004	923	3,631	18,025
2003	1,335	6,135	12,349



Occupational Therapy Out-Patient Activity



Future Plans

To complete implementation stage of computerised data management system.

To implement the move and development of Occupational Therapy Services, especially Rheumatology in the Bone and Joint Ambulatory Day Care Centre.

Complete the move of Occupational Therapy Service from St. Camillus to New Psychiatry Building. This is seen as an exciting and challenging development, as apart from the new environment, the Occupational Therapy Service hopes to extend some of its services to being more Community based.

In Rheumatology, the the multidisciplinary education group and the inflammatory arthritis group continue to develop. The service also plans a relaxation group for service users once the move to the new building has been completed.

Hands and Plastics have been offered as part of basic grade rotation.

Introduction of Peer Group/Problem Based Learning for Basic Grade Therapists.

Occupational Therapy Out-Patient Activity

Year	New Referrals
2004	1,356
2003	1,496

Year	No. of Attendances
2004	3,413
2003	4,844

Year	Total Treatment Units
2004	10,770
2003	8,535

Chaplaincy and Pastoral Care Department

Allied Health Professional and Support Services

Chaplaincy Department

The Chaplaincy Department, like every other department within the hospital represents a very specific and professional discipline. It works with other professional disciplines in providing an overall or Holistic Service that is geared to total Patient Care and Ministry to the family.

The Department is interdenominational in character and all members of the department are professionally trained for their work.

The Department has two main functions:

- To provide Pastoral & Sacramental Service to patients, their families and staff.
- To provide Educational Programmes for the 'In-Service Training' of student chaplains.

Chaplaincy Staff

Since the last annual review, there have been changes in personnel. Sr. Daphne O' Gorman was appointed to the Chaplaincy team in December. Sr. Daphne is a Religious Sister of Charity and has formerly worked in Child Care. Daphne trained in CPE here at St. Vincent's University Hospital and was accredited by the Healthcare Chaplaincy Board of Ireland in October. Daphne is a very welcome addition to the Chaplaincy team and we wish her well in her new career.

Service

When patients are admitted to hospital in need of healing for a physical or emotional illness, they often bring with them other concerns that range over the psychological and spiritual dimensions of their lives. Being aware of their many concerns, the Chaplaincy Department responds at different levels:

- In the administration of the sacraments and in providing appropriate spiritual care and counselling to patients.
- The need for this care is acutely felt at times of great anxiety, such as major surgery and especially terminal illness.
- Much time is spent with families of patients, supporting those shattered by the terminal illness of those closest to them, and later in consoling the bereaved.
- The Chaplaincy Department continues to provide a twenty-four hour service in the hospital. The team consists of three full time Roman Catholic priests, two part time priests, two Church of Ireland ordained Ministers, and three chaplains from the Religious Sisters of Charity. The department is very fortunate to have over forty lay volunteers who serve as Ministers of the Eucharist and assist in bringing the Blessed Sacrament to patients on a daily basis.

During the past year, the chaplains attended at over seven hundred deaths and made 60-80 pre-operation visits per week to patients who had requested a visit. Added to these were the informal visits made on wards when time permitted.

Staff Support

The members of the Chaplaincy Department work closely with the other health care professionals and are available to provide counsel and support to staff when the need arises.

Religious Services

The chaplains provide liturgical services in the main hospital chapel, on the wards, and in the oratory during Advent, Lent and on special occasions. One memorable occasion was the special Mass in February to commemorate the World Day of the Sick. A large number of patients attended with the kind assistance of many staff and volunteers. The Sacrament of the Sick was administered during a very prayerful and moving liturgy.

Five Masses were held during the month of November for deceased patients. The next of kin and family members of patients who died during the past year were invited and it gave bereaved families an opportunity to come together and remember their loved ones in a 'Candle Ceremony of Remembrance'. There was a very large attendance at the Masses and bereaved families were very appreciative of the support and consolation they received. For many families, it was their first time back in the hospital since the death of their loved one and they found it brought a sense of closure to their experience. Bereaved staff, and deceased members of staff and their families were particularly remembered at a special Mass organised by the Mission Effectiveness Committee.

Seminars

During the year, seminars on pastoral care were given to the student nurses, at the in service training programmes for Overseas Nurses, at induction programmes for new nurses, and for new employees.

The chaplains attended the Annual Conferences of the National Association of Hospital Chaplains and of the Dublin Diocesan Committee of Hospital Chaplains as well as other relevant study days.

Professional Bodies

At present, a member of our Chaplaincy team sits on the Executive Committee of the Dublin Hospital Chaplains' Association.

Clinical Pastoral Education

The primary focus of the C.P.E. course is the spiritual needs of the patients. The course aims at enabling the student develop a better understanding of patients' needs and how to respond to these. The participant's personal ministry is enhanced through interpersonal group experience and other methods of reflection on how one relates to patients (people in crisis). Attention to the participant's personal development and supervision of the individual's pastoral work promotes a greater self-assurance and effectiveness in ministry. Formal lectures focus on areas such as the theory of pastoral care, loss and bereavement and personal development.

Participants are welcome from all religious denominations. While these courses were originally aimed at providing practical pastoral training to clergy, seminarians, and members of religious orders, in recent years there has been a growing interest in this area among lay people.

During the past year, the Chaplaincy Department hosted an intern year in Clinical Pastoral Education. Four student-chaplains participated from September through to July. One other joined this group from January to July. The stability and continuity of this course offers students a special learning experience. This also enhances the quality of Pastoral Care offered to patients.

Sr. Dervilla O' Donnell (Supervisor in Training) shared leadership with Fr. Joe Cahill in directing the final unit of CPE from September to December. Nine student chaplains participated in the course.



Pharmacy Department

Allied Health Professional and Support Services

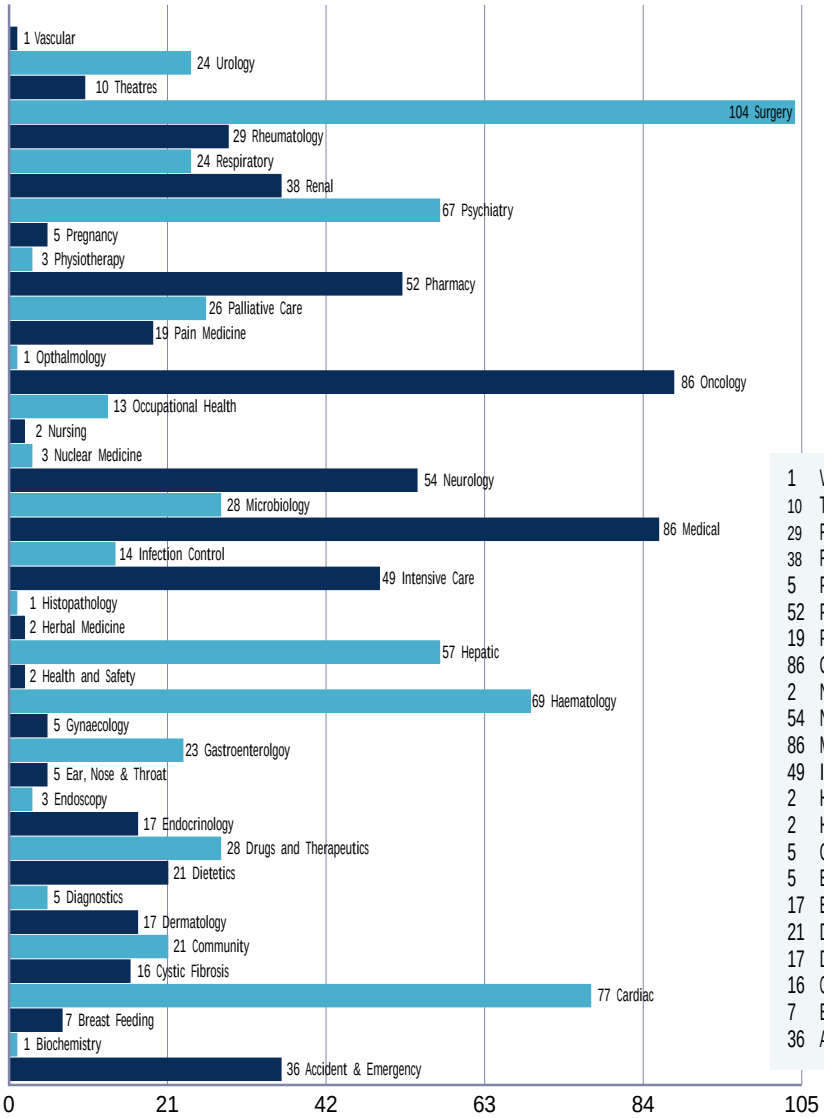
Service Developments / Activities

In 2004, there were considerable challenges for the Pharmacy Department in meeting service needs. This was mainly due to an increase in activity over the previous year. For example there was an increase of 6.4% in the number of items issued from the dispensary.

Medicines Information (MI) Service

In 2004, Medicines Information (MI) answered 1,173 enquiries, an increase of 3.7% compared with 2003 (1,132 enquiries).

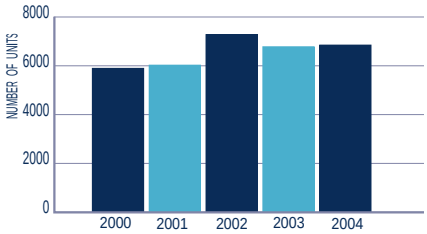
Breakdown of enquiries by directorate for the organisation, in 2004 are shown below.



Aseptic Compounding Service

The number of units prepared in the Aseptic Unit (ASU) in 2004 was 6845. Since 2000 the number of units prepared has increased by 20%.

Number of Units compounded



These figures do not reflect the complexity of work and they do not take into account quality assurance and research activities. In 2004, a capacity planning formula was devised to measure workload, taking these additional parameters into account. This formula was successfully used to negotiate additional staff for the ASU.

1	Vascular 0.95%	24	Urology 2.06%
10	Theatres 0.86%	104	Surgery 8.94%
29	Rheumatology 2.49%	24	Respiratory 2.06%
38	Renal 3.27%	67	Psychiatry 5.76%
5	Pregnancy 0.43%	3	Physiotherapy 0.26%
52	Pharmacy 4.47%	26	Palliative Care 2.24%
19	Pain Medicine 1.63%	1	Opthalmology 0.09%
86	Oncology 7.57%	13	Occupational Health 1.12%
2	Nursing 0.17%	3	Nuclear Medicine 0.26%
54	Neurology 4.64%	28	Microbiology 2.41%
86	Medical 7.39%	14	Infection Control 1.2%
49	Intensive Care 4.21%	1	Histopathology 0.09%
2	Herbal Medicine 0.17%	57	Hepatic 4.9%
2	Health and Safety 0.17%	69	Haematology 5.93%
5	Gynaecology 0.43%	23	Gastroenterolgoy 1.98%
5	Ear, Nose & Throat 0.43%	3	Endoscopy 0.26%
17	Endocrinology 1.46%	28	Drugs and Therapeutics 2.14%
21	Dietetics 1.81%	5	Diagnostics 0.43%
17	Dermatology 1.46%	21	Community 1.81%
16	Cystic Fibrosis 1.38%	77	Cardiac 6.62%
7	Breast Feeding 0.6%	1	Biochemistry 0.09%
36	Accident & Emergency 3.1%		

Patient Controlled Analgesia Audit

The Pharmacy Department and the Departments of Anaesthesia and Nursing carried out a review/audit of PCA usage in St. Vincent's University Hospital (SVUH) and St. Vincent's Private Hospital (SVPH). As a result, the following changes were implemented:

- Introduction of a new PCA pre-filled device.
- Revised documentation e.g. PCA prescription sheet and monitoring guidelines.

The PCA service will be re-audited in 2005.

St. Vincent's Healthcare Group Prescriber's Guide and Formulary

The 3rd edition of the St.Vincent's Healthcare Group Prescriber's Guide and Fomulary (2004/2005) was circulated in August.

Accreditation

A lot of time and effort went into preparations for the Accreditation survey. The Pharmacy Department's contribution to the process was capably managed by Fionnuala Kennedy.

Project Work

Work on operational policies for the new building started in 2004. Considerable progress has been made on identifying the gaps between existing resources and those required to meet service needs in the new building.

Discharge Medication Policy

A new discharge medication policy for the Emergency Department was agreed between medical, nursing and pharmacy representatives. This service provides discharge medication packs for patients who anticipate a delay in having their prescription dispensed.

Medication Incident Reporting

The IT Department designed a software programme for recording data on medication incidents. Work has commenced on inputting medication incident reports (MIRs) from 2004 on this database. The Drugs and Therapeutics Committee will use this information for compiling strategies for medication incident avoidance.

Achievements

- Niamh O' Hanlon won the award for the best presentation of original research at the United Kingdom Medicines Information (UKMI) annual conference in Warwick.
- Sinead Murray received a Diploma in Clinical Pharmacy from Queens University, Belfast.
- Niamh O'Hanlon was awarded an MSc in Health Services Management by the University of Dublin, Trinity College.
- Lisa Hammond and Brian Spooner graduated in November after completing their pre-registration year.
- Laura McCabe registered for an MSc in Hospital Pharmacy, University of Dublin, in January 2004.

Lectures and Workshops

The pharmacy staff delivered a comprehensive selection of lectures and educational support for many under-graduate and postgraduate medical, nursing and pharmacy programmes.

Publications and Posters

Gonzalez Z, Schaffer K, O'Sullivan S, Fitzgerald S, Fenelon L.
Evaluation of Diagnostic Studies and Antibiotic Treatment on Admission to a Dublin Teaching Hospita.
Poster presentation at the National Pharmaceutical Congress (Spain), Alicante, October 2004.

Kennedy F.
PCA Use in a Large Acute Hospital.
HPAI Clinical Conference, Dublin, April 2004

Spooner B.
Hospital Discharge Prescriptions and their Impact on Community Pharmacy and the Drugs Budget.
Thesis as part of pre-registration training programme.

Hammond L.
An Investigation of Medicines Information Sources used by Medical and Nursing Personnel at SVUH.
Thesis as part of pre-registration training programme.





Conferences

All Ireland Cystic Fibrosis Meeting; Killarney, February 2004. C. Keane.

British Travel Health Association Conference; London, March 2004. N. O'Hanlon.

European Association of Hospital Pharmacists Conference; Seville, March 2004. F. Kennedy.

Seminar on Cancer-related Fatigue; Dublin, March 2004. E. Marsden.

HPAI Clinical Conference; Dublin, April 2004. C. Keane, F. Kennedy, C. Kingston.

UK Pharmaceutical Technicians Conference; Belfast, April 2004. A. Flynn.

United Kingdom Medicines Information (UKMI) Annual Conference; Warwick, September 2004. N. O'Hanlon, M. Broderick.

North/South Pharmacy Conference; Dundalk, September 2004. C. Bogue, L.Hammond.

Haematology Seminar; Dublin, September 2004. K. Feeley.

British Oncology Pharmacists Association (BOPA) Conference; Birmingham, October 2004. K. Feeley, E. Marsden, A.M. de Frein.

29th Annual UK Psychiatric Pharmacists Conference; Reading, October 2004. R. Knox.

World Congress of Cardiac Rehabilitation Meeting; Dublin, May 2004. C. Kingston.

North American Cystic Fibrosis Conference; St. Louis, October 2004. C. Keane.

National Pharmaceutical Congress (Spain); Alicante, October 2004. Z. Gonzalez Sanchez.

UK Hospital Pharmacy Conference; London, November 2004. N. O'Hanlon.

American Society of Health System Pharmacists Midyear Clinical Meeting; Orlando, December 2004. S. Murray.

UK and ROI Liver Pharmacists Group; Meetings throughout the year attended by F. Kennedy.

Plans for 2005

- Installation and implementation of a new IT software programme in the pharmacy.
- Review of drug storage systems in clinical areas.
- Publication of 4th Edition of SVHG Prescriber's Guide and Formulary.
- Re-audit of PCA usage.

Physiotherapy Department

Allied Health Professional and Support Services

The Physiotherapy Service encompasses an inpatient and outpatient service, a GP direct access scheme, a rheumatology specific GP direct access scheme, an ergonomic service, Moving & Handling training programme and an Instructors training course.

Staffing

- The physiotherapy staffing complement consists of 33.5 wte posts.
- Staffing levels were reduced by 1 wte January - May '04, holiday locum cover by 75% and on call locum cover by 100%.
- Ms Grainne O'Hara was appointed Deputy Physiotherapist Manager in May'04.

Service Developments

The Physiotherapy Department is organised under three core specialities, which encompass the scope of physiotherapy practice and education namely:

- Cardio Respiratory Care
- Musculoskeletal Care/Rheumatology
- Rehabilitation

Physiotherapy Cardio Respiratory Service

The number of patients requiring Non Invasive Ventilation (NIV) continues to rise. A proposal to set up a 5-bedded NIV ward was submitted to Mr Jermyn, Group CEO, in March '04 by the multidisplinary NIV review committee. This has been agreed in principal.

Developments in cardiology have included an obesity management programme for cardiac patients. Current research is investigating the effect of exercise on heart failure patients. Study to be completed 2005.

The Cystic Fibrosis Service's activity levels continue to exceed the physiotherapy staffing of 1.5 wte. In 2004, 0.5 wte staff physiotherapist was allocated to CF service as an emergency measure from within the physiotherapy staff complement.

A multidisplinary approach to the care of the critical ill patient is in line with best practice. A consultative process with the medical and nursing teams involved in the Critical Care Division is in place in preparation for the transfer to the new unit.

Physiotherapy Musculoskeletal Service

Hand Therapy Service recorded a significant rise in activity levels 2001-2004 (133%). The current staffing levels of 0.5 wte senior post does not meet the needs of the service. The importance of physiotherapy intervention in terms of clinical outcome post hand surgery is evident.

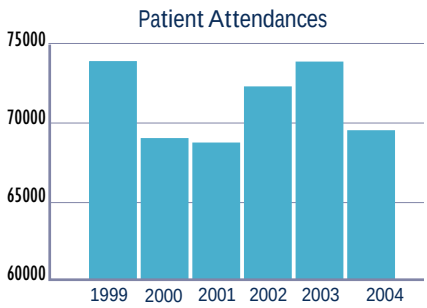
The Working Backs Programme was implemented successfully in 2004. This innovative programme offers a streamline evidenced based management pathway for staff with low back pain.

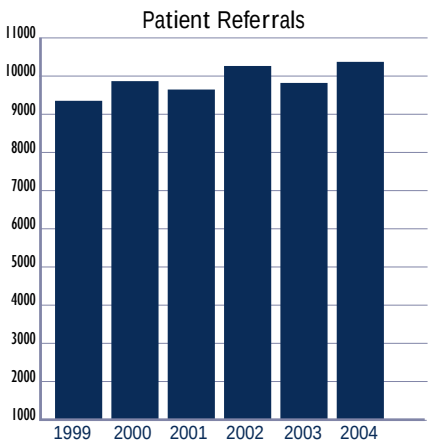
Physiotherapy Rehabilitation Service

The Neurology Outpatient Service has also seen a significant increase in referrals (220 referrals in 2004 compared to 106 referrals in 2003). This growth may be attributed to the demand for rehabilitation and lack of similar services in other hospitals in the area. The number of patients referred to the stroke service is in line with 2003 activity.

Physiotherapists are involved with the multidisplinary diabetic programme. The need to further develop this programme to include an exercise component is supported by research.

Departmental Activities





Outstanding/Significant Achievements

There is strong evidence of ongoing education and research at postgraduate and undergraduate level. 84 clinical placements are provided annually for physiotherapy undergraduates from UCD.

Awards

- Martina Fitzpatrick awarded the Abbott Education Bursary
- Martina Fitzpatrick awarded the O'Neill Bursary for research
- Noel. S, Conlon.S, Dockery M, Leahy P & O'Shea. D, awarded prize for poster presentation at International Association of Physical Therapists Working with Older People Conference. May '04.

Postgraduate Degrees

- Keith Smart, MSc by research UCD
- Karen Cradock, 2nd year MSc by research TCD
- Aoife Caffrey, 1st year MSc (Sports Physio)
- Ruth Gibson, 1st year MSc (Sports Physio)
- Sarah O'Donnell MSc (Physio) North Western University, Australia. Awarded prize for 1st place
- Grainne O'Hara commenced Leadership Programme
- Six SVUH Physiotherapy Education Bursaries were awarded to staff undertaking postgraduate degrees and diplomas.

Oral Presentations

French H, Cahalane E, Cooke G, Farrell S, Fitzpatrick M, Leahy P, Keown A.M, O'Grady A.M, O'Shea N, Smee C, *"Green Belt" Approach to conducting a Continuing Professional Development (CPD) Project'*
ISCP Conference 5th November 2004.

Cahalane E, Cooke G, Byrne M, Farrell S, Fitzpatrick M, French H, Leahy P, Keown A.M, O'Grady A.M, O'Shea N, Smee C, *'The Use of Focus Groups in a Qualitative Study of CPD'*, - ISCP Conference. November 2004.

Smart K *'Clinical Reasoning and Classification of Pain by Experienced Musculoskeletal Physiotherapists'*. Chartered Society of Physiotherapists Annual Conference. UK 2004

Murphy A *'Positioning & Handling of Stroke Patients'*, SVUH Study Day. Jan 2004

Poster Presentations

Kennedy N, Duffy A, Cussen G, Wallace L, Nolan M, Fitzpatrick M, Craig A, Dunne-Shannon U, Grant M, Buckley K, McGrane E, and Keogan F, *'The Demographic Profile of Patients with Rheumatoid Arthritis Attending for Physiotherapy Treatment; Results from a Survey of Eight Physiotherapy Departments'*

Irish Society of Rheumatology (ISR) and Irish Rheumatology Health Professional Society (IRHPS) Scientific Meeting, October 2004. Awarded IRHPS Poster Prize.

Keogan F, Duffy A, Cussen G, Wallace L, Nolan M, Fitzpatrick M, Craig A, Dunne-Shannon U, Grant M, Buckley K, McGrane E, and Kennedy N, - *'Provision of Gait Aids, Orthotics and Splints to Patients with Rheumatoid Arthritis; Results of a Survey From Eight Physiotherapy Departments'*

ISR and IRHPS Scientific Meeting, October 2004.
ISCP Conference, November 2004.
British Society of Rheumatology Conference, April 2005.

Conferences attended 2004

Physiotherapy staff attended a wide variety of national and international conferences

Plans 2004

Implement the Dublin Academic Teaching Hospitals Continuing Professional Development Project, a continuous professional development programme for staff grade physiotherapists, developed 2004 by DATH Physiotherapy Managers and Clinical Specialists.

Develop data system to incorporate ICD-10.

Develop early intervention rehabilitation programme.



Department of Preventive Medicine and Health Promotion

Allied Health Professional and Support Services

Staff

Professor Cecily Kelleher
MD, FRCPI, MPH, FFPHM, MFPHMI (Head of Department)

Ms Jacinta Barnewell
RGN, SCM, Adv. Dip. Montessori Education
(Health Promotion Officer from November 2004)

Dr. Anna Clarke LRCP&SI, MB, MPH, FRCPI, FFPHMI, FFPH

Ms. Denise Comerford RGN, RM (Health Promotion Coordinator)

Ms. Frances Conlan (Secretary)

Ms. Kirsten Doherty BSc, MPH (Health Promotion Officer)

Ms. Hilda Gallagher RGN, RM (Health Promotion Officer)

Ms. E. McIntyre RGN, RSCN, H. Dip. Health Promotion
(Health Promotion Officer to September 2004)

Ms. Veronica O'Neill RGN, RM (Health Promotion Nurse)

Ms. Carol Pye RGN, RM (Health Promotion Officer)

Ms. Vivien Reid MSc, Dip Diet. (Clinical Specialist Dietitian)

Ms. Brenda Whiteside RGN, RM (Health Promotion Officer)

Allied UCD staff

Professor Leslie Daly, MSc, PhD, FFPH

Dr. Dominique Crowley MB, MPH, MFPHMI, MFPHM (to April 2004)

Dr. Patricia Fitzpatrick MD, MPH, FRCPI, FFPHMI.

Service Developments/Achievements

Service to Patients

24 Hour Ambulatory Blood Pressure Monitoring Service
This service is provided for both inpatients and outpatients of hospital consultants and general practitioners.

Cardiac Rehabilitation

This department's support of cardiac rehabilitation continues through the monthly cardiac lifestyle information session. An information stand was provided at a GP Heartwatch Workshop in November.

A Heart Health Research Bursary was awarded to Hilda Gallagher to evaluate systematically pre-hospital administration of aspirin to patients with suspected acute myocardial infarction.

Smoking Cessation Services

This service has continued to develop, with intensive interventions for in-patients and out-patients, stop smoking courses, and a weekly drop-in session. Staff education sessions took place in most wards, in several departments, and at staff and intern induction days to increase awareness of smoking and the service available. Development of a smoking education DVD commenced with Dun Laoghaire College of Art and Design.

A research programme, funded by the EU and the Cardiovascular Health Strategy, includes an almost complete follow-up study of 1100 in-patients, at six months post discharge. Smoking cessation support and free nicotine replacement therapy are offered to staff, in conjunction with the Pharmacy and Occupational Health Departments.

Service to Staff

In-house health promotion training sessions were presented to all departments. These sessions are now included in medical interns and general staff induction programmes.

"Healthwise SVUH" continued to be published quarterly. The contribution from staff throughout the hospital is appreciated.

Health promotion information stands were provided, including Cancer Awareness for men, co-ordinated by Divisional Nurse Manager Tanya King, and Smoking Cessation, manned by the health promotion team. A COPD Awareness Day with spirometry testing was offered by the Pulmonary Department. A Heart Health Awareness stand was co-ordinated by the Cardiac Rehabilitation Department.

During National Healthy Eating Week an information stand was provided for staff and patients. The theme was 'What's your portion…size matters'. A measurement session 'Are you an Apple or a Pear?' was advertised on the stand. Measurements were compared to the Ashwell Shape chart indicating action needed to reduce weight and waist size. Two pedometers were raffled as prizes.

Service to the Community

Education and Training Days

Training days for health professionals were run in conjunction with other hospital departments. Topics were:

- Colorectal Cancer
- Palliative Care
- 2-Day Stop Smoking Courses in conjunction with the Irish Cancer Society

In association with the Health Promotion Unit at the Department of Health and Children, a national research seminar was provided for Community Dietitians relating to audit and research.

Health Promoting Hospitals

St.Vincent's University Hospital is one of over ninety hospitals affiliated to the National Health Promoting Hospital (HPH) Network. Denise Comerford and Dr Anna Clarke are members of its National Executive. Denise Comerford continued as secretary to our own hospital committee.

Health Promotion Seminars for Students

Six seminars were held, each attended by approximately 100 transition year students; topics covered were alcohol awareness, cancer awareness, smoking cessation and healthy eating. A stop smoking course was run for transition year students as part of their entry to the Young Scientists Exhibition. Smoking Awareness stands were provided at the UCD Medical Science Awareness Day for 2nd year students and the UCD No Smoking Awareness Day

Significant Publications

Reid V. *The influence of Cardiac Rehabilitation on lifestyle change – one year follow up.* European Journal of Cardiovascular Prevention and Rehabilitation. 2004 Vol. 11 Supp. 1. p.13

Doherty K. *A service analysis of a hospital-based smoking cessation service – positive outcomes.* Proceedings of the 12th International Conference on Health Promotion Hospitals, Moscow, May 2004

Reid V. *Fruit intake of hospital staff – how does it compare with national data?* Proceedings of Nutrition Society Summer Meeting, Dublin, July 2004. (In press)

O'Donoghue S. Positive impact of the anti-smoking legislation on hospitalised cigarette smokers. Proceedings of the Winter Scientific Meeting of the Faculty of Public Health Medicine, Dublin, May 2004

Gallagher H. *The external and internal factors affecting the delivery of healthcare: are nurses prepared for the challenges of the new Health Service Reforms?* Proceedings of the National Council for the Professional Development of Nursing and Midwifery Conference

Speech and Language Therapy Department

Allied Health Professional and Support Services

Staffing

This year, our senior post in ENT was made permanent and Susan McElwee took up this position, in January 2004. We continued our link post with St. Michael's Hospital, this service has been running for nearly two years. Susan Starr completed her 12-month contract and returned to Australia. Claudia Schäfer who worked in the department since March 2002 took a one-year career break to return to Germany. Claudia is now working in a community based adult post in Berlin. We successfully filled both of these posts. Una Deane joined the department in June 2004. Una worked in Cork University Hospital, since graduating from TCD (June 2002). Sharon McNulty, who graduated in June 2004, joined the department in August 2004.

Service Development

ENT Service

- We continued our twice-weekly Joint Voice/Digital Stroboscopy Clinics with Mr. Russell and Mr. Charles.
- We commenced a Fibre Optic Endoscopic Evaluation of Swallowing (FEES) assessment clinic (June 2004).
- Two of the Senior Speech and Language Therapists (ENT and Neurology) continue to participate in the multidisciplinary tracheostomy policy working group.
- Fionnuala Duffy, Speech and Language Therapy Manager, was a member of the DEEPO quality improvement team.
- Voice Problems: Prevention Programme for Primary School Teachers
  Following the 2003 audit of our Joint Voice/Digital Stroboscopy Clinic (ENT OPD), we were able to see that over 60% of the patients presenting with vocal cord pathology (vocal cord nodules) were Primary School Teachers. This programme commenced in September 2004. To-date, 16 schools have been involved (284 teachers). This programme is the first of its kind in the Republic of Ireland.

Link Post with St. Michael's Hospital

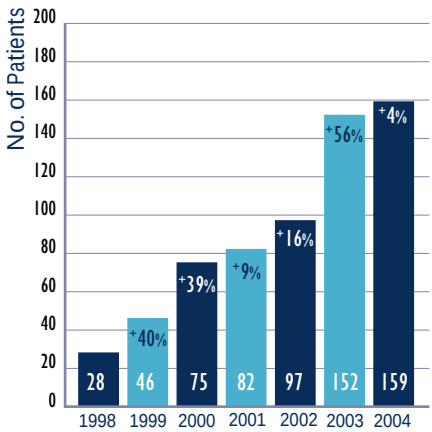
- Deirdre Kidney and Una Deane conducted training sessions for nursing staff on the management of swallowing problems at ward level.
- Patients from St. Michael's Hospital continue to avail of the specialist clinics – Videofluoroscopy and Joint Voice Clinic.

Input to the Neurology Service.

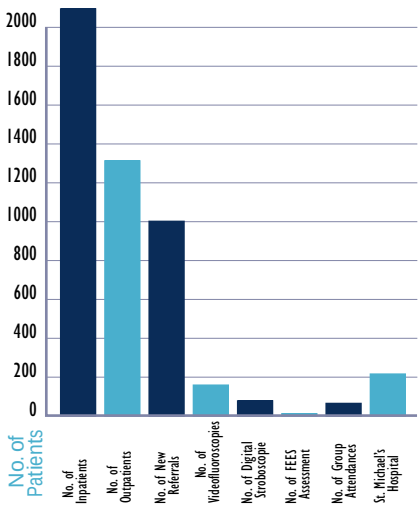
- This service has increased significantly over the last 12 months. Deirdre Kidney and Una Deane continue to participate in ward rounds and have made a number of presentations at the weekly journal club.
- We have also commenced a group treatment programme for patients with Parkinson's disease.

Videofluoroscopy Service

The Speech and Language Therapy Department and Dr. Dermot Malone, Consultant Radiologist, run this specialised clinic jointly. It continues to be the only videofluoroscopy service in the East Coast Area Health Board. Referrals are accepted from St. Michael's Hospital, St. Vincent's Private Hospital, St. Colmcilles, National Rehabilitation Hospital, and Royal Hospital Donnybrook.



Departmental Statistics



Departmental Statistics

No. of Inpatients	2,095
No. of Outpatients	1,313
No. of New Referrals	1,002
No. of Videofluoroscopies	59
No. of Digital Stroboscopies	78
No. of FEES Assessment	12
No. of Group Attendances	65
St. Michael's Hospital new referrals	216
Total	4,940

Stroke Service/Care of the Elderly

Staff attached to this service attend the weekly journal club on a regular basis. These staff members also presented at one of the stroke specific sessions on 'Assessing for depression in patients with dysphasia'.

Fionnuala Duffy, Speech and Language Therapy Manager, presented at the first annual Study Day, in January 2004.

Deirdre Kidney, Senior Speech and Language Therapist, participated in the Parkinson's information sessions organised by Dr. Diarmuid O' Shea, in Carew House.

Quality Innovation Award

We were delighted to win an award for our patient information booklet on 'Swallowing and Nutrition'. This is a joint project with our colleagues in the Department of Nutrition and Dietetics.

Training and Development

- Throughout 2004, the department was involved in a number of presentations.
- Ruth Maxwell, Senior Speech and Language Therapist, continued to provide input to nursing students on Our Lady's Ward.
- Susan McElwee, Senior Speech and Language Therapist, provides ongoing input to physiotherapy students.
- The department provided several placements for Speech and Language students, including a final year TCD student and a first year from UL.
- We also continued our extensive in-service training programme, which was particularly useful given the number of new staff in the department.
- Three members of the department have commenced postgraduate degrees (1 – IPA & 2 –TCD).

Significant Courses / Conferences

Various members of the department attended a number of different courses throughout the year.

- Special Interest Group in Voice Disorders-Voice Training for Teachers.
  Dr. Stephanie Martin 19th February 2004.Mater Hospital.

- The State of the Art of Dysphagia Management.
  Dr. Joseph Murray 3 days – St. James's Hospital, 2nd – 4th June 2004.

- IASLT – Conference 26th /27th November 2004.
  'Celebrating innovations in Service Development and Clinical Practice'.

  Three members of the department presented papers at this National Conference.

  "Do Speech and Language Therapists working in adult dysphagia use the same labels when referring to foods and fluids?"
  Ruth Maxwell, Senior Speech and Language Therapist.

  Laryngotracheal Separation for intractable aspiration.
  Fionnuala Duffy, Speech and Language Therapy Manager.

  Assessment of Oropharyngeal Dysphagia in Amyotrophic Lateral Sclerosis.
  Deirdre Kidney, Senior Speech and Language Therapist.

We also displayed two poster presentations at this Conference.

Development of a Multidisciplinary Tracheostomy Management Policy in an Acute Care Setting.

A practical guide for helping families and carers understand aphasia.
(SVUH and NRH)

Plans for 2005

- To upgrade two of our senior posts to Clinical Specialist.

- To continue our involvement in student education.

- To continue the research project on a 'Pharyngeal Retention Scale' with Dr. D. Malone and Prof. L.Daly (UCD).

- To set-up a Voice Laboratory in the new ENT OPD.

- To develop a database system.

- To complete a project on 'aphasia friendly' information with the Stroke and Care of Elderly teams.

Department of Catering

Service Departments

Staff

Staff training and development is ongoing in the Catering Department in order to comply with Current Food Hygiene and Health and Safety Regulations and continue to progress the implementation of HACCP.

Mr. Alphonsus Maguire, Chef Grade II, retired in June 2004 after forty-seven years service. Alphonsus started his career in St. Stephen's Green in 1957. I wish to thank him for his loyalty and support and wish him good health and happiness in his retirement.

Mr. Billy Rooney, Kitchen Porter, also retired in June 2004 after twenty years service. May he too enjoy good health and happiness.

We are glad to welcome all new staff who joined the Catering Department in 2004. In particular, Ms. Paula Grogan, Catering Officer, with main responsibilities for the food service in the Cafeteria.

Ms. Nora Kavanagh graduated in May'04 after successfully completing the Supervisory Management Course in D.I.T. Cathal Brugha Street. Nora started her career as a Catering Assistant and, through her own perseverance, returned to study in her own time while she continued to work at St.Vincent's University Hospital for the past three years. Congratulations Nora and every success in the future.

Service Development/Activities

The main kitchen upgrade continued through 2004. All the old steam operated equipment was discarded and replaced with modern electric and gas appliances. A major rewiring was carried out to give badly needed electric capacity and to facilitate a more efficient service. The monitoring of food temperature controls was implemented in each ward kitchen and all relevant staff were trained to carry out this function. This now completes the food chain traceability required to comply with the HACCP system.

The Catering Department, through the Director of Catering, is involved in a North/South catering project funded by the National Partnership Forum looking at nutritional and ethnic food requirements for patients and staff.

In February 2004, a one-day workshop was held in Newry which Hospital Managers, Financial Controllers, Procurement, Dieticians, Catering Managers, Supervisors, Chef and Trade Union Representatives attended. Ethnic and religious groups were in attendance at the workshop.

The issues raised were used to compile a questionnaire which was issued to Hospitals nationwide to :

- Identify the numbers of ethnic groups in various communities.
- Find out the numbers of different nationalities using our hospitals.
- Collect data on the availability of supplies in Ireland and set up a supplies register.

The aim is to produce a booklet, which will be used as a toolkit to assist and provide guidelines on menu interpretation, protocol for meal ordering and other useful information for people from different communities.

Here at St. Vincent's University Hospital Halal, Kosher and other ethnic dishes havebeen introduced into the staff menus and are also available to patients. The experiment is proving popular.

Departmental Statistics

The Catering Department caters for 500 in-patients and over 2,000 staff. Added to this is the demand from visitors and patients to use our Cafeteria, which itself has increased its turnover beyond all expectations.The Out Patients Snack Bar is used by at least 500 people every day. A full meal service is provided to patients awaiting admission in the Emergency Department. The Catering Department also provides a wide range of meals and snacks for functions, seminars, meetings etc throughout the hospital.

Future Plans

In order to address the problem in the staff restaurant of long queues at service points and at the cash desk, plans have been drawn up to provide a better layout and to offer several service points in an enclosed food hall. A carvery unit manned by one of our Chefs will provide added choice and variety for our customers. More emphasis will be on healthy eating, even though a small section is reserved for the "chip fancier". It is hoped this will be in operation within the year.

Enhanced delivery of meals to our patients is still a priority, even though the progress may seem slow. We hope to replace all the hot/cold food trolleys soon and offer an improved menu selection allowing the patient more time to choose from a wider range of dishes, as well as lighter meals and snacks.

We will continue to serve the needs of our patients, staff and visitors in a caring and courteous manner.

Theatre Sterile Services Unit / Central Sterile Supplies Department

Service Development / Activities

2004 will be remembered as the year of Accreditation. The preparation, the paperwork, the time involved presented no small challenge to us all. Despite the hard work involved, it was a satisfying process for this department as it allowed us to look critically at what we do, how we do it and at the same time recognising opportunities for improvement.

A lot of time and effort was spent this year in preparation for the move to our new department in 2005. Equipping lists, operational policies and gap analysis have become part of our lives. In the autumn, organised staff visits to the new building produced very positive feedback, as the dream is fast becoming a reality.

Staff

Early in the year Angela Ormonde left us to join the Laboratory team in the hospital, and Jacqueline Donnery left to seek pastures new in the United States. We welcome Brian O'Leary and Linda Finn as new members of the team.
TSSU Supervisor Aileen Byrne graduated from Tallaght DIT with a National Diploma in Sterile Services Technology. A glutton for punishment, Aileen has returned to college to study for a Bachelor of Science degree.

Ms. Anne Grumley, retired in September and we thank her for the support and wisdom that she showed to this department during her time as Divisional Nurse Manager.

Statistics

2004 showed an increase in the volume of instrument sets and other devices reprocessed to over 60,000 items. The increased workload and the limitations of the current working environment meant another demanding year for TSSU. We look forward with great anticipation to 'new beginnings' in 2005.



General Services Department

Service Departments

Telephone and Communication Services

The Telephone Services Department provides switchboard and communication services to approximately 12,000 internal and external callers per week. The department also co-ordinates our internal and external fire and emergency communication protocols, and is responsible for maintaining internal directories for the telephony and paging systems.

The service operates on a 24-hour basis, and is staffed by a team of 12 full and part-time staff members.

The service continues to benefit from ongoing capital investment as part of the hospital re-development programme. During 2004, this investment included the commissioning of a new internal paging system, involving the successful overnight changeover of 650 pager users. These investments will ensure that our new facilities can be successfully integrated into our existing communications arrangements.

Security Department

The hospital is committed to providing a safe and secure environment for patients, relatives and staff. Our Security Department plays a key role in the co-ordination of security and safety activities, with a particular focus on fire safety management and emergency response procedures.

The service is staffed by a team of 14 members, offering a 24-hour service. The department continued to meet the challenges associated with ongoing construction activities on site and looks forward to providing security services to the completed facilities in 2005.

Portering Services Department

The Portering Department provides support services to a broad variety of hospital departments through the provision and supervision of Portering staff. The areas serviced include Theatres, Laboratories, Radiology and Patient Clinics. Whole- hospital activities such as waste management, post and patient transport are also co-ordinated within the department. The current staff compliment is 67.

Training continued to be a focus for 2004.The department commenced a programme of re-training all staff in patient and non-patient moving and handling, and the first of our supervisors graduated from the hospital's Diploma in Supervisory Management Programme.

In late 2004, the department began planning for the commissioning of the new waste marshalling facilities and we look forward to the challenges and opportunities arising from their planned opening in 2005.

Cleaning, Hygiene & Household Services

The hospital takes seriously its obligation to meet the highest standards of cleaning and hygiene in support of the care of patients. All cleaning and hygiene activities are guided by best Infection Control practices, and operate to a pre-agreed cleaning specification. During 2004, the hospital engaged in a comprehensive review of this specification with a view to ensuring we continue to meet our changing needs.

Cleaning activities are provided under a contracted service with approximately 70 cleaning operatives and supervisors working in close partnership with the hospital.

Our Household function co-ordinates rest, welfare and accommodation facilities for staff and patient relatives, and is also responsible for co-ordinating and monitoring various contracted household services.

Medical Records and Patient Services

Staffing

During 2004, a total of 9 Grade IV Officers and 13 Grade III Officers commenced, in addition to summer locums and part-time temporary staff. This is an indication of the hard work and commitment given by our long-term permanent staff with regard to orientation and training that is on going within the department.

The following permanent Grade V appointments were made:

- Thomas Slattery, HIPE/Casemix Department
- Gaye Moffat, Emergency Department

Staff participated in the following courses on offer to line managers:

- Communication and Motivation
- Managing Team Performance
- Recruitment and Selection Skills
- Equality and Diversity Training
- Personal Development Planning

The following staff completed a 5 Day Module Supervisory Course which was offered by the Office for Health Management:

Tara Kelly (Cardiology), Ruth Gavin (Emergency Department), Mary White (Rheumatology), Rachel Boyle (Secretarial Services), Jenni Cross (Orthopaedics/Pain Management), Mary Kavanagh (Outpatient Department), Angela Corrigan (Theatre), Lorraine Murray (Medicine for the Elderly)

Service Developments/Activities

- Involvement with the Accreditation process continued for many of our staff and the Peer Review Survey took place in May.

- A new patient chart was introduced on February 11th. Workshops were held for all Medical Records and Ward Secretaries (98 attended). The changes implemented were significant with regard to structure and filing order. Attention to the changes is ongoing and an audit process is planned for the New Year.

- A very important HIPE deadline was successfully met in September, thanks to the hard work of all Coders, Casemix Assistants and Support Staff in the Medical Records filing room.

- A File Management Working Group was formed with the East Coast Area Health Board & St. Vincent's University Hospital in planning for the move to the new Psychiatric Unit. Mary Moran, Candy Molloy, Rosaleen McGuire and Tricia Mc Donough, along with Dermot Cullinan (ICT Department) are working with this group on the development of procedures for chart management in the new unit and coordination of information flow between the unit and various outpatient sites.

- Various IT projects, that involved many of our supervisors, were ongoing in 2004, such as:-

  H2H installation in St. Anne's with plans for other day care areas

  Bi-directional between Maxims and PAS

  Scanning of Emergency Department records

  Development of the new Data Management Warehouse

Human Resources Department

Service Departments

Overview

Looking back at the activities for 2004, it is apparent that the Human Resources (HR) Department had another busy year. As well as the normal range of HR functions, the department was involved in a number of projects, the most significant being the IHSAB Accreditation and advancing the modernisation and change agenda.

The hospital wide accreditation initiative impacted on HR and three sub-groups were established to address the HR standards. The Partnership approach to accreditation adopted by these teams, which involved the three hospitals in the group, proved very successful. Representatives from Department Heads, Line Management, Staff, Trade Unions and the HR Department across the three sites, worked collaboratively to identify and document key strengths and opportunities for improvement, with a prioritised list of areas for attention under Quality Improvement Plans. Significant work was carried out by the teams formed to address the HR standards and the partnership approach taken was commended by the Accreditation survey team. Work is continuing on our Quality Improvement Plans for the next stage of the process.

The hospital continued to progress a range of new change management initiatives, while consolidating the advancement of existing measures, with particular emphasis on enhanced customer services initiatives. A significant focus was also placed on the development of standard operating procedures for the move to the new building, with real and worthwhile engagement from staff in this process. The relevant pay increases under Sustaining Progress and Benchmarking were paid to staff on foot of the report submitted to the Health Services Performance Verification Group.

In July 2004, Noel Cassidy, who joined St. Vincent's University Hospital as Personnel Officer in April 1973, retired. All his colleagues in HR and throughout the hospital wish him a very happy retirement. The HR Department extends every good wish to all our staff who also retired during the year.

Service Developments/Activities

The recruitment division was involved in a high volume of activity across a wide variety of grades throughout the year. The hospital continued to experience shortages in certain specialised areas and successful efforts were made to recruit from both at home and abroad. The "Recruitment and Selection Skills" training for interview panel members up-skilled our staff in best practice recruitment and selection techniques. A review of policies, procedures and practises in Recruitment and Selection is planned for the year ahead.

The HR Department initiated and supported a range of training and development interventions, including educational financial support. Some of the key initiatives are listed below.

- The hospital was successful in an application to the ERHA for funding a Management Development Programme in collaboration with the other Dublin Academic Teaching Hospitals and Our Lady's Hospital for Sick Children. The programme, which commenced in April 2004, contained six stand alone modules viz Managing Team Performance, Dignity at Work, Communications and Motivations Skills, Recruitment and Selection skills, Employment Law and Conflict Handling, and Equality and Diversity Training. It is expected that this programme will continue during 2005.

- During 2004 the hospital offered, in conjunction with the University of Limerick, a five-day Supervisory Management Programme for Front-Line supervisors to fourteen staff from a cross section of departments. 55 managers and local union representatives took part in the three pilot Joint Problem Solving sessions. This programme may be repeated in 2005.

- The 2nd Chance Key Skills Programme, supported by the HSNPF, proved very popular and was deemed a success. This programme was developed to assist staff improve their skills in certain areas such as English as a second language, writing skills, word processing, and report writing. The programme commenced May 2004 and concluded September 2004. A total of 18 staff completed the programme and many requests have been received to run this programme during 2005.

- Throughout the year, the hospital continued to roll-out Personal Development Plans (PDP's) with staff in our Pathology Department. This initiative will continue in 2005, with more departments becoming involved.

- Many of our staff were afforded the opportunity to attend Master Classes

organised by the Office for Health Management. The guest speakers included leaders in Industry and Academia and they shared their practical views on implementing the leadership visions in the Health Service.

- The National College of Ireland two-year Diploma in First Line Management commenced in September 2004 for the third year and the programme continues to attract huge interest. 2004 was the first graduation year for St.Vincent's Healthcare Group and forty-five staff graduated.

As Partnership is now in its third year of operation, the Partnership Committee took the opportunity to review its vision and values statements and its Service Plan at a one day workshop in June. A renewed commitment was given to further developing and extending the partnership approach in the hospital. Underpinning partnership activity is the clear understanding that management, trade unions and staff have agreed under Sustaining Progress to work collaboratively to modernise the health services; to improve services to patients and clients, improve the quality of work-life for staff and provide value for money.

Our main achievements reported under the Partnership Service Plan in the hospital during the year included supporting the development of St. Vincent's University Hospital Intranet, which was piloted in five departments and extended to other areas/departments by year end. Other projects which received support are referred to elsewhere in this section.

In December, the St.Vincent's University Hospital Partnership team participated in the HSNPF major national conference and exhibition, where all health agencies exhibited the results of partnership activity over the previous five years.

The Industrial Relations and Employee Relations Division of the HR Department also had a demanding year. Most issues were dealt with at local level and, where necessary, certain issues were referred for third party intervention. The hospital continues to be fully compliant with the Industrial Relations provisions of Sustaining Progress.

2004 saw The Health Service National Joint Council become the primary forum for the management of industrial relations in the health service. It oversees the conduct of industrial relations and decides on an efficient method of dealing with any matter arising.

The following nationally agreed policies were launched during the year: -

- *"Dignity at Work Policy for the Health Service"*. This policy has a strong preventative focus and emphasises the duty of every staff member to maintain a working environment in which the dignity of everyone is respected. Training to support this policy will be provided to designated support contact persons once selected and general training/awareness for other staff.

- The *"Framework for Dispute Resolution in the Health Services"*, which was produced following discussions between the HSEA and the Health Service Trade Unions, provides clear and definitive procedures for discussion and negotiation on all matters which affect staff in the health service.

- *"Grievance and Disciplinary Procedures for the Health Service"* came into effect on 1st May 2004 and superseded all existing local procedures.

- *"An Equal Opportunities/Diversity Policy & Strategy Objectives for Health Service"* commits health service employers to treat all employees and potential employees equally in terms of recruitment and selection, pay, conditions, training, work experience and opportunities for career progression.

Purchasing & Procurement Department

Activity & Service Developments

Activity increased in 2004 as a result of additional funding made available to the hospital late in the year. The workload within the department also increased and all staff performed well within this very busy environment.

	2002	2003	2004
Total Purchase Orders	18,783	17,166	18,217
Total Value	€21,787,514	€23,943,843	€29,857,219

The increase in expenditure can be attributed to Capital Funding and also the procurement for the project development, which took place in the last quarter of 2004.

Inventory Management

A programme to count and analyse stock held at ward level was initiated. Providing data on valuation, excess and potential out of date stock, and giving the opportunity to redistribute stock, thus increasing stock turns and reducing inventory investment.

Top Up System

With existing resources the Top Up Stock Control System was rolled out to include St Paul's Ward and Theatre General Ad Hoc gowns and drapes.

e-Procurement

Personnel in the Purchasing Department have been actively involved in the introduction of e-procurement systems from suppliers and hope to develop this technology further with the introduction of the new email system in 2005.

Savings

Total Savings for 2004 was €240,000, which includes the Hospital Procurement Group new tenders and contracts.

Capital Equipment

The Purchasing Department tendered and purchased on behalf of the hospital €3 Million worth of equipment this year. Many areas throughout the hospital benefited from receiving new equipment.

New Developments for 2005

It is planned to have the new contract implemented for Custom Procedure Trays (CPT) in 2005 for all of the main operating theatres. This should bring about more efficiency for theatre case management and also achieve further value for money.
It also envisaged moving CSSD stores to the main stores.

Project Procurement

The procurement process for all new equipment for the project is now well underway. 2004 saw the tendering process initiated for the following tendering packages:

- PACS / RIS
- Diagnostic Imaging Equipment
- Portable Biphasic Defibrillator & Monitor and Emergency Trolley
- Office & General Furniture
- Network Equipment Tender
- Pathology Analysers
- Dialysis Equipment
- Wall Mounted Diagnostic Sets

2005 will see the tendering process initiated on a phased basis for the remaining equipment requirements. It is envisaged that there will be 41 different types of tendering packages (including those listed above).



Technical Services Department

Service Departments

Overview

The Technical Services Department provides the engineering and maintenance service for the hospital facility. The department is responsible for providing the following services:

- Maintenance of existing building and facilities
- Maintenance of grounds, plant and equipment
- Management of fire protection, emergency lighting systems
- Energy management
- Refurbishment of existing accommodation
- Engineering and technical advice on the installation of new equipment
- Provision and upgrading of infra-structural services such as water, electricity, steam, heating, ventilation, medical gases, telephony etc. within the hospital
- Project management

Staff

The current compliment of staff is 40. Jimmy Ward, a carpenter, retired during the year. Jimmy was a familiar sight throughout the hospital, pushing his tool cart while attending to the numerous requests for minor jobs to be carried out. We wish him well for the future. 3 new staff joined the department during 2004:

- Richard Buckley, Carpenter
- Christopher Lindsay, Groundsman/helper
- Niall Crowley, Groundsman/helper

Service Developments/Activities

There was significant progress in the main hospital project development. The Breast Check Building and Waste Marshalling Yard were both completed during the year. The new main hospital building progressed rapidly through the year and the building entered into the builder's commissioning phase as the year drew to a close.

The Technical Services Department worked closely with the design team and contractors in integrating the new developments with the hospital's existing infrastructure, and facilitating the fire upgrades and lift refurbishment within the existing hospital.

In addition to the above, many minor capital projects were completed. This was achieved through a combination of our own in-house staff of electricians, carpenters, plumbers and painters and external contractors. A summary of these projects is as follows:

- Refurbishment of Provincial House to provide a Cancer Support Centre and overnight accommodation for relatives of Liver Transplant patients
- Refurbishment of the 3 sets of public toilets on the ground floor of the main Hospital
- Total upgrading of St.Brigid's Ward to provide two 2-bed intensive care wards, an intensive care isolation cubicle, a standard isolation cubicle and a 4-bed ward, all of which are aspergillus protected
- Provision of air-conditioning in a number of areas to improve working conditions and patient comfort
- Alterations and redecoration of Carew House basement to facilitate the relocation of the Neurology Department

Various infrastructure improvements were carried out in line with ongoing policy of improving reliability and service:

- Complete upgrade of electrical wiring in the kitchen
- Various roof repairs
- Replacement of the heating pipe work serving St.Rita's
- Upgrading of heating pipe work and controls in the plant rooms serving the ward block. This has resulted in much improved temperature control in the ward block.

Maintenance activity was ongoing with the increasing demands of routine and preventative maintenance and breakdown maintenance in both existing and new buildings. This effort is supported by the computerised maintenance system, Pemac and the Building Management System.

Future Plans

It is planned to develop the computerised maintenance system (Pemac) to enable staff to log work requests directly on the system via the hospital Intranet. This will be a more efficient and effective means of registering work requests and will provide a facility for feedback and updates to staff.

The year ahead will provide significant challenges for the department as we get familiar with the new building and take on the responsibility of running the new building and the equipment and plant therein.

2004 Committee Membership

Board of Directors

Chairman:
- Professor Noel Whelan

Members:
- Mr Edmond J. Bergin
- Sr Eugene Butler
- Ms Louise English
- Mr Stewart Harrington
- Professor M. X. FitzGerald
- Mr Joe Leyden
- Sr Anne MacEneaney
- Dr Brian Maurer
- Ms Gemma McCrohan
- Mr Partick Meade
- Mr Michael Meagher
- Dr Risteárd Ó Laoide
- Mr W. R. Quinlan
- Mr Conor Sexton
- Dr Michael Somers

In Attendance:
- Sr Therese Culhane
- Ms Pauline Doyle (to April 2004)
- Mr Eamonn Fitzgerald
- Mr N. C. Jermyn
- Mr Cormac Maloney
- Ms Raphael McMullin (from May 2004)
- Mr Michael Redmond

Group Medical Executive

Chairman:
- Dr Risteárd Ó Laoide

Honorary Secretary:
- Dr Alan Watson

Members:
- Dr Conor Collins
- Dr Tom Crotty
- Dr Seamus Donnelly
- Dr David Fennelly
- Mr Arnold Hill
- Professor Kevin Malone
- Dr Ken McDonald
- Professor Niall O'Higgins
- Dr Diarmuid O'Shea
- Dr Tom Owens
- Dr Stephen Skehan

Group Executive

Chairman:
- Mr N. C. Jermyn (Chair)

Members:
- Mr Ken Bale
- Ms Marianne Byrne
- Mr Noel Cassidy (to July 04)
- Ms Gretta Colbert
- Mr James Crowe
- Ms Pauline Doyle (to April 04)
- Mr Eamonn Fitzgerald
- Mr Ian Maguire
- Mr Cormac Maloney
- Dr Brian Maurer
- Ms Raphael McMullin (from May 04)
- Mr Seamus Murtagh
- Dr Risteárd Ó Laoide
- Ms Sorcha O'Quigley (from August 03)
- Mr Michael Redmond
- Mr Peter Sheehan
- Ms June Stanley
- Mr Neil Twomey

Audit Committee

Chairman:
- Dr Michael Somers

Members:
- Mr Ken Bale
- Mr James Crowe
- Mr Eamonn Fitzgerald
- Mr Nicholas C. Jermyn
- Mr Cormac Maloney
- Mr Patrick Meade
- Mr Michael Meagher
- Mr Seamus Murtagh
- Mr Michael Redmond
- Mr Conor Sexton
- Ms June Stanley

Group Finance Committee

Chairman:
- Mr Conor Sexton

Members:
- Mr Ken Bale
- Mr James Crowe
- Ms Louise English
- Mr Eamonn Fitzgerald
- Mr Stewart Harrington
- Mr James Hussey
- Mr Nicholas C. Jermyn
- Mr Cormac Maloney
- Dr Brian Maurer
- Mr Patrick Meade
- Mr Seamus Murtagh
- Mr Michael Redmond
- Ms June Stanley

Mission Committee

Chairperson:
- Sr Eugene Butler

Secretary:
- Ms Theresa Ward

Members:
- Mr Ekundayo Badmus
- Ms Barbara Cantwell
- Ms Therese Carey
- Dr Michael Casey
- Fr Liam Cuffe
- Ms Marie Culliton
- Ms Ciara Field
- Ms Anne Grumley
- Mr John Harraghy
- Mr John Hickey
- Mr Nicholas C. Jermyn
- Sr Angela Kelly
- Ms Gemma McCrohan
- Dr Risteárd Ó Laoide
- Dr Diarmuid O'Shea
- Ms Sheila O'Toole
- Ms Phil Pyne Daly

Project Team

Chairman:

Mr Nicholas C. Jermyn,
Group Chief Executive

Members:

Mr Eamonn Fitzgerald,
Group Deputy Chief Executive

Mr Gary Wright
Project Manager

Mr Paul Marlow
Director, Cyril Sweett Boyd & Creed

Mr Peter Kerruish
Associate Director, Boyd & Creed

Sr Anne MacEneaney,
Sister Superior

Ms Pauline Doyle (to April 04)
Director of Nursing

Ms Raphael McMullin (from May 04),
Acting Director of Nursing

Mr Pat McDonnell
Technical Services Manager

Mr John O'Callaghan
Technical Services Consultant

Dr. Risteárd Ó Laoide
Chairman, Medical Executive

Dr Lynda Fenelon
Consultant Microbiologist

Professor Diarmuid O'Donoghue
Consultant Gastroenterologist

Mr Arnie Hill, Chairman
Division of Surgery

Mr Justin Geoghegan
Consultant General Surgeon

Mr William R. Quinlan
Consultant Orthopaedic Surgeon

Mr Wilf Higgins
Engineering Advisor, Department of Health & Children

Mr Paul de Freine
Deputy Chief Architectural Advisor, Department of Health & Children

Mr Terry Woulfe-Flanagan
Principal Quantity Surveyor, Department of Health & Children

Mr Brendan McGrath
Structural Engineering Advisor, Department of Health & Children

Mr Laurence Hayes,
NDP Manager, Eastern Regional Health Authority

Ms Fionnuala Duffy,
Senior Commissioner, Eastern Regional Health Authority

Ms Martine Curran,
Projects Co-ordinator, Eastern Regional Health Authority

Health & Safety Committee

Chairman:

Mr Eamonn Fitzgerald
(Chairman)

Members:

Ms Margaret Brittain
Ms Bernie Butler
Mr Larry Clancy
Mr Eugene Crummy
Ms Emma Darcy
Ms Kathryn Feeley
Dr Lynda Fenelon
Ms Theresa Flynn
Ms Nuala Gannon
Mr Pat Gargan
Mr Gerry Gorey
Dr Paul Gueret
Ms Pauline Leahy
Ms Peggy Lowry
Ms Mairead Markey
Mr Pat McDonnell
Mr Jerry Noone
Ms Valerie Oldham
Ms June Stanley
Mr Peter Sheehan
Ms Catherine Toole
Mr Fergus Clancy

Health Promotion Committee

Chairman:

Dr Tim McDonnell

Secretary:

Ms Denise Comerford

Members:

Ms Margaret Boland
(from November 04)
Ms Maeve Broderick
Ms Sarah Brophy
Ms Siobhain Bulfin
Dr Anna Clarke
Ms Ann Donohue
Ms Margaret Doyle (from October 04)
Ms Fiona Lavin
Ms Pauline Leahy (to March 04)
Ms Peggy Lowry
Ms Ann McFadden
Ms Justine McGrane
Ms Barbara Murray
Ms Anne O'Reilly
Ms Vivien Reid (to September 04)
Ms Mary Shore
Ms Angela Smith (to March 04)
Ms Catherine Smith (April to June 04)
Ms Catherine Toole
(from September 04)

Ethics and Medical Research Committee (October 2004)

Chairman:

Dr Doug Veale

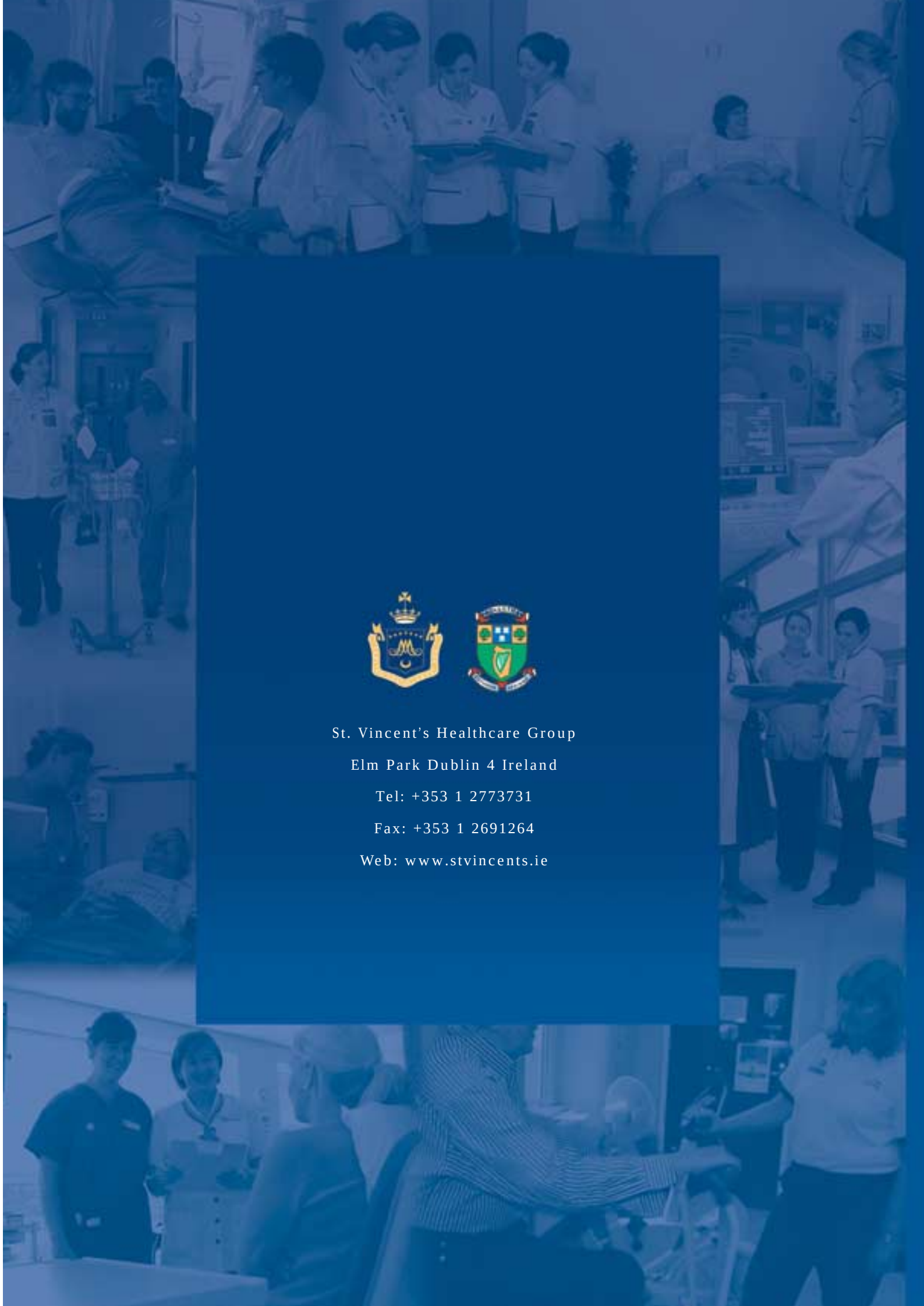
Expert Members:

Dr Kieran Crowley
Alternate Chairman
Mr Eamonn Fitzgerald
Ms Tanya King
Dr Dermot Malone
Dr Michael Moriarty
Deputy Chairman
Ms June O'Shea
Mr John Ryan
Dr Kieran Sheahan
Mr Michael Tolan
Dr Eoin Tiernan

Lay Members

Mr Joe Leyden
Ms Ciara Murray
Mr Leon Newcome
Ms Siobhan Reynolds
Ms Amelia Smith
Mr Frank Smith





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